



Medical Operations Manual

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Tier 1

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Change Summary

The below entries summarize in short the general changes made throughout the manual. For detailed, line-by-line changes, contact Corporate.Publications@airmethods.com to request a copy of the Highlight Pages.

Rev. No.	Release Date	Change Summary	Requester
Previous revisions from IR - 15 change summaries have been archived and available upon request through Corporate Publications.			
16	06/29/2023	Updates to Out Cover; Section 3.1; Section 14.1; Section 14.2.1.1; Section 14.2.1.2; Section 14.2.1.3; Section 14.2.1.5; Section 14.2.1.8; Section 14.2.1.9; Section 14.2.1.11.3; Section 19.9.1; Section 19.14.2; Section 33.2; Section 37.2.1.2; Section 37.2.2; Section 37.2.2.2; Section 75.2.5; and Section 76.2	Clinical Director II
17	12/23/2024	Updates to Out Cover; Changes to Sections 1.1.5; 2.1; 3.2; 4.1; 4.1.1; 4.2; 5.2; 6.7.3; 7.2; 8.4; 10 (New Chapter); 11.1; 11.2.1; 12.2; 15.2.1; 15.2.1.2; 15.2.1.3; 15.2.1.4; 15.2.1.6; 15.2.1.8; 15.2.1.12; 16.1.1; 16.1.3; 16.2; 18 (New Chapter); 19.1; 19.2; 19.4; 20.2; 21.3; 21.4; 21.5; 21.6; 21.7; 21.8; 21.9.2; 21.10; 21.11; 21.12; 21.13; 21.14; 21.15; 21.18; 28.2; 29.1-29.8; 31 (New Chapter); 33 (New Chapter); 34.1; 34.2; 36 (New Chapter); 37.2-37.5; 41.2.2; 44.2; 44.5.1; 44.6; 44.7; 50.1-50.4; 51.1-51.4; 54.1; 54.3; 55.2; 59.3-59.5; 60.2; 60.3; 60.5.1; 60.5.3; 60.5.4; 60.7; 60.8; 62 (New Chapter); 63 (New Chapter); 64.2-64.7; 66.1-66.2; 67.1; 68 (New Chapter); 69 (New Chapter); 70.1-70.2; 71.2; 72.1-72.2; 74.1; 75.2; 78.5; 79.1-79.2; 81.2; 83.2; 86.2; 87 (New Chapter); 90.2; 92.3; 95.1-95.2; 96 (New Chapter), and Appendix	Clinical Director II

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1 *Introduction*

This manual provides policy and high-level procedural guidance based on industry-standard philosophies. All medical team members for the Community-Based Services, and employees of other functional areas (where applicable), are responsible for knowing and complying with information contained within this manual. Deviations may occur; however, individuals are held accountable for their actions/non-actions.

This manual establishes guidelines and procedures for integrating standard medical activities into daily business practices. Its intent and purpose are to broaden knowledge of medical principles, safety practices, and provide fundamentals that are useful in the medical function. This manual prescribes minimum standards for operating practices. This manual is not a substitute for regulatory compliance of any other requirements.

Care has been taken to ensure that this manual is not contrary to any applicable federal or state regulations. However, errors do sometimes occur despite efforts. If you find such a conflict, the regulations or certificate will take precedence. You are required to bring any such contradiction to the attention of your department head.

NOTE Any differences, per individual program, will be documented in program-specific addendum to this manual and posted on FlightDeck > Resources > Corp Pubs > Medical Services Publications > MOM Program Differences.

1.1 Medical Mission Statement

1.1.1 Policy

Air Methods' Community-Based Services responds to transport requests both from an accident scene or a hospital. A collaborative, multi-disciplinary approach is utilized to care for adult and pediatric patients. Transport and care are done in accordance with the Commission on Accreditation of Medical Transport Systems standards and are made available and accessible to all patients.

Each Air Methods licensed rotor-wing program, to the extent applicable, shall provide emergency air ambulance services to all patients without discriminating in the provision of treatment or transportation based on race, sex, age, color, religion, sexual preference, national origin, ancestry, handicap, medical condition, or ability to pay.

1.1.2 Mission

Air Methods is committed to providing outstanding Air Medical Transport Services to our community in a manner that is consistent with our Core Values while delivering measurable benefits to our customers, employees, and investors.

1.1.3 Vision

Air Methods' vision is to be a long-lasting leader and a preferred air medical transport provider through world-class service.

1.1.4 Core Values

- Integrity, honesty, and ethical behavior are the foundation of our culture.
- Decision-making will occur at the appropriate level closest to service delivery.
- Creativity, innovation, quality performance, and leadership are encouraged and highly valued.
- We are the stewards of the resources entrusted to us.
- We strive to make our work safe, healthy, fun, and fulfilling.
- Dignity, fairness, and respect are a right for all.
- We are committed to enhancing the quality of life and professional growth for ourselves and the communities we serve.

1.1.5 Goals

- Be a reflection of the mission of Air Methods, LLC.
- Provide quality assessment, stabilization, quality care, and safe transport of appropriate patients in a cost-effective manner.
- Coordinate with agencies and organizations to ensure efficient response and safety-focused operations.
- Be responsive and supportive serving as an educational resource for health care providers, public safety services, and the public.
- Promote public awareness, understanding, and support of the medical transport system through ongoing cooperation and collaboration with referring and receiving institutions, the media, and the public.
- Adhere to high-quality standards by ensuring continuous quality improvement in all aspects of the program involving patient care, staff development, safety, operations, and effective communications with those receiving our service.
- Provide support services to local communities in the event of disasters or events in which there may be benefits from the helicopter, critical care transport vehicle, or medical team.
- Perform in a financially responsible manner in all aspects involving operations, medical, educational, and other areas using effective cost-containment measures.
- Assess community needs and effectiveness of services as perceived by our customers.

1.2 General Document Control

1.2.1 Document Revision

The Corporate Publications Department oversees the creation and revision of Air Methods controlled documentation. Manual owners or their proxies may submit suggestions and corrections for incorporation into a manual by submitting a completed Document Change Request (DCR) through [Freshservice](#). Changes will be summarized in the change summary table and indicated throughout the manual by change bars. For further details on manual revisions and control, refer to the [Document Control Manual](#) on FlightDeck.

The controlled document will be available on [FlightDeck](#).

1.2.2 Emergency Deviation

Should any process or procedure fail in this document, the following is used as a deviation until the [Document Control Manual](#) can be set right.

2 Safety Policy

AIR METHODS SAFETY POLICY

Air Methods' Safety Management System is a quality management approach to controlling risk, and provides the organizational framework to support a sound safety culture. While Section 2.2 Safety Objectives documents current safety objectives, the Air Methods Safety Policy will be used as guidance for setting and reviewing safety objectives. The Air Methods Accountable Executive has the sole authority, through the VP of Safety, to modify the Safety Policy described in the Safety Management System. Air Methods' Safety Policy is posted prominently at each facility, communicated throughout the organization by being published in all Company operations manuals, and included in all SMS training courses.

Air Methods' Safety Policy from CEO:

1. **Commitment to SMS:** The Safety Management System, as described by the FAA and International Civil Aviation Organization (ICAO), is an essential building block to increasing operational safety. As such, Air Methods and its executive management are committed to implementing the Safety Management System, safety objectives, and its processes in a timely manner. This includes a commitment to providing the resources needed to implement and maintain the SMS.
2. **Continual Safety Improvement:** All Air Methods teammates recognize the moral obligation to safety and the competitive advantage an excellent safety record brings to the organization. Accordingly, Air Methods' management will set safety objectives (See Section 2.2 Safety Objectives) and regularly review these objectives on an annual basis as part of the Executive Safety Council's responsibility using the Air Methods' Safety Risk Management (SRM) as guidance.
3. **Management of Risk:** Aviation will never be without risk. Using System Risk Management (SRM), Air Methods will manage the inherent risks associated with aviation operations to a level deemed acceptable by our regulators, executive management, and risk acceptors.
4. **Compliance with Regulations:** Air Methods requires strict compliance with regulatory requirements as a method of managing risk. The Air Methods' Internal Evaluation Program, FAA oversight, quality audits, and proactive teammate reporting will assist in determining compliance with the regulations.
5. **Non-Jeopardy Policy for Safety Reporting:** Open communication between teammates and management is vital to the success of Air Methods. A tenet of this organization is that respectful communication of all safety-related subjects is welcomed. Especially important in the area of safety is that teammates be empowered to report real or perceived threats to the safe operation of aircraft and equipment as well as the safety of the work environment. A policy of Air Methods is that there will be no retaliation against any person who brings information on items or issues that they deem pertinent to the safety of operations or personnel. Any person coming forward with safety concerns will be given consideration regardless of the final outcome of any suggestion, report, or comment.
6. **Teammate Safety Responsibility:** The responsibility for safety performance does not fall on one particular individual or department. While setting safety policy and goals is the duty of management, all teammates, from the CEO to the newest teammate, are responsible for acting in a manner consistent with safe operation, applicable laws, regulations, and procedures in locations where operations are conducted. Unacceptable behavior and conditions for disciplinary action are cases of willful noncompliance with regulations or Company procedures or when a criminal act has been committed. Unacceptable behavior and conditions for disciplinary action are further detailed in the Teammate Handbook (latest revision).
7. **Emergency Response Program:** Air Methods maintains a robust Emergency Response Plan that provides for the safe transition from normal to emergency operations. The Emergency Response Plan is approved by the Accountable Executive and is available on the Air Methods' FlightDeck Safety page, and/or a hard copy at every station. Additionally, Chapter 3 details Accident/Incident Investigation procedures.


Kellyn Williams, CEO, Air Methods Corporation

2.1 Safety Management System (SMS)

This SMS manual is only applicable to the Air Methods certificate at this time. Subsidiaries Blue Hawaiian Helicopters and United Rotorcraft operate on different certificates and will maintain their own respective SMS and supporting manual. However, those SMS programs and manuals are to be closely designed and aligned with those of Air Methods, LLC with deviations only accommodating differing missions, working environments, and resources.

All Clinical Services department leaders shall ensure a safety reporting mechanism exists for all employees and contractors which aligns with those requirements outlined in the Air Methods SMS Manual. This includes uninhibited reporting of all incidents and irregularities that could compromise the safety of our operations and would have otherwise remained unidentified. The Safety Reporting Programs are open to all employees that wish open, anonymously, or to confidentially submit safety concerns. Front line teammates are required to report hazards, issues, concerns, occurrences, incidents, as well as propose solutions and safety improvements through the reporting mechanisms available to them.

Use of the Safety Risk Management (SRM) is used to manage analysis, hazard identification, risk assessment, and mitigation (See SMS Manual 3.0) when SRM triggers are present.

Each CBS base location shall have a dedicated and trained Field Safety Representative (FSR). To ensure safe and compliant operations at each Air Methods facility, Air Methods has established the Field Safety Representative Program. The FSR program consists of educated teammates in good standing of safety, who maintain the responsibility of monitoring, implementing, and promoting safety at their respective facilities. FSRs report directly to their respective Manager and coordinate with the appropriate Regional Safety Director (RSD). The program is owned by the Air Methods Sr. Director, Regional Safety and administered by the Regional Safety Directors in their respective regions. For more information on the FSR Program, refer to the FSR Standard Operating Procedures.

3 *Access and Security to Air Methods IT Applications*

3.1 Purpose

To ensure that a process is in place to identify when employees and contractors are no longer with their respective organizations so that their access to certain Air Methods IT applications is disabled.

3.2 Procedures

- For Air Methods teammates, once the termination is processed in UltiPro, the system sends a nightly file to IT, and the Air Methods teammate is removed from all applicable systems. |
- Protean currently deactivates any account if someone does not log into the system in a 90-day time frame.
- All teammates will adhere to the Teammate Handbook Policy Security and Data Monitoring.
- All teammates will adhere to policies in the Access Control Policy (now known as IS-G-80 Acceptable Use Policy) and Air Methods Password Standards. |

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4 *Aircraft, Base Medical, and Equipment Supplies*

4.1 Policy

To ensure that all medical support equipment and supplies at each base of operation are checked regularly, the following guidelines will be adhered to:

- The flight program will determine stock par levels which will allow support of operations for a period of 30 days.
- Base supplies will be monitored daily.
- All aircraft will be stocked with an extra bundle of cables to include an EKG cable, Blood Pressure cable, Pulse Oximeter cable, and Defibrillation cable and an additional End Tidal CO2 sampling line airway adapter. This extra bundle will be kept in the patient care area of the cabin at all times.
- A backup battery-powered portable suction device will be checked at the start of each duty shift to ensure it is operational and secured in the patient compartment.
- All medical equipment carried in the aircraft that utilizes an AC power supply cable shall be stored with a backup battery (if applicable) and each appropriate power supply cable in the aircraft.
- Medical equipment used during patient transport will be Air Methods equipment. Each clinician will be trained on the use of this equipment, and records of annual competency shall be maintained. If the need arises to utilize equipment from the hospital or EMS agency the clinician must be trained and competent in the use of the equipment prior to utilizing the device for patient care. An equipment in-service can be provided at the bedside by the sending clinician if needed. Clinicians are required to contact the Clinical Director before using non-Air Methods medical devices for patient care. Clinicians are required to seek the Pilot-in-Command (PIC) approval to secure and utilize the non-Air Methods medical device on the aircraft.

- IV solutions can be warmed in their manufacturer plastic over pouches not exceeding 104 degrees Fahrenheit. Per manufacturer recommendations, once IV fluids have been in the warming cabinet for their maximum time period, the containers should be removed and identified as having been warmed. They should not be subsequently returned to the warmer. The IV fluids may continue to be used until their factory-labeled expiration date, provided they have not been warmed more than once. This includes all IV solutions and pre-mixed medications in IV solutions (e.g. Mannitol).
 - The current maximum time period is 14 days.
 - IV solutions placed in the portable warmer shall be labeled with tape (marking should not be done directly on the bag) indicating the date the solution was placed in the warmer and the date the solution is to be changed.
 - IV solutions that have had the outer bag removed will be replaced after being open 24 hours.
 - Refrigerated medications that have been removed from refrigeration must be marked with the applicable out-of-fridge date. The date marked on the vial should be the new expiration date for the medication and the date must be clear and legible. Take care not to exceed the manufacturer's printed expiration date when marking the out-of-fridge date.
- Medications that require refrigeration must be dated with the out-of-refrigerator expiration date when removed from the refrigerator. The date must be written in a conspicuous area that is not removable (i.e., the cap) and written in an impervious marker.
 - At no time should the out-of-refrigerator expiration date exceed the manufacturer's expiration date.
 - All manufacturers have different lengths of time before medications expire once removed from the refrigerator. It is the responsibility of the Clinical Base Lead (or designee) to review the medication insert pamphlet for the out-of-refrigerator expiration date. Medications may be dated according to the length of time noted in the medication insert pamphlet. In the absence of a medication insert pamphlet, you may search for the package insert at the following website, [DailyMed](#). It is important to ensure that you have selected the correct manufacturer if there is more than one.

- In the absence of expiration date guidance from the medication insert pamphlet, the following expiration guidelines will be utilized:

TABLE 1. Expiration Guidelines

Medication Name	Days to Expiration Out-of Refrigerator
Ativan (lorazepam)	60 days
Succinylcholine	90 days
Cardizem (diltiazem) (pre-mix)	60 days
Nimbex (cisatracurium)	21 days
Rocuronium	60 days
Vasopressin	One (1) Year

- Medical equipment will be checked for the following:
 - All components are functional.
 - Batteries for medical equipment are maintained and conditioned periodically as recommended by the manufacturer and the prescribed Air Methods battery management schedule.
 - All AC power supply cables are functional.
- All medical gases will be checked for the following:
 - Compressed gas tanks are full, and sufficient quantities are available to support daily operations utilizing the PACO tool.
 - Re-supply tanks will be checked for content to allow operations for a determined time prescribed by the program.

4.1.1 The following actions are mandatory for each shift

- Aircraft supplies and equipment will be checked at the beginning of each shift and whenever there is a clinical flight team member change. A daily aircraft checklist will be completed that will allow for documentation of:
 - All medical equipment, medical gases including amount, date/time accuracy on medical monitors; medication and supply inventories; monthly expiration date, checks of medications and disposable supplies, daily aircraft cleaning, and disinfection.
 - Check and document aircraft cabin temperature in the region-specific Air Methods Daily Checklist in Protean.
 - Clinicians will follow the manufacturer and/or Air Methods clinical direction for the proper storage and temperature control for medications and IV fluids.

- Clinicians will follow the aircraft checklist and complete region-specific Air Methods Daily Checklist in Protean.
 - Aircraft checklists will be monitored online by medical leadership for completion of duties.
- Narcotics will be checked at any change of crew in accordance with the Controlled Substances standard.
- During the shift change aircraft check, the Zoll monitor batteries must be rotated.
 - The rotation order is as follows: charger > aircraft spare > monitor > charger.
- When completing the aircraft check clinicians are responsible for ensuring the aircraft oxygen supply system is fully operational and there is an adequate oxygen supply to safely complete a patient transport.
- Clinicians are required to turn on the aircraft oxygen supply system and test each flow meter on the medical wall to ensure adequate flow (recommend allowing oxygen to flow no less than 20 seconds from the meter). Depending on the aircraft configuration clinicians may need to request assistance from the pilot to turn on the oxygen supply system.
- Clinicians are required to complete the same oxygen flow check with each new portable oxygen tank onboard the aircraft.
- If a problem occurs with oxygen during transport, clinicians are required to immediately check the flow of oxygen again at the med wall.
- Complete the PACO tool at the beginning of the shift to validate all oxygen availability in the aircraft.
- Prior to the start of each patient transport, performing the PACO tool calculation is required to ensure there is enough oxygen to complete all legs of the transport.
 - Mission Ready Minimum Levels are as follows:
 - Onboard Gaseous Oxygen - 1200 PSI.
 - Portable Tanks - 1500 PSI.
 - Liquid Oxygen - 3.5 Liquid Liters.
 - If oxygen consumption is unknown base the calculation on 15 liters per minute for pre-transport.
- Current oxygen levels will be documented in the aircraft daily checklist within Protean as well as on each patient flight in the patient chart.

- If any non-standard oxygen tanks are utilized; including composite tanks, a conversion factor must be calculated and used for the PACO tool. The process is as follows:
 - Identify the oxygen tank's total gaseous volume. (Obtain from the oxygen distributor via direct communication or on the manufacturer's website).
 - Identify maximum fill pressure. (The pressure of the tank when it is first opened).
 - Divide total gaseous volume by maximum fill pressure to obtain that specific tank's conversion factor.
 - Example: Composite tank A comes filled at 1800 PSI when first opened. The manufacturer's website reports the tank can hold a total of 2400 liters. This equates to 2400L divided by 1800 PSI = 1.33 for the conversion factor.
 - If there are any questions regarding the tank conversion factor, a consultation with clinical leadership or patient safety is required.
 - Despite oxygen tank duration/calculations the pressure gauge should routinely be re-assessed.
 - Exceptions to the minimum PSI may be made in coordination with the Clinical Director. There may be select patient missions where it is safe to transport a patient with less than 1200 PSI oxygen onboard where the patient has minimal oxygen needs and there is an adequate available supply. The Clinical Director should be consulted regarding these situations prior to accepting a mission.

4.2 Repackaging Medical Supplies

4.2.1 Policy

Repackaging Medical Supplies

4.2.2 Purpose

To guide the conditions under which single-patient-use medical supplies and devices are repackaged to ensure infection prevention and patient safety.

4.2.3 Procedure

- Nonsterile items (e.g., a box of blood pressure cuffs) may be repackaged for patient use. Sterile items (e.g., endotracheal tubes, and intravenous catheters) must be stored in their original packaging until the time of use.

- Proof of the item's expiration date must be maintained in accordance with standard practice and regular review of medical supply expiration dates and package integrity must be conducted according to policy. When an item is repackaged, the expiration date from the original packaging carries forward.
- Corrugated tubing for use with the H900 humidifier may be obtained in bulk and cut to an appropriate length for a base's existing aircraft configuration to provide safe humidification for patients.
 - Prepare a clean area to cut and package the tubing. The table can be cleaned with disinfection wipes and allowed to dry according to the package time. Wash hands with soap and water and don gloves. Prepare scissors by thoroughly wiping them with disinfection wipes and allowing them to dry according to the package time.
 - Use an aseptic technique to cut the corrugated tubing with clean scissors and immediately place the tubing into a new Ziploc-style bag. Seal the bag and label it with the expiration date from the original package.

5 Aircraft Patient Limitations

5.1 Purpose

To guide the particular mission limitations of each type of aircraft in use by the Community Based Services Division.

5.2 Policy

The flight crew will follow all manufacturer's placards, operating procedures, and recommendations when utilizing patient litters and loading systems.

All Air Methods, LLC litters have been designed to meet the following FAA regulations for occupant weight, maximum load factors, and inertial forces during flight conditions: 14 CFR 27/29.303, 27/29.337, 27/29.561(a)(b), and 27/29.785 (f)(2).

The Air Methods Litter (PN 108-1080/108-1555) weight limit is based on the limitation when carrying the litter on the ground. The ALLS system was analyzed for a weight of 550 lb. on the very end of the platform when swiveled out for loading; therefore the 300-500 lb. weight limit was more critical.

For aircraft utilizing the Life Port Systems Aero Sled RS P/N 100-5108-1, the rated load per the manufacturer is one (1) human patient that the seat belts will secure around. When utilizing the Aero Sled with the cargo deck for isolette operations the rated load is placarded on the cargo deck in pounds.

TABLE 1. Ground Weight Limits

Aircraft	Patient Loading System	Ground Handling Limit
AS350	Air Methods Articulating Lightweight Loading System (ALLS)	300 lb. - 500 lb.
Bell 407	Air Methods Articulating Lightweight Loading System (ALLS)	300 lb. - 500 lb.
Bell 412	Air Methods Articulating Patient Loading System (APLS)	300 lb. - 500 lb.
Bell 429	Air Methods Roll-On Fold-Up Litter (ROFU)	315 lb. - 500 lb.
Bell 430	Air Methods Articulating Patient Loading System (APLS)	300 lb. - 500 lb.
BK117	Air Methods Roll-On Fold-Up Litter (ROFU)	315 lb. 500 lb.
BK117	Air Methods Ferno 28	400 lb.
BK117	Air Methods Ferno 35A	500 lb.
BK117	Air Methods Stryker MX Pro	650 lb.
EC130	Air Methods Lowered Patient Loading System (LPLS)	300 lb. 500 lb.
EC130	Air Methods EZ Lift 1300 Litter	300 lb. - 500 lb.
EC135	Air Methods Aft Loading Litter (no legs)	300 lb. - 500 lb.
EC135	Air Methods Roll-On Fold-Up Litter (ROFU)	315 lb. - 500 lb.
EC145	Air Methods Tracking Patient Loading System (TPLS)	300 lb. - 500 lb.
EC145	Air Methods Roll-On Fold-Up Litter (ROFU)	315 lb. - 500 lb.
EC145	Air Methods Ferno 35P	700 lb.
EC145	Air Methods Ferno 93	650 lb.
EC145	Air Methods Stryker MX Pro	650 lb.

*Certain circumstances may require different weight limitations – see the manufacturer’s guidelines.

While the LifePort loading system will likely handle much greater loads, occupants weighing more than 450 pounds may require additional support from clinical flight team members to ensure proper loading and transition and to prevent premature wear or other damage from occurring to the loading system.

Due to the useful load and seating configuration of the PC-12 fleet, these aircraft are often tasked with flying larger patients not capable of being transported by rotor wing aircraft. Patient condition, support equipment, available personnel, weight distribution, height, and anatomy should all be considered when evaluating whether the aircraft and team can accommodate larger patients.

All limitations listed in the current Pilatus PC-12 Airplane Flight Manual, LifePort, Inc. Patient Loading Utility System supplement and supporting documents apply. FAA Approved AFMS Revision E was current at the time of this writing, but later Revisions should be referenced as necessary.

The following limitations will apply to patient size for the PC-12 with LifePort PLUS Aerosled installed:

- **Maximum size** - Maximum patient size in any dimension or measurement will be ultimately determined by each crew member's ability to safely exit the aircraft in the event of an emergency. At no time will a crew member's ability to exit their seat be inhibited in any way. No part of a patient, equipment, or combination thereof will negatively impact the safe egress of a passenger seat. Crew members will, at all times, have access to an emergency exit point in the aircraft and will be familiar with its operation. A portion of the patient protruding into the aisle is not considered blocking access to an emergency exit.
- **Weight** - For patient transport, there are no manufacturer weight limitations for the occupant Aerosled, or Load Ramp. Patient weight limitation will be based on having adequate personnel on site, or support equipment installed (loading hoist or similar) to assist with safely loading. Adequate personnel will be determined by the crew tasked with the patient transport. When possible, additional help should be requested from the sending and receiving facilities or ambulance crews. The health and welfare of our teammates and patients is paramount.
- **Girth (circumference)** - Maximum patient girth (also called the circumference) will be determined by what can be safely secured utilizing the shoulder and torso straps. Seat belt extensions that are approved by the manufacturer may be utilized as necessary.
- **Width** - Maximum patient width will be limited to 30 inches for aircraft with the corporate or executive interior installed. Maximum patient width will be limited to 34 inches for aircraft with the EMS or medical interior (smooth inboard cabin wall adjacent to the patient). Careful attention should be paid when maneuvering the 90° turn from the load ramp to the litter platform, as the patient may become unstable and tip to the side potentially causing injury to the patient or crew member. Please reach out to your Aviation or Maintenance COE leadership team if you are unsure of your aircraft configuration.

Air Methods clinicians working at CBS bases will not decline flights for patient weight without first making patient contact and/or speaking with the appropriate Clinical Director and Area Manager. Hospitals will often report patient measurements inaccurately and we will do our best to respond, evaluate, and support our customers. Pilots will accommodate this need by providing transportation to the sending facility so the on-site evaluation can be made.

Pilots should contact their Aviation management team for any questions about this policy or its application.

6 *Altitude Considerations for Patient Transport*

6.1 Purpose

To define considerations for altitude effects in certain patient populations and to protect patients from environmental extremes.

6.2 Policy

The maximum altitude allowed during patient transport is outlined in the General Operations Manual (GOM). Refer to GOM for operational considerations.

- Clinical flight team members will administer supplemental oxygen to patients as outlined in the Air Methods Patient Care Guidelines.
- The following are recommended maximum altitude limitations for patients with specific medical conditions in an unpressurized aircraft cabin.
 - 5000 feet or less, as safety allows:
 - Respiratory disease with a vital capacity of <900 ml (adult)
 - Recent MI
 - Cardiac valve defect
 - Space occupying lung lesions (abscess, cyst, and tumor)
 - Anemia
 - <8.5 gm Hgb
 - Hct <25%
 - Lowest safe altitude (less than 1000 feet is adequate).
 - Decompression sickness
 - Untreated pneumothorax
 - Cerebral air embolism
 - Intra-cranial air
- The pilot-in-command will have the final determination of the lowest safe altitude.

6.3 Altitude and Oxygen Delivery

The pilot can work with the crew to adjust the altitude so that optimal oxygen delivery is provided for the patient.

- Since altitude affects oxygen delivery, administer supplemental oxygen as needed to:
 - Cardiac patients
 - Shock or impending shock
 - Head injuries
 - Eye injuries (hypoxia dilates blood vessels in the eye, leading to increased intra-ocular pressure).
 - Known pulmonary disease/pulmonary injury.
- In the event of rapid decompression (in the following order):
 - Administer oxygen to yourself first
 - Administer oxygen to your pilot if not already done
 - Administer oxygen to your patient
 - Administer oxygen to any other crew members
- Be familiar with signs/symptoms of hypoxia in the patient or crew members:
 - Visual disturbances
 - Euphoria
 - Drowsiness
 - Dizziness
 - Hot/cold flashes
 - Tingling of extremities
 - Headache
 - Hyperventilation (early symptom)
 - Cyanosis (late symptom)

6.4 Gas Expansion

Trapped gas expands as the aircraft ascends and contracts as the aircraft descends.

6.5 Patients at Risk for Gas Expansion

- Gastrointestinal patients.
- Air leak/potential air leak (e.g., pneumothorax, pneumopericardium, pneumomediastinum, penetrating head injury).
- Orthopedic injuries (swelling may increase with altitude: monitor air splints and casts closely).

6.6 Effects of Pressure Change on Equipment

- Drip chambers: as the gas expands and contracts, the IV rate will increase and decrease if the IV is not regulated by a pump.
- ETT cuffs: monitor for air leaks.
- Chest tubes: a Heimlich valve should be inserted between the chest tube and the water seal. If a chest tube has been removed within a few hours before flight, pneumothorax could reoccur.
- Blood pressure cuffs: Deflate completely after taking a BP.

6.7 Other Physiological Considerations

6.7.1 Temperature and Humidity

For every 1000-foot increase in altitude, temperature drops 2 degrees Centigrade. Humidity also decreases as altitude increases. Provide proper warmth for the patient and monitor skin temperature. Provide humidified oxygen when practical to avoid drying of secretions in the respiratory tract and/or endotracheal tube.

6.7.2 Motion Acceleration

Acceleration in motion can lead to a change in heart rate; pooling of blood in extremities; changes in rate, rhythm, and depth of respirations, increased intracranial venous pressure.

6.7.3 Motion Sickness

Motion is sensed by the semicircular canals in the inner ear. This may lead to spatial disorientation or motion sickness. Refer to Air Methods PCGs for medical treatment.

Non-medical treatment may include:

- Laying supine with head movement restricted.
- Visual fixation on a stationary object.
- Cooling.
- Loosening of restrictive clothing.

6.7.4 Noise/Vibration

- Especially common in helicopters.
- Tends to decrease communication with the patient.
- Mitigation may include providing a headset for the patient to allow communication and for noise attenuation.

6.7.5 Physiological Reaction to Vibration

The physiological response to excessive vibrations may include increased:

- Pulmonary ventilation
- Heart rate
- Perspiration
- Motion sickness
- Vasodilation and related heat loss

6.8 Altitude-Associated Changes in Physiological and Environmental Parameters

Tables describing the following may be found in the Appendix:

- Altitude-Associated Changes in Physiological and Environment Parameters - See Table 2: Altitude Associated Changes in Physiological and Environment Parameters.

6.9 Altitude and Flight Physiology/Stresses of Flight

Tables describing the following may be found in the Appendix:

- Expected Oxygen Delivery - See Table 3: Expected Oxygen Delivery.

6.10 Time of Useful Consciousness

Tables describing the following may be found in the Appendix:

- Time of Useful Consciousness - See Table 4: Time of Useful Consciousness.

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7 Ambulance Assist for Air Medical Transport

7.1 Purpose

To establish guidelines by which ground ambulances can be utilized to augment helicopter patient transport.

7.2 Policy

- Due to remote landing zones and the unavailability of helipads in close proximity to facilities and agencies that utilize the air transport service, it may be necessary to utilize ground ambulance transport for some aspects of patient transport.
- If ground transport is required to complete a transport:
 - Patients transported in ground ambulances will be transported only on stretchers which are approved for and can be locked into those ground ambulances.
 - The clinical flight team must also transport all needed critical care equipment off the aircraft (regardless of what is already on the ambulance) to ensure all necessary equipment is on board the ambulance while in transit to or from the aircraft and this equipment will be adequately secured in the ambulance. Refer to Air Methods Ground Transport Checklist - 5711 (a sample of this form can be found in the Appendix of this manual – the most current version of the actual forms are located on the FlightDeck > Resources > Company-Wide Forms (Air Methods)).
 - Prior to transferring in the ground ambulance, the clinical flight team must ensure that there is an adequate amount of onboard oxygen, a working inverter, and working onboard suction. In addition to this, the clinical flight team should always have their own portable oxygen tank and portable suction with them as a backup.

- The clinical flight team will be required to use seatbelts and shoulder harnesses in rear-facing seats and lap belts in side-facing seats as provided in the ground ambulances. The clinical flight team must also ensure that the patient is appropriately secured prior to the movement of the ambulance.
- All such ground transports are subject to applicable anti-kickback requirements, and in no case shall any remuneration be offered, given, or accepted to or from a ground ambulance provider in exchange for referrals or the promise of referrals. The purpose of such ground transports is solely for patient care and continuity of care for Air Methods' patients.

8 Cardiac Monitor, ePCR Data Import

8.1 Purpose

To define the policy for storing, viewing, and transferring patient data from a cardiac monitor to an external system, such as a web-based cloud server and to an electronic patient care record. For the purpose of this policy, the word "Monitor" or words "EKG Monitor" will be used to refer to a Cardiac Monitor.

EKG monitors collect and store a continuous record of patient data based on the specific monitoring equipment connected to the patient. This data can be used to import recorded values into electronic patient care record systems through a web portal hosted by the EKG monitor manufacturer. The use of this technology assists in documenting accurate patient data and reduces the time needed to manually input data. This policy will define how that data is used once it is imported into a patient's care record.

8.2 Policy

- Each Air Methods CBS base location will have an EKG monitor configured to communicate through the base's Wi-Fi access point and the clinical iPad or crew cellphone hot spots. Each Air Methods CBS region will host an online data repository for the collection of patient care data collected by the EKG monitors. EKG data transfer configuration instructions can be found in the user [Section 8.4 Job Aid: Zoll X-Series Data Upload and Import](#).

- Air Methods CBS base EKG monitors are configured to import patient vital signs at 5-minute intervals into the ePCR and other events that may occur at any time during a patient transport and ECG, SpO₂, ETCO₂ strips at 10-minute intervals or for events such as defibrillation, cardioversion, or external pacing. All available vital signs and other monitoring records flagged for import in Zoll Online should be imported into the ePCR. Data should be reviewed for accuracy. For cases where data is known to be inaccurate, it should be deselected and not imported into the ePCR. It is the professional and legal responsibility of the clinicians to ensure that all documentation is accurate. Missing data or specific vital signs, such as an SpO₂ reading should be briefly explained in the related activity log entry identifying the cause, measures taken to alleviate the issue, and any assessment info used to ascertain that the reading is not accurate (such as assessment of mental status or capillary refill). Please see the Section 8.4 Job Aid: Zoll X-Series Data Upload and Import for additional details.
- Once data has been imported into the ePCR, it is the responsibility of both clinical teammates to validate the accuracy and completeness of the data. All reviewed for accuracy data collected from the EKG monitor will be displayed in the activity log of the ePCR and monitoring strips are uploaded as attachments.
- It must be understood that the clinical transport environment includes factors that can affect the quality of patient data collected by electronic monitoring equipment. Clinical teammates must monitor equipment closely during patient transports to troubleshoot equipment problems and correct issues in a timely manner. It is encouraged that Clinicians will not troubleshoot equipment problems for greater than one vital sign period during patient transport (i.e., 5-10 minutes dependent upon acuity and type of transport). Clinicians are encouraged to utilize other means of data collection with the Zoll EKG Monitor if greater than one vital collection period and document as such. Below are some examples of environmental factors prevalent in transport that can adversely affect electronic monitoring equipment. This list is not all-encompassing, other patient and environmental factors may exist that prevent monitoring equipment from collecting accurate data.
 - Patient extremity temperature
 - Patient movement
 - Vibration
 - Altitude
 - Direct sunlight

- Attachments that are imported from the EKG monitor into the ePCR must not be removed from the ePCR or altered in any way. Unless otherwise stated as above such as inaccurate readings documented by specific situations. Additional attachments containing accurate electronically collected data may be uploaded in addition to the imported attachments when needed.

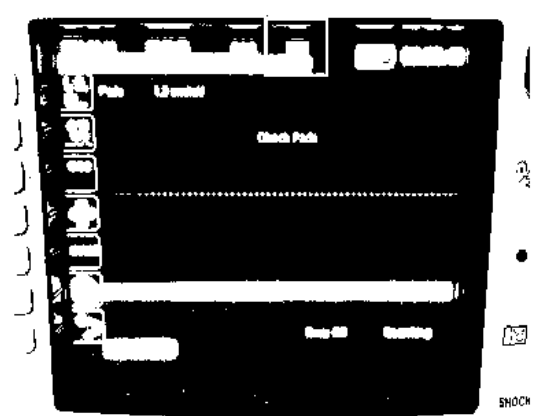
8.3 Data Use and Auditing

- Data collected by Air Methods EKG monitors and imported to the manufacturer's web portal is subject to auditing by authorized members of the clinical leadership team only. Data recorded in Air Methods ePCRs may be subject to validation by comparing it to the data recorded directly by the EKG monitor and exported into the manufacturer's web portal.

8.4 Job Aid: Zoll X-Series Data Upload and Import

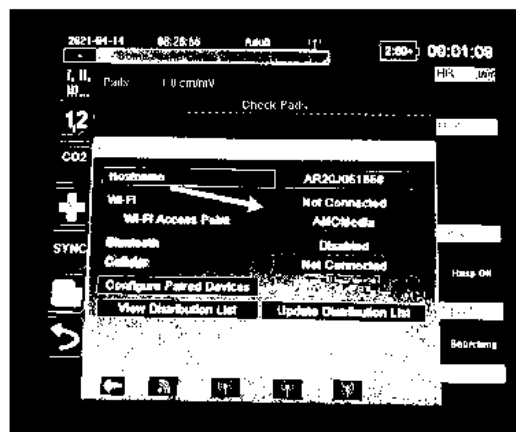
Summary: Instructions for uploading and importing saved Zoll X-Series data.

FIGURE 1: Part I - Data Upload Instructions

1. Turn on the monitor and identify if the monitor is connected to the Wi-Fi. If the ANTENNA ICON at the top has GREEN lines on the side, it is connected, and you can skip to step 6. If the ANTENNA ICON at the top has YELLOW dots on the sides, it needs to be connected to the Wi-Fi to continue. Proceed to step 2.	
---	---

2. Connected to Wi-Fi

- Use the arrow buttons on the right of the monitor so that the blue box highlights the ANTENNA icon. Click the circle select button.
- Use the arrows to scroll to the Wi-Fi Access Point and click the circle select button.

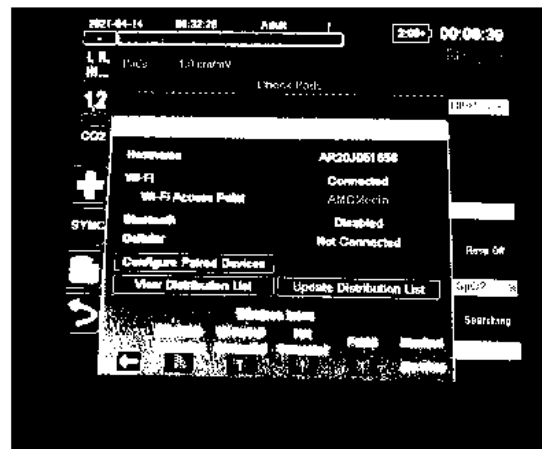


3. Assure the Air Methods Media is selected and there is a GREEN check next to the access point name. Use the arrows to scroll down to the back arrow and click the circle select button.



4. Once connected the Wi-Fi Access point Name will turn Green. Use the arrows to navigate to the back arrow at the bottom and click the circle select button.

- If the Air Methods Media is not available, go to step 5. Skip this step if you are connecting to the Air Methods Media Network.



5. Connect to another Network:

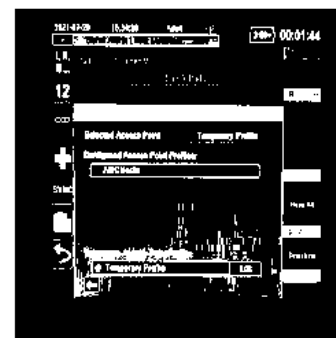
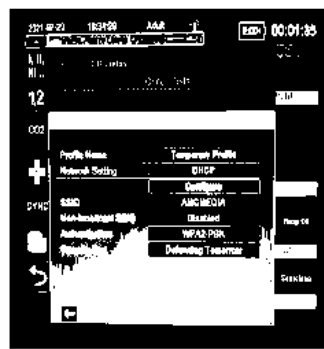
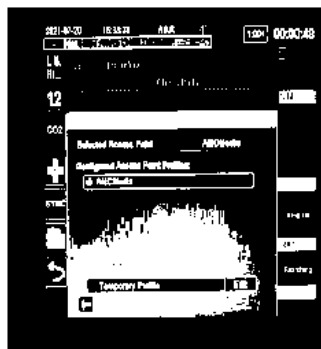
If you do not have the Air Methods Media network, but there is another network available, you can set up a temporary network.

- Go to the available networks from step 3.

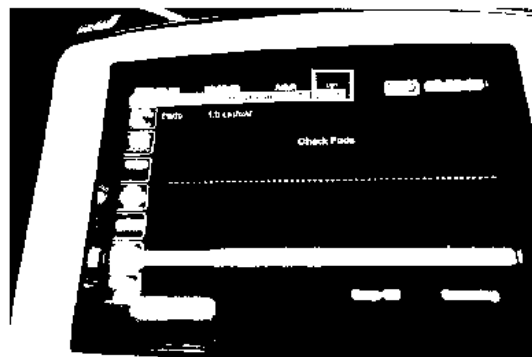
- Scroll to the bottom and select the "Edit" next to the Temporary Profile.
- Enter the Network Name in the SSID field and the network password in the Security Key Field.
- Return to the previous menu and select the temporary Profile. The monitor will now connect to this network.

*****It is recommended to program the Temporary Profile to sync with the Air Methods Clinical iPad Hotspot.**

The Temporary Profile will be saved until it is changed. You can only have one temporary profile at a time.

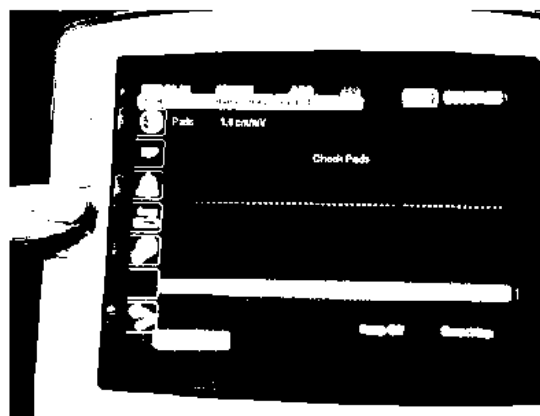


6. Once connected you will see the ANTENNA ICON at the top has GREEN lines on the side.



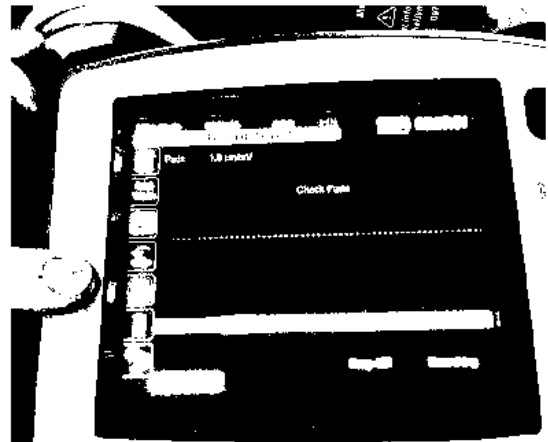
7. Select the files to upload -

- Navigate to page 2 of the monitor menu by clicking the ARROW Soft Key on the bottom left.
- Click the Soft Key next to the LOG ICON Soft Key.

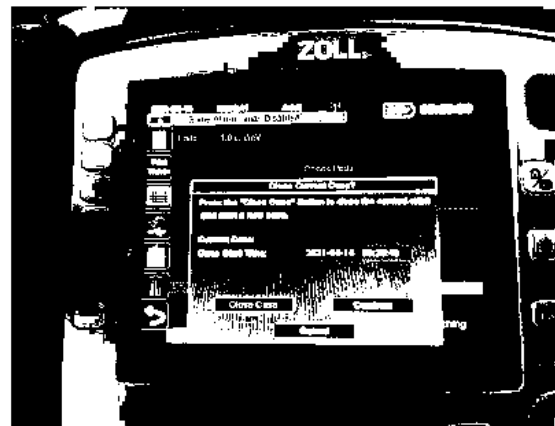


8. Select the files to upload -

- a. Click the Soft Key next to the LETTER/ ENVELOPE ICON.



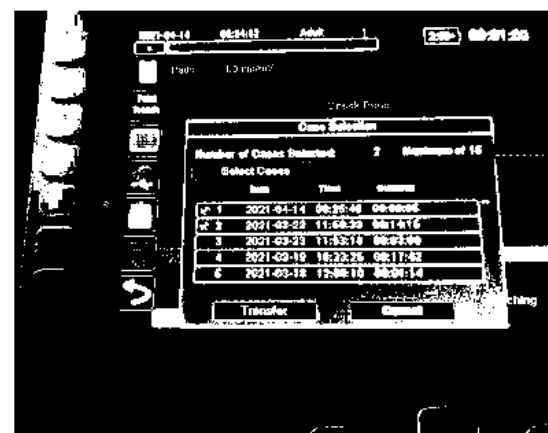
9. The monitor will prompt you to close the current case. You will need to do this if you are in the case that you intend to upload. If not, you can select continue.



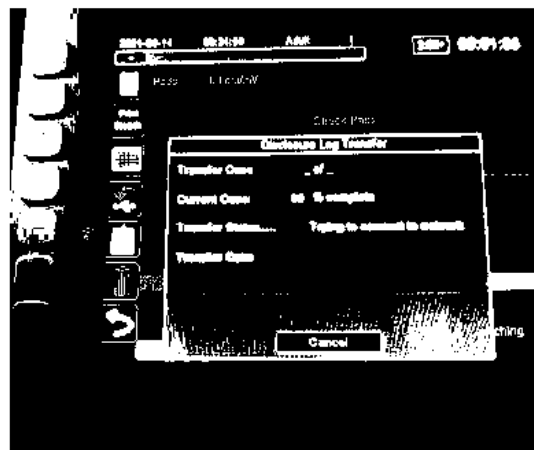
10. Next use the navigation arrows and the circle select button on the right to select the case you want to upload. You can upload a maximum of 15-cases at a time.



11. Once you have selected the cases, click the HOME Button on the right to jump to the TRANSFER button at the bottom. Click the circle select button to upload the cases.



12. You will then see the Disclosure Log Transfer status box until the upload is complete.



13. Once the upload is complete, you will see a completions verification message. YOU can then power off the monitor and move on to Part II.

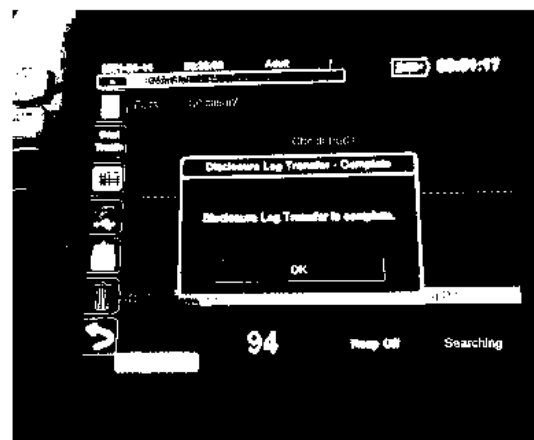
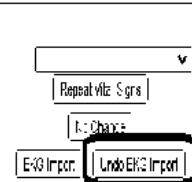


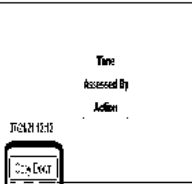
FIGURE 2: Part II - Data Import Instructions

[illegible]

17. If you discover that you have accidentally imported the incorrect case, you can click on the "Undo EKG Import" button on page 8 and that will return the data to the Zoll cloud.



18. On page 8, only actual vital signs will be included in the import to the vital signs field. Other information such as "HR Methods" "BP Methods" and "ECG Method" do not import.



19. First, you must log into a single activity event/set of vital signs and enter the details, and click Save.

Time	Assessed By	Action	HR	BP	SpO2	Temp	Respiratory Rate	ECG Method	ECG Result
1/1/2024 12:00:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:01:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:02:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:03:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:04:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:05:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:06:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:07:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:08:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:09:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result
1/1/2024 12:10:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result

20. Next, click the "Copy Down" button that is below the timestamp for the activity event that you edited.

21. All of the information (non-vitals) will then Copy Down into all the fields below the set of vitals selected.

IMPORTANT:

- The "copy down" button will only add detail to empty fields. So, if you start at the top, all the subsequent vital signs will have the same detail copied down.
- If there is a change, you will need to go into the appropriate event entries/vital signs and edit the required elements individually.
- You cannot go into an event with details already added and change it, then use the "Copy Down" button again.

End-Result following Copy Down

Time	Assessed By	Action	HR	BP	SpO2	Temp	Respiratory Rate	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:00:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:01:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:02:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:03:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:04:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:05:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:06:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:07:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:08:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:09:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result
1/1/2024 12:10:00	John Doe	Save	72	120/80	98	36.5	18	ECG Method	ECG Result	ECG Result	ECG Result

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9 *Carry-On Baggage*

9.1 Purpose

To ensure that all baggage carried on Air Methods aircraft is free from potentially hazardous materials and stowed in accordance with the General Operations Manual.

9.2 Policy

- Prior to flight, the clinicians will notify the pilot there is a request to carry on baggage.
- The pilot will follow the General Operations Manual Policy 2.45 (Safety Belts, Child Restraints, and Carry-On Baggage). All pilots must ensure to the extent possible that passengers, carry-on baggage, and cargo are checked and found free of hazardous material and unauthorized weapons.
- Clinicians will adhere to the General Operations Manual Policy 2.7 (Cell Phone and Portable Electronic Devices (PED)) utilization.
- If an item is hazardous or prohibited or the pilot is unsure, then that item shall not be permitted on the aircraft.
- The pilot will make the determination concerning baggage for purposes of weight and balance.
- Items with a Li-ion battery will be stowed where they can be accessed by a flight crew member.
- Clinicians will adhere to the General Operations Manual Policy 2.6 (Carriage of Weapons) when managing a situation involving deadly or dangerous weapons and aerosol products.
- Refer to the Patient Care Documentation policy regarding proper citation of the patient's belongings.

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10 Clinical Case Reviews

10.1 Purpose

The clinical case review is a confidential activity designed to evaluate a clinical case through the lens of our just culture, with the intent to identify opportunities to improve patient safety and healthcare quality.

10.2 Participants

The purpose of a clinical case review is targeted towards the improvement of patient safety and healthcare quality, so standard clinical case review attendance is limited to clinical attendees. This environment promotes teaching and learning in a small group environment to promote psychological safety and clinically-oriented discussion. Participants are:

- Clinicians who were involved in the patient care
- Medical Director(s)
- Regional Clinical Director II, as able to attend
- Regional Clinical Director I
- One representative from Clinical Education
- One representative from Patient Safety

If a critical clinical event occurs, the group may include additional clinical leaders, such as the Clinical Vice President and the Regional Clinical Director II, as well as additional participants from Clinical Education or Patient Safety. Comprehensive IRB process may follow according to policy. Issues specific to a base may be shared after the call with the Clinical Base Lead (CBL) by the Clinical Director(s).

If an operational or customer service concern is reported during the clinical case review, the clinical care can be reviewed with the listed clinical participants and any necessary feedback will be relayed to other COEs or external stakeholders by the Clinical Director(s).

For evaluation of nonclinical operational concerns (aviation safety, human resources concerns, medication compliance investigation, multidisciplinary operational concerns, etc.), the purpose of the meeting shifts and participants from other COEs may attend the case review. Clinicians will be made aware of the purpose of the call ahead of time. Coordination of this type of meeting often takes place before or after a standard clinical practice case review, separate from the clinical teaching and learning which occurs inside of a clinically-focused review. Clinical care and guidance and any sharing of protected health information (PHI) takes place inside of a clinical case review to ensure corporate compliance with Air Methods' Health Insurance Portability and Accountability Act (HIPAA) policy.

10.3 Scheduling

Because the purpose of the clinical case review is clinical education and healthcare improvement activity, Regional Clinical Leadership will schedule clinical case review calls to try to coordinate the group to accommodate all participants. The minimum attendance should include a Medical Director, a Regional Clinical Director I/II, an Clinical Educator, and a representative from Patient Safety.

The invitation to the call should explain to the participants that:

- The purpose of the case review is to discuss patient care and clinical operations.
- To request that the clinicians review the patient care record prior to the call and come prepared to present and discuss the case.
- The focus of the call is clinical education and healthcare systems improvement.
- The lens of our evaluation is Air Methods' commitment to a Just Culture.

10.4 Format

The Regional Clinical Director I or Director II leads the call. They begin with the policy purpose statement and will explain the usual process of the call to participants, noting that the purpose of the call is to improve the clinical practice of clinicians and to improve the clinical systems in which clinicians practice, to the benefit of both clinicians and patients.

The clinicians will present the case to the group and reflect upon their clinical practice and any successes and opportunities noted. Feedback and clinical education will follow; the Medical Director is asked to contribute their comments and ask questions first, followed by Clinical Education, the Regional Clinical Directors, and then last, the Patient Safety team.

The proceedings are strictly confidential, in compliance with Air Methods' healthcare compliance policy and intent to protect patients' PHI. Any questions or concerns about the clinical case review process may be escalated to Clinical Leadership.

11 Clinical Communication Guide

11.1 Purpose

To provide guidance to Air Methods clinicians on when and who to contact in the leadership team.

11.2 Policy

The following list will serve as a guide to determine when to reach out to leadership and who the appropriate leader to call is, depending on the topic. There are times when unusual occurrences happen during the clinical team member's shift which would necessitate the need for the clinician to contact a member of the leadership team for notification or guidance.

Calling leadership to report an unusual event, or adverse event is a proactive safety action. The following events should result in a call:

11.2.1 Clinical Director

1. All patient and teammate injuries regardless of the extent of injury.
2. Blood transfusion issue.
3. Any customer complaints that are connected to patient care. This can be from a patient, their family, the EMS agency, referring or receiving hospital, police, fire agencies, and/or bystanders.
4. Whenever a call is received from law enforcement, an attorney, or a government agency.
5. Failure to complete a clinical advanced skill successfully.
6. Delays in transport greater than two hours.
7. Oxygen issues that affect patient care.
8. Exposure to a communicable disease.
9. Medication errors or omissions.
10. HIPAA violations.
11. Inability to reach the Medical Director to discuss a patient.
12. Requests that are outside your scope of care.

13. Controlled Substance Issues - missing, broken, tampering, and/or problems with Cubex.
14. Questions regarding Advanced Directives/DNR orders.
15. Medical care provided to a non-transport patient or teammate.
16. Issues with specialty team transport.
17. Aircraft/ground ambulance issues that affect patient care and have not been resolved through aviation, maintenance, ambulance operator, and area manager.
18. Loss of patient belongings.
19. For an individual or team briefing after a difficult transport.
20. Issues related to disaster response that affect patient care.
21. Potential substance abuse of another teammate if the Area Manager is unavailable.
22. Licensure or credentialing issue.
23. Medical equipment failure resulting in out-of-service time (First call Area Manager, then Clinical Director).
24. DEA and CLIA registrations.
25. Transport logistics involving/impacting patient care.
26. Medical supply questions and/or concerns.
27. Unusual patient requests or situations.
28. Clinical care concerns.
29. Open patient care records in emsCharts - clinicians must have permission to leave the base with an open/unfinished patient care record.
See Section 60.8 Open or Incomplete Patient Care Record Requirement.
30. Specialty team professional licensure and certifications.
31. emsCharts questions/assistance.
32. Protean questions/assistance.
33. Management of professional licensure and certifications - Air Methods Credential team (Clinical Directors involvement when there is a breach in policy).

The Clinical Director will determine if any of the above events require contact with the Patient Safety Director/Manager or the Director of Clinical Compliance.

11.2.2 Clinical Education Manager

1. Orienteer schedule, feedback/concerns and progress.
2. Clinical training requirements, schedule, and/or concerns.
3. Outreach Education content and contact hours.
4. Specialty certification review courses (FP-C, CFRN).
5. American Red Cross (ARC) course requests must be sent to the Regional Clinical Education Manager.

11.2.3 Area Manager

6. State/agency EMS licensure (primary contact, clinical offers guidance).
7. Marketing/PR approval and/or concerns (include Business Development as applicable).
8. Customer complaints (include Business Development and COE leadership as applicable).
9. Clinical schedule (clinical offers guidance).
10. Shift extension request.
11. Time out request.
12. Clinical personnel issues/concerns.
13. Clinical employee disposition (title changes, transfers, and separation).
14. Work related injuries.
15. Base needs (utilities, furniture, etc.).
16. Clinical Leads report to the Area Manager (Clinical Leads refer to the Clinical Director for all patient care-related questions/concerns).
17. Open or missing specialty team patient care records and supporting documentation (Clinical Director will offer assistance as needed).
18. Clinical conference education requests.

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12 Clinical Conference Attendance

12.1 Purpose

To standardize a process to support clinicians in attending conferences for contact hours and/or presenting on behalf of Air Methods.

12.2 Clinical Training Allowance

- A clinical training allowance of \$600 annually will be provided for attending clinical, leadership, or career training which would benefit growth in their role at Air Methods, and request reimbursement for the course cost. This allowance should be utilized first for attendance at a conference, however, financial support for presenting at a conference may be approved outside of this allowance. The selection process outlined in this policy is not applicable to the use of this \$600 allowance and that process remains available beyond the Clinical Training Allowance. The Clinical Training Allowance is only applicable for the cost of the applicable course or conference and not the associated travel expenses.

12.3 Selection Committee

- The Area Manager and Clinical Director II if the clinician is attending the conference for clinical education purposes.
- The Director of Clinical Research and Director of Clinical Education is added to the selection committee if the clinician is presenting content during the conference.

12.4 Application for Attendance of Conference

- The clinician must apply no less than 60 days before the conference start date.
- Conference attendance application is associated with the following link:
<https://app.smartsheet.com/b/form/d702ed5f4ac6414d88ff1bcd0bd552e0>.

12.5 Selection Standards

- Candidate must be in good standing to include:
 - No disciplinary action in the past 12 months.
 - No credentialing lapses in the past 12 months to include meeting the seven-day renewal requirement.
 - Must follow clinical weight policy or show consistent improvement as outlined in the policy.
 - Full-time and part-time clinicians who are not currently on Leave of Absence are eligible. Part-time clinicians must meet the staffing requirements.
 - Conference attendance will not result in out-of-service time.
 - Must have a current Air Methods approved specialty certification.
 - If the clinician is presenting content during the conference they are required to submit their presentation to their Clinical Director II, Director of Clinical Education, and Director of Clinical Research no less than 30 days before the conference starting date. The Clinical Director II, Director of Clinical Education, and Director of Clinical Research must approve the presentation before presenting during the conference.

12.6 During Conference

- The clinician will assist with the conference marketing booth activities as requested during breaks or outside of educational presentations.
- The clinician will wear a flight suit if applicable otherwise dress in business casual during the conference.
- Clinicians' conference hourly pay and expense reimbursement will be determined before attending the conference by the selection committee.
- The clinician is expected to represent themselves and Air Methods professionally.

12.7 Upon Completion of Conference

- The clinician is expected to present a presentation reflecting key talking points associated with one or more of the educational offerings they attended. The talking points should include at a minimum a summary of the topic(s), learning points, and reference research findings and how they impact the air medical industry if applicable.
- Clinicians' Clinical Director II and Clinical Education Manager must review and approve the presentation content before presenting it to their peers and leadership. If the presentations include clinical research content the Director of Clinical Research must be included in the approval process.
- The Clinical Senior Leadership Team must be aware of the date of the presentation.

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13 Clinical Council and Committees

13.1 Purpose

To outline the various committees involved in clinical operations for Air Methods.

13.2 Policy/Procedure

1. The Senior Vice President (SVP) of Clinical Services and the two Vice Presidents (VPs) of Clinical Services will co-chair the Air Methods Clinical Council. This committee will meet quarterly at a minimum and will consist of the following attendees:
 - a. Director of Clinical Quality
 - b. Senior Clinical Leadership Team
 - c. Physician's Advisory Board
 - d. Clinical Directors
 - e. Clinical Education Team
 - f. Patient Safety
 - g. Research Director
 - h. Compliance Director
2. During the Clinical Council, the Senior Leadership Team will provide a general update and each of the standing clinical committees will provide a report to the group.
3. The following committees and subcommittees report to the Clinical Council:
 - a. Research Committee
 - b. Patient Safety Committee
 - c. Operational Committee
 - i. Supply Subcommittee
 - ii. Equipment Subcommittee
 - d. Education Committee

- e. Compliance Committee
 - i. Forms Subcommittee
 - ii. Medical Operations Manual Subcommittee
 - iii. Utilization Review Subcommittee
 - f. Quality Committee
 - i. Patient Care Guideline Subcommittee
4. At any point the Clinical Council recognizes a specific need not being met within the clinical COE an additional Subcommittee may be added on a temporary or long-term basis.

14 *Clinical Crew Configuration for Medical Transport Missions & Safety Officer Duties*

14.1 Purpose

To define minimum staffing patterns acceptable for completing medical transport missions as defined by state and regulatory requirements.

14.2 Policy

- Minimum staffing requirements for all patient care missions will be comprised of two Air Methods clinical personnel.
- For critical care or air medical missions, at a minimum, one of the team must be a registered nurse. Both clinical team members will be qualified to provide critical care in the air medical transport environment as dictated by state and local regulatory requirements and in meeting CAMTS standards.
- Completion of any patient care transport mission with staffing below the above-stated minimum requirements is prohibited.
- Each program may define the minimum staffing patterns acceptable to meet state and regulatory requirements and CAMTS standards when completing specialty medical transport missions that fall within the program's scope of care. Some examples of staffing considerations include specialty transports (Neonatal, pediatric, IABP/VAD, etc.).
- Any modification of required staffing minimums due to specialty team missions must be reviewed and approved by the Vice President of Clinical Services and the Regional Vice President.

14.2.1 Safety Officer's Role in Specialty Transports

- A safety officer is defined as a regularly scheduled clinician who meets all Air Methods' annual educational and safety requirements.
- The safety officer must be educated regarding in-flight and surface treatment modalities, altitude physiology, general aircraft and ambulance safety, and emergency procedures.

- It is recommended that a safety officer should be present on all specialty team transports in which the specialty team crew members do not meet the GOM standard of a medical flight crew member.
- Duties of safety officers will include the following:
 - Ensuring the transport crew is seated with their seatbelts fastened.
 - Closing of aircraft doors (and rear clamshell if applicable).
 - A medical interior resource for specialty teams (mounts for medical equipment, aircraft oxygen, medical air compressor, and aircraft suction).
 - Completing final walk-around and ensuring aircraft is safe for take-off.
 - Utilizing radios for air and ground communications as needed.
 - Assisting pilot with emergency procedures as needed.
 - Maintaining currency and utilize NVGs during transport as needed.
- Any modification of the above-listed duties must be reviewed and approved by the Regional Clinical Director and Vice President of Clinical Services. Additional requirements may be added depending on the program area.
- If a safety officer is not present or cannot be utilized, the following annual educational requirements for the crew as set by CAMTS 12th edition standards should be met:
 - a. Altitude physiology.
 - b. Day-and night-flying protocols.
 - c. EMS communications (radios) and familiarization with the EMS system.
 - d. Extrication devices and rescue operations (ranging from familiarity to explicit training depending on the service's mission statement) (RW).
 - e. General aircraft safety. (It is strongly recommended to have the aircraft physically present when providing this training.) This training addresses: (RW/ FW).
 - Aircraft evacuation procedures (exits and emergency release mechanisms) to include emergency shutdown - engines, radios, fuel switches, electrical and oxygen shutdown.
 - Aviation terminology and communication procedures to include knowledge of emergency communications knowledge of emergency communications frequency.
 - In-flight emergency and emergency landing procedures (i.e., position, oxygen, securing equipment).
 - Safety in and around the aircraft, including national aviation rules and regulations pertinent to medical team members, patient(s), and lay individuals.

- Specific capabilities, limitations, and safety measures for each aircraft used, which includes specific training for backup or occasionally used aircraft.
- Use of emergency locator transmitter (ELT).
- Minimal safety requirements on ground support ambulances used away from base for fixed wing operations, for example, an adequate number of functioning seat belts for all team members, no loose equipment.

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15 Clinical Flight Shift Scheduling and Risk Assessment Tools (Shift Extension and Time Out)

15.1 Purpose

To ensure staffing patterns promote competent and safe practices for Air Methods Clinicians while mitigating the risk of fatigue.

15.2 Policy

15.2.1 Section I: Clinical Staffing Practices

Clinical staffing practices will adhere to the following criteria:

- Scheduling will demonstrate strategies to minimize duty-time fatigue and will be monitored closely by management.
- In accordance with CAMTS Standards, clinicians must have at least 10 hours of uninterrupted rest prior to any scheduled shift of 12 hours or more, or prior to any on-call shift where they could potentially be scheduled for a shift of 12 hours or greater.
- Clinicians scheduled to work beyond 24-hour flight duty shift, for any reason, are required to clock out at the end of 24-hours and clock in for the start of their extended shift.
- The Table 15.3: Extended Duty Risk Assessment Matrix will be completed anytime a teammate must extend their shift except if the cause of the shift extension is due to a patient care mission near the end of their shift.
- Area Managers and Crew Scheduling will continually monitor clinical flight schedules to minimize duty-time fatigue and shift extensions. Clinical Directors will collaborate with Area Managers and Crew Scheduling to identify ways to mitigate duty-time fatigue and shift extensions.
- Scheduled clinical shifts will not routinely exceed 24 hours in length and will meet the following criteria:
 - Clinical personnel are not required to routinely perform duties beyond those associated with the transport service.
 - Clinical personnel are provided with access to and permission for uninterrupted rest after daily medical personnel duties are met.

- The physical base of operations includes an appropriate place for uninterrupted rest.
- At bases where consecutive shifts are not normally scheduled, shift extensions greater than 24 hours may be approved pending the outcome of the Extended Duty Risk Assessment. Otherwise, clinicians must have a minimum of 10 hours scheduled time off prior to returning for an additional shift.
- Base locations that are approved by the Senior Vice President of Clinical Services for routine scheduling of consecutive 24-hour shifts must meet the following criteria to maintain compliance with CAMTS Standards:
 - The approved base location averages less than one transport per day.
 - Clinicians are provided at least 10 hours of rest in each 24-hour shift period.
 - Base location is considered a remote site (one-way commute is more than two hours in duration).
 - A fatigue-risk management tool is utilized.
- Base locations approved for consecutive 24-hour shift scheduling will undergo quarterly monitoring by the Clinical Director and Area Manager to assess for clinician fatigue and ensure they continuously meet the CAMTS criteria.
- An employee may not be extended longer than two consecutive 24-hour shifts without a minimum of 10 hours of uninterrupted rest prior to returning to flight duty.
- Newly hired clinicians will be scheduled for four (4) twelve (12) hour shifts each week during the first three weeks at the base following new hire orientation in Denver. If the clinician's commute from home to the assigned training base is greater than two hours each way, the schedule will consist instead of two (2) 24-hour shifts per week. Any changes to the set schedule should be approved by the Regional Clinical Educator or Clinical Education Manager. A clinician may be assigned to a base for training that is different from the base where they are normally assigned and were hired; in some situations, the training location/base may require overnight travel. All travel expenses will be paid for in accordance with the Air Methods travel and training policies.

15.2.1.1 Staffing and Scheduling Requirements

- All clinicians are assigned to a permanent base except float clinicians who are assigned to a region.
- Base assignments and/or shift assignments may be altered to fill open positions to keep aircraft in service.
- It is not acceptable for a clinician to refuse a base assignment or mode of transport.

- To avoid complacency and foster teamwork, clinicians' schedules may be adjusted as needed.
- All schedule changes/switches require prior approval by the clinician's Area Manager.
- Full and part-time clinicians are responsible for working holidays that fall on their permanent rotation. A vacation request is not permitted for a scheduled holiday shift. Clinicians may trade a holiday shift with their Area Manager's approval.
- If the base staffing falls below the standard four (4) nurse/four (4) paramedic configuration due to an extended absence (such as open position or leave of absence) the schedule at the affected base may be altered. A minimum of a two-week notice will be given if a schedule rotation will be altered, either temporarily or permanently.
- Schedules are created for regular clinicians up to a year in advance in UKG Dimensions. The purpose of creating schedules far in advance is to support the balance of clinicians' vacations and cover the business. Once schedules are posted for the year, clinicians can enter their vacation into UKG Dimensions for the entire year.
- Although schedules are created far in advance, they may be adjusted due to staffing changes at the home base. These staffing changes, i.e., resignations and hiring of new staff, may create a new schedule pattern that will change advanced schedules. If these schedule pattern changes occur, clinicians will be notified of these changes well in advance of the actual schedule. Regardless of schedule pattern changes, approved vacation will be honored.
- It is recommended that careful consideration be taken when scheduling part-time clinicians during the same flight shift.
- It is recommended new clinicians (less than one year) are not scheduled together on the same flight shift.

15.2.1.2 Shift Call Off

- At Air Methods, teammates are expected to arrive on time and maintain regular attendance. Punctuality and consistency are essential to operational success.
- Teammates will receive points for the attendance issues identified below. All attendance violations are considered under one category in terms of progressive discipline. Area Managers and Human Resources will track attendance policy violations. Calculations of attendance infractions are as follows:

TABLE 15.1 Shift Call Off

Call off but teammate obtained coverage for shift that does not exceed 48 hours of scheduled time within the work week for the covering teammate.	0 points
Call off but teammate obtained coverage for shift which will exceed 48 hours of scheduled time within the work week for the covering teammate.	0.5 point
Call off with no coverage obtained	1 point
Holiday Call off with no coverage obtained	2 points
No Call No Show	3 points

- Teammates may request that an absence be excused by contacting their Area Manager. Such requests will be considered and reviewed on a case-by-case basis by the Area Manager and the Human Resources team. This includes, without limitation, requests for bereavement leave. Documentation may be required by the Area Manager and/or Human Resources.
- Absences shall be excused, and teammates shall not receive points, to the extent that an absence involves jury duty, civic duty leave, military leave, workers' compensation injury or leave, leave under any federal or state medical leave law, leave under any state or local paid sick leave law, or any absence that is considered protected leave under applicable federal, state, and/or local law.**
- Once a teammate has exhausted all protected leave available to them under federal, state, and local law, points may be assigned as set forth above.
- Teammates are required to report an absence by calling the Area Manager. Teammates must report each scheduled day they are absent.
- If a teammate is placed on the schedule for a shift or training event by volunteering or through the scheduling process, it will be treated as a regular shift and the guidance within this policy applies.

- Written communication will be provided to the teammate for each occurrence that accumulates point(s). Attendance issues will result in progressive disciplinary action up to and including termination based on the following point system, calculated on a rolling 12-month basis:

TABLE 15.2 Absentee Warnings

Written Warning	3 points
Final Written	4 points
Termination	5 points

- If a teammate is a No Call No Show for three consecutive scheduled shifts, it will be considered job abandonment or voluntary termination without notice.
- Teammates may have one attendance point deducted by volunteering and working a twenty-four hour shift or greater which qualifies for a short notice shift incentive (shifts that become vacated by sick call, injury, LOA, or termination within 72 hours of the beginning of the shift vacated). This may occur once on a rolling 12-month basis.

15.2.1.3 Short Notice Shift Incentive

- **Definition**
 - Short notice shifts are defined as shifts that become vacated by sick call, injury, LOA, or termination within 72 hours of the beginning of the shift vacated.
- **Purpose**
 - Provide additional incentive to clinicians who agree to work a shift that has been vacated within 72 hours meeting the definition of a short notice shift.
- **Guidance**
 - When a shift has been vacated within 72 hours of the start time, meeting the definition of a short notice shift, a clinician who agrees to work 12 or more hours of the shift will be provided a short notice shift incentive. A short notice shift incentive of \$500 will be provided for each short notice shift covered by a clinician for a minimum of 24 hours coverage. A clinician who covers a short notice shift for less than 24 hours but greater than 12 hours will receive a \$250 incentive. Coverage of open shifts that have been vacated greater than 72 hours and do not meet the definition of a short notice shift will not be eligible for the short notice incentive regardless of when the clinician agreed to cover the shift.

- If awarded a shift that qualifies as a short notice shift the request for the short notice incentive can be made by completing a Short Notice Shift Incentive Request form after the shift has been completed. The Area Manager will be required to provide details verifying the short notice criteria and approve the incentive. Once the short notice incentive is approved or denied you will receive an automated email back. If a clinician has received attendance points due to a short term call off within the rolling 12-month period, they will receive a deduction of one attendance point. This may occur one time during the rolling 12-month period.
- **Enforcement**
 - Any falsification of information entered into the short shift incentive form may result in disciplinary actions, up to and including termination of employment.

15.2.1.4 Short Week Schedule Guidelines

- A scheduled short week is defined as a week that does not have 48 hours of scheduled shifts or a combination of scheduled shifts and PTO hours that combine to be 48 hours.
- If a regular clinician has a short week, they should volunteer for open shifts local in nature to their assigned base. Local in nature is defined as an open shift at a base within 90 miles of the clinician's home base. This can be done as far into the future as you prefer to resolve the short weeks in your schedule. Crew Scheduling will frequently monitor UKG Dimensions for these sign-ups and approve the shifts.
- Crew Scheduling or an Area Manager may schedule you for an open shift within 72 hours if you do not sign up for open shifts during a short week and your home base, local Jump Crew, or a local base has an open shift.
- Clinicians are expected to accept short-week assignments that are local in nature and do not conflict with your existing schedule. If that assignment occurs, Crew Scheduling and the Area Manager will communicate the details of that assigned shift to the clinician.
- Crew Scheduling and your Area Manager will attempt to make the assignments as far in the future as practical. Exceptions to these assignments must be approved by the Area Manager. A clinician may guarantee the time off in their short week by requesting vacation.

15.2.1.5 Shift Sign Up Guidelines

- Crew Scheduling will be monitoring and approving shift sign-ups in UKG Dimensions. Overtime sign-ups will start to be approved after Float schedules are finalized. If a sign-up is for a shift beyond the local region and is not initiated by a Float, Crew Scheduling will work with the Area Manager to approve sign-up.
- If a clinician signs up for a partial shift that will create a 36-hour schedule, Crew Scheduling will not approve the sign-up. Shift sign-ups beyond a 24-hour period for non-48-hour bases will be approved by the Area Manager.

15.2.1.6 Training Schedule Guidelines

- Clinicians are required to attend two trainings a year: Cadaver Lab and HPS Lab. Crew Scheduling receives information from Clinical Education to determine when and where labs will be held. Crew Scheduling will schedule all clinicians to attend these labs.
- Scheduled lab training dates are considered scheduled shifts.
- Any necessary changes to lab schedules should be sent to Crew Scheduling for rescheduling of lab shifts.

15.2.1.7 Vacation Request Guidelines

- Clinicians should enter their vacation requests in UKG Dimensions. Remember to enter the start and end times of the request to align with the operating hours of your home base.
- Clinicians should request vacation in their short week to prevent additional schedules of shifts in that week. Enter a vacation request for that date in UKG Dimensions.
- Vacation requests should only be submitted for the actual days of paid time off. Unavailability can be entered in UKG Dimensions and used to block out time around the vacation.
- If clinicians are requesting vacation less than 60 days in advance, this approval is at Area Manager's discretion. The clinician is typically required to find coverage for their schedule shifts for the vacation request to be approved. Send a notification of coverage to crewscheduling@airmethods.com and copy the Area Manager and the employee agreeing to cover the shift.
- If a vacation request needs to be canceled or changed, work with your Area Manager.

15.2.1.8 Holiday Schedule Guidelines

- Air Methods recognized holidays as listed in the Teammate Handbook.
- Full and part-time line clinicians and traveler clinicians are responsible for working holidays that fall on their scheduled shifts. A vacation request is not permitted for the day prior to and on a recognized Air Methods holiday. Clinicians may trade a holiday shift with another clinician with their Area Manager's approval.
- If the clinician is able to find coverage for the holiday shift resulting in a Short Week occurrence (see Section 15.2.1.4 Short Week Schedule Guidelines) the clinician should use PTO to ensure they do not have a short week for a holiday week.
- A vacation is not permitted for the travel day to work a holiday shift.
- Per Diem clinicians are required to provide availability to work at least one holiday listed above per year. Availability for a day before or a day after a holiday may also satisfy the holiday requirement at Air Methods discretion.
- Within a calendar year, Float clinicians are required to:
 - Provide availability for a minimum of one of these holidays: New Year's Day, Thanksgiving Day, Day after Thanksgiving, or Christmas Day.
 - Provide availability for a minimum of one of these holidays: President's Day, Memorial Day, Independence Day, or Labor Day.

15.2.1.9 Full-time (FT) Clinician Commitment

- Expected to work scheduled shifts, training, and other duties as assigned resulting in 2,496 hours annually.

15.2.1.10 Part-time (PT) Clinician Commitment

- Expected to work an average of one 24-hour shift per week also utilizing a set clinical shift rotation.
- Part-time clinicians are limited to an average of 29 hours per week (1,508 hours within a calendar year) without prior approval from their Area Manager and a Human Resource Business Partner.
- During orientation part-time clinicians must provide a minimum of 24 hours per week for flight shifts and training requirements.

15.2.1.11 Per Diem Clinician Commitment

- Expected to work two 24-hour shifts within 30 days.
- Per Diem clinicians are limited to an average of 29 hours per week (1,508 hours within a calendar year) without prior approval from their Area Manager and Human Resource Business Partner.

- Per Diem clinicians who are not scheduled for a flight shift in 60 days are required to work a 12-hour ride-along flight shift with a preceptor before returning to the flight line.
 - Re-orientation includes the following:
 - Review patient care guideline changes and company communications.
 - Refresh intubation/airway skills.
 - Review medical equipment and aircraft familiarity.
- Per Diem clinicians are expected to positively facilitate the acquisition of shifts as their situation best permits through one/all of the following three methods:
 - Signing up for open shifts as far in the future as possible.
 - Providing availability in UKG Dimensions.
 - By the first of the month, availability should be provided from the beginning of the next month through to the end of the third month (or as much of your schedule that is known at that time). Example: on July 1st, availability should be provided from August 1st through October 31st.
 - Availability can be adjusted if needed due to other commitments.
 - Availability should include at least one holiday per year. Availability for a day before or a day after a holiday may also satisfy the holiday requirement at AM discretion.
 - Being easily reachable and available (through on-call or another method) to cover last-minute openings.
- If availability is provided three months in advance, Centralized Scheduling may schedule the Per Diem clinicians.
 - Per diem clinicians may be scheduled for shifts at bases within their home base area or 90 miles from their home base (whichever is greater) as needed.
 - Scheduling will not assign Per Diem clinicians to more than three shifts in a month despite availability, without the consent of the Per Diem clinicians.
 - Per Diem clinicians will receive communication from Centralized Scheduling if they are scheduled on the days they are marked as available.
- If Per Diem clinicians sign up for shifts, it is assumed that this is their consent to work the number of shifts signed up for within the month.
 - Shift sign-ups are approved by Centralized Scheduling after Clinical Float schedules are finalized, about 45 days in advance to two weeks in advance.

- During orientation Per Diem clinicians must provide a minimum of two 12-hour periods of availability per week for flight shifts and training requirements. The initial orientation period should be considered but not limited to 6-8 weeks.
- Compliance will be monitored by the Area Manager with non-compliance subject to progressive discipline.

15.2.1.12 Float Clinician Commitment

- Floats should provide as much open availability for the schedule as possible. Inability to be scheduled for two (2) shifts per week because of providing limited availability (less than five (5) days per week), in the absence of requested benefit time, will result in performance management. Float Clinicians are required to provide availability for one (1) Saturday and one (1) Sunday per month, this does not need to be of the same week.
- Float schedules are provided by Centralized Scheduling 14 to 60 days in advance.
- Self-scheduling by floats is not permitted.
- Float clinicians should not schedule their travel home to where it would impede accepting a transport before their shift end time.
- When calling out sick in the Float role you are required to notify the Area Manager of the base you are working at by phone call, then notify your direct supervisor. All short notice call-outs will be required to utilize sick time to equal 48 hours for that pay period week.

15.2.1.13 Travel Force Clinicians

- Clinical teammates assigned to the travel force will be scheduled at their assigned bases utilizing shift patterns allowing for multiple shifts with rest periods at their assigned base locations.
 - Shift Patterns may consist of but are not limited to 24 hours on, 24 hours off for two-week periods followed by two weeks off.
- The base location for a travel force member will be assigned by the centralized scheduling team. Base location is subject to change based on need as determined by the centralized scheduling team.
- The clinical credentialing team will approve any assignment or movement of a clinical travel force member.
- Additional licensure outside of the assigned base should be pursued under the guidance of the centralized scheduling team.

- Travel clinicians will be paid from the time they arrive at their home airport until they arrive at their base housing during their commute. When traveling home, pay will be provided from departure until arrival at the home airport.
- Travel clinicians are responsible for booking their travel.
- Travel will be scheduled to allow for required rest time before their shift.

15.2.1.14 Manager, Director, or Educator Clinical Flight Shift Work-over Guidelines

15.2.1.14.1 Eligibility:

- A current salaried manager, educator, or director employed at Air Methods is eligible to work as a flight nurse or paramedic for work-over pay providing all requirements listed below have been met:
 - Must have flown as a clinician with an Air Methods CBS program within the past year or have worked in a clinical setting meeting Air Methods requirements for hire (ICU critical care/Emergency Nursing for RN; busy 911 service or Critical Care Transport for paramedic) for three of the past five years.
 - Must have flown as a clinician with an Air Methods CBS program or have completed the full clinical orientation process to include 3rd rider.
 - Current credentials (licenses, certifications) required for the specified base to be covered.
 - Credentials must be uploaded and validated in Protean by the Credentialing Team prior to being scheduled.
 - Current training and education to include:
 - Annual sim lab.
 - Annual cadaver lab.
 - Quarterly intubations.
 - LINK training (to include aviation/safety modules).
 - Crew member aviation hands-on training.
 - NVG currency.
 - Must be validated by the teammate's supervisor prior to scheduling a flight shift.
 - Must be confirmed by the pilot on duty.
 - Confirmation from an education manager that all training and education are current must be provided before being scheduled.

- Must be current with annual OCC health requirements to serve as a Flight Nurse or Flight Medic.
- Initial eligibility and continued competency will be approved/monitored by the Education Manager and Clinical Director II within the region where the shift is being covered.
- Teammates requesting to complete work-over shifts should complete the Work-over Application form and list the bases where work-over shifts would be scheduled.
 - Use of this form will prompt a review by Education, Clinical Leadership, and the Credentialing Team to ensure all requirements have been met.
 - Follow this link to submit a Work-over application >> [Workover Request](#).
- Once eligibility has been approved for each step, Cubexhelp, emsChartshelp, and Crew Scheduling will be notified to grant applicable access and provide credentials if needed. However, it will be the responsibility of the teammate requesting the work-over to ensure access to Cubex and emsCharts is obtained. This should be completed no less than 24 hours prior to any scheduled shifts. You may follow up with Cubex here >> cubexhelp@airmethods.com and you may follow up with emsCharts here >> emschartshelp@airmethods.com.
- Failure to maintain requirements will result in immediate suspension of ability to fill work-over clinical flight shifts.

15.2.1.14.2 Scheduling and Frequency of Work-overs:

- Teammates who meet eligibility requirements may cover shifts during the week or on weekends and holidays.
- Clinical Directors and Clinical Educators must obtain approval from a Clinical Services VP for any shifts requested Monday - Friday during normal business hours.
- Area Managers and other leaders who meet eligibility requirements must obtain approval from their Regional VP for any shifts requested Monday - Friday during normal business hours.
- Teammates who are approved to cover shifts Monday - Friday will only be paid for the 12-hour night portion of a 24-hour shift (the day portion of a 24-hour shift is not eligible for work-over pay).
- Teammates may not exceed more than three work-over shifts during any month without prior approval from the Clinical Services VP (for Clinical Directors/Clinical Educators) or Regional VP (for Area Managers and other leaders who meet eligibility requirements).

- Teammates must confirm eligibility requirements with Education and Credentialing and coordinate shift coverage with Crew Scheduling.
- Teammates will be required to work a minimum of one 24-hour flight shift (or partial shifts totaling at least 24 hours) every three months - does not include training and meeting requirements.

15.2.1.14.3 Pay for Workover Shifts:

- Teammates shall receive a payment for the shift worked that equates to \$33.33 per hour.
- Specifically:
 - \$800 for 24-hour shift.
 - \$400 for 12-hour shift.
- There will be no pro-rated payment for any shift worked between 12 and 24 hours in length.
 - There will be no pro-rated payment for any shift worked less than 12 hours.
 - There will be no pro-rated payment for late flights, holdovers, or any flight-related work beyond a scheduled shift.
 - No payment will be made for any shift less than 12 hours.
 - Payment remains the same regardless of time of day, location, or date.
 - There is no additional payment associated with holidays worked.
- There is no payment associated with the day half of 24 shifts worked during the week (Monday through Friday).
- There will be no shift incentives paid for work-over shifts.
 - Payment is the same for both nurse and paramedic-qualified managers/directors/educators.
 - There is no payment for any associated training, travel, or other work associated with being prepared to work a flight shift.
 - No aggregation of partial shifts is allowed to create a 12, 16, or 24-hour shift.

- The "Manager/Director/Educator Clinical Shift Workover Payment Request" form must be completed following the flight shift and submitted to the teammate's supervisor and the VP of Clinical Services with oversight of the region where the shift was worked.
 - Jeff Marr - Southeast, North Central, PacWest, North, West.
 - Kelly Miller - Northeast, South Central, Midwest, East, South.
- The VP of Clinical Services will forward to payroll for processing.

15.2.1.14.4 Travel:

- If travel to a base location will require air travel or hotel, the teammate's direct supervisor will need to approve the shift prior to scheduling.
- Travel time will not be permitted to accrue towards work-over shift payment.

15.2.2 Section II: Time-Out

15.2.2.1 Purpose:

Clinicians have the right to initiate a "time-out" and be granted a reasonable rest period if they (or fellow crewmembers) determines they are unfit or unsafe to continue their flight shift. This is available to all clinicians no matter what length of their shift.

15.2.2.2 Procedure:

- The decision to take the aircraft out of service must be proactive. A clinician is responsible for making this decision upon completion of patient transport.
- The clinician initiating a Time-Out will immediately notify their teammates and the communication center specialists to place the base out of service. This action will prevent the acceptance of another transport request while managing a time-out request.
- The clinician initiating a Time-Out will contact the applicable Area Manager or Administrator on Call (AOC) after notifying the communication center or based on regional policy.

- The Area Manager and clinician will review the reason for initiating a time-out request and determine the appropriate course of action if any further follow-up is needed.
 - The Area Manager or AOC will subsequently follow up with the communication center and notify of the duration and/or conditions of the temporary out-of-service status. The Area Manager or Administrator On-Call will notify the communication center when the aircraft is placed back in service following the time-out.
 - The clinician initiating a "time-out" occurrence is required to submit a Time-Out form within Protean.
- The Area Manager will monitor transport volumes and frequency of time-out submission to evaluate current staffing patterns and means to mitigate potential fatigue.

15.2.3 Section III: Fatigue Risk Assessment Tool

15.2.3.1 Purpose:

Outline the procedure to evaluate if a clinician can extend their duty time in excess of 24-hours.

- Clinical Shift Extension Risk Assessment is required to be completed in the following conditions:
 - Bases with consecutive 24-hour shift scheduling.
 - Consecutive 24-hour shift scheduling. In this situation, the Area Manager or Administrator On-Call only needs to be notified if the teammate cannot complete their normally scheduled shift(s).
 - Occasional unforeseen circumstances may arise at bases that are not pre-approved for consecutive 24-hour shift scheduling (e.g., sick call, personal emergency, open shift unable to be filled). In these situations, the on-duty clinician may be asked to extend their shift. Prior to any extension, the Area Manager or AOC must be notified and approve the extension. Pre-scheduling of extensions greater than 24 hours from the shift opening is strictly prohibited.

15.2.3.2 Procedure for 24-Hour Flight Shift Extension:

1. In the instance of an unanticipated open shift, the Area Manager or AOC will communicate with the clinician to determine if they are willing and able to extend into a consecutive shift for up to, but not to exceed an additional 24 hours.
2. If the clinician is able and willing to extend into a consecutive shift, the clinician will complete the Extended Duty Risk Assessment in Protean. They will tally the question's scores to determine the cumulative score. This cumulative Extended Duty Risk Score will determine how many hours they can extend.

3. The Area Manager or AOC will review the extended duty risk assessment with the clinician and discuss the ability to extend past the normal scheduled shift.
4. The clinician will communicate with the Pilot-in-Command (PIC) advising the length of the approved extension.
5. The Area Manager or AOC will update Crew Scheduling by email confirming the duty extension for unexpected shift openings (excludes bases that are pre-approved for consecutive day staffing patterns).

TABLE 15.3 Extended Duty Risk Assessment Matrix

#	Criteria	Low Risk	Medium Risk	High Risk
1	How many hours of work did you engage in 24 hours before the start of your flight shift? (<i>This is the day before the first 24-hour shift</i>).	The clinician was off duty and did not work (including other jobs).	The clinician worked for 12 or less hours.	The clinician worked for more than 12 hours
		1	2	3
2	How many hours of sleep did the clinician achieve prior to the start of the first 24 hours of work?	7 hours or more	6 to 7 hours	Less than 6 hours
		1	2	3
3	How many hours of transport time (Ground and Air) did the clinician complete in the first 24-hours? (<i>Calculated from time departed the base to time returned to the base</i>).	Less than 6 hours	6-12 hours	Greater than 12 hours
		1	2	3
4	How many hours of uninterrupted sleep did the clinician obtain in the previous 24 hours on shift?	7 hours or more	6 to 7 hours	Less than 6 hours
		1	2	3
5	What was the quality of sleep* during the uninterrupted hours from question 4? <i>See the chart below for sleep quality definition.</i>	Good	Average	Poor
		1	2	3

Add the points from the 5 criteria to identify the length of time that a clinician may be extended past 24 hours.

SCORE	# OF HOURS THE EMPLOYEE CAN BE EXTENDED PAST 24-HOURS
13-15	If the score is 13-15, the clinician may not be extended. The clinicians must have a minimum of 10 hours of uninterrupted rest prior to returning to flight duty.
8-12	If the total score is 8-12, the clinician may be extended for 12 hours (if extending beyond 12 hours the clinician must complete a 36-hour risk assessment).
7 or Less	If the total score is 7 or less, the clinician may be extended for a total of 24 hours only.

TABLE 15.4 Definition of Quality of Sleep

* How to define quality of sleep:
• Good - Fell asleep within approximately 10-20 minutes. Slept soundly and did not arouse during the night. • Average - May have aroused out of sleep state occasionally but was able to fall back asleep quickly. • Poor - Difficulty falling asleep/Frequently aroused out of sleep state/Difficulty falling back asleep.

15.2.3.3 Procedure for 36-Hour Flight Shift Extension:

1. Clinicians will continue to follow the Air Methods Medical Operations Manual policy regarding the "Fatigue Risk Assessment Tool" after their 24-hour shift.
2. Clinicians will complete the 24-hour extended duty risk assessment matrix following the current policy and submit their assessment using the Extended Duty Risk Assessment in Protean.
3. If the clinician scores seven or less, they may extend for another 24 hours without the need to complete a second risk assessment.
4. If the clinician scores 13 or higher they are not permitted to extend and must have a minimum of 10 hours of uninterrupted rest before returning to a flight shift.
5. If the clinician scores between 8-12 they are required to complete the 36-hour risk assessment tool (refer to table 1) in Protean at or before the 36-hour of their flight shift.
6. The extending clinician is required to contact the Area Manager to discuss the cumulative risk assessment score and determine if they are fit to remain on duty.
7. Clinicians have the right to "time out" following the Medical Operations Manual policy Section 15.2.2 Section II: Time-Out

TABLE 15.5 Extended Duty Risk Assessment - 36-Hour Reassessment

#	Criteria	Low Risk	Medium Risk	High Risk
1	How many hours of sleep did the clinician achieve in the previous 12 hours?	7 or more hours	5-7 hours	5 or less hours
		1	2	3
2	What was the quality of sleep experienced in question number 1?	Good	Average	Poor
		1	2	3
3	How many hours of transport time (Ground and Air) did the clinician complete in the previous 12 hours? <i>(Calculated from departed to returned to base)</i>	Less than 5 hours	6-7 hours	8 or more hours
		1	2	3
4	What was the previous 24-hour risk assessment score?	7 or less	8-12	
		1	2	

SCORE	# OF HOURS THE EMPLOYEE CAN BE EXTENDED PAST 36 HOURS
9-11	If the score is 9-11, the clinicians may not be extended. The clinician must have a minimum of 10 hours of uninterrupted rest prior to returning to flight duty.
4-8	If the total score is 4-8, the clinician may be extended for the remaining 12 hours.

15.2.3.4 Summary:

- A clinician is not required to extend past their normal scheduled shift if they are not willing and able to do so.
- An extended clinician may cancel the remainder of the shift at any time if they feel safety may be compromised or they do not feel up to completing the extension. This should be communicated before a request for service is received.
- Clinicians who are scheduled for consecutive 24-hour shifts at pre-approved bases are only required to contact the Area Manager or AOC if they have a score of nine or greater that would result in aircraft/base out-of-service time.

- An employee may not be extended longer than 2 consecutive 24-hour shifts without a minimum of 10 hours of uninterrupted rest prior to returning to flight duty.
- All program staffing matrix records will be maintained electronically in Protean for a period of three years.

15.3 Unscheduled Absences

- Definitions:
 - Unscheduled absence is defined as: (1) any absence from work that was not both requested by the employee and approved in advance by the employee's Area Manager or (2) leaving early from a work site without approval. Employee use of an approved leave of absence, activation of a State and/or Federal leave of absence, bereavement leave, Worker's Compensation, or any other pre-approved or scheduled absence shall not constitute an unscheduled absence.
- It is the intention of Air Methods to allow the employee reasonable latitude in unscheduled absences. Understandably, personal and family illnesses, as well as unseen emergencies, occasionally arise. Consecutive absences due to illness or non-work related injury will be considered one incident if these absences are due to one particular reason and occur within seven days.
- Unacceptable unscheduled absence patterns which may result in counseling and disciplinary action include, but are not limited to:
 - Patterns of absences occurring immediately before or after holidays, weekends, or scheduled vacations.
 - Greater than two unscheduled absences on a weekend (Friday through Sunday).
 - Greater than one unscheduled absence with seven days of a designated Air Methods holiday.
 - Unscheduled time off after denial of a request for scheduled time off.
 - Three or more unscheduled absences in a rolling 12-month period.

NOTE These examples are guidelines. The Area Manager, in consultation with Human Resources, will consider each case individually.

- If an on-coming clinical crew member does not show up for their scheduled shift, the off-going clinical crew member notifies the Area Manager immediately. If the off-going clinician agrees to hold over, a risk assessment must be completed.

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16 Clinical Leadership Structure

16.1 Purpose

- To establish the leadership structure within Air Methods.
- To provide the leadership structure to the Physician Medical Directors to ensure transparency.

16.1.1 Chief Nursing Officer (CNO) and Senior Vice President (SVP) of Clinical Services

- Provide leadership and direction to the Clinical Department and represent the clinical department on the Executive Leadership Team.
- Maintains oversight of performance related to clinical quality, clinical training and education, patient safety, clinical financial goals, strategic objectives, and key performance indicators (KPIs).
- Drives the development and implementation of the clinical department's revenue plan, budget, and communications.
- Collaborates with customer experience/service, develops operational goals, drives safety culture, and policy/procedure compliance in accordance with Air Methods strategic direction, mission, vision, and values.

16.1.2 Vice President (VP) of Clinical Services

- Provides leadership and direction to areas within their assigned regions to achieve regional financial goals, strategic objectives, and key performance indicators (KPIs).
- Is responsible for the overall operations of regions assigned, collaborates with SVP in the development and implementation of the region's revenue plan, budget, communications, collaborates with customer experience/service, develops operational goals, drives safety culture, and policy/procedure compliance in accordance with Air Methods strategic direction, mission, vision, and values.
- Leads the execution of strategic initiatives for Air Methods.
- Champions patient safety and quality in operations which are paramount in all Air Methods operations.

16.1.3 Clinical Services

- Clinical directors provide oversight for patient safety and quality of care at each of their assigned bases. Clinical Directors advise the Area Managers, Vice Presidents, and other teammates regarding clinical care, safety, quality, and compliance matters. All medical directors report to Clinical Directors for purposes of submitting activity records and should coordinate their activities and educational presentations for Air Methods clinical teams, and other relevant activities with their Clinical Director.
- Clinical Education Managers and Clinical Educators provide recurrent education for all clinical teammates.
- Director of Patient Safety and manager(s) report to the Safety department, but also work with the clinical leadership team.
- Clinical Base Leads are flight nurses and paramedics who report to Area Managers, but have additional responsibilities related to chart review and oversight of clinical quality and safety at their specific base locations.

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17 Clinical Leveling Policy

17.1 Purpose

To create a path for professional development within Air Methods for all clinicians, through the completion of various objectives, that demonstrate a commitment towards career advancement.

17.2 Policy

Air Methods affords three levels of career opportunity for each front-line clinician role within the organization:

- Flight Nurse I, II, and III
- Flight Paramedic I, II, and III
- Flight Nurse Float I, II and III
- Flight Paramedic Float I, II, and III
- Clinical Base Lead Nurse I, II, and III
- Clinical Base Lead Paramedic I, II, and III

Upon hire, clinicians are placed at Level I regardless of experience, education, skills, licenses, certifications, etc.

Each year, clinicians will be given one opportunity to advance to the next level assuming minimum requirements for the level have been met. Clinicians must be self-motivated to initiate the leveling process.

To be eligible for advancement to the next level, clinicians must not have received disciplinary action (greater than verbal counseling) within the 12 months before the closing date for the Clinical Leveling packet submission. Clinicians are required to meet certain minimum requirements regarding experience, credentials, clinical education, etc. as defined in the "Clinical Leveling Requirements and Objectives" (separate objectives for Clinical Base Lead, Clinical Float, Non-Clinical Base Lead, and Non-Clinical Float Clinicians). Only full-time clinicians are eligible for the highest level; level III.

If minimum requirements are met, the clinician will then need to complete a series of objectives as listed in the "Clinical Leveling Requirements and Objectives." These objectives serve to demonstrate a level of commitment and dedication to the profession based on behavioral criteria. The clinician will complete various objectives and compile a packet of information to support the completion of those objectives.

Clinicians submitting packets for consideration must turn them in by the deadline specified in the instructions provided each year. Packets must be professional in appearance and include pictures, dates, examples, and other details to support the completion of the objective.

Area Managers, Clinical Directors, Clinical Educators, and the Clinical Credentialing Team will collaborate on the review of the packet and determine if all minimum requirements and objectives have been met. Clinicians who complete all minimum requirements and objectives will be advanced to the next level and will receive a leveling incentive in the form of a pay adjustment commensurate with the pay range of the next level.

Area Managers will complete the workflow in UltiPro to change the clinician's title to reflect the next level and pay adjustment commensurate with the pay range of the next level.

Clinicians currently in a Level I category who meet Level III requirements may complete objectives and submit a packet to advance to Level III without first progressing through Level II. The pay adjustment is commensurate with the pay range of the next level.

Clinicians who are no longer employed in full-time status and who do not continue to meet minimum requirements to be a Level III year over year will be returned to a Level II. Any clinician who drops a level will be subjected to a corresponding removal of pay adjustment commensurate with the pay range of the next level. Exceptions may be made for full-time clinicians who were displaced to part-time or per diem status as a result of a base closure. These clinicians will have six months to remain in a Level III status while waiting to transition to a full-time position elsewhere within Air Methods.

Clinicians do not need to submit a packet each year to remain at a Level II or a Level III. Clinicians in a Level III status must continue to meet all minimum requirements. Minimum requirements will be reviewed annually for all Level III clinicians to determine if they remain in compliance.

Clinical Leveling documents can be found on the [FlightDeck 2024 Clinical Leveling Library](#).

18 Clinical Peer Review

18.1 Purpose

The Air Methods Clinical Quality Management plan describes peer review as the first step in the Continuous Quality Improvement (CQI) process. The CQI plan is intended to improve clinical quality, reduce clinical risk, enhance patient safety and ensure clinical compliance. During the peer review, both clinical performance and documentation is evaluated by answering set questions specific to the mission type and skills performed. In addition to providing quality data, the peer review provides an opportunity to have an open dialogue about each case and escalate a transport for additional evaluation by Clinical Leaders and Educators.

18.2 Process

It is the responsibility of each clinician to perform the assigned Peer PCR reviews in Protean. This is a daily shift requirement, to check for and complete open peer reviews.

Peer reviews require a thoughtful and productive comment. The feedback in each peer review should follow these basic principles for providing a constructive evaluation:

- Ensure the tone is respectful and professional.
- Be curious and open-minded, observe, don't attempt to interpret.
- Approach the review as a conversation, ask questions, allow the team to explain why they may do something differently.
- Focus on actions and provide potential solutions.
- Avoid feedback overload.
- Be open to criticism of ourselves and our review.

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19 Clinical Weight Policy

19.1 Scope

This policy applies to all clinicians flying in all Air Methods aircraft types.

19.2 Purpose

Air Methods is focused on supporting a healthy workforce while continuing to provide exceptional patient care within the communities we serve. The health and well-being of our teammates and patients is always our priority. This policy is designed to promote our life-saving care to all patient(s) within every community we serve. This policy is also designed to make certain that Air Methods flight missions meet the safe operating requirements of Air Methods aircraft, including certifying that the maximum payload capacity allowable for Air Methods aircraft will not be exceeded at any time.

19.3 Policy

This policy applies to all currently employed clinicians and applicants for a clinical flight position. The weight of each clinician shall be subject to a maximum weight limit of 225 pounds while wearing an empty flight suit and boots ("Weight Limit"). The Weight Limit is an essential requirement and function of flight crew positions. If a current clinician's weight exceeds the Weight Limit, his or her direct manager, in collaboration with the Clinical COEs and HR Business Partners, will work closely with the clinician to develop a healthy and reasonable plan to meet the Weight Limit within a specific time frame. Formal wellness plans will be developed as outlined below and impacted clinicians will be required to participate in and complete such wellness plans and to demonstrate, over time, sustained weight loss to achieve the Weight Plan. Failure to participate in a wellness plan or meet the expectations by the agreed-upon date may result in further disciplinary action, up to and including termination of employment. Clinical applicants who exceed the Clinical Weight Limit will not move forward in the recruiting or hiring process.

19.4 Procedure

Every Air Methods clinician will be required to participate in weigh-ins at established intervals. Weights are required to be taken at the base via the company-provided scale with the clinician's direct manager or other Air Methods management personnel present. The displayed weight must be given to the Pilot in Command (PIC) to be entered into Complete Flight. Weights are required to be updated monthly or quarterly depending on the clinician's weight, and at other times as requested by the Company. If weigh-ins are not completed during the established weigh-in timeframes for the reasons of vacation time or unforeseen sick time, the clinician is expected to complete the weigh-in on his or her next scheduled shift. If an in-person weigh-in is not feasible based on travel or scheduling conflicts of the clinician's manager, a virtual conference weigh-in at the base is acceptable via Air Methods-approved video conferencing systems. Each clinician will also have an official weight documented at his or her annual Occupational Health physical.

19.4.1 Weigh-In Cadence

- All clinicians must complete weigh-ins every quarter before the following dates each year: February 1st, May 1st, August 1st, and November 1st.
- Clinicians who weigh 215 pounds and above will be required to complete monthly weigh-ins.

If a clinician exceeds the Weight Limit, the following procedures will apply:

- Individual wellness plans will be developed to support and outline the applicable expectations with specific deadlines for the clinician to meet the Company Weight Limit policy. If the clinician believes the deadline to be unreasonable, the clinician should discuss with his or her direct leadership and provide any support (medical or other) for a different deadline. Clinicians have the right and are encouraged to consult a medical professional for advice on the wellness plan so that the plan is designed on an individual basis that allows safe weight loss.

- Consistent weight loss progress is considered 1-2 pounds per week. Sustained weight loss means meeting and maintaining the company Weight Limit guidance for six consecutive months.
 - For clinicians weighing between 226 and 235 pounds, the clinician's direct manager will verbally set weight loss expectations. Consistent and sustained progress is required as defined above. If consistent and sustained weight loss is not met, the clinician will be moved to a formal wellness plan.
 - For clinicians weighing more than 235 pounds, a formal wellness plan will be developed by the clinician's direct manager in collaboration with the clinical COE and HR Business Partner establishing the necessary expectation to reach the company's Weight Limit Policy, with an expected deadline for compliance.
- As long as the clinician is successfully meeting the goals of the wellness plan, the clinician may remain on active flight status. The wellness plan will include monthly weigh-ins with the clinician's direct manager to certify weight loss progress.
- The above process will restart for any clinician who completes their wellness plan, but at a future date comes out of compliance with the company's Weight Limit Policy.
- If a clinician is unable to meet the Weight Limit at the expiration of his or her wellness plan or fails to show consistent progress established in his or her wellness plan for three consecutive months, the clinician will no longer be eligible for a flight crew position and depending on the individual circumstances may be placed on paid or unpaid leave, or may be terminated from employment.

19.4.2 Compliance with Federal, State, and Local Laws

Supervisors, managers, and human resources personnel should keep all weight results confidential, disclosing the results only to those on a strict "need to know" basis. Air Methods will comply with all applicable laws, rules, and regulations of federal, state, and local governments, and other appropriate private and public regulatory agencies.

19.4.3 Disclaimer

The Company reserves the right to change this Policy at any time. This Policy does not limit the Company's right to hire, discipline, or terminate any employee. This Policy does not create a contract of employment with any employee, and the relationship between the Company and any employee is "at-will".

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20 Continuum of Care

20.1 Purpose

To ensure Air Methods Community Based Services (CBS) standard of care is provided to patients from the onset of care, throughout transfer, and upon transfer of care at the most appropriate receiving facility.

20.2 Policy

- Transport will be completed, regardless of mode, once patient contact has been made. The exception to this would be the refusal of transport by a mentally competent patient or after a complete discussion with the sending facility, Medical Control, the receiving facility, and the patient's family in the event of a patient's imminent death (case by case basis). If the patient's clinical condition is outside of the clinical team's scope of care then the Air Methods clinical team must remain with the patient until the appropriate team arrives; Medical Control should be consulted. All assessments of patient condition and determination to transport or not transport a patient must be made after in-person assessment by the Air Methods clinical team. Medical Control should be contacted via the communication center so that the relay of information is recorded. Air Methods clinicians will not relinquish care to a facility or unit unless they can provide an equal or higher level of care.
- Patients who are in a NICU and will be received in a NICU may be declined, if the program does not do NICU transports, without going to the bedside in deferral to a specialty NICU team. Inter-facility patients who will be received in a NICU may also be declined in deferral to a NICU team, if the program does not do NICU transports, without going to the bedside in deferral to a specialty NICU team. If a NICU team is not available for a timely response then the decision to transport should be made after responding to the bedside, assessing the patient, and in consultation with the referring and receiving physicians, Air Methods Medical Director (contacted via AirCom), and the clinical AOC (when available). Scene patients (out-of-hospital referrals) will be transported regardless of age or weight.

- If an unscheduled landing makes it necessary to complete transport by ground, the clinicians will accompany the patient to their destination, with all applicable supplies and equipment. If it is necessary to relinquish care to another flight program, it must be determined that the standard of care will remain the same. If it is necessary to complete the mission by ground, the clinicians will provide the same quality and level of care during the ground transport, following the policy Section 70 Patient Transports Utilizing a Ground (Surface) Ambulance and the applicable patient care guidelines. Any changes in the mode of transport will be documented in the patient care record, and a PSAP report will be filed.
- In determining receiving hospitals for scene responses, the Air Methods clinicians will transport patients to the closest appropriate facility whenever possible. Major trauma will be transported to Level I or Level II trauma centers, stroke patients to hospitals with specialized stroke care, AMI patients to hospitals with staffed cardiac catheterization labs, major burns to a Level I or Level II burn center, HROB patients to a hospital with OB services and a Level III or Level II NICU, etc.
- The Air Methods clinicians will receive a full clinical report from the referring facility or agency and give a full report to the accepting facility.
- The clinicians must ensure a copy of the patient record is provided to the receiving facility.
- For inter-facility response, both clinicians will respond to the patient's bedside with all applicable supplies and equipment including cardiac monitor, portable oxygen, ventilator, airway equipment to include CMAC and BVM, complete critical care medication bag and infusion pump, if on medication drips.
- For scene response, both clinicians will respond to the patient with all applicable supplies and equipment including cardiac monitor, portable oxygen, airway equipment (including: CMAC, BVM and suction), and ALS meds.

NOTE Clinicians are encouraged to respond to scenes with blood products and administration supplies whenever safe and prudent to do so.

- If the clinicians believe a change in the mode of transport is required for any patient, they must consult online Medication Control before making the change, EXCEPT in cases of inclement weather, patient weight restriction, or cardiac arrest in progress.

21 Controlled Substances

21.1 Purpose

Nurses and paramedics assigned to Air Methods Community Based Services (CBS) programs share the responsibility of maintaining the security of narcotics. No rationale exists for breaching the legal and ethical accountabilities related to narcotics handling. Patient safety is imperative and health promotion related to impairment is essential among healthcare providers employed by Air Methods.

21.2 Policy/Procedure

- Definition of a controlled substance: when referenced in this policy, Air Methods will consider a controlled substance to be any substance that is a narcotic, benzodiazepine, medications governed by DEA schedules (I, II, III, IV, V classification) plus Etomidate, Zofran, Benadryl, and Phenergan.
- All controlled substances (those listed above and per Section 812 of the Title 21 United States Code (USC) Controlled Substances Act) will be secured per this policy and accounted for each time a change in clinical crew occurs or every 24 hours, whichever occurs first.

21.3 Medbank (Cubex) General Processes

21.3.1 Medbank (Cubex) General Record-Keeping Requirements

- Controlled drug transactions will be maintained electronically using Medbank (Cubex).
- All transactions pertaining to the acquisition, handling, and disposition of controlled drugs will be maintained in accordance with all regulatory requirements.
- The Medbank (Cubex) transaction log will include all receipt and disposition transactions, and the current inventory level of each drug at all times.
- Specific instructions to perform various functions in the Medbank (Cubex) system can be found in the Medbank (Cubex) Guide.

21.3.2 Back-up Battery Power or Generator Back-up

- All Medbank (Cubex) units will have back-up battery power or generator back-up in the event there is a power loss. There should be no interruption in service to the unit.
- In the event the battery can no longer provide power, please notify the Clinical Director.
- **When a new battery APU is received at the base, it should be labeled with an expiration date of three (3) years. The APU should be replaced with a new one when the current one expires.**
- Medbank (Cubex) machines **should not be routinely plugged into the battery power source.**
- The medication refrigerator **should not be plugged into the backup battery.**
- In the event of power failure at the base, the Medbank (Cubex) can then be plugged into the battery for use. This preserves battery power for emergent Medbank (Cubex) access when needed.

21.3.3 Back-up Keys

- Back-up keys to access the Medbank (Cubex) will be kept in a lock box with a changeable combination lock that only the Clinical Director will have access to.
- In the event of a Medbank (Cubex) access failure, the crew should contact the Clinical Director to gain access to the meds as needed to perform job functions. Back-up keys provide access to the Otterbox only, should it be secured in the Medbank (Cubex) at the time of a power failure.
- Access to all remaining medications within the Medbank (Cubex) remains secure and tamper-evident. If access to additional medications is required, contact your Clinical Director and see the Medbank (Cubex) Guide for instructions.
- During any Medbank (Cubex) malfunction, it may become necessary to document medication transactions on a Controlled Substances Perpetual Inventory Log - 5424. The Clinical Director should immediately be notified if there is a Medbank (Cubex) malfunction to determine if this manual documentation is required. If needed, the Controlled Substances Perpetual Inventory Log - 5424 can be found on FlightDeck> Resources> Company-Wide Forms (Air Methods).

21.3.4 Medbank (Cubex) Maintenance

- If at any time an employee of Medbank (Cubex) must perform on-site maintenance on stocked Medbank (Cubex) machines, Air Methods will ensure that at least one clinical employee will be in attendance to supervise the access and handling of company-owned controlled substances. The Clinical Director must be notified when a Medbank (Cubex) employee will be accessing the system.
- As soon as it is practical following any maintenance where a Medbank (Cubex) employee has gained access to a stocked Medbank (Cubex) machine, a full cycle count will be performed by the on-duty clinical crew and any discrepancies discovered reported to the Clinical Director immediately.
- Discrepancies that are created during Medbank (Cubex) maintenance and/or installation will be resolved via cooperation between the Medbank (Cubex) employee and the Air Methods clinical chaperone.
- Resolution should occur before completion of the maintenance and/or installation. If a discrepancy cannot be resolved, the Clinical Director and the Director of Clinical Compliance must be notified immediately.

21.4 Shift Change at the Medbank (Cubex)

- Shift change at the Medbank (Cubex) with **all** on-coming and off-going clinicians present at the Medbank (Cubex) together must occur anytime there is a change in any clinical team members within a shift; it is NOT acceptable for any clinician to walk away from the Medbank (Cubex) during an active shift change process.
- Shift change at the Medbank (Cubex) will include the following processes:
 - I. The off-going crew must inspect the Otterbox medications for quantity, clarity, and tampering and place it in the Medbank (Cubex) before the on-coming crew takes possession.
 - II. The on-coming crew (nurse and paramedic) will issue the Otterbox, perform an accessed cycle count, and OPEN the Otterbox to visually inspect the Otterbox medications for quantity, intact packaging, and clarity of solution. This cycle count will include all medications accessed during the previous shift and always count the Otterbox medications.
 - III. In cases where a clinical team must extend their shift beyond 24 hours, the accessed cycle count must be completed at the 24-hour mark or as soon as possible.
 - IV. If an oncoming clinical team is not available, the off-going clinical team must complete the accessed cycle count.

- Under certain circumstances, and only with the Clinical Director's approval, a shift change can occur away from the Medbank (Cubex). See the procedure outlined in the section of this policy titled: Section 21.5 Controlled Substance Hand-Off when away from the Medbank (Cubex).
- If a flight comes in close to shift change and the Medbank (Cubex) shift change has not been completed, the off-going crew must take the flight in order NOT to delay the transport.
 - I. If relief for one of the clinicians has arrived early and agrees to take the flight, the shift change process must occur between all clinicians present and there should be no delay in accepting the flight.
 - II. Example: On-coming RN arrives early, flight request comes in, the pilot is checking the weather, on-coming RN is willing to take the flight - Medbank (Cubex) shift change would occur to include the off-going RN, on-coming RN, and off-going medic.
 - III. DO NOT delay acceptance of the flight; once the Medbank (Cubex) shift change process has been completed, the on-coming RN and off-going medic will take the flight.

21.5 Controlled Substance Hand-Off when away from the Medbank (Cubex)

- This procedure is to be followed in situations where the off-going crew is away from the Medbank (Cubex) at the time of shift change and a variation of one of the following examples occurs:
 - I. Example 1: Aircraft made a provisional landing at an FBO for maintenance and the on-coming crew can relieve the off-going crew at the FBO.

- II. Example 2: The off-going crew is at the co-located hospital preparing to package a patient for transport and the on-coming crew can relieve the off-going crew prior to any patient care occurring.

NOTE If a patient transport was completed prior to the hand-off where controlled substances were used on the flight, the crew must contact the Clinical AOC/Clinical Director prior to completing a hand-off of the Otterbox. At no time will the on-coming crew take over care of a patient from the off-going crew during the middle of a patient transfer without approval from the Clinical AOC or Clinical Director.

NOTE Any other scenario, other than the two examples above, that may result in a shift change occurring away from the Medbank (Cubex) requires the approval of the Clinical Director or Clinical AOC.

- Procedure:
 - **Step 1:** Off-going crew will open the Otterbox, and confirm the contents are intact without evidence of damage or tampering. The broken PIN tag will be handed to the on-coming crew.
 - **Step 2:** The on-coming crew will also confirm all medications are present per the standard inventory for the base as represented on the Controlled Substances Perpetual Inventory Log - 5424, a copy of which should be carried in every medication bag or medical crew clipboard. The on-coming crew will retain the broken PIN tag for input into the Medbank (Cubex) upon their return to base. A picture of the contents of the Otterbox must be obtained by the on-coming crew and uploaded into Protean upon return to base using the "Controlled Substance Hand-off" verification form. This form can be found in the Forms section of Protean.

NOTE If a missing or damaged medication is discovered during the hand-off by either the off-going or on-coming crew, the Clinical Director must be notified immediately to determine whether the hand-off can continue.

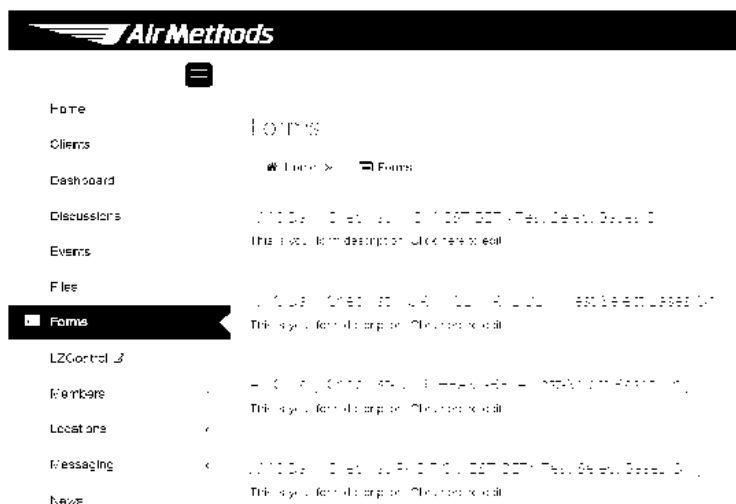
- NOTE** For the purposes of the perpetual inventory log use during an Otterbox hand-off, all Controlled Substances being carried in both Otterboxes (if two Otterboxes are being carried) must be accounted for in the log.

FIGURE 21.1: Controlled Substances Perpetual Inventory Log

[illegible]

- **Step 4:** When the on-coming crew returns to base, they will perform a return transaction of the Otterbox to the Medbank (Cubex) and will enter the PIN tag number that was retained from the hand-off process. The on-coming crew will then issue the Otterbox and perform the required cycle count as per the standard shift change process.
- The on-coming crew will then complete the “Controlled Substances Hand-off” form in Protean and upload the picture of the Otterbox contents as well as the completed “Controlled Substances Perpetual Inventory Log - 5424.” The form is accessed in Protean by going to the “Forms” tab and selecting the Controlled Substances Hand-off form. Select “Open” and complete all form entry fields, upload the Otterbox picture and the Perpetual log, and then click “Submit.” A copy of the Forms Tab and the form itself are below for reference.

FIGURE 21.2: Protean Forms Access



21.6 Medbank (Cubex) Cycle Count

- A full Medbank (Cubex) cycle count will be completed at the beginning of every month by the Clinical Lead and Clinical Director or can be delegated to the on-duty Clinical Team.
- A cycle count of accessed items from the previous shift is required to be completed at each shift change by the on-coming nurse and paramedic.
- A cycle count of accessed items should include any restocked medications from the previous shift.
- The steps to complete the various types of cycle counts can be found in the Medbank (Cubex) Guide.

21.7 Controlled Substances Administration/Waste

- Administration and waste dosage of controlled substances used on a patient flight will be verified by two Air Methods medical personnel. Wasting controlled substances is to take place at the receiving hospital and must be witnessed by both clinical flight team members. Unused controlled substances should **NOT** be taken back to the base and wasted there.
- Empty vials are to be disposed of in appropriate sharps containers or designated medication disposal receptacles at the receiving hospital.
- Documentation in the ePCR will include the date, time, medication name, amount of waste, and names of both clinicians witnessing the waste. Clinicians must take care to assure administration dosing in Medbank (Cubex) matches the total doses documented in the ePCR.
- Wasted medications are automatically calculated in Medbank (Cubex).
- The steps to replace used medications and document waste in Medbank (Cubex) can be found in the Medbank (Cubex) Guide.

21.8 Medbank (Cubex) Discrepancies

- If at any time the Medbank (Cubex) alerts that a discrepancy has occurred, a pop-up message will occur which indicates "Count Different. Are You Sure"? The user will be given the opportunity to re-count and must immediately notify the Clinical Director via email.

- All cycle counts and count-backs are “blind” meaning the machine will not alert the user as to the number of medications that are logged in its system.
- Clinical Directors are responsible for resolving all Medbank (Cubex) discrepancies within 48 hours of occurrence. If a discrepancy is not able to be resolved the Clinical Director will notify the Clinical Compliance Director.

21.9 Otterbox

21.9.1 Otterbox Container

- A Controlled Medications hard-sided box (Otterbox) will be stocked in accordance with established par levels.
- Each base will have the option to stock/carry two Otterboxes - one to be carried by each clinician on duty.
- The Clinical Director will make the final determination on which clinician(s) shall carry the Otterbox(s) and what configuration of medications will be carried in each Otterbox based on allowable EMS regulations, base mission profile, and average length of transport. Medications carried will not exceed maximum quantities as outlined in the standard aircraft checklist without permission from the Clinical Director and Director of Clinical Compliance.
- The Otterbox used by each base will be the standard container specifically designed for use by Air Methods CBS bases.
- The controlled substances stored in the Otterbox will be secured in modular foam inserts specifically designed for the particular vial, ampule, etc., and specific to the concentration of the medication. To reduce the possibility of medications rolling/falling out of the container upon opening and to reduce the risk of breakage, all medications will be secured in a foam insert. It is not acceptable to stock medications in the Otterbox unprotected.
- Only controlled substances will be stored in the Otterbox. All non-controlled medications (including Etomidate, Zofran, Benadryl, and Phenergan) should be stored in the aircraft's main medication container.

21.9.2 Otterbox Storage

- The aircraft supply of controlled substances will either be secured on the flight clinician's person and stored in the approved Air Methods Otterbox container with a numbered/tagged lock **OR** always secured in the Medbank (Cubex).

- **It is NOT acceptable to store the Otterbox on a shelf, in a helmet, or anywhere other than the Medbank (Cubex) or clinician's flight suit while waiting for a flight request.**
- In cases where only one clinical crew member is on shift and must take the Otterbox for a specialty team transport, consult with the Clinical Director for guidance.
- When a flight suit is removed and not in the continuous presence of a clinical flight team member, the Otterbox must be stored in the Medbank (Cubex) (such as during exercise, washing the aircraft, etc.).

21.10 Controlled Substance Ordering Process

- The following process will be followed when there is a need to order controlled substances.
 - I. All orders for controlled substances shall be placed through Coupa via the Air Methods approved vendor.
 - II. Controlled substances orders will be placed separately from any other medication or supply item.
 - III. The Clinical Base Lead or designee will place the order for controlled substances.
 - IV. If utilizing the DEA's Controlled Substance Ordering System (CSOS), the Clinical Director shall approve the order. CSOS is the preferred method for controlled substance ordering. Each Clinical Director should ensure that each CBS base has a primary CSOS signer and an alternate signer. Only Clinical Directors may fill the role of CSOS signer.
 - V. If there is a circumstance that necessitates the use of a DEA 222 form, the Clinical Director shall obtain the Medical Director's signature and forward the completed form to the controlled substance vendor, via FedEx, for order fulfillment. These forms must be kept locked on Air Methods owned or leased property secured in the Medbank (Cubex). At no time will 222 forms be kept at any employee's private residence.
 - VI. DEA licenses must be stored in the Medbank (Cubex) unless State or Local regulations mandate otherwise.

- VII. A narcotic management file, which should contain all records of controlled substance orders, copies of completed 222 forms (where applicable), and all associated records related to controlled substances should be maintained for a period of two (2) years. Controlled Substance files will be secured in a locked file container and the key will be secured in the Medbank (Cubex). Files must be accessible and readily available as needed for audit by the DEA.
- VIII. A secondary digital file will be maintained as a backup.

21.11 Tracking/Receiving of Controlled Substances

21.11.1 Tracking Controlled Substances

- All controlled substances are delivered from the supplier via FedEx on Tuesdays through Fridays of each week.
- On Tuesday through Friday mornings, the Duty Crew, Clinical Base Lead, and Clinical Director will receive an email providing notification of all controlled substances due to be delivered that day. The notification will include the medication (name and quantity) being delivered that day along with the FedEx tracking number.
- The duty crew will click the link in the email or go to the FedEx website, enter the tracking number under "Manage Deliveries", and request an email or text notification when the package is delivered.
- If the package is delivered off-site, the duty crew should receive a notification from FedEx and will retrieve the package as soon as possible.
- Clinical Directors will also enter the tracking number following the same process to be aware of when the package has arrived to ensure it has been received and stocked.

21.11.2 Receiving Controlled Substances

- All controlled substances will be shipped by the supplier in a manner that requires a signature at time of delivery.
- All McKesson shipments that arrive at the base from the supplier requiring a signature must be signed for by a credentialed clinician.
- If a credentialed clinician is not available, arrangements need to be made for an alternate delivery time and/or date.

- Verification/tracking will occur to ensure medications have been placed into the Medbank (Cubex) **immediately upon receipt** - all shipments of controlled substances will be tracked and audited from shipment to delivery, to placement in the Medbank (Cubex).
- While it is the responsibility of the flight nurse to “receive” the controlled substance delivery in Coupa so the order can be closed in the Coupa system, **it is the responsibility of both clinicians on duty to ensure proper accountability and storage of the controlled substances.**
- When controlled substance orders arrive, the package will be inspected and opened by both clinicians on duty. Both clinicians are responsible for locating the packing slip and will verify the count ordered is correct. The clinicians will inspect the package and medications for evidence of any tampering or damage during shipping.
- The packing slip must be signed and dated by the duty crew receiving the package and note any discrepancies.
- Controlled substances will be placed into the Medbank (Cubex) without delay. It is NOT acceptable to store the controlled substances outside of the Medbank (Cubex) to wait for the Clinical Base Lead or someone else to stock the delivery.
- If the clinicians receive a flight request before the controlled substances can be appropriately stocked in the Medbank (Cubex), the clinicians will follow the process to store the controlled substance in the approved Safe located at the base. The process for storing controlled substances in the Safe can be found in the Medbank (Cubex) Guide.
- **ALL** controlled substances must be logged into the Medbank (Cubex) for tracking purposes. If there is a problem with the order, the controlled substances must be logged in the Medbank (Cubex) and the Clinical Director must be contacted for guidance.
- Internal auditing will be conducted to ensure all controlled substance orders are received and secured.
- In cases where only a single flight clinician receives a controlled substance delivery at the base, the Clinical Director should be notified immediately to assist with the process of stocking the delivery in Medbank (Cubex).
- Clinicians will return to the email containing the shipment notification, complete the confirmation process, and upload a scanned copy of the packing slip.
- The original packing slip will be added to the controlled substance management file as part of the permanent record.

21.12 Medbank (Cubex) Storage/Security

21.12.1 Storage/Access

- Medbank (Cubex) should be used for the storage of controlled substances.
- In addition to controlled substances, Diphenhydramine, Etomidate, Ketorolac, Ondansetron, and Phenergan will be stored in the Medbank (Cubex).
- There are five (5) scenarios in which controlled substances may be stored outside the Medbank (Cubex), in an approved Air Methods safe with a key lock, or an approved Air Methods refrigerator with a Q-lock. In all scenarios, the Medbank (Cubex), safe, and refrigerator used to store controlled substances must be in view of the security camera system (once that system has been installed) and behind a door that locks.
 1. If a Medbank (Cubex) is unavailable for any reason, controlled substances must be stored in an approved Air Methods safe with a key lock.
 2. Diphenhydramine and Ondansetron may be stored in the approved Air Methods safe with a key lock if there are bin/storage limitations in the Medbank (Cubex). These medications will be stored in the approved Air Methods lock box within the safe. Clinical Director approval is required to move Diphenhydramine and Ondansetron storage to the safe.
 3. Controlled substances that must be refrigerated will be secured inside the approved Air Methods lock box within the Air Methods-approved Q-locked refrigerator.
 4. Controlled substances were delivered to the base but were unable to be stocked immediately into the Medbank (Cubex) bins due to a flight request.
 5. Controlled substances that have been removed for disposition to MedFlats, per the appropriate process, and pending FedEx pick-up.
- The step-by-step instructions for how to perform various processes such as moving, storing, accessing, and accounting for controlled substances stored outside of the Medbank (Cubex) can be found in the [Medbank \(Cubex\) Guide](#).
- **It is NOT acceptable to store controlled substances outside of the Medbank (Cubex) environment for any other reason without the permission of the Clinical Director and the Director of Clinical Compliance.**
- Access to controlled substances will be limited to the on-duty clinical flight team.
- In some instances the storage and security of controlled substances is regulated by state and regional EMS protocols and those protocols will be followed.

21.12.2 Exceptions

- If there are states with differing requirements for the storage/security of controlled substances, the Clinical Director must establish a local or statewide policy exception.

21.12.3 Security at the Base

- When there is concern about the physical security of controlled substances at a base location, these medications will be stored at a more secure location such as a hospital or central office as permitted by state law.

21.12.4 Security at the Medbank (Cubex)

- For security reasons, clinicians should **NEVER** walk away from the Medbank (Cubex) without selecting "Exit" on the menu screen. The inactivity time-out will be set to 120 seconds for each machine.
- **Both the clinician who initiates a transaction at the Medbank (Cubex) and the clinician witnessing the transaction must remain present at the Medbank (Cubex) for the duration of the transaction - it is NOT acceptable for either clinician to walk away from the Medbank (Cubex) during an active transaction, to include Full Cycle Counts.**

21.12.5 Transport of Controlled Substances

- If it is necessary to transport a controlled substance from one location to another, outside of a patient's transport, local DEA guidelines and/or state pharmacy board guidelines for transport must be determined and followed.
- The Clinical Director will be responsible for contacting the DEA and/or the state pharmacy board when applicable to determine allowable transport means for the area and the intended purpose of the transport. The Clinical Director will obtain confirmation in writing from the DEA and/or the state pharmacy board regarding approved practices for the transport of controlled substances.
- Without direct permission from the Clinical Director, it is not permissible for Air Methods employees to place controlled substances in a personal vehicle for transport or other purposes. If approved by the Clinical Director, there must always be a minimum of two (2) clinicians, wearing Air Methods issued flight suits, present during the transport.

21.12.6 Security at the Otterbox

- All Otterbox containers will be secured with a Security Tag Lock in the appropriate designated location on the Otterbox.
- Tag locks must be changed at every shift change, regardless of whether the Otterbox was accessed on the previous shift.
- Additionally, tag locks must be replaced every time the Otterbox is opened to access controlled substances for patient administration.
- Tag lock PINs MUST be entered in the PIN field in the Medbank (Cubex) each time the tag lock is replaced. Medbank (Cubex) will prompt the clinicians to enter the tag lock number when placing the Otterbox into the Medbank (Cubex) and when removing the Otterbox from the Medbank (Cubex).

21.13 Audits

21.13.1 General Audit Information

- Purchasing and distribution records will be reconciled monthly by the Clinical Base Lead and randomly by the Clinical Director.
- Par levels will be managed with the least amount of controlled drugs kept on-site to operate effectively.
- Clinical Base Leads, Clinical Directors, and the Clinical Compliance Director will perform ongoing audits of narcotics management including security, documentation, and compliance to established processes.
- Documentation of such audits will be maintained by the program for a minimum of three years.

21.13.2 Monthly Controlled Substance Audits Performed by the Clinical Base Lead

- The Clinical Base Lead will perform QA audits monthly using the Smartsheet platform.
- Audits will be performed on 100% of all controlled substances administered during the month.
- Audits will be completed within 40 days of the controlled substance transaction.
- If the base is located in a state with a Controlled Substance and/or Pharmacy License requirement, the base will also follow the audit guidelines imposed by the Controlled Substance and/or Pharmacy License.

- To audit the medication, the CBL should receive an automated C14 - Transactions by Patient by Med report from Medbank (Cubex) to cover the month that is being reviewed.
- Any discrepancy identified between the amount administered/wasted in the patient chart and the amount administered/wasted in the Medbank (Cubex) will trigger a review by the Clinical Director. Clinical Directors will investigate all discrepancies and provide follow-up/resolution utilizing the process in Smartsheet.
- Monthly audits will include the following medications: Fentanyl, Morphine, Dilaudid, Ketamine, Propofol, Versed, Valium, Ativan, Etomidate, Benadryl, Zofran inj., Zofran ODT, and Phenergan.
- Examples of other Medbank (Cubex) transactions to be audited monthly by the Clinical Base Lead are listed below. Any discrepancy or inappropriate transaction will require an explanation in writing to be provided to the Clinical Director by the 5th of the month following the month in which the transaction occurred.

“B” Transactions

- This is an Issue transaction indicating a medication has been removed from Medbank (Cubex). This transaction may apply to a patient administration, Wasted Entire Vial on a Flight, Reverse Distribution, or Expired Inventory.
- “B” transactions for controlled medications found under Duty Crew require immediate notification to the Clinical Director.
- Controlled substances that have been Issued “B” to a patient or Wasted Entire Vial on a Flight should be checked for consistency by reviewing the ePCR.
- Controlled substances that have been Issued to Reverse Distribution should be reconciled with the retained reverse DEA form 222 or MedFlats return form.
- Controlled substances that have been Issued to Expired Inventory should be compared to a corresponding Restock “S”: transaction for Expired controlled substances in the same amount and type of medication.

“R” transactions

- This is a Return transaction indicating a medication has been returned to Medbank (Cubex) from the same bin it was issued from.
- This will only happen under rare circumstances with controlled substances and Expired controlled substances and the Clinical Director should be aware.

“S” transactions

- This is a Restock transaction indicating new medications were added to the Medbank (Cubex) either from a supply order, adding medication that has expired to the expired bin, or restocking medications received from another base.
- Restock “S” transactions for medications received from the supply vendor should be reconciled to the packing slip for those medications to ensure the number ordered and received matches what was restocked into Medbank (Cubex).
- Examples of transactions that will **not** be included in the audit, with explanation:
 - **“P” transactions**
 - This is a cycle count transaction indicating a crew has initiated either a full or accessed cycle count of the affected bin. This may result in a discrepancy, but discrepancies are addressed outside the purview of this audit.
 - **“DI” transactions**
 - This is a discrepancy indicating that a count back or cycle count number did not match the number of medications in the Medbank (Cubex) database. Discrepancies are addressed separately and not included in this audit.
 - **“SK” transactions**
 - This is a skip transaction indicating a transaction was initiated to access this medication, but the transaction was skipped by the crew. The medication bin will not open on skip “SK” transactions so no auditing of these transactions is necessary.
 - **“DS” Transactions**
 - This is a destock transaction indicating an administrator has de-assigned a medication bin and may have removed medications from that bin. Destock “DS” transactions are monitored by the Clinical Director.

21.13.3 Monthly MedFlats Controlled Substance Audits Performed by the Clinical Directors

- The Clinical Director will perform monthly QA audits on all controlled substances returned via Reverse Distribution using the MedFlats platform.
- The Clinical Director will receive a notification from MedFlats following receipt of controlled substances at their location.
- The Clinical Director will compare the controlled substances received by MedFlats against the Medbank (Cubex) transactions issued to “Reverse Distributions.”

21.13.4 Quarterly Clinical Director Audits

- Each quarter the Clinical Director will perform a random audit and provide results to the Director of Clinical Compliance. Random audits can include reviewing expiring medication processes, monthly cycle count compliance, usage and waste trends, etc. Effective quarterly controlled substance audits should vary from base to base and quarter to quarter.
- Clinical Directors are authorized to inspect all inventory, records, security, and procedures for controlled drugs.

21.14 DEA

21.14.1 DEA Registration

- All Air Methods CBS bases using controlled substances must be registered with the DEA and any other applicable state agencies (DOH, BNDD, etc.).
- For flight programs with multiple bases, a separate DEA and state registration is required for each base of operation.
- Exceptions are in cases where the base is located at a hospital and the hospital dispenses the controlled substances.
- It is the responsibility of the Clinical Director to renew all DEA licenses and to be knowledgeable of all state and federal mandates and reporting requirements.
- DEA licenses will be stored in the Medbank (Cubex) unless local EMS regulatory or pharmacy requirements require otherwise.

21.15 Expired Drugs - Reverse Distribution - MedFlats

21.15.1 Expired Pharmaceuticals

- All unopened, expired pharmaceuticals will be disposed of through the appropriate vendor.
- **Non-controlled medications** will be disposed of using the “MedPro” Pharmaceutical return process.
- **Controlled Substances** are to be disposed of through the “MedFlats” medical return/disposal service unless otherwise required in accordance with specific state requirements. The Clinical Base Lead will be responsible for the disposal process. MedFlats shipping supplies will be ordered through McKesson and should maintain a stock of two boxes available at the base.

21.15.2 MedFlats Accounts for Reverse Distribution

- The Clinical Compliance Director will be responsible for the creation of a MedFlats account for each separate base of operations and will provide access to the respective Clinical Director and Clinical Base Lead.
- Each Clinical Director will maintain a MedFlats account to review/receive medication shipment/delivery notifications for their bases.
- Medications that are checked out of the Medbank (Cubex) to "Reverse Distribution" should be reconciled with the "Shipments Received" notification provided by MedFlats as part of the monthly reverse distribution audit completed by the Clinical Director.
- The Clinical Base Lead will be responsible for the actual reverse distribution process to return unused controlled substances.

21.15.3 Frequency of Disposal

- Expired medications should be held in the Medbank (Cubex) for no longer than 90 days.
- MedFlats materials and reverse 222 DEA forms should be ordered to allow for the shipment of expired substances on the dates below:
 - January 5, April 5, July 5, and October 5 (the 5th of the month beginning each quarter, annually).
 - The Clinical Director will be notified if these shipment dates cannot be met for any reason.

21.15.4 Pharmaceutical Returns/Disposal using MedFlats

- Visit www.MedFlats.com and watch the video tutorial on how to use MedFlats properly.
- Do not ship sharps, medical waste, or damaged/open medications to MedFlats.
- Do not overfill the box.
- Do not ship Schedule CIII-V or other pharmaceuticals with Schedule II shipments. Schedule II Controlled Substances must be returned separately from any other pharmaceuticals. (See below Special Handling of Schedule II controlled substances (C2)).
- Shipments to MedFlats are returned in MedFlats shipping boxes that are available for procurement through McKesson Medical.

21.15.4.1 Assemble the MedFlats Shipping Box

1. Assemble the bottom of the box by folding the two bottom flaps in then folding the larger flap over the two smaller flaps.
2. Pull off the paper covering from the adhesive strip and firmly press and seal the tab with the exposed adhesive to the outside of the box.
3. Once assembled, line the box with the enclosed plastic bag.

21.15.4.2 Creating Inventory and Filling Your Return

1. Register your flat by logging into your MedFlats account (www.MedFlats.com) and click the Create New Inventory button in the center of the screen to enter your MedFlats ID and tracking number. DO NOT utilize the return shipment label provided with your MedFlats box. All returns should be shipped utilizing Air Methods' FedEx account and entering the FedEx tracking number (see instructions below). This is completed through Okta under Pitney-Bowes.
2. During the flat registration process, you will be asked if this is a "Standard" or "C2" return. Standard registration includes Schedule III-V or other RX medication tracked in the Medbank (Cubex). DO NOT ship Schedule III-V, RX, or OTC drugs with your Schedule II flat or combine your Schedule II flat with another flat.



This is your MedFlat ID.
It is always placed on the upper right corner of your MedFlats box lid.

3. After registering your flat, create an inventory by entering the NDC of return items into the online inventory system. Place all items listed inside of the lined MedFlats box. Print a copy of the inventory and FedEx shipping label (which will show the package tracking number) and file it with your Clinical Director via email.
4. If return/disposal is a CS III-V (e.g., Versed, Valium, Ativan, Ketamine), items will be issued to REVERSE DISTRIBUTION in the Medbank (Cubex). This must be completed by two Air Methods medical personnel.

21.15.4.3 Sealing and Shipping Your Flat

1. Seal the remaining flap by pressing firmly on the adhesive strip to ensure a strong seal. DO NOT utilize the return shipment label provided with your MedFlats box. All returns should be shipped utilizing Air Methods Pitney-Bowes account selecting FedEx Ground. If FedEx Ground is not available, FedEx Air should be selected.



vs.



2. When creating a shipment on Pitney-Bowes, the following services are required:
 - a. Select "Direct Signature Required" from "Special Services"
 - b. Enter the email addresses of the individual preparing the shipment and the respective Clinical Director and select the following notification types:
 - i. Ship
 - ii. Tendered
 - iii. Delivered

21.15.5 Special Handling of Schedule II Controlled Substances (C2)

- What is a C2 (Schedule II Controlled Substance)? Schedule C2 controlled substances are defined by the Drug Enforcement Administration (DEA) as substances that have a high potential for abuse which may lead to severe psychological or physical dependence. Some examples of C2 drugs include narcotics like Morphine, Fentanyl, and Hydromorphone.
- The DEA monitors Schedule II drugs very closely and requires the use of "DEA Form 222" before any controlled substances are shipped to MedFlats for return or disposal. The DEA has strict laws that forbid the undocumented shipping of Schedule CII controlled substances.
 1. Register your flat by logging into your MedFlats account (www.MedFlats.com) and entering your MedFlats ID and tracking number. DO NOT utilize the return shipment label provided with your MedFlats box. All returns should be shipped via FedEx utilizing the Air Methods Pitney-Bowes account and utilizing that tracking number.

2. During the flat registration process, you will be asked if this is a “Standard” or “C2” return. Select “C2” from the drop-down. Please note that once a flat is registered as a C2 it cannot be re-registered for use as a Standard return/disposal. Only Schedule CII controlled substances can be included in a C2 flat.
3. Create an inventory by entering the NDC of returned items into the online inventory system. Print a copy of this inventory list and email it to your Clinical Director as notification of the pending return/disposal of controlled substances.
4. At this time, you will submit your inventory, but you CANNOT ship your flat to MedFlats for processing until you receive the C2 Kit, and corresponding DEA Form 222, from MedFlats.
 - i. The items to be returned/disposed of must be returned to the base double-locked cabinet or Medbank (Cubex) machine and included in the base inventory of controlled substances noted in the controlled substances perpetual inventory log until shipment occurs. Once your inventory is submitted, MedFlats will send you a C2 Kit that includes the DEA Form 222.
 - ii. Please note that once a flat is registered as a C2 it cannot be re-registered for use as a Standard return/disposal.
5. Once you receive your C2 Kit, complete DEA Form 222.

IMPORTANT Depending on how many products were entered into inventory, you may have more than one DEA Form 222. The DEA Form 222 will expire 60 days from the date issued. You will need to complete the form(s) as directed below.

- a. Fill out the following fields in the “Part 4: To Be Filled in By Supplies” portion of the form:
 - i. “Number Shipped”: The number of products for the line item.
 - ii. “Date Shipped”: The Date you ship your MedFlats.
- b. Remove all items to be returned/disposed of which are listed on the inventory and corresponding DEA 222 Form from the Medbank (Cubex) by issuing to “Reverse Distribution.” This process must be completed by two Air Methods medical personnel. (The items included must match the quantity, item description, and NDC that is listed on the 222 form. Additional items cannot be added once the 222 form is issued and must be returned in a separate shipment).
- c. Pack the MedFlats Box carefully to avoid damage to medications in transit.

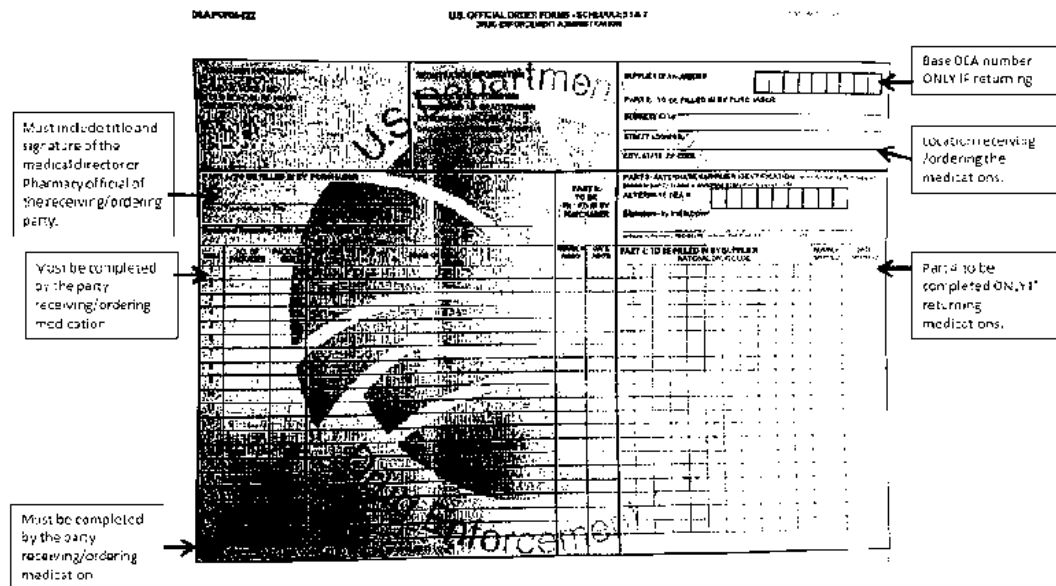
- d. Create a shipment in Pitney-Bowes utilizing FedEx Ground or FedEx Air, schedule a pick-up, and print the label.
- e. Log into the MedFlats portal and click open the inventory with the status indicating "Awaiting Form 222."
- f. Enter your DEA Form number found in the middle portion of the form under the heading Order Form Number and click submit.
- g. You will be prompted to validate your MedFlats ID number. A UPS shipping label will automatically generate in a new tab. **DO NOT** utilize the UPS return shipment label. All returns should be shipped utilizing Air Methods Pitney-Bowes account selecting FedEx Ground. If FedEx Ground is not available, select the standard FedEx option. Enter the FedEx tracking number in the MedFlats tracking field and click submit.
- h. Include a photocopy of the DEA Form 222 and the Inventory Report inside the MedFlats box with the medications being returned.
- i. Seal the box and secure it for scheduled pick-up.
- j. **The Original DEA 222 form** must be retained with the base controlled substance records with a copy of the Inventory Report or items being returned.
- k. **A copy of the DEA Form 222 and Inventory Report** of items being returned must be included with the return.
- l. **A scanned copy of DEA Form 222 and Inventory Report** of items being returned must be emailed to the Clinical Director.
 - i. **The Clinical Director** must email a **scanned copy of DEA Form 222 and Inventory Report of items** being returned to DEA email address DEA.orderforms@usdoj.gov and upload a copy of the documentation to the Smartsheet.
 - ii. The Clinical Director will confirm medications removed from Medbank (Cubex) under "Reverse Distribution" transactions match the quantity and type of medications sent to MedFlats.
 - iii. The Clinical Director will forward this to the responsible Medical Director.

21.15.6 Completing DEA Form 222 for Reverse Distribution

- Instructions: DEA Form 222 will be issued by MedFlats and sent to the base via UPS.
 - Part 1 - This will be previously completed by MedFlats.
 - Part 2 - This will be previously completed by MedFlats.
 - Part 3 - This section is not pertinent and is to be left blank.
 - Part 4 - Complete this section by adding count in "Number Shipped" and "Date Shipped."

FIGURE 21.4: Single Page DEA Form 222 Instructions

DEA 222 Updated version released 2020



21.15.7 Issuing Medications for Reverse Distribution Shipment

Once medications are ready to be shipped, follow the procedure below. Perform an ISSUE TRANSACTION for the expired medications.

- Obtain an inventory report from the Clinical Base Lead or Clinical Director. Using this list, determine the number of medications needed to return to MedFlats.
- Login to the Medbank (Cubex).

- Select "Reverse Distribution" from the patient list.
- Select Issue/Return.
- Select Issue Items.
- Type to search for items you wish to ship via MedFlats and with the items selected, click Next.
- Enter the quantity (found in your inventory report) of the selected expired medication and select Next.
- Repeat the previous two (2) steps until you have a list of all expired meds to issue.
- Select Issue Items Now.
- Have a second clinician scan their fingerprint to serve as a witness - **THIS CLINICIAN MUST REMAIN PRESENT AT THE Medbank (Cubex) THROUGHOUT THE TRANSACTION.**
- Input the quantity of medications that are in the compartment once opened and select Next.
- Verify the total being issued is accurate and select Next.
- Repeat until all expired items have been issued and then select Exit.
- Place all meds in the MedFlats container and follow the process for Storing Packaged Controlled Substances before Shipment/Restocking.

21.16 Storage of Packaged Controlled Substances Prior to Shipment to MedFlats to Stocking in the Medbank (Cubex)

21.16.1 Purpose

- To allow for the storage of packaged controlled substances on arrival to the base when it's not feasible to stock in the Medbank (Cubex) (example: when a flight request comes in before the controlled substances can be stocked.).
- To allow for storage of controlled substances that have been packaged to ship to Reverse Distribution while awaiting FedEx pick-up.

21.16.2 Procedure

- A Standard keyed external medication safe or locking keyed cabinet will be made available at each base.
- The only key(s) available for this will be stored within the Medbank (Cubex) as "Key External Medications."

- When a package containing controlled substances is received at the base and cannot be immediately stocked for any reason such as Medbank (Cubex) availability or flight request, or a package containing controlled substances is prepared for MedFlats return and awaiting shipment, the package will be stored in the locked safe or cabinet with the key issued from the Medbank (Cubex) and returned to the Medbank (Cubex)
- This package should be stocked as "Pending Shipment" in the Medbank (Cubex) and each package accounted for as one each.
- Packages referenced in this policy should be sealed and remain sealed upon placement within the safe/cabinet.
- If an issue arises where the package is open and will need to be stored within the safe/cabinet referenced, the Clinical Director must be notified prior to approval.
- **At no point will the key(s) to the external medication safe/cabinet be stored on a person or any location outside of the Medbank (Cubex).**

21.17 Loss Reporting

21.17.1 Loss Reporting

- Any theft, loss, tampering, or clarity of solution will be immediately reported to the Clinical Director by phone.
- Subsequent notification will be made to the Clinical Compliance Director, or the Senior Vice President of Clinical Services as soon as known.
- Clinicians should not leave the base without authorization from the Clinical Director and the Clinical Compliance Director.
- Clinicians will need to provide separate written statements detailing the circumstances of the loss, tampering, etc. Pictures should be included as needed.
- All reportable agencies including DEA, pharmacy suppliers, and law enforcement agencies will be notified as directed by the Clinical Director in conjunction with the Clinical Compliance Director and the Legal Department.
- If warranted, Corporate HR will be notified after a discussion between the Clinical Compliance Director and the Senior VP of Clinical Services for follow-up and drug testing as indicated.

21.17.2 Breakage/Missing Caps

- Any defects including breakage and missing plastic caps will be immediately reported by phone to the Clinical Director.
- All reports of breakage or missing caps should be reported through Protean using the Controlled Substance Damage form found in the forms section.
- Pictures should be uploaded to the Controlled Substance Damage form whenever possible.
- The Clinical Director and Clinical Compliance Director will be provided with a copy of the completed form through a Protean email notification.
- If warranted, Corporate HR will be notified after discussion between the Clinical Compliance Director and the Senior VP of Clinical Services for follow-up and drug testing if indicated.

21.18 Controlled Substance Accountability when a Medbank (Cubex) is Unavailable

21.18.1 Purpose

It may occasionally be necessary to account for and secure controlled substances outside of Medbank (Cubex). Medbank (Cubex) failure, new base start-ups, or an overabundance of on-hand controlled substances are some examples of when it might be necessary to store medications in key lockable safes and track medication using paper-controlled substance perpetual inventory logs. The Regional Clinical Director (RCD) will be responsible for assuring all the necessary components listed below are on hand at the base if a Medbank (Cubex) will be unavailable. The RCD will also assure the Clinical Base Lead (CBL) and all clinical teammates assigned to the base are familiar with this policy and procedure and how to properly use the perpetual inventory log when a functioning Medbank (Cubex) is unavailable.

NOTE This procedure is not an all-inclusive policy addressing the accountability and documentation requirements for controlled substances. This policy only outlines the procedural changes needed to function without a Medbank (Cubex) on site.

21.18.2 General Record-Keeping Requirements

- Controlled drug transactions will be recorded in black or blue ink on Air Methods paper perpetual inventory logs.
- All transactions pertaining to the acquisition, handling, and disposition of controlled drugs will be maintained in accordance with all regulatory requirements.
- Documentation logs are to be signed at a minimum of each shift change and whenever there is a change in medical crew within a shift. All crew members, including off-going and on-coming nurses and paramedics, present at shift change are required to attend the narcotic count. The on-coming nurse will count and visually inspect each vial for intact packaging and quantity/clarity of solution. All four crew members must sign verifying the integrity and correctness of the narcotic count.
- A perpetual inventory record will be maintained which documents each receipt and disposition transaction for each drug. Three separate logs are required to be maintained.
 - I. Usable “in-date” medications
 - II. Expired medications
 - III. Otterbox “aircraft supply” medications

21.18.3 Storage and Security

- Controlled drugs must be stored in a securely double-locked, substantially constructed cabinet that is primarily attached to the base location.
- Controlled drugs inside the cabinet will also be in a separate box sealed with a keyed lock.
- Refrigerated controlled substances will be secured inside a keyed locked refrigerator.
- In addition, the controlled substances inside the refrigerator will be in a separate box sealed with a keyed lock.
- The nurse and paramedic on duty will each be responsible for carrying a key to one of the designated locks. At no one time should any one clinician control both sets of keys that could grant individual access to the controlled substance inventory.
- Access to controlled drugs will be limited to the on-duty medical crew.
- For circumstances when the keys must be secured due to no clinical staff at the base, the keys will be secured in two lockboxes (one each for the paramedic and nurse) with a code that can be changed. For instances when securing keys in the lockboxes is necessary, complete the below steps.

- I. Complete a full controlled substance count and record findings in the perpetual inventory logs.
- II. Assure all medications are secure including the Otterbox. The Otterbox should be red tag sealed and secured in the medication safe. Make a note of the seal number.
- III. The nurse and paramedic will each secure their keys in their respective lockbox.
- IV. Change the combination on the lockbox and communicate to the Clinical Director the new combination and the number on the serialized tag lock applied to the Otterbox.
- V. The Clinical Director will communicate individually to the on-coming crew the new combination lock and the number on the Otterbox tag lock.
- VI. When the on-coming crew accesses the Otterbox, send to the Clinical Director the number found on the Otterbox seal and report any discrepancies found during the controlled substance count.
- VII. Any discrepancies found must be reconciled before the staff from the off-going shift leave.

21.18.4 Equipment Needed for Controlled Substance Security and Tracking when Medbank (Cubex) is Unavailable

- A substantially constructed key lockable safe that can be secured to the base location. This safe needs to be large enough to hold at least two (2) key lockable boxes inside.
- Three (3) key lockable "Cash Box" style boxes large enough to hold the base supply of usable (in-date) medication, expired medication, and refrigerated controlled substances and substantial enough to reveal if tampering has occurred.
- Three (3) separate perpetual inventory logs, one each for usable (in-date) medication, expired medications, and Otterbox medications.
- Two (2) wall-mountable key lockboxes with changeable combination locks. Lock hasp and key padlock to secure the medication fridge.

21.18.5 Key Allocation

- The nurse key ring will include:
 - I. Exterior lock key for the main medication safe.
 - II. Exterior lock key for the medical fridge.
- The paramedic key ring will include:
 - I. Interior lock box keys
- At no time will any one person have control of both key rings.
- Keys will be kept in sight of their respective owner at all times unless secured in their respective key lock box.
- Spare keys will be in the sole control of the Clinical Director.

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22 Crew Briefing, Shift Change, and Post Flight Debriefing

22.1 Purpose

To outline pertinent information that will be exchanged collaboratively between all clinical flight team members as well as any passengers pre-flight. To establish a process to facilitate ongoing communications post-mission between aviation, communications, and clinical flight team members to improve all aspects of operations. Debriefings will be conducted as soon as practical after each flight and consist of all involved parties including the communications specialist.

22.2 Policy

22.2.1 Clinician Shift Change Briefing Guidelines

- The clinician shift change briefing will occur immediately following a shift change or when there is a clinician change during a scheduled flight shift.
- All transitioning clinicians are required to participate in the shift change process.
- At a minimum, the briefing will include the following information.
 - Aircraft status
 - Medical equipment status and battery rotation
 - Medical supplies status or needs
 - Pertinent information impacting shift and/or flight operations.
 - Open items for follow-up
 - Controlled medication count in accordance with the Controlled Substance Policy

22.2.2 Flight Shift Brief Guidelines

- Clinicians will brief the pilot in command at the beginning of each shift, after completing the clinician brief.
- The flight brief will include the duty clinicians, pilot in command, AirCom communication specialist, and when applicable ride-along personnel and specialty transport clinicians.

- The flight briefing will include information outlined in the General Operations Manual Policy Passenger and Medical Crewmember Briefing. Depending on regional differences pre-approved items may be added to the briefing content.
- During the flight briefing, clinicians will verify with the pilot in command and AirCom communication specialists their current professional licensure meets the licensure requirements for the specific base location. Based on the clinician's professional licensure AirCom communication specialists, pilots in command, and clinicians will verify transport limitations. Clinicians are required to contact the Clinical Director if there is a question or concern about meeting the base professional licensure requirements.

22.2.3 Post-Flight Debrief (PFD) Guidelines

- The purpose of the post-flight debrief process is intended to discuss concern(s) identified during all phases of flight operations.
- Standard items addressed in the post-flight debrief include but are not limited to the following items.
 - Safety aspects, such as pre-flight inspection/walk-around, securing equipment, patient loading/security, landing zone selection/security, Crew Resource Management, weather, etc.
 - Communications – internal and external including radio/telephone communications with ground agencies, hospitals, and the communications center.
 - Decision-making and judgment calls.
 - Overview of flight from the customer's/patient's perspective, including how the crew's performance would be viewed by the sending/receiving agencies or facilities, as well as the patient and family members.
 - Significant issues involving the communications center will be reviewed in a conference call involving all crew members, otherwise a designated crew member will call the communications center to provide any necessary feedback, as well as receive any input from the communications specialist. This will be followed up with a notation on the Post Flight Debrief.
- If a concern occurs during flight operations a verbal debriefing should take place with all crew members as soon as practical and before accepting another transport request.

- Post-debrief clinicians are required to submit a Post Flight Debrief summary within Protean (the PFD process within Protean is progressing through a developmental stage). All affected crewmembers should review the debrief comments prior to submission.
- Clinicians have the option to submit any concerns utilizing the Air Methods AIDMOR reporting process.
- Regional and when applicable corporate leadership will review all post-flight debrief submissions and provide feedback for loop closure.

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23 Customer Satisfaction

23.1 Purpose

To establish a guideline for evaluation of overall customer service.

23.2 Policy

All Air Methods CBS programs will survey their requesting/receiving agencies utilizing the RSQ911 electronic surveys to evaluate the service provided. Patient satisfaction surveys will be distributed by Patient Business Services (PBS).

- It is an expectation that flight team members will distribute the RSQ911 survey to our customers. This includes pre-hospital, interfacility, and receiving facility staff.
- Flight team members will enter the survey code into emsCharts to link the survey with the flight.
- Management will receive alerts when survey results are not within the parameters. This notification includes the appropriate communications center.
- Upon receipt, survey results will be shared with program management, discussed at staff meetings, and as applicable, with the communications center, Medical Directors, and team members involved with the transport.

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24 Duty Readiness

24.1 Purpose

To ensure that clinicians report for work fit to perform the duties and responsibilities as defined by their job descriptions safely and efficiently.

24.2 Policy

- Clinicians will report to work physically and mentally prepared and capable of performing their duties.
- As defined by CAMTS Standards, clinicians require at least 10 hours of rest with no work interruptions prior to any scheduled shift of twelve hours or more.
- Hearing protection will be worn at all times when operating in or around the aircraft or other excessively loud equipment. Disposable earplugs and helmets are provided for every employee assigned to flight operations. Properly sized helmets will be worn, and properly secured, with visors down while in flight or around running/hot rotor wing aircraft. Annual audiograms will be completed to monitor for hearing loss.
- It is recommended that clinicians evaluate themselves and each other for signs of mental or physical impairment that might jeopardize operational safety. If mental or physical impairment is noted by any teammate or his or her co-worker, the affected individual should be asked to excuse himself or herself from duty immediately and the base will be placed out of service pending the arrival of a replacement teammate. The Area Manager or AOC will be notified immediately.
- No medication or substance, prescription, or over the counter, that impairs judgment or performance will be consumed within eight hours of the start of the shift or for the duration of action of the medication or substance, nor will they be consumed while on duty. Reasonable cause drug testing may be requested if the Area Manager or AOC suspects a teammate of being under the influence of a substance that impairs judgment or performance.
- To avoid barosinusitis, baro-otitis media, and possible rupture of the tympanic membranes, flight team members should not fly with a head cold, upper respiratory infection, or congested/blocked sinus passages.

- Clinicians will be free of illness or injuries that would prohibit them from carrying out their duties as defined by their job descriptions and potentially harming themselves or others. The employee will undergo an evaluation by the contracted occupational health provider to assess their fitness for duty if a medical condition arises that could potentially impact their ability to meet the job requirements.
- Teammates should inform Human Resources when they learn they are pregnant. The flight team member is responsible for informing their physician of the activity required to fulfill their position duties and providing the physician with a copy of their job description. A copy of the job description signed by the physician and a written statement from the physician indicating the flight team member may continue to perform flight duties will be required following each prenatal visit and submitted to Human Resources. If the physician determines the employee is not able to perform job requirements, non-flying assignments may be considered in consultation with Human Resources. A medical release signed by the attending physician will be required when returning to duty post-partum.

25 Disaster Response Policy

See the [FEMA Disaster Response Manual](#) on FlightDeck > Resources > Corp Pubs > Compliance Policies and Procedures.

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26 Dispatch Procedures

See the Communications Policies and Procedures Manual, Dispatch Procedures section.
This manual is on FlightDeck > Resources > Corp Pubs > Communications Publications.

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27 Disposal of Medical Waste

The exposure control plan in 8.2 of the SMS Manual guides discipline and roles regarding bio-hazards in the air medical environment. All teammates are responsible for the content found in this document.

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28 Dress Code and Helmets

28.1 Purpose

Air Methods Community Based Services (CBS) programs and clinicians will reflect a positive and professional image to the community while on duty.

28.2 Policy

- Upon hire, all clinicians will be issued two uniforms specific to their job description. It is optional for the clinician to purchase additional uniforms at their own expense.
- All clinicians will report to work with two uniforms that are clean and in good condition. A new uniform will be provided annually, as necessary. Worn uniforms will be turned into the Area Manager or his/her designee.
- Clinical uniforms should not be worn off duty or out in the community to prevent wear and tear and cross-contamination. Uniforms are appropriate for outreach and public relations events while working for Air Methods.
- An appropriate jacket will be provided to all crewmembers. Each clinician is responsible for proper care and treatment of the jacket. Jacket replacement will occur on a case-by-case basis with the Area Manager's authorization. All company-issued equipment must be returned at the time of separation.
- With prior approval of their respective Area Manager, boots will be purchased by each clinician and reimbursed up to \$100 maximum annually. Acceptable foot attire consists of black boots, over the ankle, with hard toes and soles. Boots should be polished and in good condition.
- Flight clinicians working 24-hour shifts are expected to be in their flight suit from the shift start until 20:00, seven days/week. Flight clinicians may change out of their uniform while in the crew rest (sleep) quarters. They must, however, be in uniform when exiting their crew rest (sleep) quarters, except for performing duties around the base (washing aircraft, cleaning hangar, etc.). The Area Manager can make exceptions to the uniform requirement worn while in quarters based on environmental circumstances.
- Clinicians may only apply Air Methods-approved program patches on their flight suit and/or helmet.

- The alternate dress while on duty after business hours must be in good repair and professional in appearance. No exceptions or alternatives will be allowed.
 - Uniforms must always be zipped to the level of the nametag.
 - Turtlenecks may be worn under the flight suit. Acceptable solid colors must coordinate with the flight suit. The turtleneck must be cotton, clean, and in good repair. Avoid polyester or other man-made fabrics.
 - Tee shirts may be worn under the flight suit. Acceptable solid colors must coordinate with the flight suit or flight program colors. The tee shirt must be cotton, clean, and in good condition. No visible logos are allowed, other than corporate or flight program logos.
 - Company-issued photo name badges are part of the uniform and should always be worn.
 - A government-issued identification such as a driver's license shall be carried by each clinician, and professional licensure/certification in states that requires it.
 - Ball caps are not to be worn around turning rotors, nor is it acceptable to wear into a referring or receiving facility unless it is specifically a program-logo cap. Stocking caps may be worn around the aircraft but are not acceptable to wear into a referring or receiving facility.
 - The use of technical/tactical rescue vests (load bearing vests or MOLLE vests), which are worn over company issued flight suits, is **prohibited**.
- Hair will be kept clean and groomed. Hair will not extend below the collar of the flight suit. If hair extends beyond the flight suit collar, it will be worn up or pulled back as to allow no hair to move beyond the crewmember's shoulders for safe operation around moving helicopter rotors. Hair color and style must be conservative; no bright or unprofessional colors are allowed.
- Facial hair should be neatly trimmed, professional in appearance and must not interfere with the N95 or respirator testing or fit.
- Jewelry: No more than one necklace will be worn at the program and must always be kept inside the undershirt. No more than one set of matching earrings will be worn. They will not extend greater than 1/8 of an inch below the earlobe. No facial or body piercing will be visible while on duty except for pierced ears. No more than two rings will be worn while on duty at the program. One bracelet and one watch may be worn.
- Fingernails will not extend more than 1/4" beyond the tip of any finger. Neat, professional-appearing fingernail polish may be worn. Artificial nails or artificial nail tips are not permitted. Nail jewelry of any sort is not permitted.

- No visible tattoos are permitted. Tattoos must be covered by Company-approved uniform or clothing.
- When attending Company meetings, training, or events staff must wear business casual clothing, which may include neat jeans or conservative shorts. Unacceptable for either gender: rumpled or ripped clothing, miniskirts, tank tops, underwear as outerwear, and inappropriate or revealing attire such as bare midriffs, short shorts, and flip-flops.
- An appropriate helmet will be provided to all clinicians. Helmets are considered a part of the uniform and must be worn at all times during rotor-wing operations.
- When air medical transport is converted to ground, clinicians should consider carrying helmets in the event rotor wing transport conversion may occur.
- The clinician is responsible for proper care and treatment of their helmet. The clinician is responsible for a lost or stolen helmet and will replace the helmet at the clinician's expense.
- **Helmet Wear:** All personnel directly responsible for staffing the helicopter is responsible for and wear a program-approved helmet as designated below:
 - All flight personnel must wear an Air Methods-issued helmet at all times during helicopter transports.
 - Each crewmember is responsible for having his/her helmet during flight shifts. A crewmember may be sent home if their helmet is not available.
 - Each crewmember is responsible for the normal caring, cleaning, and the reporting of problems with their helmet. The wearer will report any equipment malfunction to the lead pilot or mechanic.
 - The user may be responsible for damage to the helmet arising from misuse or carelessness.
- **Helmet Care and Cleaning:**
 - **Helmet Shell and Visor Assembly** - Wipe with a clean, soft cloth dampened with a mild soap solution; rinse with clear water and air dry. **NEVER USE ABRASIVES OR BLEACH-BASED CLEANERS ON HELMETS AND ASSEMBLIES.**
 - **Ear cups, cords, and Retention Assembly** – Wipe with a damp cloth; and allow to air dry thoroughly.
 - **TPL Cloth Cover** – Machine wash (gentle cycle) or hand wash with warm water and soap; and air dry.
 - **TPL Plastic Layers** – Hand wash with soap and water; and air dry.
 - **Helmet liners** should be machine-washed and air-dried.

- **Helmet Inspections**
 - Inspections consist of pre-flight and post-flight inspections by the user.
 - Preflight Inspection: Prior to each flight the user should inspect the helmet assembly to see that it is in good working order. This inspection should ensure that:
 - The helmet, liners, and ear cup assemblies have been properly fitted.
 - The chin strap and nape strap are properly adjusted and the retention assembly is attached to the helmet with screws that are tightened securely.
 - The visors operate properly and are clean and free of scratches and cracks.
 - All communication components operate properly.
 - The overall condition of the helmet has been checked for serviceability.
 - The user should note any component malfunction or damage to the helmet resulting from operational use and report to the program manager after each flight.
 - Periodic or scheduled inspections should occur annually or as required. Clinical flight team members are responsible for ensuring that their helmets have been inspected and cleaned and that the defective components have been replaced as necessary.
 - Qualified individuals are responsible for all helmet maintenance (except for daily care and cleaning).

WARNING Improperly worn helmets are a significant safety hazard. Crewmembers who improperly wear their helmets will be disciplined according to Air Methods' disciplinary policy.

- No unapproved personalization of helmets is allowed.
- Helmets must be returned to the clinical lead or lead pilot upon resignation or termination of the employee.

29 *ECMO (extracorporeal membrane oxygenation) Specialty Team, Transport Team, Composition, and Competency*

29.1 Purpose

To define the composition, qualifications, initial training, and competency required for Air Methods clinical teammates to provide direct patient care for patients receiving ECMO therapy during transport.

29.2 Policy

Air Methods base locations that are selected to assist in the transport of patients receiving ECMO therapy will be required to complete the following prior to the acceptance of any mission requiring Air Methods clinical teammates to provide direct patient care for a patient receiving ECMO therapy.

29.3 Initial Training

- Hands-on didactic training with Air Methods ECMO specialist to include:
 - Airframe loading/securing.
 - Familiarization with specific equipment.
 - Team dynamics with specialty ECMO team member(s) who will accompany Air Methods team.
 - Communication.
 - Playbook Development.

29.4 Logistical Needs

- The Playbook is developed and available for review. Items that must be included:
 - Configuration Pictures to include aircraft seating chart.
 - Loading/Unloading Procedures.
 - Securing Procedures.
 - EMI testing.
 - Additional Equipment needed with weight.

- Non-Air Methods team weights.
- Contact names/numbers for Air Methods specialist and ECMO program resources.

29.5 Team Composition

- The Air Methods clinical team will consist of:
 - A minimum of one Air Methods clinical teammate trained and competent by the Air Methods education team for the specific ECMO program transports.
 - A second Air Methods clinical teammate who has completed online ECMO familiarization training and reviewed the local ECMO program playbook.

29.6 Special Consideration

If one of the teammates has not completed the hands-on competency training by Air Methods Education for the specific ECMO program and only the online education portion; it is required that the team contact the Air Methods on-call ECMO specialist and Clinical Director for mission planning prior to acceptance.

ECMO AOC Number: 303.414.7616.

29.7 Annual Training

Annual Competency and new clinician training for Air Methods base locations selected to assist in the transport of patients receiving ECMO therapy.

- Hands-on didactic training with an Air Methods ECMO specialist will include:
 - Airframe loading/securing.
 - Familiarization with specific equipment.
 - Team dynamics with specialty ECMO team member(s) who will accompany the Air Methods team.
 - Communication.
 - Playbook Review.
 - Online ECMO familiarization training.
- New clinical teammates assigned to an Air Methods base location(s) selected to assist in the transport of patients receiving ECMO therapy will complete online ECMO familiarization training before being released to the 2nd seat and operate only as a secondary teammate on ECMO transports until full Air Methods ECMO competency is verified by the Air Methods clinical education team.

29.8 Approval Process

Before the base can start EMCO missions, approval is required by the following leadership:

- Clinical Director 2
- Clinical Education Manager
- Air Methods Clinical ECMO Specialist
- Regional Aviation Director
- Regional Maintenance Director
- Area Manager
- AirCom
- Local ECMO partner Hospital Leadership

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30 EMTALA

30.1 Purpose

To ensure the compliance of Air Methods Community Based Services (CBS) programs with Federal EMTALA (Emergency Medical Treatment and Labor Act) and its corresponding regulations to the extent such laws and regulations apply to Air Methods.

30.2 Policy

Air Methods complies fully and at all times with the Federal EMTALA regulations involving the treatment and transfer of critically ill or injured patients from one facility to another appropriate receiving facility, to the extent that such compliance is required by and applicable to Air Methods.

Documentation of such compliance will be noted by communication procedures, which verifies acceptance of patients at appropriate receiving facilities; also by a signed Authorization and Consent and by documentation of Medical Necessity by the referring physician on the Physician Certification Statement PCS-01, (these forms can be obtained by contacting PBSForms@airmethods.com) and by obtaining copies of COBRA and medical necessity forms from referring facilities.

30.3 Transfer Terms

According to Federal EMTALA regulations 489.24 regarding the responsibilities of Medicare hospitals in emergency cases: "If an individual at a hospital has an emergency medical condition that has not been stabilized, the hospital may not transfer the individual unless:

- The transfer is an appropriate transfer.
- The individual (or legally responsible person acting on the individual's behalf) requests the transfer, after being informed of the hospital's obligations and the risk of transfer. The request must be in writing indicating the reasons for the request and indicating that he or she is aware of the risks and benefits of transfer.

- A physician has signed a certification that based upon information available at the time of transfer; the medical benefits reasonably expected from the provision of appropriate medical treatment at another facility outweigh the increased risks to the individual or in the case of a woman in labor, to the woman and unborn child, from being transferred. The certification must contain a summary of the risks and benefits upon which it is based; or if the physician is not physically present in the emergency department at the time an individual is transferred, a qualified medical person has signed a certification after a physician in consultation with the qualified medical person agrees with the certification and subsequently countersigns the certification. The certification must contain a summary of the risks and benefits upon which it is based.
- Transfer to another medical facility will be appropriate only in those cases in which the transferring hospital provides medical treatment within its capacity that minimizes the risks to the individual's health and the receiving facility has available space and qualified personnel for the treatment of the individual and has agreed to accept the individual and to provide appropriate medical treatment.
- The transferring hospital sends to the receiving facility all medical records or copies thereof related to the emergency condition which the individual has presented that are available at the time of transfer, including available history, records related to the individual's medical condition, observations of signs and symptoms, preliminary diagnosis, results of diagnostic studies or telephone reports of the studies, treatment provided, results of any tests and the informed written consent or certification provided by the referring physician.
- The transfer is affected through qualified personnel and transportation equipment, as required, including the use of necessary and medically appropriate life support measures during the transfer. Air Methods CBS transports provides bedside to bedside service to ensure continuity of care.
- Federal guidance has previously stated that helicopters and ambulances entering hospital grounds for the sole purpose of conveying a patient from or to another hospital DO NOT trigger EMTALA obligations for the hospital owning the helipad pad UNLESS:
 - The patient's condition deteriorates while at the helipad and the medical personnel accompanying the individual requests a medical screening exam or stabilizing treatment.
 - EMS personnel, the patient or an individual acting on the patient's behalf requests a screening and treatment for an emergency medical condition.

31 *Environmental Considerations for Patient Transport*

31.1 Purpose

To define considerations that can protect crews, patients, and patient care equipment from environmental extremes.

31.2 Policy

31.2.1 General Safety

- The personal safety of our crews and patients is the primary consideration when evaluating and mitigating hazards related to temperature extremes and environmental risk factors. The pilot will evaluate aircraft limitations specific to temperature with the Pre-Flight Risk Analysis (PFRA) process and its assessment of “Additional Risk Factors,” and weather conditions must be evaluated for their impact on flight operations.
- During each shift brief, teammates are responsible for discussing how weather conditions will impact flight operations. This discussion should include potential mitigation strategies when applicable.

31.2.2 Preflight Preparation

- Flight crews must bring weather-appropriate attire and personal protective equipment (PPE) on shift and the aircraft (e.g., coat, hat, gloves, thermal wear, personal survival kit, hand warmers, etc.).
- Consider carrying additional equipment as appropriate (e.g., extra thermal mattresses, ventilator circuit, hot/cold packs, thermal blanket/cocoon, etc.) when temperatures are extreme.
- Transport requests are always evaluated on a case-by-case basis to ensure crew safety. Clinicians are responsible for contacting their Clinical AOC before declining a transport request for environmental factors so that systems issues can be identified and mitigated. For interfacility requests, teams may consider postponing lift requests until after acceptance is confirmed to mitigate the effects of temperature extremes on crews, equipment, and aircraft operation. Communicate temperature extreme turndowns and delays to AirCom so that differentiation can be made for true weather turndown vs. temperature mitigation efforts.

- Tailor risk mitigation efforts specific to your geographic region, temperature conditions, and flight time:
 - For fixed-wing operations, consider:
 - Hangar availability.
 - Temperature conditions en route and at the destination.
 - Ability to load/unload in the hangar.
 - Confirmation of ground unit availability at airport locations.
 - For rotor wing operations, consider:
 - Local flight in nature (< 20 min/flight time from base).
 - Ability to stay running on pads/scene.
 - Ability to load/unload in the hangar at the airport on the hospital hangar.
 - Patient condition and exposure to elements (scene vs. interfacility).

31.2.3 Control of Aircraft Temperature and Plan for Patient Safety

- All Air Methods CBS aircraft are climate-controlled to protect patients and teammates from temperature extremes.
- If climate control systems are not sufficient to ensure that the aircraft environment is safe for teammates or patients, clinicians must contact medical control or clinical leadership to discuss current ambient temperature and weather conditions to plan for patient safety. Consider:
 - Extremes of age: age less than 12 months or greater than 65 years.
 - Critical illness or injury, and risk/benefit profile of air transport.
 - Any patient with:
 - heat- or cold-related emergency
 - impairment of body temperature regulation
 - significant impairment of skin integrity
 - mechanical circulatory support: IABP, VAD, ECMO, etc.

31.2.4 Temperature Monitoring and Risk Mitigation Before Flight

- In the heat: if the interior of the aircraft cabin cannot be maintained between 50-95 degrees F, mitigation actions must be taken.
 - Store heat-sensitive medical equipment (cardiac monitor, ventilator, infusion pumps, etc.) and supplies (medications, IV solutions, and applicable soft goods, etc.) in the base.
 - It is recommended that clinicians store their helmets with all critical equipment so that no equipment shall be left behind when leaving for a flight.
 - Open aircraft doors after completing flight operations.
 - Place sunshield in front windscreen after completing flight operations.
 - Utilize a portable air conditioner after completing flight operations.
 - No medications or temperature-sensitive medical supplies should be stowed in the aircraft baggage compartment.
- In the cold: if the interior of the aircraft cabin cannot be maintained between 50-95 degrees F, mitigation actions must be taken.
 - Store cold-sensitive medical equipment (cardiac monitor, ventilator, infusion pumps, etc.) and supplies (medications, IV solutions, and applicable soft goods, etc.) in the base.
 - It is recommended that clinicians store their helmets with all critical equipment so that no equipment shall be left behind when leaving for a flight.
 - Utilize a portable heater after completing flight operations.
 - Bring proper blankets and packaging materials to ensure patient warmth. Referral facilities may provide blankets and thermal mattresses may be used in accordance with the manufacturer's instructions. Cover the patient's head with a hat or with towels and blankets as needed.
 - Conduct patient care preparation in a warm environment: during scene operations, consider starting treatment in the back of the ambulance or aircraft as appropriate; for interfacility transports, initiate treatment inside the referral facility and prepare all lines, fluids, and circuits tucked next to the patient and covered with blankets and insulated covers, as able.
 - Base locations subject to cold weather operations are required to purchase and carry an approved Thermal Blanket System based on the weather conditions they routinely operate in.
 - No medications or temperature-sensitive medical supplies should be stowed in the aircraft baggage compartment.

- Base locations in extreme cold temperature areas may consider placing more than one portable heater in the aircraft with pilot approval.

31.2.5 Temperature Monitoring in the Aircraft During Flight

- A continuously reading thermometer must be placed inside the cabin. The thermometer must not be affixed to the aircraft interior or held in place using Velcro, or adhesives. Possible recommendations include securing the thermometer in an aircraft pouch, drawer, or secured baggage.
- Cabin temperatures will be monitored during all patient legs and tracked for quality assurance, regardless of the mode of transportation.
- Clinicians must make every attempt to maintain the patient compartment within acceptable temperature ranges.
- Cabin temperatures must be measured and documented every 15 minutes during a patient transport until temperatures are maintained within the range of 50-95 degrees F (10-35 degrees C) for aircraft and the range of 68-78 degrees F (20-25 degrees C) for ground vehicles.
 - Clinicians are required to monitor the cabin temperature every 15 minutes; however, the temperature only needs to be documented once in the patient care record if it remains in range.
 - Cabin temperatures will be documented utilizing the "other vital sign" section within emsCharts.
 - If the cabin temperature of the aircraft cannot be maintained in accordance with CAMTS and/or state regulations the Clinical Director or Clinical AOC will be notified immediately.
 - Clinicians are required to have knowledge of and be able to reference state-specific regulations addressing temperature requirements.

31.2.6 Temperature Mitigation during Flight

- Clinicians must take action to mitigate a patient's exposure to temperature extremes and must document their actions in the patient care record (Refer to the [Clinical Documentation Guidance manual](#)).
- Clinicians must plan for weather elements when providing patient care during heat and cold extremes which may result in temperatures out of policy range during transport.
 - Cold Temperature Mitigation Strategies
 - Utilize approved Thermal Blanket Systems.

- When preparing the patient for transport consider requesting warm blankets from the sending health care providers (wrap the warm blanket around the patient within the Thermal Blanket System).
- Utilize a warming packet next to the patient in accordance with the manufacturer's guidance.
- Secure IV lines, ventilator circuit, ETCO₂ sensor, and any other cold-sensitive items next to the patient and warming packet (consider using additional warming packets if a need to contact cold-sensitive equipment).
- Cover the patient's head.
- When possible, utilize options to continuously monitor the patient's temperature throughout transport.
- Collaborate with the pilot to adjust the aircraft cabin temperature during flight.
- Hot Temperature Mitigation Strategies
 - Utilize appropriate blankets/sheets on the patient stretcher to avoid overheating the patient.
 - When appropriate, consider using cooling packets to mitigate overheating when preparing the patient for transport.
 - Protect medical equipment and supplies from extreme heat (remove/protect from direct sunlight when possible).
 - When possible, utilize options to continuously monitor the patient's temperature throughout transport.
 - Collaborate with the pilot to adjust the aircraft cabin temperature during flight including considerations for cruise altitude allowing for cooler temperatures.

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32 Exposure Control Plan

The exposure control plan located in 8.2 of the SMS Manual provides guidance for discipline and roles regarding bio-hazards in the air medical environment. All teammates are responsible for the content found in this document.

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33 First Responder Operations Clinical Guidance

33.1 Purpose

To provide clinicians guidance when engaging in scene operations for pre-approved "First Responder" base locations. Clinicians are required to complete initial and annual training before they are permitted to engage in First Responder scene operations.

Reference "First Responder Operations" Bulletin number B-2024-07 published on 06/28/2024 for details regarding First Responder operations.

33.2 Policy

- The individual base location must be approved for First Responder scene operations. A list of approved First Responder bases can be found on the Flight Operations page of [FlightDeck](#).
- Clinicians engaging in this operation must have completed initial and annual First Responder training. The required training must have been completed by both responding clinicians in this operation.
- During shift brief clinicians working at an approved First Responder base must confirm they have or have not completed the required training (Assigned Air Methods LINK Course).
- The Clinical Lead will collaborate with the Clinical Director and Area Manager to ensure the medical equipment and supplies utilized for First Responder scene operations are accounted for and operational.
 - Confirm the proper means to secure and carry medical equipment/supplies when extracting and moving a patient to the aircraft. Examples include using the aircraft stretcher or collapsible backboard and ensuring an ergonomic means to carry medical equipment/supplies such as shoulder straps or backpacks.
 - At a minimum one full D-size oxygen tank must be available for this operation.
- During First Responder scene operations clinicians will adhere to the General Operations Manual (GOM) Landing-Zone Safety and Medical Operations Manual (MOM) General Safety, specifically Scene Operations policy.

- *When responding to the patient, clinicians are required to bring all medical equipment and supplies as stated in the Continuum of Care policy, specifically addressing “Scene Response”, unless outlined in a Regional Differences policy. Clinicians are encouraged to contact the Clinical Administrator On-Call (AOC) if they are unable to bring the required medical equipment or supplies to the patient’s location.*
- The transport team should verify if additional responding resources are needed, such as the search and rescue/EMS team including estimated arrival time, location, capabilities, and contact information.
- After landing, clinicians are required to confirm a means of two-way communication with the pilot before moving towards the patient’s location. Clinicians are required to contact the Clinical Administrator On-Call (AOC) if they are not able to confirm two-way communication with the pilot.
- Clinicians must know the patient’s location in relation to the aircraft before engaging in this process.
- Clinicians are required to establish routine check-in intervals with the pilot (every 15 minutes) if accessing the patient is outside the view of the aircraft. Once outside the view of the aircraft clinicians are required to have a visual point of reference for navigating back to the aircraft.
- Clinicians must be aware of self-limitations before moving to the patient’s location (Consider personal physical and environmental limitations including proper attire and equipment. Clinicians must be aware of environmental safety concerns such as wildlife). Clinicians will operate to the lowest personal comfort level when engaging in this process.
- If the clinicians are not able to access the patient due to safety concerns or personal limitations, they must consider alternative options to access the patient (rendezvous with a search and rescue/EMS team to make safe patient contact). Clinicians are encouraged to contact the Clinical AOC to discuss safe options to access the patient.
- Clinicians are required to reference the Regional Difference policy specific to First Responder operations.

34 *Flight Paramedic to Flight Nurse Bridge Program*

34.1 Purpose

The Paramedic to Nurse bridge will provide an opportunity for qualified flight paramedics to transfer to either an open Flight Nurse position or a Flight Nurse-Paramedic position within CBS Air Methods flight programs. This will provide a professional growth opportunity for interested paramedics and allow Air Methods to retain these experienced and valued clinical crew members.

34.2 Policy

To be eligible to transfer from Flight Paramedic to Flight Nurse or Flight Nurse-Paramedic the following must occur:

- An open Flight Nurse position must exist or the Flight Paramedic may transition into a position titled “Flight Nurse-Paramedic” which will allow the Flight Paramedic to remain at his/her current base filling shifts as both a Flight Paramedic or a Flight Nurse and will allow the Flight Paramedic to be eligible for the nursing pay scale.
- The candidate must be a current or former employee of Air Methods.
 - If a paramedic obtains their nursing degree while at Air Methods and leaves Air Methods to pursue the required years of experience and returns afterward, their prior experience as a flight paramedic with Air Methods will apply.
- Three (3) years of critical care experience must be obtained and may be met via the following methods:
 - Nursing experience in a critical care area defined as ICU and ER.
 - Cath Lab, PACU, Operating Room are not considered critical care areas for this program.
 - Air Methods flight paramedic experience may be credited towards critical care experience based on the following scale:
 - 1,000 hours of Air Methods Flight Paramedic experience = six months of nursing critical care experience.
 - 6,000 hours may apply if the candidate was employed with Air Methods in a full-time Flight Paramedic position for no less than three (3) years.

- Credit for an Air Methods Flight Paramedic experience may be utilized for up to three (3) years required critical care experience. Any remaining experience must come from hours of nursing experience while employed as a nurse in a critical care unit as defined above.
 - No credit will be given for ground transport of any kind.
- Flight Paramedic candidates must have a flight nurse specialty certification to transfer to an open Flight Nurse or Flight Nurse-Paramedic position. Specialty certification requirements are explained below.
 - Flight Paramedic bridge candidates must obtain either CFRN or CTRN before transfer. Air Methods will reimburse for one attempt. If more than one attempt to obtain CFRN or CTRN is required, it will be at the expense of the bridge candidate.
- Flight Paramedics transitioning to a Flight Nurse-Paramedic position will be required to maintain all credentials necessary to fill both the Flight Paramedic and Flight Nurse positions to be scheduled in either role depending on a staffing need.
- If a Flight Paramedic transitions to a Flight Nurse-Paramedic position and chooses to let their paramedic credential expire, they are required to transfer to an open flight nurse position before the paramedic credential expiration date. The Flight Nurse-Paramedic must hold current professional licensure for the flight nurse position they choose to apply for.
- FP-C or CCP-C specialty certifications are preferred for all bridge candidates in addition to the CFRN or CTRN certification. The cost of taking/renewing both the FP-C or CCP-C and the CFRN or CTRN will be reimbursed by Air Methods, however, the bridge candidate will only be eligible for one advanced cert stipend per year in accordance with the stipend requirements.
- Flight Paramedics transitioning to a Flight Nurse-Paramedic position will be eligible for annual salary reviews and compensation at the Flight Nurse pay range and will be eligible for a corresponding pay increase at the time of transition.
- Flight Paramedics transitioning to either a Flight Nurse or Flight Paramedic-Nurse position must not have had any disciplinary action greater than verbal counseling on file during the past 12 months.
- Final approval of each candidate by the Clinical Director II is required.
 - Teammates must be reviewed and approved by Clinical Credentialing before final approval using this form >> [Clinical Credentialing Audit Request](#).
- Following a transfer, some programs may require, based on differences in scope of practice or operations, a brief period of orientation for the bridged Flight Nurse or Flight Nurse-Paramedic.

35 *Flight Readiness*

35.1 Purpose

Air Methods Community-Based Services (CBS) programs are committed to providing a safe and emergent response for all transport requests. Clinicians will maintain a state of readiness in order to minimize their response for transport.

35.2 Policy

- Readiness will be maintained to optimize response times.
- Clinicians will complete the aircraft daily checklist in accordance with the guidelines established in the Aircraft/Base Equipment and Supplies policy.
- Clinicians will complete a shift brief with the pilot-in-command.
- Any base-specific mission capabilities that may be impacted by licensure or clinical crew competency should be reported to the PIC and AirCom during the shift brief. This will include but is not limited to states where the base is licensed but a clinician is not Balloon Pump competency and ECMO competency where applicable.
- Clinicians are responsible for ensuring they have the necessary clothing readily accessible to minimize their response time during seasonal changes.
- The clinicians will respond immediately to the aircraft when a flight request is accepted by the pilot-in-command for a non-specialty team transport.
- The clinicians should not attempt to re-route a transport request to an alternate aircraft based on distance, shift change, etc., where AirCom has identified the requested aircraft as the closest appropriate aircraft. Any concerns with the asset identified by AirCom should be resolved after completion of the mission through the post-flight debrief process.
- The clinicians should not request clinical reports prior to liftoff, AirCom will communicate any known needs for special equipment prior to liftoff, as this inquiry is included in the dispatch info obtained from the referring customer.

- The clinicians will respond immediately to the aircraft for all scene and inter-facility standby requests to minimize response times if requested to respond.
- If response times are excessive, the QA/QI Post Flight Debrief process should be used to explain delays.
- The clinicians will respond to the patient's bedside with all required equipment to assess the patient's clinical condition and other clinical factors. Decisions regarding acceptance of a flight based on a patient's clinical condition should not be made remotely or via phone, except when it is not reasonable or possible to go to the patient's bedside; in this situation, the Clinical Director or Clinical AOC should be consulted immediately.
- Clinicians must contact their Clinical Director or Clinical AOC in consultation with their Area Manager before declining a transport request for any clinical reason.
- Flight requests for patients who are reported by the sending agency to exceed weight limitations for the aircraft based on that day's weight and balance calculations should be discussed between the pilot and clinicians before turning down the flight for weight reasons. Consideration should be given, when reasonable, to respond to the bedside to assess the accuracy of the reported patient weight and to assist the sending facility in finding an alternative transport option. Some factors to consider include whether the weight is being reported in kg vs. lbs. (ex. 150 lbs. may be possible but 150 kg may not), is it possible to remove the third seat, leave the third rider behind, or consider a short aircraft fuel burn to be able to accommodate the patient's weight, has the sending facility expressly stated they do not want a response to assess weight, etc. If there is any question about whether to respond, clinicians should contact their Clinical Director or Clinical AOC before turning the flight down for patient weight.
- The clinicians should evaluate the need to transport a patient by a different mode (ground, fixed wing) while at the patient's bedside whenever possible; if it is not possible to be at the patient's bedside then the Clinical Director or Clinical AOC will be consulted.
- The QA/QI process will include periodic reviews to ensure compliance and to identify and correct system or individual issues.
- At no time while on duty should a clinician leave their base of operations without notifying their Area Manager of the situation and discuss coverage plans.
- At the end of the shift if the relief nurse or paramedic is not present, the off going nurse and paramedic must call their Area Manager to notify them of the situation and discuss coverage plans.

36 General Pest Management Considerations (Bedbugs, Lice, Other Pests)

36.1 Caring for Patients with Pest Infestations

Patients infested with bedbugs, scabies, or lice may be accepted for transport by air or ground.

- Avoid skin-to-skin contact with an infested patient, and avoid close contact with their clothing, personal items, or linens.
- Move the patient from their bed/stretchers onto the aircraft stretcher with a new sheet or device. Contain the patient and their sheets in a body bag, thermal wrap, or plastic sheeting to prevent infestation of the aircraft. Use a surgical bouffant cap to cover their hair.
- Do not transport any patient belongings if possible. If you must bring items with the patient, place them inside tightly sealed plastic bags.
- When transferring patient care to the receiving facility, notify the receiving team members of the infestation and existing precautions.

36.2 Post-Flight Mitigation Actions to Prevent Aircraft/Base Pest Infestation

- Do not take your flight jacket, boots, equipment, or other clothing into the base until they have been thoroughly inspected. If pests are found, bag and seal everyone's clothing before entering the base. Clothing should be washed using a hot water cycle and high-heat drying cycle. All equipment and bags must be cleaned before entering the base or aircraft.
- Remove the patient transport stretcher from the aircraft/ambulance and wipe it down using germicidal disinfection wipes. Inspect the mattress, all straps, wheels, and other stretcher components for pests. If one or two pests are found on the cot, remove them and clean the area with disinfection wipes. If several are found, bag the stretcher mattress in plastic sheeting and contact the Area Manager to arrange for professional pest mitigation services.

- Inspect all cracks, crevices, and surfaces of the aircraft. A vacuum can be used to remove pests from cracks and crevices but the vacuum bag must be sealed and taken outside to the garbage immediately after use. If visible pests are found, vacuum again and seal the second vacuum bag, and dispose of it outside of the base area. Wash the inside of the transport vehicle with soap and water and inspect all equipment and bags for pests. If pests are found, remove them and clean all surfaces with disinfectant wipes.

36.3 General Base Cleaning and Pest Prevention

- Maintain clean living and sleeping quarters. Teammates should shower and change clothes as often as necessary to promote clean furniture use, and quarters should be frequently cleaned and vacuumed.
- Base storage/furniture: Do not store items on the floor under or near the bed. Place linens, clothing, extra shoes, and coats in plastic bins with tight-fitting lids.
- Base beds: Wash and dry bedding on a hot setting (high heat). Routinely inspect the cot mattress for holes or cracks that would provide hiding places. Replace as needed.

36.4 General Pest Control Guidance

- Report pest encounters through the AIDMOR process to ensure surveillance and loop closure.
- Air Methods follows the guidance of the Centers for Disease Control and Prevention (CDC) around parasite detection, health risk mitigation, diagnosis, and treatment. For more information, please visit the CDC website for the most up-to-date guidance.
- To best protect our teammates, Air Methods does not promote teammate use of chemical treatment for base or aircraft pest control. Any treatment of an infestation beyond monitoring, cleaning, mechanical removal of pests should be done by a professional pest control company contracted by Air Methods leadership.

37 General Safety

37.1 Purpose

To establish criteria and standards for Clinical Crew Personal Safety.

37.2 Policy

Air Methods is committed to attaining the highest level of safety while accomplishing our corporate mission. Our goal is to provide a safe and healthy working environment for all teammates and, in doing so, to support state and federal laws regarding safety. Our intention in making this strong commitment is to eliminate injuries to our teammates and accidental damage to equipment/property. It will be understood that teammates of all levels of the company will be responsible for participation in the Safety Management System (SMS) and the safe operation of our aircraft.

37.3 Passenger Safety

Additional passengers wishing to accompany the patient must attest to some basic physical abilities and well-being before being allowed to ride along. The attestation form 5312-CS will be signed by the non-patient passenger (5812-CS - Passenger Attestation). Following the passenger attestation, the passenger safety briefing must be conducted according to the General Operations Manual.

The completed attestation form should be emailed separately to DIS and not attached to the PCR.

37.3.1 Attestation Statements

1. I can walk up and down stairs unassisted.
2. I can understand and follow verbal instructions from the crew.
3. I am not currently experiencing any of the following: fever (temp > 100), nausea, vomiting, diarrhea, shortness of breath, or chest pain.
4. I understand I may be directed to wear personal protective or safety equipment as guided by the crew.
5. I understand I may be denied access to the receiving hospital based on their current communicable disease and/or hospital visitation policies.

37.3.2 Patient Statement Draft

Patients who are deemed by the clinical team to not be experiencing a critical illness and who have ambulation privileges while at the referring facility, may be allowed to walk onboard the aircraft however the following criteria must be met:

1. The patient does not utilize assist devices to ambulate, i.e., a cane or walker.
2. The patient's vital signs are stable to include blood pressure, heart rate, and they are not actively receiving fluid resuscitation or medication infusions.
3. The clinical team will always keep one hand on the patient while boarding the aircraft and the utilization of a gait belt on the patient for this purpose is strongly encouraged.

37.4 Flight Operations

- The flight team and any specialty team members will receive annual aviation training as per Air Methods CBS policy.
- Running, smoking, throwing objects, and operating a vehicle within 50 feet of the aircraft is prohibited.
- Caution will be exercised in and around the aircraft. Antennas, blades, pitot tubes, etc., can be damaged or cause injury to personnel. Damage to these items may cause aircraft grounding.
- Nothing should be lifted higher than the head when operating around rotor wing aircraft.
- Only the flight team and mechanic are approved to operate doors on the aircraft.
- All clinical flight teams will wear appropriately fitted company-provided long-sleeved Nomex flight suits with sleeves rolled down and zippers fully zipped, along with black leather boots while on flight duty. Flight suits will have reflective striping on the arms and legs.
- The flight team will always wear hearing and eye protection around a running aircraft.
- All rotor wing clinicians will wear helmets during all phases of flight operation. All rotor wing clinicians will have the helmet visor down during flight or around running/hot aircraft.
- At no time will a clinician contact one of the oncoming clinicians before their arrival at the base to discuss pending missions or request ETAs to the base.

- At no time will a member of the clinical team contact a pilot before the commencement of his or her shift to discuss pending missions, patient information, or operational concerns that may influence the pilot's decision-making in flight acceptance.
- The pilot WILL NOT be informed of the patient's age or condition before making a "go/no-go" decision.
- The Pilot-in-Command is the final authority for the safety of the flight, selection of landing zones, continuing or canceling a flight due to weather, all loading of equipment, and anything else related to the operation of the aircraft.
- Clinicians will perform a safety walk-around before boarding the aircraft.
- Clinicians will wear seat belts and shoulder harnesses during all take-offs, landings, and in-flight whenever patient care permits.
- Clinicians will notify the pilot before unsecured/unbuckled from their safety harness and when re-secured/buckled in.
- Medical equipment must be secured in an approved manner during all phases of flight. When a mount is installed and available the medical device should be secured in the mount. Clinicians must maintain visibility and access to the medical device when secured. If an aircraft mount is not available, clinicians must secure the medical device using Air Methods-approved securement equipment in a manner approved by the PIC. At no time will medical equipment share a seat belt intended for patient use while a patient is secured to the stretcher (head, side rails, or between/beneath/top of the patient's legs). Clinicians are encouraged to refer to the Carry-On Baggage and Equipment Storage and Security policy for guidance when securing specialized medical equipment. Clinicians should contact the Clinical AOC if they are unable to secure medical equipment or supplies in accordance with this guidance.
- Clinicians are expected to stay alert on all legs of the transport to assist the pilot in staying alert.
- Clinicians will assist the pilot in monitoring for other air traffic, clearing the aircraft tail, clearing the main rotor blades, and in general watching for any hazards that should be brought to the pilot's attention.
- Clinicians are responsible for terminating a flight if the patient's needs cannot be met due to illness of the crew, medical equipment failure, or perceived threat to the well-being of those on board.
- Clinicians will never disembark the aircraft without the pilot's permission.

- Seat belts will be buckled in place when not in use.
- Local/state/federal regulations regarding Personal Flotation Device (PFD) use in flight over water will be adhered to.

37.5 Scene Operations

- Whenever prudent, one clinical team member will stay outside the aircraft, either outside the rotor-disc in the pilot's view or connected via the ICS, during startup at unsecured landing sites. This will ensure the area remains secure until the pilot indicates he is ready to take off.
- Company-provided high-visibility, reflective safety vests will be worn on all scene flights in accordance with the ANSI-SEA 107 standard.
- Air Methods clinicians will not participate in heavy extrication or rescue efforts.
- Air Methods clinicians will not engage in ground search and rescue efforts without the assistance and presence of a ground EMS department, fire department, or search and rescue team.
- If Air Methods clinicians are requested to provide patient care or assessment during an extrication or rescue effort, then all appropriate safety measures should be taken before engaging in patient contact. Clinicians must decline assistance if they do not have access to needed specialized protective equipment or safety gear.
- When responding to a scene involving a victim of a violent situation/altercation always consider the safety of yourself, your team, and your aircraft.
- Verify if you are responding to a secured scene or if this is in a safe zone. If in a safe zone, ask AirCom to obtain the proximity of the landing zone in relation to the incident to determine whether this is an adequate distance from the scene.
- If you are responding directly to the scene, inquire with AirCom if the scene is safe and has been secured by appropriate law enforcement. Inquire if the assailant is in custody or if they are actively in pursuit.
- If the scene is not safe, communicate with AirCom and local agencies to identify an alternate landing location to which the patient may be transported.
- Always be vigilant to continually monitor scene safety.

38 Hazardous Material Response

38.1 Purpose

To define the role of Air Methods in managing incidents involving contamination, and ensure the safety of personnel while appropriately caring for patients exposed to hazardous materials. Air Methods reserves the right to deny transport of patients deemed by the Incident Commander as a Hazardous Materials Incident or a scene involving a reportable quantity of hazardous materials release.

38.2 Procedure

38.2.1 In General

- A hazardous material is defined as any substance which causes physical or mental disturbances upon exposure or which requires specific decontamination of clothing or equipment after exposure.
- Air Methods will not transport any patient/patient belongings/clothing exposed to hazardous materials or respond to scenes involving "significant hazardous materials exposure" as determined by the Incident Commander.
- The communication specialist will inquire at time of transport request whether hazardous materials are involved.
- Each rotor wing aircraft will have onboard (available by print copy or on iPad) a DOT Hazmat Emergency Response Book of materials which could pose a threat to the clinical transport team or render a transport inappropriate.
- In the event of unanticipated contact with hazardous materials, the helicopter will be repositioned upwind and at a safe distance from the contamination site. Patients will be thoroughly decontaminated by specially trained pre-hospital care providers prior to loading the patient into the aircraft. In the event of unanticipated contact with hazardous materials, the decision to accompany such a patient by ground ambulance (only after the decontamination process) is at the discretion of the Air Methods clinical crew.
- Initial/Annual Training: All crewmembers will receive initial and annual recurrent Hazardous Material (HAZMAT) training.

- Remember to take into consideration that the recommendations suggested in the HAZMAT guidebook are the minimum safe distances. Due to the nature of a chemical's flammability or explosive potential, greater distances away from the scene may be prudent.
- Avoid low-lying areas as many asphyxiates and explosive vapors are heavier than air. It is imperative that the aircraft approaches a HAZMAT incident cautiously to avoid exacerbating a hazardous situation by operating an aircraft in the immediate area. The rotor wash of the aircraft may introduce an element into a HAZMAT incident that is not in the interest of scene safety and patient care.

38.2.2 Flight Request

- AirCom will provide the crew all HAZMAT related information available when received from the requesting agency.
- Crewmembers should consult the U.S. Department of Transportation Emergency Response Guidebook (ERG) prior to departing for the transport if en route time is not sufficient to ensure the safety of the crew on arrival at the incident. The entire crew will be made aware of pertinent information regarding the contaminant.

38.2.3 En route to the Scene

- The ERG will be kept readily available in the aircraft and communications center to allow situation assessment as new information is received en route.
- Radio contact should be established with the landing zone as soon as possible. The ground personnel should confirm the presence of the hazardous agent(s).
- Special attention should be paid to the direction of the wind and distance of the landing zone from the scene. The pilot will maneuver the aircraft upwind as necessary to remain contaminant free.

38.2.4 On the Scene

- Ensure the safety of the flight crew and aircraft.
- Circle the scene at a safe distance upwind, approximately 3-5 miles, selecting flight routes to avoid downwind danger zones.
- Do not land until a safe landing zone is secured.
- Upon landing contact the incident commander or the incident commander's designated representative for the patient care sector.
- At no time will crewmembers enter the warm or hot zone of the incident.

- Confirm that the patient has been decontaminated by ground personnel prior to assuming care.
- **NO HAZARDOUS MATERIALS SHALL BE TRANSPORTED ON THE AIRCRAFT.**
- Prior to departure from the scene, contact the landing zone coordinator to verify any changes to the hazardous material, wind direction, or other conditions in the vicinity of the landing zone.
- After the termination of the call, any contaminated crewmember, equipment and/or aircraft will be properly decontaminated.

38.2.5 Methamphetamine Decontamination

It is critical for all clinical crews to identify potentially contaminated patients and mitigate risk to flight crew during transport of burn patients. The proliferation of methamphetamine labs predisposes all emergency responders to exposure to hazardous materials. Chemicals used in methamphetamine labs include but are not limited to: anhydrous ammonia, red phosphorous, acetone, sodium hydrochloride, iodine, phosgene, and ephedrine. Symptoms of exposure to these chemicals include but are not limited to: chest pain, difficulty breathing, dizziness, eye irritation, and collapse. Due to the clandestine nature of these labs, rarely will emergency responders be alerted to the potential hazards of the scene.

- Communication center receives a call for the transport of a burn patient.
- Communication center determines the nature of the fire through the requesting dispatch center.
- If the source of the fire is not known or methamphetamine lab cannot be ruled out, the communication center will request the patient be decontaminated by fire personnel on scene.
- Guidelines for this process is that the patient will have all clothing removed and be washed with soap and water.
- In an effort to minimize hypothermia, the patient will then be covered with clean dry sheets and blankets following decontamination.

38.3 Specific Duties

38.3.1 Communications Specialist

The communications specialist will obtain as many of the details regarding the incident as is practical including the nature of the hazardous material that was spilled. The communications specialist will also access the appropriate Material Safety Data Sheets online. The requesting agency will be contacted to setup a landing zone one mile upwind by either the communications specialist or the pilot while en route.

38.3.2 Flight Clinicians

The flight clinicians will assist the communication specialist and pilot with safety issues of transport. Patients exposed to hazardous materials (e.g., gasoline and diesel fuel) must be thoroughly decontaminated and clothing removed by specially trained pre-hospital care providers prior to transport. Under no circumstances will the flight crew enter the contaminated area. The patient must be thoroughly decontaminated by EMS personnel before transport. Generally, this involves removing the entire patient's clothing and washing with soap and water. All open wounds should be copiously irrigated with water before dressings are applied. The patient's clothing and other personal effects shall remain at the scene of the incident.

38.3.3 Pilot

If the pilot suspects the LZ is downwind of a hazardous material spill, he/she will notify the flight crew as well as Incident Commander and reposition aircraft to a safe location. It is recommended that the helicopter will land no closer than one mile upwind of the site of the incident.

38.4 Inter-facility Transports

Patients who were exposed to a hazardous material and are in a hospital setting should have the same decontamination considerations as all of the above scene response guidelines. Patients who have not been fully decontaminated will require this intervention prior to air medical transport. The flight clinicians need to take care not to become exposed; they should be diligent to ascertain the patient's decontamination status and ensure this is done prior to patient contact.

39 *HIPAA and Protected Health Information*

See the HIPAA Manual located on FlightDeck > Resources > Corp Pubs > Compliance Policies and Procedures.

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40 Hot/Cold Loading and Unloading

40.1 Purpose

To ensure there is comprehensive and safe policy for the Hot - Cold Off-loading of patients.

40.2 Policy

- The flight crew, patients, and passengers will always adhere to all safety regulations/ Crew Resource Management principles to include General Operations Manual policy "Loading and Unloading Patients or Passengers." Any problems, deviations, or safety concerns will be reported using an applicable Air Methods reporting tool.
- The flight crew will assess the medical need for hot loading/unloading of equipment, personnel, or patients. Examples are departures from rooftop helipads or scene calls when the helicopter is left running, and critical patients requiring emergency off-loading.
- The pilot must give permission for hot loading/unloading. Operational or safety considerations may override medical needs.
- If a third rider (orientee or family member) accompanies the flight crew on a patient transport, the third rider must be accompanied to and from the aircraft by a member of the flight crew. It is recommended that the aircraft be shut down for on/offloading when a third rider is present. However, this decision can be left to the pilot and crew on duty depending on the situation and location.
- It is preferred, but not required that hospital security or other designated personnel be present at the hospital helipad when the helicopter is landing and departing.
- Only the minimum number of trained personnel are allowed under the rotor disk during the hot loading/unloading process. All personnel and passengers should wear hearing and eye protection, if possible.
- The flight crew must assign one ground personnel member on scene during loading and unloading to be in charge of the stretcher to ensure it does not roll away and come in contact with any part of the aircraft.

- Ensure loose items are secured (i.e. Blankets) when hot loading/unloading.
- No one will be allowed past the designated safety zone dictated by airframe during hot loading/unloading.
- The pilot may be required to remain at the controls of the running helicopter, except to provide security, as defined in the General Operations Manual.

40.3 Guidelines for Patient Movement

- Take an appropriate time out prior to patient movement to ensure all safety measures are in place to reduce harm to patients and crew members.
- Do not move a patient until you have **confirmed** with each other you have the following:
 1. All equipment is secured - use of approved aircraft mounts for **every** flight.
 2. Adequate lifting assistance to load and off-load the patient from the aircraft is present.
 3. Air Methods clinicians will ensure safe and proper stretcher movement near and around the aircraft to include stretcher control during patient loading and off-loading procedures. Stretcher control includes ensuring the stretcher brakes are engaged and/or assigning a teammate to physically hold the stretcher in place during patient transition.
 4. Air Methods clinicians are responsible for ensuring all medical devices (cardiac monitor, ventilator, IV infusion pumps, IV fluid warmer(s), cardiac assist devices, etc.) are properly secured during patient movement to include loading and off-loading procedures.
 5. Air Methods clinicians are responsible for ensuring all ventilator circuits, IV tubing, and other invasive device lines or drains are properly secured during patient movement to include loading and off-loading procedures.
 6. One member of the Air Methods clinical team will remain at the head of the stretcher for all intubated patients and assure correct tube placement after each move of the patient.
 7. No less than two people with all four hands on the stretcher for patient movement.
 8. Stretchers are at an appropriate height, do not roll with the patient in highest position.
 9. Ensure the patient's weight is centered on the stretcher prior to movement.

41 Initial and Continuing Medical Education

41.1 Purpose

To establish minimum standards for initial and continuing medical education, which will ensure clinical preparedness and competency.

To outline the phases and expectations of new employee orientation.

41.2 Policy

41.2.1 Initial Orientation Program

41.2.1.1 Didactic Component

The didactic topics may be taught by several different methods: traditional lecture with discussion, self-directed learning tutorial, web-based, or case-scenario teaching, etc. However, there must be documentation that ensures competency through written objectives and examination testing prior to independent practice (completion of orientation). The Topics will include but are not limited to:

- Advanced airway management
- Air Medical Resource Management (AMRM)
- Altitude physiology/stressors of flight
- Aircraft fundamentals and safety to include radio communications for aircraft and EMS and crew resource management.
- Cardiac emergencies to include acute coronary syndrome (ACS), cardiogenic shock, twelve lead ECG interpretation, arrhythmia recognition, valvular dysfunction, aortic emergencies, hypertensive emergencies, and pericardial tamponade.
- Environmental emergencies to include hyper/hypothermia, near drowning, and electrical injuries.
- FEMA Disaster Management training (IS100, IS200, IS700, IS800, IS5A, or Hazmat Awareness Equivalent (Firefighter, Operations, Technician or State Academy course).
- Hazardous materials recognition and response

- Hemodynamic monitoring, pacemakers, automatic implantable defibrillator (AICD), intra-aortic balloon pump (IABP), central lines, pulmonary and arterial catheters, ventricular assist devices (VAD), Impella, and extracorporeal membrane oxygenation (ECMO).
- High-risk obstetrical emergencies to include preterm labor (PTL), pregnancy-induced hypertension (PIH), placenta previa, abruptio placenta, HELLP syndrome, and emergency childbirth.
- HIPAA training
- History of air medicine, philosophy, and indications for transport
- Infection control
- Industry associations and standards to include CAMTS
- Patient Care Guidelines (PCG) exam
- Metabolic and endocrine emergencies to include diabetic ketoacidosis (DKA)
- Multi-trauma to include maxillofacial, neurological (head and spine), thoracic, abdominal, and orthopedic)
- Neonatal emergencies (respiratory distress, cardiac, and surgical anomalies)
- Neurological emergencies to include intracerebral hemorrhage, subarachnoid hemorrhage, seizures, and cerebral vascular disease
- OSHA training
- Compliance issues and regulations
- Oxygen quality controls to include: hazard awareness, how to read cylinder levels, basic understanding of Compressed Gas and associated connections, and safely transporting liquid oxygen cylinders.
- Pediatric medical emergencies to include seizures, meningococemia, croup, and epiglottitis
- Pediatric trauma
- Pharmacology
- Quality Management
- Radiographic interpretation (chest)

- Respiratory emergencies to include acute respiratory distress syndrome (ARDS), reactive airway disease (RAD), pulmonary embolism, pneumothorax, acid-base balance, arterial blood gas interpretation, mechanical ventilation, and capnography interpretation.
- Scene management to include safety, disaster management/triage, rescue/extrication.
- Sleep deprivation, sleep inertia, circadian rhythms, and recognizing signs of fatigue.
- Stress recognition and management
- Survival training
- Thermal, chemical, and electrical burns
- Trauma systems and trauma scoring
- Trauma in pregnancy
- Toxicology
- Just Culture Education
- Threat and Error Management

41.2.1.2 Clinical Component

Measurable objectives need to be developed and documented for each patient type listed below. These Clinical experiences need to be scenario-based practical training utilizing a human patient simulator capable of demonstrating physiological changes resulting from treatments and therapies performed. For those programs that require in-hospital clinical rotations, they will be completed during orientation, as required by local and regional regulation; see regional differences. All documentation will be maintained at the program.

- Complete Orientation in Greenwood Village, Colorado – Clinical Indoctrination
- Local orientation – To include a clinical needs assessment, flight internships, completion of competencies, and orientation items. See Education Blueprint.
- Completion of TPATC within three (3) months of hire, if new hire possesses a valid trauma certification, and Neonatal Resuscitation (NRP) must be completed prior to successfully completing local orientation (moving to 2nd seat).

- All clinicians will receive a flight internship beginning as a 3rd crew member.
 - During the flight internship, the orientee will remain under the direct mentorship of preceptors. While in the aircraft, the orientee and preceptor will be with the patient. At no time will the preceptor be in a separate portion of the aircraft during patient transport.
- Completion of a minimum of five (5) flights; flights evaluated for quality by the Regional Clinical Education Manager (RCEM).
- Clinicians with prior flight experience, who are approved by the clinical educators for fast track, will require a minimum of three (3) flights.
- Greenfield bases without access to volume may use simulated patient contacts at the discretion of the education and clinical team.
- SIMX (Simulation Experience) or Virtual Reality (VR) scenarios can be used in place of flight volume, but this must be approved by the RCEM (replaces 1 flight maximum).
- SIMX, Cadaver - New Hires will have five (5) Successful Intubations in each category (adults, pediatric, infants) during initial clinical orientation in Denver.
 - 15 intubations total to complete initial orientation.
 - Must include the use of a backup airway device.
- Completion of HROB Modules on Learning Management System (LINK)
- Completion of safety forms TF-5642, TF 118 (if the flight program utilizes clinicians for aircraft refueling).
- RSI Exam – passing grade 80%
- PCG Exam– passing grade 90%
- Demonstrate Competency in all Invasive Skills
- Complete all Flight and Shift Evaluations by Preceptor(s) and complete update calls with RCEM. All flights, certifications, Clinical Competencies, and orientation items must be logged in Protean before moving to 2nd seat.
- Specialty Equipment training ex. IABP, ECMO (if applicable).

- The orientee will be assessed at the fifteen-day mark to assess educational needs. Demonstrates Successful Completion of ALL SIMX or VR Scenarios (4) - Adults, Peds, Infant, and HROB (different from initial assessment scenarios). Additional educational and SIMX will be provided if needed.
 - Adult critical care (cardiac, medical, surgical)
 - Pediatric critical care
 - Labor and delivery
 - Emergency care
 - Pre-hospital care
- If SIMX is utilized, a video recording of the performance is done for debriefing/ educational purposes if technology limitations allow.
- The orientee will meet with RCEM and CE to review all the above documentation via Protean. The orientee's performance will then be reviewed with the Medical Director of oversight. A decision will be made as to whether the individual is cleared to practice independently upon recommendation of the RCEM and final approval from the Medical Director. The decision will be communicated to RCD.
- Documentation of all completion requirements must be completed via Protean.
- Advancement to 2nd seat provider is considered off local orientation.
- When an HR workflow is complete and the new hire is moved to post orientation (2nd seat), they will become the area manager's responsibility.
- Completion of Specialty Review Course within six months from the start of the post-orientations phase.
 - Submission of certificate of completion to RCEM/CE.

41.2.2 Annual Continuing Education Requirements

Clinicians are required to complete assigned clinical education/training requirements before or on the established deadline date. Clinicians who fail to meet the clinical education/training deadlines are subject to progressive discipline including previous coaching occurrences.

Clinicians are required to complete quarterly assigned education and competencies before or on the 20th of the last month of the quarter. Failure to complete the assigned clinical training requirement before or on the 20th will result in documented verbal coaching. Failure to complete this requirement on or before the close of business on the last day of the last month of the quarter will result in progressive discipline with a 30-day completion requirement.

Clinicians are required to complete the Ad-Hock clinical education/training requirement before or on the assigned completion date. Failure to complete this requirement will result in documented verbal coaching with an assigned due date. Failure to meet the newly assigned due date will result in progressive discipline.

Clinicians are required to complete quarterly intubations before or on the 20th of the last month of the quarter. Failure to complete the quarterly intubation requirements on or before the 20th of the last month in a quarter will result in documented verbal coaching. Failure to complete this requirement on or before the close of business on the last day of the last month of the quarter will result in removal from flight status and a progressive discipline with a 30-day completion requirement.

41.2.2.1 Didactic Component

- Aviation safety
- Altitude physiology and stressors of flight
- Advanced airway management
- Air Medical Resource Management (AMRM)
- Aircraft fundamentals and safety to include radio communications for aircraft and EMS and crew resource management.
- Cardiac emergencies to include acute coronary syndrome (ACS), cardiogenic shock, twelve lead ECG interpretation, arrhythmia recognition, valvular dysfunction, aortic emergencies, hypertensive emergencies, and pericardial tamponade.
- Environmental emergencies to include hyper/hypothermia, near-drowning, and electrical injuries.
- Hemodynamic monitoring, pacemakers, automatic implantable defibrillator (AICD), intra-aortic balloon pump (IABP), central lines, pulmonary and arterial catheters, ventricular assist devices (VAD), Impella, and extracorporeal membrane oxygenation (ECMO).
- High-risk obstetrical emergencies to include preterm labor (PTL), pregnancy-induced hypertension (PIH), placenta previa, abruptio placenta, HELLP syndrome, and emergency childbirth.
- HIPAA training
- History of air medicine, philosophy, and indications for transport
- Industry associations and standards to include CAMTS
- Metabolic and endocrine emergencies to include diabetic ketoacidosis (DKA)

- Multi-trauma to include maxillofacial, neurological (head and spine), thoracic, abdominal, and orthopedic)
- Neonatal emergencies (respiratory distress, cardiac, and surgical anomalies)
- Neurological emergencies to include intracerebral hemorrhage, subarachnoid hemorrhage, seizures, and cerebral vascular disease
- Compliance issues and regulations
- Oxygen quality controls include hazard awareness, how to read cylinder levels, a basic understanding of Compressed Gas and associated connections, and how to safely transport liquid oxygen cylinders.
- Pediatric medical emergencies to include seizures, meningococemia, croup, and epiglottitis
- Pediatric trauma
- Pharmacology
- Quality Management
- Radiographic interpretation (chest)
- Respiratory emergencies to include acute respiratory distress syndrome (ARDS), reactive airway disease (RAD), pulmonary embolism, pneumothorax, acid-base balance, arterial blood gas interpretation, mechanical ventilation, and capnography interpretation
- Scene management to include safety, disaster management/triage, rescue/extrication
- Thermal, chemical, and electrical burns
- Trauma systems and trauma scoring
- Trauma in pregnancy
- Toxicology
- Hazardous material recognition and response
- Infection control
- Medical protocol test
- OSHA (Company modules)
- Sleep deprivation, sleep inertia, circadian rhythms and recognizing signs of fatigue
- State EMS rules and regulations regarding ground transport
- Stress recognition and management

- Survival training
- Just Culture education
- Threat and Error management

41.2.2.2 Clinical Component

Measurable objectives need to be developed and documented for each patient type listed below. These Clinical experiences need to be scenario-based practical training utilizing a human patient simulator (SIMX) capable of demonstrating physiological changes resulting from treatments and therapies performed. (See Education Blueprint for details).

- Adult critical care (cardiac, medical, surgical)
- Pediatric critical care
- Labor and delivery
- Emergency care
- Pre-hospital care
- Quarterly Intubation requirement to establish minimum standards for training, ensuring clinical preparedness, and competency.
 - Endotracheal intubations (Minimum of one per quarter in each patient age category; infant, pediatric, and adult as per mission profile to include rescue airway placement).
 - Quarterly intubation training is the responsibility of the CBL/Preceptor with Regional Education team oversight.
 - All endotracheal intubation attempts on mannequins must be successful to include the following critical criteria for all patient age categories:
 - Demonstrate/verbalize the correct medication.
 - Demonstrate/verbalize the right dose of medication/dilution if indicated.
 - Identification of critical lab values that can influence medication selection/dosing.
 - Proper use of the RSI checklist.
 - Proper use of the MACC checklist.
 - Proper use of backup airway devices.
 - Correct use of all required equipment as outlined in the Blueprint.
 - Obtain a passing score on the Quarterly Intubation scorecard.

- Assessment of minimum standards for the following concepts (not part of the critical grading criteria for the Quarterly Airway Validation):
 - Demonstrate ability to make an accurate differential diagnosis.
 - Demonstrate ability to interpret provided lab values.
 - Demonstrate ability to calculate and use basic mechanical ventilation settings.
- Remediation guidance.
 - Removal from flight status and completion of a PIP will be evaluated and decided on a case-by-case basis by the RCD and RCEM upon failure of any of the critical criteria.
 - CBL/Preceptor must immediately notify the RCEM of any failures by email/text/verbally, then submit the completed scoring sheet within 24 hours.
 - At the discretion of the RCD and RCEM, the clinician may be placed on an ELP if:
 - The clinician is unable to demonstrate the ability to calculate and use basic mechanical ventilation settings.
 - Other identified clinical gaps that are not considered critical criteria.
 - The clinician must complete the PIP within seven days; a regional educator must complete remediation with the clinician.
 - Failure of the PIP will lead to a Multidisciplinary Meeting to include: RCEM, the Area Manager, Clinical Director, Medical Director, and an HR Business Partner.
- Documentation of Quarterly Intubations.
 - The CBL/Preceptor will document the intubations via Smartsheets, use either the link or the QR code provided to complete the scoring tool. This form allows for multiple validations by the CBL/Preceptor to be entered per clinician (Adult/Peds/Infant) if all three were completed on the same day.
 - Example of Smartsheet form for logging airways.
 - The employee number of the CBL/Preceptor and the clinician are required for the successful completion of this process.
 - The clinician will receive an email prompting them to validate their quarterly intubation. This is a two-step validation process and daily email reminders will be sent until it is completed.

FIGURE 41.1: Quarterly Airway Validation Scoring Report

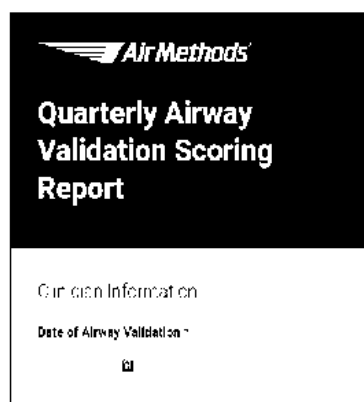


FIGURE 41.2: Example of Validation Email



- The due date for quarterly intubations, the skill, and the two-step validation process is the 20th of the last month within the quarter.
- Yearly Invasive procedure lab (mannequin, cadaver)
 - See Air Methods Tube Thoracotomy education blueprint
 - For those programs that practice Chest Tube Thoracotomy, training will be done on a bi-annual basis, i.e., cadaver lab and in-person/virtual learning.
- Yearly Ventilator Lab

- Skills maintenance program must be documented and comply with the number of skills required in a set period according to the regional differences policy of each program.
 - All clinical teammates are accountable for recurrent didactic and skills training for special medical equipment and procedures associated with their base and regional assignments. Special medical equipment and procedures consist of the following: Intraortic Balloon Pump (IABP), Extracorporeal membrane oxygenation (ECMO), invasive procedures to include thoracotomy. Failure to complete the stipulated training will result in the clinical teammate's suspension from managing specialty medical equipment or performing invasive procedures such as thoracotomy and may result in progressive discipline.

For BLS, ACLS, PALS and NRP Air Methods will provide at no cost to the clinician the "E Learning" version of these courses. Each clinician will be provided with a key for BLS, ACLS, PALS, and NRP in a range from three (3) to six (6) months prior to the expiration of their current certification.

The 8th edition of NRP has two levels of providers Advanced and Essentials. In these cases, the "Essentials" (lessons 1-4) provider will not be accepted. The clinician will be required to complete the "Advanced" level curriculum (lessons 1-9) with applicable skills/validations.

These keys will originate via a request made from the clinical educator to the Air Methods American Red Cross Training Center. The clinician will be notified via Air Methods email of the key and the procedure for which to log into the Healthstream LMS to complete the course(s). BLS, ACLS, and PALS keys are valid for 60 days after issuance, while NRP has a 30-day key validity period. The clinician will then complete the didactic portion of their class while on duty. No overtime shall be incurred to complete the didactic portion. After successfully completing the online didactic portion of the course, the clinician will be responsible for coordinating the skills verification portion of the course by direct communication with their respective regional education team members.

After completing both the didactic and skills portions of the course, a course completion card will be issued by the Air Methods American Red Cross Training Center for BLS, ACLS and PALS. NRP Course completion cards will be issued by the NRP instructor who conducts the actual skill verification portion of NRP.

If a clinician chooses to recertify through other means, such as a traditional class in a discipline that "E Learning" is offered, the clinician is not eligible for reimbursement of course cost.

It is the responsibility of the individual clinician to recognize their own educational needs independent of any requirements. Each clinician has a professional responsibility to maintain and continue to gain knowledge in their respective professions.

All curricula will be reviewed on an annual basis. Didactic topics will be provided by a member of the Air Methods Education team or a qualified subject matter expert (SME) that has been approved by the Air Methods education team.

To qualify as a SME the individual must provide evidence of prior experience within the subject and attend SME training by the Air Methods education team.

42 *Inter-facility Transport without Confirmation of Accepting Physician and/or Facility*

42.1 Purpose

Air Methods is committed to offering a dedicated clinical air transport team upon transfer request regardless of patient acceptance at time of dispatch unless otherwise specified by requesting physician or medical control physician (MCP).

42.2 Policy

- Clinicians will respond to the patient's bedside without delay.
- Clinicians will not remove the patient from the sending facility until acceptance has been confirmed or COBRA/EMTALA has been verified. Clinicians may also relinquish care back to the primary care provider if acceptance at an appropriate facility cannot be obtained and after consultation with the requesting physician or MCP and Clinical Director.

42.2.1 Indications

- Transporting clinicians may make patient contact after consulting with the requesting health care providers.
 - Clinicians will only assist in patient care under direction of the primary care provider requesting transport.
 - Clinicians will not assume full patient care until acceptance has been confirmed or further direction obtained from the primary care provider and MCP.
 - Clinicians should begin the packaging process if the patient diagnosis is time sensitive in an effort to minimize transfer delays.
- Clinicians will notify AirCom, Clinical Director, and/or MCP at specific time intervals as necessary.
 - Clinicians will notify dispatch AirCom, Clinical Director, and/or MCP 30 minutes after initial patient contact if acceptance has not been confirmed.
 - Clinicians will again notify AirCom, Clinical Director, and/or MCP 60 minutes after initial patient contact if acceptance has not been confirmed.
 - Clinicians will notify AirCom, Clinical Director, and/or MCP if there is change in patient condition at any time.

- Relinquish Care – Clinicians may relinquish care back to the primary care provider and be released from patient care and returned to service after consultation with the MCP and Clinical Director in the following situations:
 - If acceptance has not been obtained within 60 minutes of flight crew initiating patient contact.
 - If patient's clinical condition is not deemed stable for transport and/or demonstrating signs of rapid deterioration. Transporting clinicians will consult with the primary health care provider, Clinical Director and MCP before relinquishing patient care.
 - If due to the delay of waiting for acceptance the flight cannot be completed as planned due to outside factors (e.g., weather deterioration, pilot duty time).
 - Any flight crewmember can refuse or abort the mission if at any time, in their judgment, they feel that the situation is unsafe.

42.2.2 Special Considerations

In the event that the delay in patient acceptance adversely effects flight completion, alternate arrangements may be considered. **The arrangements should be in collaboration with the primary care provider and MCP when applicable.**

42.2.2.1 Deteriorating Weather

In the event of deteriorating weather at origin or destination, the transporting clinicians may:

- Elect to abort the flight and be absolved from patient care/responsibility.
- Request alternate appropriate receiving facility that has adequate weather for air transport as confirmed by pilot on duty.
- Complete the patient transport utilizing ground transportation. The transporting clinicians will accompany the patient to the receiving facility.

42.2.2.2 Duty Time Out

In the event of duty time out for the pilot, the following options may be considered:

- Other aircraft may be requested to complete mission. If time allows, primary transporting clinicians will remain with patient until such time that the secondary transporting clinicians arrive, obtained report, made patient contact and proper patient handoff can be completed.
- Dependent on time constraints, it may be possible to leave primary transporting clinicians at referring facility to continue patient care while pilots change shift with oncoming pilot returning to complete mission.
- If there is not another aircraft available and an immediate pilot shift change is not possible, primary transporting clinicians may transport the patient via ground ambulance. The oncoming pilot will then reposition to the receiving facility to intercept transporting clinicians after transfer of patient care, weather permitting.

Any deviations or special considerations not listed such as delays in contact times, customer requests, or other issues there should be communication and consultation between the Clinical Director, Area Leadership, and MCP.

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43 *Laptop/iPad Usage during Air Transport*

43.1 Purpose

To provide guidelines for Air Methods transport personnel for use of laptops or the clinical iPad in transport.

43.2 Policy

The use of an Air Methods laptop or clinical iPad is prohibited during periods of sterile cockpit requirements including pre-takeoff, takeoff, and landing procedures. In addition, the laptop or iPad should be stowed for the aforementioned critical phases of flight. The laptop or iPad is prohibited for use at night if there is no light barrier separating the pilot from the rear cabin. The laptop or iPad can be used by clinicians if approved by the Clinical Director and at the discretion of the pilot-in-command. The laptop or iPad are permitted during cruise flight under the following conditions:

- Clinicians will adhere to the General Operations Manual policy regarding Cell Phone and Portable Electronic Device (PED) Utilization.
- Only one clinician is permitted to use a computer in flight at any time. The other clinician will assist the pilot as needed with visual situational awareness.
- The laptop or iPad must be EMI tested and each device is approved based upon the manufacturer of the aircraft.
- There is a light barrier separating the pilot from the rear cabin and the barrier is sufficient enough to keep the computer light from interfering with the pilot's night vision or degrading the effectiveness of NVGs. If light interference is identified use of the laptop or iPad will be immediately discontinued.
- The laptop or iPad wireless router is maintained in airplane mode during liftoff, flight, and landing.

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44 License/Certification/Accreditation Requirements

44.1 Purpose

To define the responsibility of Air Methods Community Based Services (CBS) clinical teammates in obtaining and maintaining required licenses, certifications, and accreditations for skills maintenance and job performance as well as to establish minimum standards for verification of current mandatory certifications by clinical teammates to ensure clinical preparedness and competency.

44.2 Policy

- Clinicians must have all valid licenses, certifications, and accreditations required to perform their job duties as described in specific Air Methods CBS Job Descriptions. Clinical teammates are required to obtain and maintain the required licenses, certifications, and/or accreditations. Failure to comply with this requirement will result in disciplinary action and may result in termination.
- All clinicians are responsible for notifying HR and their Clinical Director if they have been notified of an active investigation or suspension from any state licensing agencies or accrediting body which may affect a required license or certification.
- Protean automatically generates notification emails when certifications are coming due for expiration. The Clinical Credentialing Team will provide email notification at the 30, 14, and 7 day mark for expiration. However, it will remain the responsibility of clinical teammates to maintain continuous licensure, certification, and/or accreditation without lapse. Grace periods specified by accrediting bodies will not be valid for Air Methods CBS clinical teammates. Extensions by an issuing authority or government entity for exigent circumstances will be considered.
- Evidence of current licensure and certifications will be maintained and validated in Protean or the credential will not be considered current.
 - Upon initial employee orientation, the Regional Clinical Education Manager and/or Clinical Director will review the new employee's certification cards for currency. If the new employee has any lapsed certifications, they cannot begin orientation. The clinical teammate is responsible for uploading copies of the credentials into Protean following the appropriate process. The Clinical Credentialing Team will be responsible for logging into Protean and validating the credentials.

- All clinical teammates are responsible for notifying HR and their Clinical Director if they have been suspended or excluded from any Federal Health Care Programs or Medicaid.
- All clinical teammates will, upon receipt of a new credential, upload it to Protean as a new certification for validation by the Clinical Credentialing Team. The copy of all credentials uploaded in Protean will be sourced directly from the issuing authority whenever possible (ex. proof of a valid nursing license will be printed directly from NURSYS; proof of a valid paramedic license will be printed directly from the state EMS website, etc.).
 - The Clinical Credentialing Team will re-verify the credential directly from the source whenever possible to ensure nothing on the copy uploaded to Protean has been altered.
 - For those credentials that cannot be verified directly from the source, the clinical teammate will upload the actual card or certificate to Protean. The clinical teammate will scan a copy of the card. Once the card has been uploaded, the Clinical Credentialing Team will validate the credential after examining the document provided to ensure nothing has been altered.
 - For a Certification card to be valid, it must come from its originating organization e.g., ACLS, PALS, BLS must come from the American Red Cross (ARC), American Heart Association (AHA), or American Safety and Health Institute (ASHI), and NRP from the American Academy of Pediatrics. Any third party certification cards that provide a test and subsequent certification “based on” standards such as the American Heart Association’s will not be acceptable. Examples of this would be cards from third party online vendors that do not result in the issuance of a card with the originating organization’s logo affixed.
 - Reach out to Clinical Credentialing for source verification.
- All courses must be approved as indicated in the CAMTS standards, such as BCLS, ACLS, PALS, etc. A complete listing of approved courses may be found in Appendix B of the CAMTS standards.
- Requests for HealthStream keys for BLS, ACLS, PALS and NRP will originate directly from the Clinical Educator 3-6 months prior to the expiration of the employee's current credential. The Clinical Educator will manage employee certifications and generate the key requests in accordance with the standard key issuance cycle that has been set by the education department.

- Clinicians are required to upload all credentials in Protean and validated by the Clinical Credentialing Team no less than seven (7) days before the expiration date. The seven-day countdown ends on the day before the printed expiration date on the credential. Clinicians are not permitted to shift on the day a certification expires. An example of the submission requirement is listed below. Using this example, the new credential must be uploaded in Protean before midnight on the 23rd. Failure to have a credential uploaded and validated in Protean seven (7) days prior to expiration will result in progressive disciplinary action for the clinical teammate.

FIGURE 44.1: Credentials Expiration Calendar



- Any Air Methods CBS clinical teammate who fails to maintain required licenses, certifications, and/or accreditations will be immediately removed from active duty without pay until such time as license, certification, and/or accreditation is reinstated. Clinical teammates will not be permitted to be scheduled on the day of their required credential expiration. Failure to achieve reinstatement within 30 days may result in termination. Exceptions may be made for extenuating circumstances such as military or medical leave of absence.

- Air Methods offers online recertification courses for BLS, ACLS, and PALS through the American Heart Association, American Red Cross, and NRP through the American College of Pediatrics. The online portion of these courses via Healthstream should be completed during flight shifts. All course requirements must be completed no later than 7-days from expiration (to include skills verifications to be completed at a base using the appropriate Red Cross manikins). **If a clinical teammate fails one of the above courses, they will be responsible for purchasing further keys to complete the online course.**
- All trauma course renewals will be completed via the on-line TPATC course and all clinical teammates are required to obtain and maintain this transport specific trauma certification, with the course fee paid by Air Methods. The course should be completed during scheduled flight shifts. If other trauma courses (TNCC, etc.) are obtained from another employer or from an outside company, Air Methods will not pay for time, travel, or fees associated with any outside trauma course, except where a provider is required to hold an additional certification as required by state or local EMS regulations. **TPATC is still required even if a teammate holds another trauma certification.**
- A Trauma certification (TPATC, TNCC, PHILS, ITLS, ATCN, ATLS) will be required before Indoc.
 - Online TPATC is the 1st choice if there are no state requirements for others; TPATC must be completed in Post Orientation (within 90-days) if the new hire has a valid trauma certification.
 - No Paramedic Audits of Trauma Courses will be accepted.
- If a candidate is hired and has a trauma certification other than TPATC, they have 90 days to obtain TPATC post-release from orientation (2nd Seat).
 - The teammate will be required to upload a 90-day acknowledgment form in Protean for tracking purposes during their 1st shift.
- If an applicant does not have a trauma certification and is selected for a position, Air Methods will provide a TPATC link at Air Methods cost and an offer made contingent upon completion of TPATC before INDOC.
 - If the candidate does not complete the required TPATC course, the job offer will be rescinded.
 - Proof of completion must be submitted to Clinical Leadership and the Credentialing Team seven (7) days before the start of INDOC.

- Air Methods will accept BLS, ACLS, PALS and pertinent trauma course renewals obtained through another employer or an outside company, however, Air Methods will not pay for time, travel, or fees associated with these courses. Additionally, the clinical teammate must comply with the requirement to have the credential uploaded and validated at least seven (7) days prior to expiration. AHA, ARC, and ASHI credentials must be able to be verified by the Clinical Credentialing Team directly from the issuing authority. Rosters showing course completion will not be accepted as temporary evidence of course completion. If a clinical teammate will not be able to meet the requirement to have the credential completed seven (7) days prior to expiration, the clinical teammate should plan to complete the course through the Air Methods Healthstream process.
- The following records will be maintained on all current and past employees (former employee records will be maintained in Protean under the "Inactive/Expired" tab):
 - State RN Licensure or APRN (advanced practice/nurse practitioner) license and state paramedic certificate.
 - All certification cards as required by the FN/FP job descriptions.
 - All initial and continuing education and training files.

TABLE 44.1 New Hire Nurse/Medic Credential Requirement Summary Chart

Nursing/Paramedic License	State License for Nurses (RN or PRN)/Paramedic Certification
BLS	BLS
ACLS	ACLS
PALS, PEPP or ENPC	PEPP will be converted to PALS once hired; PALS must be completed by the end of third-rider portion of orientation. ENPC will be converted to PALS once hired; PALS must be completed by the end of the third-rider portion of orientation
NRP	Must be completed by the end of the third-rider portion of orientation. Issued during orientation in Greenwood Village, Colorado Online access key - Healthstream
Trauma Course	A Trauma certification (TPATC, TNCC, PHTLS, ITLS, ATCN, ATLS) will be required before Indoc. TPATC will be required within 90 days of release from orientation.
FEMA Certification	IS100, IS200, IS700, IS800, IS5A or Hazmat Awareness Equivalent (Firefighter, Operations, Technician, or State Academy course)
Specialty Certification Paramedics	CCP-C or FP-C (FP-C/CCP-C - Incentive per the MOM policy)
Specialty Certification Nurses	CCRN, CEN, CTRN, and CFRN (CFRN/CTRN - Incentive per the MOM policy)

44.3 Education Requirements

Includes intubations, HPS (human patient simulation) lab classes, cadaver labs, and other assigned clinical classes, courses or requirements:

- Schedules for HPS and cadaver labs and any other educational classes will be communicated by the Clinical Educator to each clinical teammate via Air Methods email.
- Each clinical teammate is responsible for ensuring enrollment and completion of all required labs and courses; if an approved absence or leave of absence has been scheduled then the teammate should coordinate a make-up course/class with the Clinical Educator prior to the absence. This plan should be recorded and communicated with the clinical teammate via Air Methods email.

- If a clinical teammate does not attend a required course and was not on approved time off or a leave of absence then the clinical teammate will be removed from flight status and placed on unpaid leave. If a make-up course/class is available (make-up course/class availability is not guaranteed) it must be completed within 30 days, or the clinical teammate will be separated from employment.
- Any exceptions to this will require approval from the Vice President of Clinical Services.

44.4 Completing the Requirements

- Clinical teammates who have not completed a mandatory requirement to include uploading any required documentation into Protean by a scheduled deadline (to include on-line training, document acknowledgment, etc.) will be removed from flight duties and placed on unpaid administrative leave pending completion of this requirement.
- If a clinical teammate has not communicated his/her plan to complete the required tasks or has not completed the requirement within three business days of a scheduled flight shift, the Area Manager will coordinate with crew scheduling to backfill the teammate's applicable future shifts to avoid a business hardship (last-minute open shifts, overtime, out of service aircraft).
- Clinical teammates who complete required tasks/training in close proximity to the deadline may be required to alter their scheduled flight shifts if local schedule changes were made to ensure continuity of in-service operations.
- Clinical teammates who do not complete requirements by the assigned deadlines will enter the progressive disciplinary process.

44.5 Specialty Certification, Support and Guidance

Air Methods is committed to supporting the CAMTS requirements regarding advanced certifications for Flight Nurses and Flight Paramedics. Air Methods must also remain flexible considering operational requirements and local, county, or state requirements. As regulations have evolved and continue to evolve over time, there exists different advanced certification requirements based on a clinician's date of hire, job role, and base assignment. Please see the below matrix for advanced certification requirements based on job role and date of hire. Local regulations that require clinicians to obtain a specialty certification at a faster pace than is required by Air Methods will supersede the Air Methods requirements.

TABLE 44.2 Flight Nurses and Paramedics

<u>Flight Nurses</u>	
Date of Hire (Prior to 10/1/2021)	Date of Hire (10/1/2021 or later)
Acceptable Certification - CEN, CFRN, CCRN (adult), CTRN	Acceptable Certification - CFRN, CTRN
Required to obtain an advanced certification within two (2) years of hire	Required to obtain an advanced certification within two (2) years of hire
For All Flight Nurses: Effective October 1, 2021, newly obtained RN advanced certifications must be either a CFRN or CTRN per CAMTS 12th edition standards. RNs who hold a CEN or CCRN (adult) obtained prior to October 1, 2021 will be compliant until that certification expires. Newly acquired specialty certifications obtained after October 1, 2021 must include at minimum either the CFRN or CTRN.	
For nurses who currently do not have advanced certifications, if the CEN or CCRN (adult) is obtained prior to October 1, 2021, the nurse will remain compliant until that certification expires or the nurse reaches his or her 2-year date of hire anniversary, whichever is later, at which point a CFRN or CTRN must be obtained.	
<u>Flight Paramedics</u>	
Acceptable certifications for those individuals that are employed as Flight Paramedics with Air Methods are one of the following: FP-C or CCP-C.	

Air Methods is committed to assisting our clinical employees with successfully obtaining a specialty certification.

44.5.1 Procedure

- Any flight nurse or any flight paramedic who does not have a transport-specific specialty certification at hire will be required to complete the Specialty Cert Acknowledgment Form. (Accepted transport-specific certifications are CFRN or CTRN for nurses and FP-C or CCP-C for paramedics). This form will be required to be uploaded to the “Specialty Certification Acknowledgment Form” category as a “placeholder” for the certification. Instructions on the form provide guidance regarding the expiration date to enter in Protean.
- Flight nurses and Flight paramedics who do not have a transport-specific specialty certification at hire will be required to make their first attempt at a transport-specific specialty certification within one year from their hire date.
- If the clinical teammate does not pass the exam, they must re-test during the next six months (on or before 18 months after hire).
- If the clinical teammate has not successfully passed the exam by their 2-year anniversary they will be terminated.
- Local requirements that require clinicians to obtain a specialty certification at a faster pace than is required by Air Methods will supersede the above Air Methods requirements.

44.5.1.1 Support

- Clinical teammates will be reminded through Protean notifications that they must sit for and successfully pass one of the above specialty certifications specific to their job description, as stated above.
- An online review course is provided free of charge to all clinical teammates.
- Purchasing Keys: (Completed by Clinical Educator)
 - Employees must request keys through the Regional Clinical Education Manager via email.
 - Clinical Educators will coordinate the purchase of course keys.
 - Employees must allow a minimum of five business days to receive the key, once requested.
- Modules can be reviewed as many times as desired for up to one year after initiating the modules and then will become inactive. Please note Air Methods requires the entire course be completed within 90 days once the key has been provided.
- The Clinical Educators will review the completion reports sent by the vendor each month.

- If clinical teammates choose to take an external review course for CEN, CCRN, CFRN, CTRN, FPC, or CCP-C, they must complete an Education Conference Application Form and meet those requirements for discretionary approval by the Clinical Director.
- If, within the first two years of employment (or individually established timeline for completion), a clinical teammate is unsuccessful in passing the exam, they are highly encouraged to engage the assistance of their Clinical Educator. The Clinical Educator will assist in identifying and addressing their areas of weakness and will assist in developing an education plan that addresses those concerns and prepares the employee for completion of the identified specialty certification exam.

NOTE In states where there are legal requirements that differ from the 2-year CAMTS expectation, Air Methods will honor the expectations of the state.

- Clinical teammates who already have a specialty certification but would like the CEU hours may be given the recertification key by the Clinical Educator. This CEU certification key will only be given once per 4-year period or period of certification validity.

44.6 Professional Certification Financial Support

To provide guidelines for financial support for industry-specific professional certification for Air Methods Community Based Services (CBS) clinical personnel.

The following are the requirements and process for reimbursement of a certification exam fee:

- Initial certification: A reimbursement for the exam fee and any mileage on personally owned vehicles to and from the testing will occur following successful completion of the exam and when proper documentation is presented to the Clinical Director and Area Manager. Air Methods will reimburse an initial certification for CFRN, CTRN, FP-C, and CCP-C. Reimbursement will be made via an expense report.
- Renewal fees for re-certification will be reimbursed for the CFRN or CTRN for Nurses and FP-C or CCP-C for Paramedics. Renewal fees for other specialty certifications are not reimbursable.
- All fees will be reimbursed at the professional association membership rates (ASTNA, AAMS, IAFFCCP, ENA, and AACN); fees that exceed member rates will not be reimbursed. The cost of membership for any professional association is the sole obligation of the clinician and will not be reimbursed by Air Methods.

44.7 Reimbursement for Licenses/Credentials

Due to the nature of HEMS, flight clinicians frequently are required to hold licenses or credentials that may be additional to the standard requirements for their discipline. This may include additional state licensure, regional required credentials, or other specialty certifications. Air Methods offers financial support for these additional licenses/credentials that are required. The following offers guidance on credentials eligible and ineligible for reimbursement.

44.7.1 Licenses/Credentials NOT Eligible for Reimbursement

- Primary state discipline-specific license.
- External Advanced Certification Review Courses (Air Methods offers an internal specialty care course at no cost to the clinicians).
- Renewal courses offered internally such as ACLS, BLS, NRP, PALS, TPATC, etc.
 - **EXCEPTION:** Reimbursement will be provided if the state or other regulatory body requires a different course such as PHTLS or BLS, ACLS, PALS through the American Red Cross is not accepted. Obtain approval from your Clinical Educator, Clinical Director, or Area Manager prior to registering for a course.
 - Obtain the necessary approval through the Credential/License Reimbursement Exception form in Smartsheet.

44.7.2 Licenses/Credentials Eligible for Reimbursement

- Additional licenses that are required for the clinician's assigned base or region are identified.
- Flight Nurses who also have their paramedic credential will be reimbursed for the renewal of this license/credential.

- Licenses/Credentials that are required for staffing outside of the clinician's home base/region.
 - Credentials that are obtained to work outside of the clinician's home base or region must be approved prior to the initial renewal of the credential. Approval must include both the Area Manager and Clinical Director in the region that will benefit from the additional licensure and the clinician's assigned Area Manager.
 - It is the clinician's responsibility to submit the Credential/License Reimbursement Exception for approval.
 - If approved, the fees will be coded to the benefiting regional GL Code provided by the approving Area Manager.
 - Obtain the necessary approval through the Credential/License Reimbursement Exception form submission.

45 Long Range Transports

45.1 Purpose

A long-range transport is defined as one in which the patient is in the aircraft for three hours or greater, on-way, and may be associated with an overnight stay.

45.2 Policy/Procedure

- Clinicians will ensure there is a means to contact AirCom/Communication center during patient transport using the aircraft sat-phone or on-board radio. Clinicians will use this means of communication to provide patient updates when appropriate and seek medical control consultation.
- If the patient's condition deteriorates during transport clinicians will follow the Patient Care Guidelines and seek medical control consultation. When necessary, clinicians will coordinate with the pilot and communication center to identify the closest appropriate location to land and transport the patient to a hospital for stabilization.
- Clinicians are required to complete the PACO tool in accordance with policy to ensure an adequate oxygen supply during patient transport. If necessary, clinicians will coordinate with aviation, maintenance, and communication specialists to identify additional oxygen supply resources. It may be necessary to carry additional oxygen fittings, connections and or tools for servicing the aircraft oxygen supply.
- Clinicians will coordinate with the communication specialists to ensure the ground ambulance providing transport has an adequate oxygen supply for patient transport.
- With patient transport exceeding two-hours, clinicians are required to perform the following actions to prevent and document pressure ulcers.
 - Assess and document presence of pressure ulcers prior to, during and after transport.
 - Clinician will implement measure to reduce pressure ulcers during patient transport (if not contraindicated reposition the patient to reduce pressure points, ensure the stretcher and securing belts are not generating pressure points).

- During refueling procedures with the patient on-board, clinicians will coordinate with the pilot to verify safety procedures are followed to include adequate cabin light, oxygen supply and electrical power. Clinicians will monitor the aircraft cabin temperature and take action to mitigate exposure to excessive heat or cold temperatures. Clinicians will coordinate with the pilot to conduct fire-watch procedures during aircraft fueling.
- Clinicians will ensure all medical devices have adequate back-up battery supply are readily accessible in case of a power failure (each medical device is required to have an electrical plug in power supply. The cardiac monitor and ventilator are required to have one battery in the device and a minimum of one spare battery in the patient cabin. Specialty devices such as an IABP or other cardiac assist devices are required to have a backup power supply in accordance with the manufactures recommendations).
- Guideline for transport resulting in an overnight stay.
 - Flight team will coordinate their overnight stay with their Area Manager.
 - Flight team will coordinate with each other to establish a meeting point and time for updates and/or return to the aircraft.
 - It is recommended flight team members do not venture out alone.
 - It is recommended flight team members do not participate in high-risk activities; example bungee jumping or riding scooters.
 - The aircraft must be in a secure location when being left unattended.
 - For instances when temperature is a concern overnight, measure must be taken to protect equipment on the aircraft for excessive cold or heat. Electronics, including ECG monitor, Vent, IV pumps, etc., must be kept within manufacturers acceptable temperature range.
 - Medications, including controlled substances should be removed from the aircraft and kept by the clinical crew. The controlled substance Otterbox(es) must stay in possession of the clinical crew. If a safe is available in the clinician's hotel room, that safe may be used to store controlled substance. If a safe is unavailable in the hotel room, and the clinician's need to leave the room to attend to personal matters, the controlled substances should be stored in the clinician's flight suit and the flight suit should be hidden from view. When leaving the hotel room, care should be taken to assure all entry points are well secured and locked.

46 Mandatory Reporting Requirements

46.1 Purpose

To provide guidelines for reportable incidents to appropriate state and local agencies. To ensure prompt attention and proper follow through of all unusual occurrences that occur within the clinical division.

46.2 Policy

- All unusual occurrences and errors, as defined in the Patient Safety Action Program, will be reported in Protean within 24 hours. Protean will automatically forward PSAP report notifications to the appropriate level of leadership for review and proper handling.
- The Clinical Director will complete a thorough investigation and document resolution in Protean within seven days for a Priority 1 event and within 30 days for a Priority 2 event.
- Each program will adhere to state and local laws/mandatory reporting requirements. All reports made by a clinician to any state must be documented in the patient care report, and shall include the report number provided by the agency. These reporting requirements may include:
 - Suspected child abuse or maltreatment
 - Suspected domestic violence
 - Suspected elderly abuse
 - Patient abuse/assault
 - Any other reportable event as dictated by regulations
- All disclosures of Patient Information to third parties shall be in accordance with HIPAA and state privacy laws. Please refer to the Company's HIPAA manual for further guidance. See also, Medical Operations Manual policy Section 50 Medical Records, Legal Requests, HIPAA, and Confidentiality.
- A first report of injury form is to be completed as soon as possible for all work-related injuries and sent to the Risk Management Department per Company policy. Area Manager should also be notified.

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47 Medical Control

47.1 Purpose

To establish criteria and standards for Medical Control for Air Methods Community Based Services programs.

47.2 Policy

- Off-line Medical Control refers to the general concept of control/authorization not related to a specific patient transport but residing in written materials. Off-line medical control resides with the program Medical Director. The Medical Director has specifically authorized the Air Methods clinical crew to perform procedures and administer medications based on written policies and procedures. The clinicians are functioning under the Medical Director's oversight within their Patient Care Guidelines.
- Patient care guidelines are intended to optimize patient care, are informed by a systematic review of evidence and an assessment of the benefits and harms of alternative care options. All deviations from the Patient Care Guidelines require contact with the medical control physician to review the alternate plan of care, and receive guidance regarding the patient being transported.
- Air Methods programs should define what parameters and disease processes require medical control input during patient transport. This must be prescribed by the program Medical Director.
- Online Medical Control (OLMC) refers to an authorization related to a specific patient transport. Because of the breadth of the agency's standards of care, OLMC is required only in specified situations. The Air Methods program Medical Director or his/her designee will be the resource for on-line medical control.

Clinical Director and the program Medical Director will identify physician(s) who will assume responsibility for medical control consultation. Physician(s) functioning as medical control are required to meet the following criteria:

- Board certified in Emergency Medicine or their specialty equivalent.
- Receive documented (roster reflecting date and names) initial and annual training for the following criteria.
 - Transport program overview to include the clinician requirements, clinical training, and scope of care.
 - Transport program aircraft capabilities and mission limitations.
 - Transport program specialty services capabilities (IABP, ECMO).
 - Transport request profiles; scene versus inter-facility.
 - A general overview of the Air Methods Patient Care Guidelines to include access for reference when necessary.
 - Overview of altitude physiology and stresses of flight as applicable to the program mission profile.
 - Review local, state, and federal health care compliance regulations applicable to patient care.
 - Termination of resuscitation in the field role and responsibilities.
 - Review the process for requesting a patient care transport (all transport requests are generated through AirCom/Communication Center). We do not automatically respond to an event or disaster.
 - Review appropriate utilization for air medical transport.

Medical Control physician(s) are required to be available 24/7/365.

- Clinical Director will ensure AirCom/Communication Center and program clinicians are provided information regarding designated medical control physician(s) and schedule of availability.

Clinicians are required to follow the Medical Control Consultation policy within the Patient Care Guidelines when consulting with a physician.

It is strongly encouraged, however, that open dialogue and consultation with medical control be established whenever:

- The current plan of care is not consistent with the Patient Care Guidelines.
- The clinicians require direction in formulating an appropriate plan of care is required.

- Any unusual or unforeseen situation relating to patient care or inter-agency relations during transfer of care.
- Clinician's disagreement in plan of care.
- As dictated within the Patient Care Guidelines.

If contact with the program Medical Director or designee is not possible, one of the following options may be utilized:

- Referring physician: Clinician consults or receives physician order(s) from the referring physician. Clinicians are required to document physician order(s) in accordance with the Clinical Documentation Guidance Manual, Chapter 28.
- Receiving physician: Clinicians may contact the receiving physician during transport to obtain orders.
- If clinicians are unable to reach Medical Control, they should contact the Clinical Director who can facilitate reaching a physician for consultation.
- If the receiving or referring physician cannot be contacted refer to the Patient Care Guidelines.
- The Medical Director for the program will be advised post flight.

47.3 CAMTS Standards for Medical Direction

47.3.1 02.01.00 Medical Direction

See the Medical Direction section of the CAMTS standards posted on the CAMTS website.

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48 Medical Control Orientation

48.1 Purpose

This policy is intended to ensure an appropriate understanding by Medical Control physicians of the capabilities and expectations of clinical flight team members. This will aid in the provision of optimal patient care by the flight team.

- Physicians who are to assume the role of medical control physicians will be introduced to the Air Methods clinical system by the Clinical Director. Policy, protocols, and procedure documentation will be provided.
- Designated medical control physicians must be knowledgeable of the transport program protocols and ensure proper medical control during transport as requested for all patient types within the given scope of care of the flight program. This physician should be an appropriately designated physician or the patient's receiving attending physician.
- Each program will have a record of training, both initial and recurrent, for medical control physicians and Medical Directors to ensure compliance with CAMTS standards.

48.2 Guidelines

Physicians who agree to participate as regular medical control physicians will review and demonstrate understanding of the following items:

- Phone numbers/pagers for emergency contact of crewmembers.
- Appropriate dispatch number for flight initiation.
- Who may initiate flight response.
- COBRA regulations with regard to helicopter response.
- Dispatch: location and qualifications.
- Service area.
- Types of requests: weather check, standby, definite need.
- Types of transports: Interfacility, on-scene, specialty.
- Flight interruptions: aborts, delays.

- Diversions: more than one simultaneous request.
- Options for alternative transport means if simultaneous requests.
- Diversion considerations: weather, fuel, pilot time out, etc.
- Turnaround times.
- Communications: radio, cell phone, pager.
- Physician Orders: requested, received, documentation/signature, medical control release.
- Termination of resuscitation in the field.
- Flight Nurse duties/capabilities.
- Flight Paramedic duties/capabilities.
- Helicopter configuration.
- Flight Crew Capabilities/care limitations in flight.
- Infection control.
- Stress recognition and management.
- Medications/equipment on board.
- Clinical crew standards of care.

Physicians who act as medical control only occasionally through their position as receiving physician (such as NICU/PICU/high risk OB receiving physicians) will be educated as to the crew capabilities and the issues relative to their patient population during air transport.

Medical Control Orientation will be documented and retained using the Medical Director/ Medical Control Checklist found in the appendices.

49 Medical Operations Manual Committee and Policy Development

49.1 Purpose

The committee is responsible for reviewing and developing proposed policies for the Medical Operations Manual and assure regular communication regarding policy revisions.

49.2 Policy

- Committee members will participate in the development of new policy and complete a review of the established policy to ensure it meets current practices and/or regulatory requirements.
- The committee will meet every other month with January identified as the starting point for each year.
- The committee will release policy updates in April and October of each year. Policy revisions added during the bi-annual MOM review will be summarized in the "Changes Page" of the Medical Operations Manual.
- The committee will maintain records reflecting policy development and revisions.
- The committee members will consist of the following participants.
 - Committee Chair
 - Vice President of Clinical Services
 - One Clinical Director
 - Director of Clinical Compliance
 - One Patient Safety Representative
 - One Clinical Education Representative
 - Two Area Manager Representatives
 - Two Clinician Representatives
- The committee members will collaborate with the following subject matter experts.
 - Aviation Representative
 - AirCom Representative
 - Air Methods IT Representative
 - Air Methods Human Resource Representative

- Air Methods Legal Representative
 - Air Methods Occupational Health Representative
 - Air Methods Supply Chain Representative
 - Air Methods Clinical Research
 - Air Methods Maintenance Representative
 - Business Account Executive Representative
 - Clinical Scheduling Representative
 - Patient Billing Services Representative
 - Physician Advisory Board Representative
 - Back-to-Basics Representative
- All regional and program specific policies associated with clinical teammates, patient care to include all phases/modes of transport and clinical operations will require review and approval from the Medical Operations Manual committee. Possible regional and program specific policies include the following recommendations, however, not limited to the following:
 - State or local clinical credentialing requirements
 - State or local ambulance regulations
 - State or local clinical education regulations
 - State or local clinical mandated reporting
 - State or local patient care regulations impacting a clinician's clinical practice
 - Blood bank policy
 - Extreme weather conditions impacting patient transport and/or a clinician's performance and/or safety.
 - Patient triage and mode of transport guidelines
 - Each region has a folder for locating these policies on FlightDeck > Resources > Corporate Publications > Medical Services Publications > MOM Program Specific Policies.
 - Suggested revisions or additions to the MOM are welcomed from all disciplines and titles at Air Methods. To submit a proposed revision or new policy for inclusion into the MOM, please access this link and fill in all required details: Medical Operations Manual Policy Recommendations Form. A link to the MOM Policy Recommendations Form will also be available in the "External Links" icon on each region's Protean home page.

50 Medical Records, Legal Requests, HIPAA, and Confidentiality

50.1 Purpose

Air Methods, LLC often receives subpoenas or other legal requests and communication due to the nature of its business. The purpose of this policy is to ensure compliance with: (a) the Privacy Standards of the Health Insurance Portability and Accountability Act (HIPAA), and (b) other federal and state laws and regulations governing the disclosure of confidential information.

50.2 Policy

- All legal notices, requests, and subpoenas for medical records, billing statements, or records should immediately be forwarded to PBSrecords-legal@airmethods.com. PBS will log the request and forward to the appropriate department.
- All subpoenas or legal notices/requests NOT pertaining to medical records should immediately be forwarded to LegalNotices@airmethods.com. Examples of this would include: requests for audio recordings, requests/subpoena for teammates to interview/deposition, and request/subpoena for a teammate to appear at trial.
- Air Methods clinicians may not discuss company-related business, including patient care, with outside legal representatives and law enforcement unless approved by the Patient Safety Team.
 - **Exception:** All clinicians are bound by applicable mandated reporter laws in their state(s). Clinicians must comply with such laws and, where applicable, file reports of suspected child and/or elder abuse. All reports made by a clinician to any state must be documented in the patient care report, and shall include the report number provided by the agency.
- Under no circumstances should medical records, patient information, or Protected Health Information, as defined by HIPAA, be disclosed or released, either verbally or in writing, to any third parties/agencies by Air Methods clinicians or supervisors. The Custodian of Records at PBS must approve and release all records according to Air Methods policy.
- The Patient Safety Team will work directly with requesting agencies, the Air Methods legal department, and appropriate leadership team members to coordinate and handle all legal requests.

- All Air Methods clinicians will be accompanied by a manager for depositions or court appearances. The Patient Safety Team will coordinate all legal interviews, depositions, or court appearances with respective program administration.
- Clinicians requested to function in an expert witness capacity are required to contact the Patient Safety Team prior to accepting the request, in order to ensure there is no conflict of interest or other reason that would prevent the clinician from serving in such a capacity. Clinicians who are approved to serve as an expert witness will be doing so in their own capacity, and not as an employee of Air Methods.

50.3 Medical Records Requests

50.3.1 Urgent Requests

- If the request is urgent and during business hours (0800-1700 Pacific Time, Monday-Friday), the requester will be referred to the PBS Medical Records Department. The requester may call 877-800-5668 for immediate assistance.
- If the request is urgent (needed prior to the next business day) and after business hours (0800-1700 Pacific Time, Monday-Friday) the teammate receiving the request will contact the Clinical Director or the Clinical AOC for assistance. Urgent requests are narrowly defined for only two requester types: Transplant Services and Coroners/Medical Examiners as both of these are permitted under HIPAA without patient authorization. Others requesting an urgent response will be immediately referred to management staff for review.
- The Clinical Director/AOC must validate the requester is who they represent themselves to be and the organization they represent. The request needs to include four components:
 1. Who they work for, their title, and role.
 2. What record(s) do you need including patient name, approximate date of transport, and type of records required.
 3. Why they need the records (required by HIPAA to document the need).
 4. What is the best way to get them the record.

- The Clinical Director/AOC will ask the requester to fax or email the request directly to the Clinical Director/AOC. When the Clinical Director/AOC has received the request, they will review it to ensure it meets the above requirements.
 - If all the above information is present and meets the “urgent” request categories, the Clinical Director/AOC is authorized to release only the minimal necessary (exactly what is stated in the request).
 - Once the information is released to the third party, the Clinical Director/AOC will send an email to DIS@airmethods.com with the following information:
 - Subject Line: After-Hours Medical Records Release.
 - All four of the components above.
 - The official request (as either an attachment or forward of email) from the requester.
 - DIS will enter the information into the billing system for the purposes of tracking.
 - If the request is missing any of the verification information above, the Clinical Director/AOC must not release the information requested until the missing data is received and verified.
 - If the information cannot be verified or is not all present and no records were released, the Clinical Director/AOC will send an email to DIS@airmethods.com with the following information:
 - Subject Line: After-Hours Medical Records Denied.
 - Any of the above components that were received with the request.
 - The official request (as either an attachment or forward of email), if received.
 - The reason for the denial of records release.
 - DIS will enter the information into the billing system for the purposes of tracking.
- If the request includes these components, a HIPAA compliant request, and the record has been created and has been locked in Air Methods designated patient care charting system, the record may be released.
- If the Clinical Director/AOC is faxing the record to the requester, an Air Methods HIPAA compliant fax cover sheet must be utilized.
- If the Clinical Director/AOC is emailing the record to the requester, the email must contain a HIPAA disclosure statement and the word “Confidential” must be in the subject line of the email. This ensures that Outlook encrypts the email.

50.3.2 Non-Urgent/Routine Requests

- Requests for copies of medical records or additional records must be immediately forwarded to PBS at PBSrecords-legal@airmethods.com.
- For verbal requests, the requester should be directed to contact the Air Methods medical records department via one of the following methods:
 - Direct Phone: (909) 915-2930.
 - Main Office Phone: (877) 800-5668; option 3.
 - Fax: (402) 952-2413.
 - Email: PBSrecords-legal@airmethods.com.
- All requests for medical records will be referred to PBS and released in compliance with HIPAA and other applicable law. Absolutely no records may be provided or reviewed at the base level.

50.3.3 Employment Information Requests

- Requests for employee information are to be directed to Human Resources. Personal information such as verification of employment, contact information, or work schedules may not be released from anyone other than Human Resources. Requesters should be directed to call 1-800-367-5690 or to submit their request via www.theworknumber.com using code: 20597.

50.4 HIPAA and Protected Health Information

For additional information about Air Methods HIPAA policies and procedures, refer to the [HIPAA Manual](#) located on FlightDeck > Resources > Corp Pubs > Compliance Policies and Procedures.

51 Medication Administration and Storage Practices

51.1 General Guidance for Preparation and Administration of Medication

- 'Medication' is defined here as any drug/fluid or biologic agent ordered for administration by Air Methods clinicians during the course of clinical care.
- Medication must be prepared and administered in accordance with applicable federal and state laws. Clinicians are responsible for knowing and adhering to their local and state regulations and standards of practice where these vary from standard Air Methods guidance, including applicable licensing requirements.
- Medication may be prepared and administered on the orders of non-Air Methods practitioners only if such practitioners are acting in accordance with federal or state law, including scope of practice laws, facility policies, and medical staff bylaws, rules, or regulations. Where there is any question if orders are in conflict with these, Air Methods clinicians shall coordinate consultation with Air Methods medical direction and the non-Air Methods practitioner to determine the safest and most appropriate plan for patient care.
- Medication will be prepared and administered in accordance with policy, clinical guidance and accepted standards of practice, and/or manufacturer instructions for use (IFU) regarding:
 - Safe and aseptic handling in the air medical transport environment.
 - Knowledge of the indication, side effects, drug interactions, compatibility, and dose limits of administered medications.
 - Equipment, devices, special procedures, and/or techniques required for medication administration.

- Medication will be prepared and administered based upon the orders for treatment in the PCGs and MOM, as approved by Air Methods supervising physicians, who are considered to be the ordering practitioners for treatment, and these are considered “orders for administration” during the course of clinical care. Medication may be prepared or administered outside of this guidance according to specific orders for treatment given by referral or receiving physicians, in coordination as necessary with Air Methods physician medical directors. If medication is required outside of the parameters prescribed by the PCGs and MOM, it is the responsibility of the clinician to contact their medical director for orders to administer the medication.
- Medication shall be stored securely throughout the continuum of care, from receipt through administration and disposal. Secure storage shall ensure that medication is only accessible to those personnel who have a compliant business need for its access and use and will be conducted in accordance with all applicable local, state, and federal regulations. Medication storage must allow for protection from extreme temperature changes if the environment deems it necessary, in accordance with the standard provided by the Commission on Accreditation of Medical Transport Systems (CAMTS) and the manufacturer’s requirements for safe storage of the medication.
- Hand hygiene will be performed prior to medication handling in accordance with CAMTS standards and the recommendations provided by the Centers for Disease Control and Prevention.
- Air Methods promotes a culture in which it is not only acceptable, but also strongly encouraged, for clinicians to bring to the attention of the supervising practitioners caring for the patient and/or the medical director any concerns they have about medication selection, preparation, or administration. Any questions about medications are expected to be resolved promptly, whether they arise prior to the preparation, dispensing, or administration of the medication.

51.2 Basic Safety Practices for Medication Administration

- Right patient
 - The patient must be identified by full name and date of birth prior to transport from a sending facility; patients should be identified to the extent possible in the air medical transport environment when responding to a scene request.
 - Any medications prepared for transport and/or continued from the initial clinical team during transport must be labeled and confirmed to be intended for that patient.

- Right medication
 - Medication selection and administration must occur in compliance with all applicable federal, state, and local laws, as well as with Air Methods guidance around medication compliance and diversion protection. No controlled substances may be accepted from referring healthcare personnel except as expressly permitted under Medical Control guidance, MOM/Patient Care Guidelines direction for continuous infusions.
- Right dose
 - The dosage of the medication to be delivered to the patient must match the dose intended for the patient and their condition, and the clinician must ensure that the dose does not reflect an unsafe dosage level or application.
 - Dosage based upon a patient weight shall be assessed for safety based upon the patient's weight in metric units. The patient's weight shall be documented in metric units in the patient care record (PCR).
 - When available, medication infusions shall be delivered via a programmable infusion pump utilizing dose error-reduction systems. Medication dosage may be calculated utilizing internet-based resources and crosschecked for accuracy in dose and volume against the ranges provided in the Air Methods PCGs.
- Right route
 - The route of administration should be the most appropriate one for that particular medication and patient.
- Right time
 - The timing of administration must be assessed to ensure adherence to the Air Methods PCGs, and that this is a safe frequency for this patient.
- Right action
 - The medication selected for use must be selected for the appropriate reason/indication, with clinical signs/symptoms or diagnosis documented to support the rationale for selection.

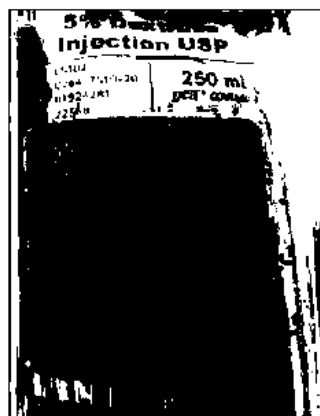
- Right form
 - The medication must be prepared in the most appropriate form, to include appropriate dilution and concentration, labeling, and delivery device.
 - Labels must be placed upon any delivery device (syringe, infusion bag, etc.) which is prepared and provided to another clinician for use. The label must include the drug name, concentration, and volume.
 - A label must be placed on any intravenous solution other than 0.9% normal saline to alert teammates to double-check the solution, concentration, and compatibility (e.g., hypertonic saline solution, fluids containing dextrose, medication admixtures, etc.). One example of a High Alert Label:

FIGURE 51.1: High Alert Label

HIGH ALERT

- Example of best practice includes using a bright medication label with clearly written medication information.

FIGURE 51.2: Bright Medication Label



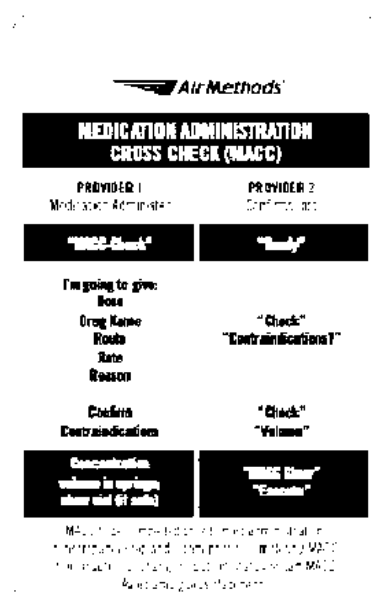
- Right response
 - The effect of the medication upon the patient must be assessed to ensure that there is no adverse effect and to determine if the medication met the patient's treatment needs.
 - Minimum monitoring for patients receiving opioids must include, at a minimum: vital signs, pain level, respiratory status, and sedation level.
 - Patients receiving paralytic medications must be assessed to determine that they are adequately sedated throughout the continuum of care, and that their response to the medications reflects sufficient sedation.
- Right communications
 - Documentation: the indication, preparation, administration, and response to the medication must be documented in the PCR as prescribed in the PCGs and MOM. Documentation must clearly reflect which clinician provided each medication.
 - Hand off: all medications administered during the course of care will be communicated to the receiving healthcare team.
 - Compliance: all medications shall be disposed of in a manner consistent with Air Methods policy on controlled substances and medication waste, and these shall be documented in the patient care record (PCR).
 - Safety reporting: any near-miss safety events, actual patient safety events, or deviations from the Air Methods PCGs or MOM concerning medication preparation or administration will be documented in a Patient Safety Action Report (PSAP) in accordance with Air Methods Safety Management System (SMS) and MOM.

51.3 Medication Administration Cross Check (MACC or “MACC Check”)

- The MACC is a standardized safety process designed to ensure that all medication prepared and administered during the course of clinical care is double-checked by both clinicians responsible for patient care. This process applies to both medications initiated by Air Methods clinicians as well as those received from a referring provider. Medications provided for transport or currently infusing when an Air Methods crew receives a patient for transport shall all be checked for safety with this process.
- The standardized script is as follows; Provider 1 is the person who will administer the medication and Provider 2 is the person who is acting to confirm the medication information prior to administration:
 - Provider 1: “MACC check.”
 - Notifies partner that they are ready for a MACC.
 - Provider 2: “Ready.”
 - Confirms that they are ready to engage in a MACC with their partner.
 - Provider 1: “I’m going to give *drug/dose/route/rate/reason*.”
 - Provides all necessary information to support safe medication administration.
 - Provider 2: “Check. Contraindications?”
 - Confirms the accuracy of the information and the rationale for administration and asks if there are any contraindication in this patient.
 - Provider 1: *confirms any applicable contraindications for the patient to receive the selected medication.*
 - Provider 2: “Check. Volume?”
 - Confirms that they understand and agree with the decisions and information provided.
 - Asks to see the vial, as applicable, and the prepared medication.
 - Provider 1: *shows the original vial with the name/volume/concentration available, and shows the delivery device (syringe, bag) and its volume and concentration, as applicable. No verbal cues are provided at this time.*
 - Provider 2: *reviews the information, confirms accuracy, and provides assent for medication administration: “MACC clear. Execute.”*

- The MACC shall be completed before any medication administration. Any discrepancy or question shall stop the process immediately and the issue shall be resolved prior to restarting the MACC and administering any medication. If there is any interruption or change in patient status, the MACC shall be restarted.
- Clear, unambiguous language shall be used by both clinicians during the check, including the review of any vials or packaging.
 - Example:*
 - Not: "Syringe of 50. I'll give 1."
 - Instead: "I have fentanyl 50 mcg in 2 mL in this syringe. I will give 1 mL, for a total of 25 mcg right now."
- If the clinician is alone in the patient care compartment and must administer a medication without a second clinician present to perform a mutual crosscheck, the single clinician must utilize the steps of the MACC with the intention to check all of the necessary elements prior to medication administration. If a second clinician of any other licensure level becomes available, it is acceptable to crosscheck the medication with them prior to administration.

FIGURE 51.3: Medication Administration Cross Check (MACC)



51.4 Medication Safety during Supply Concerns

51.4.1 FDA Drug Shortages

- A link to current FDA Drug Shortages can be found on the Protean homepage by clicking on “Clinical Operations” on the right side of the page under “News.”
- Clinical Directors and Clinical Base Leads should monitor the link frequently to determine if there will be any impact to ordering of medications that could result in a shortage at a particular base.
- Shortages will be managed on a case-by-case basis and may result in the need to transfer medications between bases or substitute a medication with an alternative medication.
- No medication may be transferred between bases without the permission of the Clinical Director and the Clinical Compliance Director.
- Substitutions with an alternative medication will be determined by the Clinical Compliance Director in conjunction with the Physician Advisory Board.
- Should it become necessary to substitute a medication with an alternative medication, the Clinical Compliance Director will obtain any appropriate guidance for indications, use, dosing, etc., from the Physician Advisory Board and will work with the Education Team to develop any required training prior to placing the medication “in-service.”
- The Clinical Compliance Director will notify the Clinical Directors and Clinical Base Leads of any changes to formulary resulting in the need to add alternative medications.
- The Clinical Base Leads will order and stock the alternative medication as required and will ensure any necessary training is completed by the clinicians at their assigned base prior to placing the alternative medication “in-service.”
- When the drug shortage has been resolved, the Clinical Compliance Director will notify the Clinical Directors and Clinical Base Leads of the resolution and whether the alternative medication needs to be removed from service.

51.4.2 Extended Use Dates for Medication in Short Supply

- Occasionally, the FDA will provide notice of certain medications in short supply that have been approved for extended use. A link to the Extended Use medications and their corresponding extended expiration dates can be found on the Protean homepage by click on “Clinical Operations” on the right side of the page under “News.”

- Clinical Base Leads must obtain approval from the Clinical Director and the Clinical Compliance Director prior to extending expiration dates on any medication.
- Extended Use Guidance is supported by applicable guidance provided by the U.S. Food and Drug Administration (FDA) with discretion and approval provided by Air Methods Clinical Leadership. Clinical Directors and the Clinical Compliance Director will confirm medication, lot numbers, and expiration dates prior to providing approval.
- If approval is provided to extend an expiration date of a medication, the Clinical Base Lead will be responsible for ensuring the extended expiration date is clearly marked on the packaging of the medication and the corresponding FDA notice that include the lot number and extended expiration date is printed and included in the clipboard on the aircraft, should the aircraft be inspected for any reason.

References:

CMS Transmittal 116, June 6, 2014: section clarifying 42 CFR 482.23(c).

2022-2023 ISMP Targeted Medication Safety Best Practices for Hospitals.

CAMTS Standards, 11th Edition.

United States Food & Drug Administration: [Drug Shortages](#). Accessed 5/10/2024

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52 Meeting Commitments

52.1 Purpose

To establish guidelines for attendance at meetings.

52.2 Policy

- Clinicians must attend 80% of the mandatory scheduled clinical meetings for their base, area, and region.
- Clinicians are required to notify the Clinical Director no less than four hours prior to the meeting start time if they are unable to attend.
- If a clinician misses a meeting, they are required to read the posted minutes from the meeting. Minutes may either be posted in print format or electronically in Protean.
- The clinician must initial reading the minutes from the meeting within one week from the posting of the meeting minutes.

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53 Mother/Newborn Identification

53.1 Purpose

To establish a distinct method for identifying newborns born outside of the hospital setting consistent with the National Patient Safety Goals established by the Joint Commission.

53.2 Policy/Procedure

1. When the non-specialty team flight crew encounters a situation where a birth has occurred outside of the hospital and mother and newborn will be separated but have not yet been identified via armbands, they will place an armband on both mother and newborn(s) that has the below identifiers legibly written on the armband so that they can be successfully reunited as appropriate at the receiving facility.
2. In order to accomplish this, the crew will need to carry armbands and a marker with indelible ink.
3. Two matching identification bands should be placed on the infant(s) on two extremities (preferably wrist and opposite ankle) and should be secured so they will not become loose and lost in blankets.
4. On the mother's band, they should write flight number and "Mother" and on the infant(s)' band they should write flight number, "Baby", sex and identify multiples as A, B, C etc. For example, mother would wear "20-12345 Mother" and twins would wear "20-12345 Baby Girl A and 20-12345 Baby Boy B."

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54 New Medical Equipment Request

54.1 Purpose

1. To provide a standardize guidance on the ethical, safe and organize evaluation of new medical equipment for Air Methods, LLC.
2. Procedure for loan and trial of medical equipment.
3. Optimize resources by using efficient medical equipment processes and life-cycle management of equipment.
4. Improve staff performance through effective education and training.

54.2 Background

Air Methods is committed to providing outstanding Air Medical Transport Services to our community in a manner that is consistent with our Core Values while delivering measurable benefits to our customers, employees, and investors.

54.3 Scope

The policy applies to all medical equipment for all entities in the community based services division of Air Methods, LLC. This includes departments who are involved in procurement, maintenance, supply, decontamination and use of medical equipment.

54.4 Process/Map out Process

This is the process for submission of new equipment for evaluation.

1. Any clinician (requester) may submit an initial request application for evaluation of a product. New Clinical Product Request Form - 5716.
2. The requester must complete a product evaluation application with approval (signature) from the clinical director and medical director.
3. The clinical director will assure the form is completed and submit to the equipment committee chairman at clinicalequipment@airmethods.com.
4. The equipment chairman will have 60- days to respond to the clinical director whether the device will be accepted, denied or more information requested.
5. The equipment committee will evaluate the product based on cost, safety, efficiency of use, value to improve clinical practice, relevance to industry, weight, maintenance and training.

54.5 Conflict of Interest Statement

A conflict of interest is defined as an affiliation or relationship of a financial nature with a Commercial Interest Organization that might bias a person's ability to objectively participate in the planning, implementation, or review of a product. The requester must include a signed conflict of interest statement with the application. (Form Attached)

54.6 New Clinical Product Request Form

54.6.1 Instructions:

1. Requester please complete this form.
2. Requester, please complete conflict of interest form.
3. Attach any other supplement information as needed.

See Appendix for New Clinical Product Request Form - 5716.

55 *Non-Assigned Aircraft Utilization*

55.1 Purpose

Provide guidance when clinicians utilize a non-assigned aircraft to complete a patient transport. This situation may occur when the primary aircraft is not available due to a maintenance event, pilot staffing, or weather occurrence.

55.2 Policy/Procedure

- Base locations planning to utilize a non-assigned aircraft to routinely complete patient transports will require their clinicians to complete initial and recurrent medical crewmember aircraft familiarization training for those airframes commonly substituted.
- The transport request will originate from AirCom who will offer the non-assigned aircraft, where applicable. This should be offered with consultation of the Area Manager and Clinical Director.
- Transporting clinicians will contact the responding pilot to verify the following information.
 - Confirm the aircraft on-board oxygen supply and spare tank options.
 - The patient loading system contains the stretcher.
 - Discuss current aircraft maintenance status and any applicable concerns
- If transporting clinicians are based at the requesting facility the following process is applicable.
 - Transporting clinicians will respond to the patient's bedside with standard medical equipment and supplies.
 - When utilizing a fixed-wing asset for patient transport the communication specialist will arrange for ground transport from the hospital to the airport.
 - The communication specialist will provide the transporting clinicians an estimated time of arrival (ETA) for the responding aircraft and ambulance, if applicable.

- Transporting clinicians will ensure they have all medical equipment, charging cords, and medical supplies (Refer to Air Methods Ground Transport Checklist - 5711) to safely complete the patient transport. An exception is permitted when a base utilizes a pre-authorized secondary set of established medical equipment and supplies which is checked and maintained regularly using a pre-approved checklist.
- If a fixed wing is arriving at the airport to complete the transport, once confirmed with AirCom the aircraft is within close range, the transporting clinicians should begin the ground leg of the transport to the airport
- When the aircraft arrives, the clinicians will verify the following items prior to departing on the flight with the patient.
 - On-board oxygen and back up supply status.
 - The patient loading system is operational.
 - LifePort systems are operational (suction, inverter).
- In all cases, clinicians will ensure all medical equipment, medical supplies, and bio-hazard items are removed from the aircraft after transport is completed. If this is not possible because the pilot departs back to home base before the team returns, the aircraft status and needs must be communicated to Clinical Director for resolution.

56 Non-Emergency Patient Transport

56.1 Purpose

Provide guidance to safely perform non-emergency transport, often to a lower level of care.

Examples of lower level of care centers/destinations include, but are not limited to:

- Acute care hospital (from higher level of care to lower level of care).
- Nursing home/skilled nursing facility (SNF).
- Long-term acute care (LTAC).
- Rehabilitation facility.
- Residence.
- Non-hospital setting.
- Ground medical transport.

56.2 Policy/Procedure

Clinicians will adhere to the following guidelines when transporting a patient to a lower level of care.

- Adhere to the Patient Care Guidelines specific for Non-Emergency Patient Transport.
- Prior to transport, discuss with the patient, family, or legal authorized representative how to respond to an acute change in patient status or decompensation in condition during transport to include alternative landing locations.
- Verify the receiving location and health care provider(s) or family members can provide continuum of patient care including but not limited to ongoing oxygen therapy, mechanical ventilation, medication administration, or enteral feeding via pump.
- Clinicians should be prepared to follow any legal document or Advance Directive guiding medical care for the patient to include Do Not Resuscitate (DNR) orders (refer to the patient care guideline and Section 92 Transport of Patients with Advanced Directives/Do Not Resuscitate/DNR for specifics).
- The transport team should discuss and review alternative receiving locations should the patient's condition change upon arrival at the scheduled destination.

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57 Occupational Health Standards

The Occupational Health Standards located in 8.1 of the SMS Manual provides guidance for discipline and roles regarding bio-hazards in the air medical environment. All teammates are responsible for the content found in this document.

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58 Out of Service Staffing

58.1 Purpose

To provide a plan for clinical coverage and compensation when a base is “out of service” due to extended aircraft maintenance or weather situations.

58.2 Policy

Decisions to release clinicians from their scheduled assignment are situational. Generally, clinicians are expected to report for scheduled duty shifts and remain on site.

- Early departures should only occur with discussion and specific approval of the Area Manager or AOC on a case-by-case basis. Subject to approval, clinicians may use vacation hours for early departures but otherwise this would be considered personal time off (unpaid).
- Clinicians are not required and should not remain at a Company work site when occupancy of the base is deemed as a threat to safety (e.g., tornados, hurricanes, blizzards). In such events, the clinician should notify the Area Manager or AOC as soon as possible of the situation and seek further direction.
- During an out-of-service event (maintenance or weather), clinicians will be paid for their scheduled shift if they remain at the base. Duties other than regular flight duties may be assigned during this period. Additionally, coverage may be assigned to an alternate base within reasonable distance during this period per discretion of the Area Manager.

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59 Outreach Education Policy

59.1 Purpose

Air Methods is committed to excellence in patient care in the communities it serves across the country. Air Methods further recognizes that its clinical professionals have specialized skills that can be used to educate other medical professionals outside of Air Methods in an effort to improve patient care, particularly in rural areas with limited access to high-quality education. The purpose of this policy is to ensure that Air Methods provides such outreach educational efforts in a consistent and compliant manner, for appropriate purposes, and in a way that ensures Air Methods' training center maintains its certifications.

59.2 Scope

This policy shall apply to all Air Methods personnel providing or coordinating outreach education programs, including but not limited to, flight nurses, paramedics, managers of flight programs, regional management, and all business development personnel. This policy is limited to outreach education, meaning clinical education provided to other medical professionals not employed by Air Methods, and this policy does not apply to internal educational efforts directed at Air Methods' own employees or contracted professionals.

59.3 Outreach Education Definitions

- Pre-approved course - a course that has been authored by the CEU Outreach Committee and vetted and approved by the chair of the CEU Outreach Committee.
- One-off course - a course developed by an Air Methods clinician for the purposes of teaching a single group or specific to a service area.
- Association developed course - a course that has been authored by a national accrediting body, such as the American Heart Association.
- CAPCE - Commission on Accreditation for Pre-Hospital Continuing Education.
- CA BRN - California Board of Registered Nursing.

59.4 Policy

59.4.1 Nationally Accredited Education Courses (e.g., ARC, TNCC, ENPC, PFCSS, etc.):

- For every accredited outreach education program provided by Air Methods, each student is responsible for purchasing or otherwise obtaining the requisite textbook(s) (e.g., borrowing, purchasing online, etc.), and Air Methods will not provide such textbooks to students. However, Air Methods will provide the certification card(s) that are given as a result of the completion of the class, and the student shall not be responsible for the card(s).
- All accredited classes must meet the requirements of the accrediting body. Additionally, courses taught as Air Methods outreach must include the necessary number of instructors to meet the policy of the accredited class, and each instructor must be a properly certified instructor to teach the class. This would not include conference style lectures, only accredited courses that require hands on demonstration with equipment. Air Methods will provide instructor materials if necessary for Outreach courses.
- For each accredited class Air Methods holds, the class must be advertised or announced to the public in some way (except if the class is held pursuant to a contract). Such advertising may include the distribution of fliers, posting on the Air Methods website or a flight program's Facebook page, word-of-mouth to referral sources and non-referral sources alike, etc. Additionally, employees should make clear to those receiving the training that such training is not in exchange for the referral of patients to Air Methods.
- Course completion cards will be issued by the designated accrediting agency. If an Air Methods employee is assisting an outside agency conduct a course (e.g. assisting an instructor, guest lecturing, participating as a skill evaluator, etc.), the Air Methods employee must meet the standards set by the agency.
- There will be no charge for instructing the classes. However, classes will be tracked to ensure that Air Methods does not perform more classes in an urban area than in a rural area. Tracking will be reviewed on an annual basis by the Outreach Manager.

59.4.2 Non-Accredited Training (e.g., CME lectures, etc.)

- Air Methods may also provide certain non-accredited clinical training and education to other medical professionals outside of Air Methods. All such training can be provided without charge, and no textbooks or certification cards are necessarily related to such training.

- All non-accredited education programs must be approved in advance by the applicable Area Manager and the chair of the CEU Outreach Committee. Request must be received no less than 7 days prior to the date the program will occur.
- If the desired education content is not available in the Air Methods Outreach bank of lectures, a 45 day advanced notification will occur so the outreach lecture content can be developed by the employee requesting the lecture. The lecture development process outlined in the CEU class requirements below will be followed.
- The non-accredited course must be submitted 7 days prior to scheduled class time to AMCoutreach@airmethods.com for approval. The content developer must receive an email back with an approval prior to presenting the class.

59.5 Procedures

- Select desired course from the available courses in the Air Methods Library.
 - Courses listed in the Air Methods Library are pre-approved courses with a CAPCE and CA BRN CEU hours.
- Outreach Course Registration and Scheduling Process
 - Classes will be scheduled and arranged through the Base Outreach Coordinator (BOC).
 - The BOC will schedule the class in Protean.
 - The instructor will provide the QR code to the participants at the completion of the course for the participants to receive CE credits.
 - At no time will the instructor leave the QR code unattended.
 - The QR code must not be shared.
 - Care must be taken to ensure the CE credits are given ethically and following the standards of this policy.
 - Participants will receive a CE certificate via the email provided during their evaluation.
 - Participants that do not complete the post-instruction evaluation will not receive CE credit.
 - Participant must complete the entire course to receive CE credit.

59.5.1 Process for Authoring New Outreach Lectures

- Pre-approved courses will be authored by the CEU Outreach Committee.
- All classes shall be created so that they may be accredited for in-person and virtual presentation.
- Lecture submission and writing criteria.
 - Must use the approved Air Methods Outreach PowerPoint.
 - Should contain 3-5 measurable objectives.
 - Slides should follow the 555 rule or 7x7 rule for PowerPoint.
 - 555 Rule
 - No more than 5 words per line.
 - No more than 5 lines per slide.
 - No more than 5 text-heavy slides per presentation.
 - 7x7 Rule
 - No more than 7 words per line.
 - No more than 7 lines per slide.
 - Slide format should retain the Air Methods Outreach PowerPoint Template formatting.
 - (i.e., font, font size, font color)
 - Disclosures slide must be included and maintained as required by the accrediting bodies.
 - Avoid use of copyrighted materials.
 - All pictures must be properly cited.
 - All information that is not from the author must be cited.
 - Information included should be based in evidence.
 - As a general rule, the evidence must be from a peer-reviewed source.
 - Evidence should be less than five years old.
 - All slides must have fully annotated notes in the note section of PowerPoint.
 - Notes should include the full details of what is intended by the slide.
 - Notes should fully outline the purpose of any picture.
 - Notes should be prepared in a manner that any person presenting the outreach class can understand and relay the appropriate information.

- PowerPoint will be formatted in the style of the 7th edition of the American Psychological Association (APA) manual.
- A page of references is required.
- A summary slide is required that summarizes the main points of the information presented.
 - Summary slide should be limited to 6-8 main points.
- Presentations shall be written so that they may be accredited for virtual or in-person education.
- Presentations created and presented by Air Methods become and remain the intellectual property of Air Methods.
- Outreach presentations may not be distributed or reproduced without express permission of Air Methods.
- Only presentations that have been written by the CEU Outreach Committee Members and approved by the chair of the CEU Outreach Committee or presentations written and submitted by a crew member, and approved by the Outreach Committee chair will be authorized for use in outreach education instruction. No outside purchased lectures, other than our approved PHTLS, TNCC, TNATC, AMLS, ACLS, PALS, PEPP, and BLS, will be allowed for instructional Air Methods courses.
- Courses completed by the CEU Outreach Committee members will be uploaded to the appropriate shared folder for review by the chair.
 - After content is approved by the chair, it will be submitted to the marketing department for review.
 - After approved by marketing, the course will be submitted to the CEU Coordinator for accreditation by CAPCE and the CA BRN.

59.5.2 Landing Zone Classes

- If a Landing Zone Class will offer CEU credits, the class must meet all of the same requirements as the Outreach Presentations.
- If CEU credits will be offered, the Landing Zone Class must be taught by a clinician.
- Only the approved Landing Zone Class PowerPoint located in the library may be used.
- Landing Zone Classes must consist of the following at a minimum:
 - Landing Zone Class must consist of a classroom portion (the PowerPoint).

- Entire Landing Zone presentation must be presented.
 - Presenter cannot choose to leave out uncomfortable portions (such as crash recovery) as these are required for CAMTS accreditation.
 - It is recommended that the classroom portion should be instructed first.
 - If possible, a landing zone with incoming aircraft should also be present.
 - If possible and reasonable, a backup aircraft should be scheduled in the event the primary aircraft is unable to attend.
 - CEUs will still be provided if an aircraft is unable to attend.
- At no time will an aircraft "show and tell" or "meet and greet" be considered a Landing Zone Class.
 - While these methods are used for PR purposes, they do not meet the requirements of CAMTS or the CEU accrediting bodies.

59.5.3 Expectations of Flight Crew when Presenting Outreach Classes

- Flight crew members must present themselves in a professional appearance.
 - This includes wearing their flight suit when presenting outreach.
 - Only approved clothing may be worn (i.e. program hat).
- When at all possible, the flight crew member presenting outreach should not be on duty.
 - This provides continuity in the presentation and ensures that the entire class is taught.
 - If the presenter leaves during the presentation, CEUs may not be provided for the class if no other approved presenter is present.
- Classroom location should have the following available:
 - Enough seats for all participants.
 - A laptop computer with the ability to download the Outreach Presentation.
 - A projector to display the presentation.

60 Patient Care Documentation Policy

60.1 Purpose

The patient's record provides the only enduring evidence of the care provided and is used to ensure continuity and quality of care, provide information required for reimbursement, ensure compliance with regulatory requirements, and to legally protect the patient and the caregiver. The documentation of care provided is critical for effective communication with the receiving facility. High quality documentation is a necessary and integral aspect of the patient transport.

Failure to comply with the documentation policies and guidelines may result in discipline. Additionally, both intentional and unintentional documentation issues could result in charges of fraud, false claims, or other legal issues for the company as well as the clinician. Your documentation must always be factual, objective, and thorough. Falsification of any documentation will not be tolerated and will result in immediate termination.

60.2 Patient Care Record – General Guidelines

1. Air Methods clinicians will complete an accurate and timely patient care record for all patient encounters and transports. An electronic patient care record (PCR) will be generated any time the transport clinicians have a patient encounter, regardless of whether the patient is transported.
2. If a flight is launched and the Air Methods vehicle arrives at the referring location, a PCR must be created, regardless of whether patient contact is made.
 - a. The PCR should document what the request was for, the name of the person canceling the transport, and the reason given.
 - b. Disposition/Outcome should be marked according to what occurred (e.g., No Patient Contact, Canceled, Patient Expired, etc.).
3. Air Methods clinicians must follow the regulations in their area for creation of PCRs. Some states require the creation of a PCR for every dispatch or every response. It is the clinician's responsibility to know the local regulations and create the appropriate records. Failure to create PCRs when required may result in licensure issues for the company.

4. The PCR must contain the FlightVector flight number, including the letters FV. It is the responsibility of the crew members to ensure the referring and receiving location/facilities are accurate in the PCR, including the address. Importing the dispatch record is the easiest way to ensure the accuracy of this information.
5. Patient care records are required to be completed prior to the end of the clinical teammate's shift, and must be completed on Air Methods owned computers or mobile devices. Any delay in documentation must be approved by the Clinical Director. All patient care records will be uploaded into Protean. This is completed by file transfer from the documentation system. If the automatic upload does not occur, the crew members should manually upload the files to Protean.

The following documents are required to be attached to the PCR (or manually uploaded to Protean):

- Cardiac Monitor Strips (including ETCO2 waveform recordings) or the Zoll X-series monitor upload file.
- Billing paperwork (PCS, ACF, and FaceSheets).
- Blood paperwork.
- Any physical paperwork generated as a result of the transport by the crew members (narcotic forms, etc.).
- Air Methods (or state required) Drop Sheet.

A transport is not considered complete until all patient records are uploaded, post flight debriefing is documented, and quality improvement/utilization review documentation is completed in Protean. At no time will any patient's records be removed from the base where the flight originated.

6. The patient care record will accurately and objectively reflect the assessments and care provided during the patient encounter, from initial contact to relinquishment of patient care at the receiving facility.
7. All treatment, along with noteworthy events related to the patient's care and/or transfer, that occurs during a patient encounter shall be documented in the PCR. Be concise and include only factual statements. Do not include reasoning, speculation, conjecture, or information unrelated to the patient's care. All documentation must be objective; subjective information should not be documented.
8. The patient care record should also include the patient's demographic and insurance information, if available.
9. The crew must attempt to obtain the medical record number from the sending (interfacility transfers) and receiving facility when face sheets are not available.

10. Following completion of the patient care record, all paper forms associated with the flight will be scanned and attached to the record. Once the paper records have been scanned and uploaded to the documentation system and the documentation process has been completed, all paperwork must be shredded. Scanned versions must be deleted and the recycle bin emptied on the computer.
11. Patient care records are to be viewed only within Protean once the PCR has been locked in emsCharts. Viewing the PCR from within emsCharts is only acceptable with prior approval of the Clinical Director.
12. Any patient belongings transported will be documented in the appropriate area of the PCR, including the name of the caregiver receiving the belongings at the completion of the transport. If no belongings are transported, clinicians should record "no belongings" or belongings in the possession of family members."
(See [Section 8 Cardiac Monitor, ePCR Data Import](#)).
13. For Specialty Team Transports completed by non-Air Methods specialty team clinicians: A condensed version of the patient care record will be entered by the Air Methods clinical flight team on duty or the specialty team clinicians. This record will include the patient name, date of birth, demographics, times, category, flight number, and insurance information. The specialty team record will then be scanned and attached to this record or handled as specified pursuant to the relevant contract with the hospital.
14. If the electronic charting system is not available and is projected to be unavailable through the end of the clinical flight team's shift, a paper PCR must be completed in the interim, with the approval of the Clinical Director. It is the responsibility of the clinical teammates to enter this record into the electronic charting system during their next scheduled shift, unless requested sooner by the Clinical Director, to ensure accurate electronic reporting of all Air Methods transports. The paper PCR will be scanned and attached to the electronic record upon completion and then shredded at the base upon completion of the electronic PCR.
15. Additional copies of PCRs will not be routinely printed or stored on site. No patient information or records will be removed from the base or transported from the base to another location.
16. Patient Care Records will either be left at the receiving hospital or faxed to the receiving hospital's Medical Records Department on a secure fax line. Crew members must take great care to ensure that the PCR is faxed to the correct fax number. Failure to ensure accuracy in fax numbers may result in a HIPAA breach.
17. All faxed PCRs will be sent with the fax cover sheet containing the required HIPAA confidentiality statement.

18. Records being sent to a receiving facility after the initial shift in which the transport occurred, or PCR's requested to be re-faxed to receiving facilities, will be handled by PBS. The request is sent to PBS by the Clinical Director by emailing pbsrecords-legal@airmethods.com.
19. In the event that communication from an insurance company (EOB, denials, requests for additional information) is received at the base, please scan the correspondence to a PDF and email it to PBSMediaDept@airmethods.com.
20. State Reports - The electronic charting system reports to many states automatically. If these reports are not available, or until state extracts are available through the electronic charting system, state forms must be completed as required by state EMS regulations.
21. All clinical teammates are responsible for the entire patient record. All clinicians must read and review the entire patient record to ensure that the information is accurate and all sections of the record match and are consistent.
22. All clinical teammates involved in the patient's care must review and electronically sign the PCR. After the record is signed, it will be locked and advanced to the next level in the system.
23. Many bases have access to the emsCharts Mobile application on the base laptop in order to minimize documentation time. It should be noted that emsCharts Mobile is not set up in the same manner as the online version. For patient encounters where the chart is initiated on the emsCharts Mobile platform, the crew must ensure that they complete documentation of a complete head-to-toe assessment and that all sections of the PCR are checked prior to locking to ensure thorough documentation. The PCR should be uploaded and completed in the online version of emsCharts. - It is the responsibility of the clinical teammate to ensure that all documentation completed in emsCharts Mobile captures the same data as the online version.
24. It is recommended that clinical issues be addressed in Protean and not by email.
25. A dropsheet will be left at the receiving facility for all patients transported by Air Methods clinicians. The dropsheet should be the one required by the state/system the program is in or at a minimum, the Air Methods standard Transport Drop Sheet (see [Appendix A - Sample Forms](#)).
26. In the event that a check for payment of transport is received at the base, the check should be placed in a sealed envelope and mailed to the correct Lockbox according to the company it is addressed to. [Air Methods Lockbox Addresses](#) can be found in the Appendix.

27. Assistance with emsCharts (errors that you cannot resolve) can be obtained from your Clinical Director. Assistance with emsCharts, such as requiring addition to specific service accounts can be obtained by emailing emsChartshelp@airmethods.com. Assistance with locking errors, such as NEMSIS issues, can be obtained by submitting a Support Ticket to emsCharts within the emsCharts system.

60.3 Changes to Patient Care Records (Addendums)

It is the responsibility of all clinicians involved in the patient encounter to thoroughly review the patient care record for accuracy and completeness before signing and locking the chart.

Once locked, the patient care record will not be unlocked or altered under any circumstances with the exception of demographic data. Correction or additions to a locked patient care record must be accomplished by an addendum. Addendums will be approved on a case-by-case basis and must be discussed with the Clinical Director.

Addendums are the exception and should not be used routinely. Generally, addendums must be completed within 24 hours of the chart's original completion date and time, after obtaining the necessary approval.

Addendums are approved by the Clinical Director and/or Clinical Compliance Director. Please refer to the table below to identify the required approval.

TABLE 60.1 Addendum Approval

Reason for Addendum	Approval Required
• Documentation of wasted medication not included.	Clinical Director/Clinical AOC
• Errors identified in documentation.	Clinical Director/Clinical AOC
• Missing treatments, interventions, and medications that were performed.	Clinical Director/Clinical AOC
• Missing patient information pertinent to the continuation of care.	Clinical Director/Clinical AOC
• Other reasoning not listed.	Clinical Director AND Clinical Compliance Director

The addendum must include:

- Date and time of the addendum.
- The section and specific information of the chart being changed/edited.

- What was previously documented or omitted.
- What is being added/edited.
- Reasoning for the correction or addition.
- No other changes should be made other than the approved content.

Once the addendum is complete the following steps must be completed:

- The updated chart including the addendum must be sent to the receiving facility to be added to the hospital record.
- DIS must be notified that an addendum was added to the Flight Number.
- The chart must be re-queued in emsCharts to send the updated chart and addendum to Protean>Oasis. This step can be completed by the Clinical Director or by submitting a request in Fresh Service > emsCharts or by sending an email request to emschartshelp@airmethods.com. The request should include the note that an addendum was added and request the flight be re-queued to sync with Protean > Oasis.

60.4 Unprofessional Documentation

Venting frustrations in the Medical Record is unacceptable. It is unprofessional to express concerns about working conditions, poor rapport with team members, care not rendered by previous caregiver, personal opinions, and other issues of concern. Use of profanity is prohibited, including patient quotes that contain profanity.

60.5 Forms

The following Air Methods standardized forms must be completed as applicable for every patient encounter by Air Methods medical personnel. Do NOT photocopy billing forms. If you need additional copies of forms, you must request them by emailing PBSForms@airmethods.com. If you have questions about the PBS forms, need additional education for your base/program, or have compliance questions regarding the forms, email PBSformshelp@airmethods.com.

At no time may a clinician use white-out or other methods to fully obliterate a mistake on the forms. Mistakes should be corrected by drawing a single line through the information, writing "error" and the initials of the person correcting next to it.

60.5.1 Authorization and Consent

ACF must be completed anytime contact is made with the patient, even if no transport is provided.

- This form must be signed by the patient unless there is a valid reason why the patient cannot sign. If the patient cannot sign, Air Methods clinical teammates associated with the patient who are present and willing to sign, may sign only when the patient legitimately cannot. Signature requirements are governed by various federal and state laws and regulations.
 - Examples when it is acceptable for the patient not to sign (list is not exclusive):
 - Unconscious or unresponsive.
 - In cardiac/respiratory arrest.
 - Confused.
 - Has an intellectual disability.
 - Physically incapable of signing (document the specific problem that prevents signing).
 - Patient is a minor.
 - A prisoner/inmate in the custody of the prison/police department.
 - Patient has a legal guardian.
 - This is not the same thing as a healthcare POA. A healthcare POA can only sign if the patient meets the above requirements.
- Scene transports will not be delayed to obtain a signature. The written consent will be obtained after arrival at the receiving hospital.
- If the patient is unable to sign and no family members or representatives are present to sign, one clinician or healthcare provider will sign as witness, listing the reason(s) why the patient cannot sign.
- If “Unable to Sign” is documented on the consent form, then section III must be completed with the specific medical or legal reason to validate the reason for no patient signature.
- When the clinical flight team signs in place of the patient or representative, then the receiving facility must sign the “Receiving Facility Acknowledgment” section (section IV) to acknowledge receipt of the patient at the facility.

60.5.2 Refusal of Transport and/or Treatment Form - 5196

See [Section 66 Patient Refusal of Treatment/Transport](#) for detailed instructions.

60.5.3 Physician Certification Statement (PCS-01)

Any time a patient is placed on board an Air Methods vehicle, this form must be completed.

This form must be filled out completely with all appropriate requested information provided. The form should be signed by the physician (or other professional specifically indicated on the form where allowed) most closely associated with the patient's condition and need for transfer and transport at the time of air medical transport. For interfacility transports, the referring physician is preferred. At no time can the clinical teammates (or our contractors, such as the specialty team crew) sign the Physician Certification Statement with the exception of the program Medical Director.

If the physician is not available on arrival of the flight team, the referring nurse (or other professional specified on the form where allowed) may sign the form or complete a verbal order from the sending physician. For scene flights, on scene EMS or first responders may sign the form. If unable, the receiving physician or nurse may sign the form. As a last resort, the flight program Medical Directors may sign the form.

60.5.4 Physician Certification Statement Addendum (PCS Addendum)

Any time a correction or addition is needed to the PCS form after it has been signed (certified), a PCS Addendum Form must be completed. Evidence and reasoning of the change must be documented on the PCS Addendum to protect the clinician and Air Methods from any potential liability (such as an allegation of fraud). If a change is needed to the PCS form, the following procedure applies:

- On the PCS Form:
 - The clinician will draw a single line through the incorrect information.
 - The clinician will write "error" next to the line and initial it.
 - The clinician will then write the correct information.
 - If the information was left blank, the clinician will simply fill in the correct information.
- On the PCS Addendum Form:
 - The clinician will fill in the transport information at the top.
 - Clinician will find the appropriate area where the correction was made and then complete the appropriate section.
 - Clinician will document why the change was made and how the information was verified.
 - Clinician will sign the PCS Addendum Form.

- After the correction is made and the PCS Addendum Form created, the clinician will send both the PCS Form and PCS Addendum Form to DIS.
- At no time will the clinician make corrections to or alter the information entered into section 1 and 2 of the PCS Form after it has been signed.

60.6 Frequency of Vital Signs

- Minimum - All patient transports require a minimum of two complete sets of vital signs. Vital signs to be monitored are as set forth in the PCGs.
- At a minimum, document vital signs every five (5) minutes for scene transports and every 10 minutes for interfacility transports.
- If vital signs are entered into the record by import from a Zoll X series monitor, vital signs frequency should remain at every five (5) minutes.
- The frequency of monitoring vital signs may be increased based on the patient's condition.
 - Vital signs will be taken after each intervention.
 - Any abnormal vital signs are to be repeated and monitored closely.
 - A final set of vital signs will be documented within ten minutes prior to relinquishment of care.
 - Patients on vasoactive medications should have vital signs monitored every five minutes.

60.7 Charting Guidance

The following are examples of additional situations that may be encountered, with guidance about the appropriate charting that should be completed in each situation:

1. Air Methods RW crew requested and responds to sending facility to package a patient and fly them to an airport to meet an Air Methods FW crew for completion of transport.
 - a. RW crew receives a flight number from Air Com and completes a separate chart with HPI.
 - b. FW crew receives a separate flight number from Air Com and completes a separate chart which includes an HPI that reflects patient was transported by RW to intercept FW crew.
 - c. Both flight numbers should be cross referenced in the HPI of both charts.

- d. RW crew will hand off DIS paperwork (PCS, Consent Form, etc.) to FW crew - FW crew will reference both flight numbers on the DIS paperwork and forward to DIS upon completion of the FW flight (RW crew should let DIS know that DIS paperwork will be submitted by the FW crew).
 - e. emsCharts Disposition
 - i. Initial RW crew selects - "Treated, Transported" - Receiving will be EMS.
 - ii. FW crew selects - "Treated, Transported" - Receiving will be hospital.
2. Air Methods RW crew co-located at a sending facility assists in packaging and riding with a patient by ground to local airport for FW crew to complete the transport.
 - a. RW crew will obtain and use the FW flight number to complete a chart and designate "G" after the flight number indicating "ground" portion of the FW flight.
 - b. FW crew will use the FW flight number (no designation after the number) to complete a separate chart reflecting care provided during FW flight.
 - c. RW crew will complete and hand off DIS paperwork to FW crew - FW crew will submit DIS paperwork upon completion of the FW flight.
 - d. emsCharts Disposition
 - i. Initial RW crew selects - "Transported by flight crew on ground ambulance" - Receiving will be EMS.
 - ii. FW crew selects - "Treated, Transported" - Receiving will be hospital.
3. Air Methods RW crew co-located at sending facility assists in packaging a patient but does not provide any transport - the transport is completed by another Air Methods FW crew or another Air Methods RW crew.
 - a. Packaging RW crew will obtain and use the flight number issued to the "other" Air Methods FW or RW crew and complete a chart. Packaging crew will use the "P" designation after the flight number to denote that the chart is a "Packaging Chart."
 - b. The "other" crew (FW or RW) will use the issued flight number (no designation after the number) to complete a separate chart reflecting care provided during their flight.
 - c. Packaging RW crew will complete and hand off DIS paperwork to the "other" crew who will submit DIS paperwork upon completion of the flight.
 - d. emsCharts Disposition
 - i. Packaging RW crew selects - "Treated, Transported by other EMS" - Receiving will be EMS.
 - ii. "Other" crew selects - "Treated, Transported" - Receiving will be hospital.

4. Air Methods crew lifts with patient on board and has to abort for maintenance or other reasons and flight is completed by another Air Methods crew.
 - a. Each crew will receive a separate flight number and complete separate charts.
 - b. Each crew should cross reference both flight numbers in the HPI of both charts.
 - c. First crew can hand off DIS paperwork to second crew - second crew will reference both flight numbers on the DIS paperwork and forward to DIS upon completion of the second part of the flight (1st crew should let DIS know that DIS paperwork will be submitted by the 2nd crew).
 - d. emsCharts Disposition
 - i. Initial crew selects - "Treated, Transported" - Receiving will be EMS.
 - ii. Second crew selects - "Treated, Transported" - Receiving will be hospital.
5. Air Methods crew lifts with patient on board and has to abort for weather or other reasons and transport is completed by ground.
 - a. Crew will complete one chart with one set of DIS paperwork using initial flight number provided for request.
 - b. emsCharts Disposition
 - i. Crew selects - "Treated, Transported" - Receiving will be hospital or EMS depending on where they divert to (it could be the sending or receiving hospital).
6. Air Methods crew responds to a scene or hospital and has to go by ground due to weather or patient condition - patient is never loaded in the aircraft.
 - a. Crew will complete one chart with one set of DIS paperwork using initial flight number provided for request.
 - b. emsCharts Disposition
 - i. Crew selects - "Transported by flight crew on ground ambulance" - Receiving will be destination hospital.
7. Air Methods crew responds to a scene or hospital, treats the patient and then aborts before lifting off and no transport is completed.
 - a. Crew will complete one chart using initial flight number provided for request.
 - b. Crew must complete the Authorization and Consent Form (ACF) and send to DIS.
 - c. emsCharts Disposition
 - i. Crew selects - "Treated, No Transport."

8. Air Methods crew responds to referring facility/location, canceled (or discussed patient with staff), no patient contact made.
 - a. Crew will complete one chart using initial flight number.
 - i. Chart completed with whatever information is known and why the cancellation occurred.
 - b. emsCharts Disposition
 - i. Crew selects - "Canceled, No patient contact."

IMPORTANT NOTE: CREWS SHOULD NOT USE AN "A" AND "B" DESIGNATION BEHIND A FLIGHT NUMBER WHEN HANDING OFF A PATIENT FROM ONE CREW TO THE NEXT.

"A" and "B" designations are used for transport of more than one patient on the same flight.

There are many possible scenarios that may not be addressed here. If you have any questions regarding the proper documentation of a patient flight, or if you are receiving conflicting information from DIS regarding paperwork required, please email pbsformshelp@airmethods.com.

60.8 Open or Incomplete Patient Care Record Requirement

60.8.1 Electronic Patient Care Records

Clinicians are expected to have completed and locked electronic Patient Care Records (ePCR) before leaving the base at the end of the shift. If unable to complete the ePCR, approval from the Clinical Director or AOC is required to extend the completion/locking. In all cases, the ePCR must be started and include controlled medication administration, airway or invasive skills interventions, and the vital sign information in the draft ePCR prior to leaving the base. In addition, the required DIS paperwork must be attached and the Debrief section of Protean must be completed prior to leaving the base.

60.8.2 Approval to Leave Base

If the clinician is approved to leave the base with an open ePCR the DIS paperwork must be completed and submitted to DISDocs@airmethods.com.

61 Patient Care Equipment Maintenance

61.1 Patient Care Equipment

Medical equipment is maintained in accordance with manufacturer specifications as well as through a preventative maintenance program. Clinical Base Leads or their designees monitor this program. Any new equipment requests must be submitted to the Clinical Operations Committee with the correct form via email at clinicalequipment@airmethods.com.

Inspections are completed in accordance with state and local regulations. It is the responsibility of the Clinical Lead and Clinical Director to ensure that each base is kept within regulatory standards.

61.2 Patient Care Equipment Preventative Maintenance

Preventative maintenance will be performed as noted below which is obtained from manufacturer recommendations. Any equipment not listed below will be maintained per the manufacturers recommended timeline.

Equipment	Preventative Maintenance Requirement
Zoll X Series	Annual PM
LifePak 15	Annual PM
Sapphire Infusion Pump	Annual PM
Alaris Minimed Infusion Pump	Annual PM
Revel	Every 3 years or 15k hours of use
LTV 1200	Every 2 years or 10k hours of use
Hamilton T1	Annual PM
IABP (Arrow or Cardiosave)	Annual PM
Buddy Life Fluid Warmer	Annual systems operations check completed locally by Air Methods clinical team member, as specified by manufacturer. Date of check should be noted on equipment.
Endotrachealtube Cuff Manometer	Annual Calibration

61.3 Calibration of Medical Equipment

Any piece of medical equipment that requires calibration (e.g., glucometers, ETCO₂ monitors) is maintained in accordance with manufacturer's guidelines. Clinical Base Leads are responsible to ensure this occurs at the required intervals. Items such as EZ-IO, dopplers, portable suction, baby pod, and C-Mac will be tested daily by the clinicians on duty to ensure proper working order.

61.4 Monthly Inventories

Monthly inventories are completed on all supplies and pharmaceuticals on board the aircraft and stored at the base. Inventories should remain at par levels as set forth by the needs of each base and should not be expired. Each base will have a list of aircraft contents including the bags.

62 Patient Care: Infection Prevention

62.1 Purpose

This policy details the safety precautions and infection control procedures implemented to protect patients from exposure to infectious disease and related hazards and is complementary to existing policy guidance within the Air Methods Safety Management System (SMS) and all other general safety and compliance program guidance.

The chief principle of this policy is to protect patients through adherence to Standard Precautions, as they are defined by the Centers for Disease Control and Prevention (CDC). Standard Precautions are the minimum infection prevention practices that apply to all patient care, regardless of suspected or confirmed infection status of the patient, in any setting where healthcare is delivered. These practices are designed to both protect healthcare providers (HCP) and prevent HCP from spreading infections among patients. Standard Precautions include: 1) hand hygiene, 2) use of personal protective equipment (e.g., gloves, gowns, masks), 3) safe injection practices, 4) safe handling of potentially contaminated equipment or surfaces in the patient environment, and 5) respiratory hygiene/cough etiquette. Exposure Management is also a high priority.

This guidance adheres to CDC guidance available at the time of publication of this policy and is subject to change based upon any new or evolving information specific to post-publication infectious disease hazards prevalent in the local community or region. Please contact your clinical director with any questions or concerns about safe patient care or infection prevention or control.

62.2 Policy

62.2.1 Hand Hygiene

Key situations where hand hygiene should be performed include:

- Before contact with a patient.
- Before performing an aseptic task (e.g., insertion of IV, preparing an injection).
- After contact with the patient or objects in the immediate vicinity of the patient.
- After contact with blood, body fluids, or contaminated surfaces.

- If hands will be moving from a contaminated-body site to a clean-body site during patient care.
- After removal of personal protective equipment (PPE).

Good hand hygiene, including use of alcohol-based hand rubs (ABHR) and hand-washing with soap and water, is critical to reduce the risk of spreading infections in outpatient settings. Use of ABHR as the primary mode of hand hygiene in healthcare settings is recommended by the CDC and the World Health Organization (WHO) because of its activity against a broad spectrum of epidemiologically important pathogens, and because compared with soap and water, use of ABHR in healthcare settings can increase compliance with recommended hand hygiene practices by requiring less time, irritating hands less, and facilitating hand hygiene at the patient bedside. For these reasons, ABHR is the preferred method for hand hygiene in most clinical situations. Soap and water should be used when hands are visibly soiled (e.g., blood, body fluids) and is also preferred after caring for a patient with known or suspected *Clostridium difficile* or norovirus during an outbreak.

If hand hygiene with soap and water or ABHR cannot be safely conducted within the preflight or flight environment, clinicians shall put on a new pair of clean gloves to accomplish the patient care task at hand.

62.2.2 Personal Protective Equipment (PPE)

Personal Protective Equipment (PPE) refers to wearable equipment that is intended to protect HCP from exposure to or contact with infectious agents. Examples include gloves, gowns, face masks, respirators, goggles, and face shields. The selection of PPE is based on the nature of the patient interaction and potential for exposure to blood, body fluids, or infectious agents. Examples of appropriate use of PPE for adherence to Standard Precautions include: use of gloves in situations involving possible contact with blood or body fluids, mucous membranes, non-intact skin, or potentially infectious material; use of a gown to protect skin and clothing during procedures or activities where contact with blood or body fluids is anticipated; use of mouth, nose, and eye protection during procedures that are likely to generate splashes or sprays of blood or other body fluids. Hand hygiene is always the final step after removing and disposing of PPE.

Clinicians must use appropriate PPE based on the anticipated exposure to infectious agents.

PPE must be donned and doffed according to the current [CDC Guidelines for isolation precautions](#) and manufacturer recommendations.

Exposure to infectious disease unknown to clinicians at the time of the flight may be referred to the Air Methods Health and Wellness Team for consultation and management.

62.2.3 Safe Injection Practices

Injection safety includes practices intended to prevent transmission of infectious diseases between one patient and another, or between a patient and healthcare provider during preparation and administration of parenteral medications.

Clinicians must follow safe injection practices, including:

- Using aseptic technique when preparing and administering injections.
- Avoiding the reuse of needles, syringes, or other equipment on multiple patients.
- Properly disposing of sharps in designated puncture-resistant containers.
- Cleansing the access diaphragms of medication vials with alcohol before inserting a device into the vial.

Clinicians will use one syringe for each medication and will not reuse used syringes for repeated medication doses.

No single-dose or single-use injection materials may be reused between multiple patients.

62.2.4 Environmental Cleaning

Cleaning refers to the removal of visible soil and organic contamination from a device or environmental surface using the physical action of scrubbing with a surfactant or detergent and water. Emphasis for cleaning and disinfection should be placed on surfaces that are most likely to become contaminated with pathogens, including those in close proximity to the patient (e.g., stretcher, sled, seatbelts) and frequently-touched surfaces in the patient-care environment (e.g., aircraft surfaces).

Clinicians must participate in maintaining a clean and hygienic environment by:

- Following established protocols for routine cleaning and disinfection of patient care area of the aircraft, equipment, and high-touch surfaces.
- Promptly cleaning up spills of blood or body fluids using appropriate disinfectants.
- Ensuring that reusable patient care equipment is cleaned and disinfected between uses.

The aircraft will be cleaned between patients and patient care equipment will be cleaned and disinfected between patients. No aircraft or equipment will be left dirty at the end of the shift without notification to the oncoming crew that the cleaning and disinfection task is incomplete and must be done. Cleaning to remove organic material must always precede disinfection because residual debris reduces the effectiveness of the disinfection and sterilization processes.

If visible blood or debris is present in the aircraft, crews will clean and disinfect the patient care area before the aircraft is sent to heavy maintenance and/or transferred to the care of the on-duty aircraft maintenance technician (AMT).

Clinicians will follow manufacturer's instructions for use of products selected for cleaning and disinfection. Refer any questions or concerns to the clinical director/AOC.

Reusable medical devices (e.g., glucometer, blood pressure cuff, etc.) will be cleaned and reprocessed according to manufacturer's instructions to prevent patient-to-patient transmission of infectious agents. Appropriate PPE shall be worn when handling and reprocessing contaminated medical devices.

62.2.5 Respiratory Hygiene/Cough Etiquette

Contain respiratory secretions in patients who have signs and symptoms of a respiratory infection, beginning at point of contact for the flight and continuing throughout the duration of the encounter. Symptomatic patients should wear a mask to contain respiratory infection and prevent spread to the aircrew.

Clinicians must adhere to respiratory hygiene/cough etiquette by:

- Covering their mouth and nose with a tissue or elbow when coughing or sneezing.
- Properly disposing of used tissues.
- Performing hand hygiene after coughing, sneezing, or handling used tissues.

62.2.6 Exposure Management

Clinicians must promptly report any incidents of exposure to infectious agents, including needlestick injuries or other sharps injuries, to their area manager and to the Air Methods Health and Wellness team, according to existing safety/occupational health protocol. Management of post-exposure evaluation and treatment will be coordinated by the Health and Wellness team.

63 Patient Movement

63.1 Purpose

To provide comprehensive guidance for safe patient movement.

63.2 Policy

63.2.1 General Safety: Packaging and Movement

- The flight crew, patients, and passengers will always adhere to all safety regulations/ Crew Resource Management principles, including all applicable General Operations Manual policies.
- Any problems, deviations, or safety concerns must be reported using an applicable Air Methods safety reporting tool. Concerns related to property not owned and managed by Air Methods must be communicated through the appropriate channels for evaluation and mitigation.
- All personnel, patients, and passengers should wear appropriately sized hearing and eye protection.

63.2.2 Patient Securement

- Patients will be secured to the stretcher during flight, and the stretcher will be securely locked in place.
- Infants transported in an isolette or infant transport device will be secured with the straps provided and according to the manufacturer's instructions and in accordance with Service Engineering approval.
- All patients will be secured to the stretcher by the FAA-approved safety belts provided.
 - The use of any transferring device, e.g., 'smooth mover' or 'man-handler' will be limited to use during transfer only. Securing the patient to the stretcher with a transferring device in place is prohibited unless the moving device can be secured utilizing additional straps or securing devices.
 - If the patient size prohibits the ability to safely secure the patient using all the stretcher belts, clinicians are required to contact the Clinical Director and consider a safe and prudent alternative mode of transport, including utilizing the next closest appropriate aircraft or ground ambulance.

- Car seats are not an approved means for securing an infant or child receiving patient care during transport, and an infant/child may not be held by an adult secured on a transport stretcher. If the patient weighs less than 40 lbs., an appropriate (for height and weight) restraint device must be used, which must be secured by an FAA-approved seat belt(s). Air Methods-approved infant and child restraint devices include the following options.
 - Pedi-Mate: 10-40 lbs. (4.5-18.1 kg).
 - Pedi-Mate + Pediatric Restraint System: 10-100 lbs. (4.5-45.5 kg).
 - Medkids Baby Board: 2-15 lbs. (1-7 kg).
 - BabyPod: 0-17.6 lbs. (0-8 kg).

63.2.3 Equipment Securement

- All equipment must be secured before the flight. Use approved aircraft mounts or approved securement devices for all equipment.
- All medical equipment and supplies must be secured before and during flight operations. Medical devices with an approved mount must be secured in the mount. Medical supplies must be secured in the aircraft pouches or drawers and carry-n bags with an Air Methods-approved securement device.
- When equipment is secured in mounts or with devices, perform a visual inspection and a physical double-check of the equipment securement before flight. All equipment that could come loose and injure a patient or crew must be double-checked to ensure that it is secure for flight. Report any loose or damaged equipment mounts immediately.
- Clinicians must ensure all medical devices (cardiac monitor, ventilator, IV infusion pumps, IV fluid warmer, cardiac assist devices, etc.) and all patient equipment (ventilator circuits, IV tubing, and other invasive device lines or drains) are properly secured during patient movement.

63.2.4 Patient Movement

- To minimize the potential for teammate or patient injury, Air Methods clinicians must adhere to the following guidance:
 - **Talk**
 - Make a plan. Communication and planning are key. Take the time to stop, communicate with your partner, decide what the plan is, and commit to communicating every step. Communicate the plan for hazards like poor light, slick surfaces, and limited personnel. Identify adequate lifting and moving assistance before moving the patient. At least two people with all four hands on the stretcher must be present.
 - Do not move a patient until you have confirmed that all equipment is secured with approved aircraft mounts and that you have adequate assistance to load and unload the patient and all associated equipment. No fewer than two people with all four hands on the stretcher shall engage in patient movement activity.
 - Before movement, Air Methods clinicians will pause the patient movement procedure and specifically assign a dedicated clinician to the head of the stretcher to monitor the movement of all patients with an advanced airway in place (“the airway guardian”). This person will ensure that the advanced airway remains patent and in place before, during, and after the patient’s movement as evidenced by continuous end-tidal capnography measurement. The team will document that this assignment and procedure was conducted according to policy and that the patient’s airway was assessed and reassessed before and following patient movement. When the movement is conducted at hand-off, a member of the receiving team will acknowledge that the airway was received.
 - Enlist the assistance of available EMS crews or hospital personnel, and position assistants alongside the stretcher to ensure that it does not tip or fall over.
 - When planning the transfer of a patient who is not secured to a long spine board, do not plan to load the patient into the aircraft with a sheet. The aircraft stretcher must be removed from the aircraft and taken to the EMS stretcher. Using additional assistance, the patient may be transferred between stretchers using a commercially available device or a sheet, as long as the patient is moving between stretchers at the lowest level possible. At no time should a patient be moved from a stretcher directly into the aircraft utilizing a sheet transfer without a supporting stretcher or sled present. This applies to both scene calls and inter-hospital transfers.

- **Tuck**

- Tuck your patient's arms and legs in under straps and blankets and ensure that all equipment is secure for the movement. Ensure that all straps are secured appropriately before any patient movement.
- Ensure that the patient is secure on the stretcher or sled. At no time is a sheet to be used to move a patient in or out of an aircraft. A sheet is only to be used for a same level move from bed to stretcher or stretcher to stretcher. The bed or stretcher should be placed at the lowest level possible for safe movement. Commercially available assistive devices appropriately rated for the patient's weight (MegaMover, LifeBlanket, etc.) should be utilized as often as possible to ensure patient and clinicians safety.

- **Track**

- Track the team movement and safety of the patient and all clinicians. Confirm and communicate clinician and patient safety at each step during the movement, and ensure visual and auditory confirmation that crew, patients, and equipment are always secure and safe. Watch to make sure that monitors, ventilators, infusion pumps, and all other patient equipment are secure, and ensure that all patient lines (ventilator circuits, IV tubing, device lines, etc.) are free and clear. Ensure that devices are used as directed by the manufacturer's instructions for use.
- Use assistive devices (e.g., MegaMover) to move patients whenever possible to ensure stability for the patient and to reduce the potential for clinician injury. While patients may be moved from one equally-level bed to another with a bed sheet and/or slide-board, a bed sheet should not be used to pick a patient up and move from one location to another.
- Air Methods personnel should always be the primary operators of an Air Methods stretcher while in transit. When Air Methods clinicians must operate stretchers and sleds not owned and maintained by the company, the following risk mitigation steps are necessary:
 - Assess the equipment before use and establish that the equipment is in good working order and safe to operate, including a test operation before use with a patient.
 - Ensure that the mechanism of operation is well understood by the clinician and the work is safe to perform.
 - Request that the agency/facility staff who own and typically operate the equipment assist the crew in its operation. Ensure that the movement plan has sufficient personnel to be as safe as possible.

- Ensure safe and proper stretcher movement near and around the aircraft to include stretcher control during patient loading and off-loading procedures. Stretcher control includes ensuring the stretcher brakes are engaged and/or assigning a teammate to physically hold the stretcher in place during patient transition. Move the stretcher in the most appropriate height for safe movement.

63.2.5 Hot and Cold Loading and Unloading

- The flight crew will assess the medical need for hot loading/unloading of equipment, personnel, or patients. Examples are departures from rooftop helipads or scene calls when the helicopter is left running, and critical patients requiring emergency off-loading.
- The pilot must give permission for hot loading/unloading. Operational or safety considerations may override medical needs.
- If a third rider (orientee or family member) accompanies the flight crew on a patient transport, the third rider must be accompanied to and from the aircraft by a member of the flight crew. It is recommended that the aircraft be shut down for on/offloading when a third rider is present. However, this decision can be left to the pilot and crew on duty depending on the situation and location.
- It is preferred but not required that hospital security or other designated personnel be present at the hospital helipad when the helicopter is landing and departing.
- Only the minimum number of trained personnel are allowed under the rotor disk during the hot loading/unloading process. All personnel and passengers should wear hearing and eye protection, if possible.
- The flight crew must assign one ground personnel member on scene during loading and unloading to be in charge of the stretcher to ensure it does not roll away and come in contact with any part of the aircraft.
- Ensure loose items like blankets are secured when hot loading/unloading.
- No one will be allowed past the designated safety zone dictated by airframe model during hot loading/unloading.
- The pilot may be required to remain at the controls of the running helicopter, except to provide security, as defined in the General Operations Manual.

63.2.6 Ambulatory Patients

- Patients who are ambulatory with unrestricted mobility privileges while at the referring facility, may be allowed to ambulate onto and off the aircraft if after a thorough assessment the following criteria have been met:
 - The patient does not utilize assistive devices to ambulate such as a cane or walker.
 - The patient's vital signs are within the acceptable range for their age and developmental stage.
 - The patient is not receiving oxygen at four (4) liters or greater as therapy for an acute disease process.
 - The patient is not actively receiving fluid resuscitation (boluses) or medication infusions.
- The clinical team will escort the patient from behind with one hand on the patient while boarding and deplaning the aircraft, and the utilization of a gait belt on the patient for this purpose is strongly encouraged.

63.2.7 Crew Safety during Patient Movement

- Company-provided high visibility, reflective safety vests will be worn on all scene flights in accordance with the ANSI-SEA 107 standard.
- Employees will at all times be cognizant of roadway hazards and request the proper agency, e.g., local law enforcement or fire department, to shut down roadways that pose any potential threat to the aircraft, crew members, or patient(s). Consider using hospital security for controlling private roadways commonly found on hospital campuses or similar areas. A public road is any road, street, or similar facility under the jurisdiction of and maintained by a public agency and open to public travel.
- Anytime flight crews are operating on or near a non-public road, or other area in which vehicles may be in operation, they will evaluate the need for, and advantages of, wearing reflective safety gear for that particular operation. Reflective safety gear can also be beneficial in non-vehicle-related activities such as providing patient care in a large field, wooded area, near large bodies of water, mass casualty scenes, etc., to provide greater visibility of the rescuer to assist with personnel accountability and scene management. In most situations, clinicians should not cross public or nonpublic roads with a patient on a stretcher unless the circumstances dictate that this is a safer option than loading/unloading the patient from an ambulance for movement.

64 Patient Packaging Services

64.1 Purpose

Air Methods recognizes that in certain situations, it may be beneficial to have one or more clinicians prepare patients for air medical transport at the bedside at the referring hospital, prior to the aircraft's arrival at the referring hospital. This purpose of this policy is to set forth two situations in which this may occur and the protocol to be followed in each situation.

64.2 Terminology

Pre-Flight Specialist (PFS) - an Air Methods flight nurse and/or paramedic who prepares patients for air medical transport prior to the arrival of the Air Methods aircraft and flight crew at the referring hospital. The PFS may be either a Dedicated PFS or an Assigned PFS:

- **Dedicated PFS** - An Air Methods flight nurse and/or paramedic who is not stationed at an Air Methods base but is instead stationed at a referring hospital, and who will prepare patients for air medical transport prior to the Air Methods clinical crew's arrival at the referring hospital.
- **Assigned PFS** - An Air Methods flight nurse and/or paramedic who is stationed at an Air Methods base either at or nearby the referring hospital, and who will prepare patients for air medical transport when the aircraft at his/her Air Methods base is unavailable due to maintenance, weight limits, or pilot or clinical staffing issues, and another Air Methods aircraft and clinical crew will perform the air medical transport.

Packaging Hospital - a referring hospital with a written agreement with Air Methods for patient packaging services by either a Dedicated PFS or an Assigned PFS. The Packaging Hospital will be either a Dedicated Packaging Hospital or an Assigned Packaging Hospital:

- **Dedicated Packaging Hospital** - A referring hospital with a Dedicated PFS stationed at the hospital.
- **Assigned Packaging Hospital** - a referring hospital where one or more Assigned PFS(s) provide patient packaging services when the local Air Methods aircraft is out of service due to maintenance, staffing or other reasons, and another Air Methods aircraft and clinical crew will perform the air medical transport.

64.3 Requirements

Dedicated Packaging Hospital Requirements

- A written agreement with the hospital, prepared and approved by the Air Methods legal department and executed by Air Methods and the hospital.

Assigned Packaging Hospital Requirements

- Except for packaging services performed on an occasional but rare basis, a written agreement with the hospital, prepared and approved by the Air Methods legal department and executed by Air Methods and the hospital.
- The local Air Methods base must be at the Assigned Packaging Hospital or nearby, so the Assigned PFS(s) can report to the Assigned Packaging Hospital without utilizing an ambulance, as directed by leadership on a case-by-case basis.
- There should be a second Air Methods aircraft in the geographic area, which is able to accomplish the transport.

64.4 Process

Process for a Dedicated Packaging Hospital:

Once the decision has been made by the Dedicated Packaging Hospital to transport a patient utilizing an Air Methods aircraft, the Dedicated Packaging Hospital will contact AirCom utilizing the customary process. Once the flight is accepted by the pilot, AirCom will dispatch the aircraft and notify the PFS via telephone or FlightVector pager to report to the patient bedside to prepare them for transport while awaiting the inbound aircraft/team.

Process for an Assigned Packaging Hospital:

Once the decision has been made by the Assigned Packaging Hospital to transport a patient utilizing an Air Methods aircraft, the Assigned Packaging Hospital will contact AirCom utilizing the customary process. When the local aircraft is unavailable due to operational issues such as maintenance, weight limits, pilot or clinical staffing issues, AirCom will dispatch the next closest Air Methods aircraft as is customary and notify the Assigned PFS(s) via telephone or FlightVector pager. The Assigned PFS(s) will respond with acknowledgment. The Assigned PFS(s) will report to the patient bedside and prepare them for Air Methods transport while awaiting the inbound aircraft. The local aircraft will be considered unavailable during the time the Assigned PFS(s) are providing packaging services. Once care has been transferred to the transporting flight crew, the Assigned PFS(s) will return to their base.

64.5 Policy

The PFS will report to the patient's bedside with all Air Methods medical equipment and supplies necessary to safely prepare the patient for transport. The cardiac monitor cables, ventilator circuit and infusion drip sets will be changed to the transporting teams' devices (the packaging base medical equipment will remain at the packaging base).

The PFS participating in the patient stabilization and preparation for transport will practice under direction of the referring physician. If the referring physician is not physically present the PFS may consult with the referring physician by telephone or utilize on-site medical control.

The PFS may perform invasive procedures identified in the flight clinicians' scope of practice and under the direction of a physically present physician.

The PFS may institute advanced cardiac life support (ACLS) therapy without a physician order while awaiting physician orders in the following circumstances:

- Initial treatment of life-threatening cardiac arrest situations.
- When failure to take immediate action may result in loss of a patient's life, severe suffering or disability.
- AT NO TIME MAY THE PFS CIRCUMVENT THE REFERRING HOSPITAL PHYSICIAN.
- In the event the flight clinician receives an order from the referring physician that the clinician determines to be outside their scope of practice or detrimental to the patient's care, the flight clinician should consult with medical control.

If the PFS is caring for a critically ill patient where continuity of patient care may be deemed beneficial, the PFS(s) may remain the primary care provider during transport subject to weight and balance considerations as determined by the pilot.

64.5.1 Alternate Scenarios:

If a rotor wing crew responds to the bedside and is unable to provide transport by rotor wing, the flight clinicians may transport the patient by fixed wing aircraft.

In the event the mode of transport is changed to fixed wing, the RW clinicians will respond to the bedside as assigned PFS(s) to package the patient for transport. These PFS(s) will then transport the patient in a ground ambulance to the responding fixed wing aircraft at the airport, and transfer care to the fixed wing team. If a patient is to be transported in a ground ambulance to the airport, there must be two flight clinicians attending the patient. If only one flight crew member is packaging, the transporting flight clinicians will need to respond to the patient's bedside per normal operations.

The PFS(s) will provide stabilization and preparation for transport utilizing Air Methods Patient Care Guidelines in conjunction with the on-site medical control. Transfer orders from the referring physician or telephone consult with the referring physician is acceptable. The referring physician will maintain medical control of the patient while in the PFS's care and should be consulted throughout the care provided.

In the event that a patient has been prepared for transport by the PFS(s) and the inbound Air Methods aircraft is canceled, the PFS(s) will remain with the patient and continue to assist with care until released by the on-site referring physician. The PFS(s) may offer to transport the patient with the local ground ambulance team, if appropriate.

The PFS(s) should make every attempt to have the patient ready for transport by the time the transporting aircraft and flight crew arrives. The PFS(s) shall provide a full verbal report of the patient's condition and care provided prior to transfer of care to the transporting flight crew. The PFS(s) and transporting flight crew must verbalize the time transfer of care took place in order to ensure that double documentation does not occur.

Each patient encounter by a PFS shall be documented in a patient care record (PCR). The PFS(s) shall create a PCR in emsCharts utilizing the flight number for the transporting aircraft. The PFS(s) shall place the letter "P" after the flight number in emsCharts. The outcome for the packaging chart is "Packaging Shell (CBS only)." The PCR should include treatments completed prior to arrival, assessment of the patient, and any interventions provided by the PFS(s). The PFS(s) must print an EKG strip and vital sign trends from the cardiac monitor as required by Air Methods documentation standards. The EKG should be mounted, uploaded to DIS, and attached to emsCharts by the PFS.

The PFS shall obtain and complete the appropriate billing forms for every patient encounter. The following forms and documents shall be completed/obtained by the PFS(s) and transferred to the transporting flight crew:

- Authorization and Consent Form (ACF)
- Physician Certification Statement (PCS)
- Referring facility face sheet
- Referring facility chart

Only one set of billing paperwork is required for the entire patient encounter. The transporting flight crew shall ensure that the above-listed documents have been completed/obtained, and that the receiving facility face sheet has been obtained prior to releasing the patient to the care of the receiving facility. The transporting flight crew is responsible for submission of the completed/obtained documents and any other relevant paperwork to DIS.

The transporting flight crew shall complete a PCR in emsCharts as per normal procedures utilizing the outcome “Treated and Transported.” The referring facility will be the pickup location and receiving facility where the patient was dropped off. Notation should be made in the PCR that the patient was prepared for packaging by an Air Methods crew.

64.6 Limitations

PFS's may not provide clinical services to non-Air Methods patients except in rare, emergency cases. If the hospital requests a PFS to provide such services, the PFS must contact their Clinical Director immediately.

All PFS services are for the benefit of the patient. No remuneration shall be offered or provided to the hospital by Air Methods personnel, unless pursuant to a written agreement approved by the Air Methods legal department.

Contact the Legal Department for further evaluation of particular packaging arrangements.

64.7 Expected Outcome

Stabilization and preparation for air medical transport will be rendered by Air Methods clinicians who are on duty and/or present at the packaging or designated hospital according to policy. The expected outcome of this process is a decrease in the time from flight request to the receiving hospital, and the provision of a high level of care to the patient to be transported. The packaging program is provided to our partner hospitals as an additional method to support our customers.

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65 Patient Records

65.1 Purpose

Due to Air Methods having to respond to after hour requests for medical records, this procedure provides proper direction.

65.2 Policy

65.2.1 Urgent Requests

If the request is urgent and after hours the requester may email DIS at dis@airmethods.com or fax the request to 888-540-3845. Urgent requests are narrowly defined for only two requester types: Transplant Services and Coroners/Medical Examiners. Others requesting an urgent response will be immediately referred to management staff for review.

Air Methods needs to validate they are who they represent themselves to be and the organization they represent. The request needs to include four components:

1. Who they work for, their title, and role
2. What record(s) do you need including patient name, approximate date of transport, type of records required
3. Why they need the records (required by HIPAA to document the need)
4. What is the best way to get them the record

If the request includes these components, is a HIPAA compliant request, and the record has been created and has been locked in Air Methods designated patient care charting system, then it will be sent out quickly. Turnaround time should be less than an hour if all the above is in order.

65.2.2 Non-urgent Requests

This applies to non-urgent requests or urgent requests during Business Hours (M-F 8 a.m.-5 p.m. Pacific Time)

If a request is not urgent or is urgent and during business hours the requester can contact Air Methods Medical Records (part of the PBS department) by calling (leaving a voicemail after hours if they simply have questions) 909-915-2930 or submit a request via email at PBSRecords-Legal@airmethods.com or by fax at 402-952-2413.

All of DIS has been trained to handle only after hour urgent requests as defined within this new procedure.

66 Patient Refusal of Treatment/ Transport

66.1 Purpose

To establish criteria for documentation of flight requests when the patient refuses treatment or transport releasing Air Methods, LLC from responsibility.

66.2 Policy

As with all medical care, patients retain the right to make decisions regarding their care. As a result, a patient may make the decision to refuse transport or specific treatments during our care. When a patient chooses to refuse care, the clinician should utilize this form and these instructions:

A refusal form (Refusal of Transport and/or Treatment Form - 5196) is utilized when a patient refuses a specific treatment that is needed or refuses transport after contact the transport crew. A sample of this form can be found in the Appendix of this manual. Actual form is located on FlightDeck > Resources > Corporate-Wide Forms (Air Methods).

Medical Control must be contacted via a recorded line for consultation and to ensure the patient has medical decision making capacity prior to allowing the patient to refuse air transport or treatments.

The clinicians must provide thorough and accurate information to the patient regarding the transport and/or treatment being refused. Verify that the information the clinician understands about the patient's condition or the events that occurred are accurate. Advise the patient of the complications and the possible outcomes that could result from the patient's refusal. Explain the severity of the patient's injuries/condition to the patient. Provide the patient with instructions on how to seek care or treatment if they change their mind. Ask the patient (or parent/legal guardian, if applicable) if they understand the instructions and if they have any questions.

It is vital that the clinicians obtain and document as much information as possible about the patient, injuries, and circumstances.

Patients that wish to obtain a copy of this form and/or the Patient Care Report may request the record utilizing the following QR code or by going to www.airmethods.com/contact and sending a Medical Record Request.

If the patient meets the requirements to be able to refuse the transport and/or treatment, the transport or specific treatment refused will be canceled.

The refusal form is then to be uploaded into emsCharts as an attachment. A complete emsCharts record will be created and include the following assessments, regardless of the patient's cooperation:

- Mental Status.
- All assessments that can be performed visually.
- Suspected alcohol/drugs (and the evidence providing that information).
- Patient's response to refusal of care or completion of the form.
- If applicable, the circumstances or reasons why the crew was unable to fully advise the patient about the risk and potential consequences of refusing transport and/or treatment.
- Advice and instructions given to the patient (or parent/legal guardian).
- Witness(es)' names and titles (including agency name or identifying number, such as a badge number).

Crew should always attempt to perform and document (if the patient allows):

- A full head-to-toe assessment.
- At least one set of vital signs.

At least two witnesses must be present for the refusal. Witnesses may be Air Methods clinicians, other healthcare providers, or bystanders on scene. All witnesses present, regardless of whether they sign the refusal form, must be documented in the PCR.

A PSAP must be completed in Protean.

67 Patient Report Guidance

67.1 Purpose

To assist the clinician in verbal discussion and report with other healthcare providers in a formatted and concise manner.

Accurate communication is vital to the safety and continuity of patient care. Whether giving a report to a receiving facility or discussing patient care with a Medical Director, communication must occur in a manner that is concise, but thorough. Having a standard method of communication will allow for all communication to occur in a manner that will protect the patient, clinician, provider, and company.

1. When communication is required, start by recognizing what the purpose of the conversation will be.
 - a. If you are seeking to obtain an order from a Medical Director, go into the conversation knowing exactly what you want the order to say.
 - b. For example, if you are calling to obtain an order to increase the ventilator PEEP to 18, have this in your mind during the conversation.
2. Utilization of a communication framework is useful in providing concise information, but in a consistent format.
 - a. Use of SBAR communication will help to ensure that you provide all of the information needed for the listener to decide or ask questions.
 - b. Air Methods clinicians are expected to utilize the SBAR format when communicating with a Medical Director or Clinical Director about potential or actual problems with a patient.

67.2 Use of SBAR Communication Format

1. Always start with the situation at hand.
 - a. Identify yourself and your position.
 - b. What is the name of your program and base location.
 - c. What is the name of the hospital or referring location you are positioned.
 - d. State what is wrong with the patient (concisely)

TABLE 67.1 Situation

S	Situation: What is the situation you are calling about? • Identify self, unit, patient, room number. • Briefly state the problem, what is it, when it happened or started, and how severe.
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2. Give the background of the problem.
 - a. Explain the patient's diagnosis or differential diagnosis to the listener.
 - i. Give an accurate description of their illness as it relates to what you are calling about.
 - b. Provide any details to the listener that are vital to the decision that needs to be made about the patient (if calling for an order).
 - c. Provide any details to the listener that are vital to the continuation of safe care for a receiving provider.

TABLE 67.2 Background

B	Background: Pertinent background information related to the situation could include the following: • The admitting diagnosis and date of admission. • List of current medications, allergies, IV fluids, and labs. • Most recent vital signs. • Lab results: provide the date and time test was done and results of previous tests for a comparison. • Other clinical information. • Code status.
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3. Provide the listener with an assessment of the patient or problem.
 - a. Describe for the listener what your assessment of the patient is as it relates to the needs you are calling about.
 - b. Only provide an assessment that is related.
 - i. For example - do not provide an assessment of the patient's central line site when you are calling about ventilator settings.

TABLE 67.3 Assessment

A	Assessment: What is the clinician's assessment of the situation?
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4. Ask for recommendations or make a recommendation for what you want.
 - a. State exactly what you want your order to state.
 - i. If you would like an order to increase your PEEP to 18, then state, "I would like to increase the PEEP to 18."
 - b. If you are unsure of what you want, then explain to the listener that you are unsure of how to proceed.
 - c. If you have an idea, but not an exact order, state that you are unsure, but believe what you would like.

TABLE 67.4 Recommendation

R	Recommendation: What is the clinician's recommendation or what does he/she want? Examples: • Patient critical (Acute change in a patient's condition requiring immediate lifesaving treatment). • Propose a higher level of admission (medical floor versus ICU). • Order changes/needs (address patient care that wasn't provided during transport).
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68 Patient Safety Action Program

68.1 Scope

The Patient Safety Action Program chiefly serves the Community-Based Services (CBS) Division of Air Methods, where clinicians provide healthcare under the guidance and supervision of Air Methods clinical and operational leadership.

The Patient Safety Team is available as a safety resource to Air Methods hospital-based service (HBS) and alternative delivery model (ADM) programs, which operate under their own clinical guidance and supervisory structures.

68.2 Purpose

At Air Methods, safety is our top priority first and always. Each day, our objective is “Everyone comes home.” The mission of the Patient Safety Action Program is that Air Methods’ Patient Safety structure, process, and outcomes will support patients and crew returning safely home after every mission. The vision of the program is that Air Methods will provide the safest possible healthcare in the air medical transport environment.

Air Methods is committed to a culture of safety where teammates report all actual and potential risks to patient safety through the Patient Safety Action Program (PSAP). The PSAP system will identify risks and their causes, review the data for trends, and use the data to inform and improve systems processes within the context of a Just Culture framework. The program functions with the underlying assumption that the critical care transport environment is a complex system in which human error and technology failure will occur. Safety analysis and mitigation is conducted with a focus on both human factors and systems processes which impact clinician and patient safety.

The Patient Safety Action Program is an integral part of the Air Methods Safety Management System and follows the principles of the Air Methods Safety Policy wherein a commitment to safety, continuous improvement, clinical risk management, compliance with regulation, non-jeopardy in safety reporting, and teammate safety responsibility are all promoted. The Chief Nursing Officer/Senior Vice President of Clinical Operations and all members of the Clinical Leadership team are responsible for integration of the Safety Management System in all aspects of clinical care delivery, including safety policy, safety assurance, safety risk management, and safety promotion.

Patient safety is a component of high-quality healthcare, and the Patient Safety Action Program supports and functions in parallel to the Clinical Quality Program to support the provision of quality clinical care by Air Methods clinicians. The process for evaluation of clinical quality and performance is outlined in the Air Methods Community Services Quality Management Plan.

68.3 PSAP Reporting Procedure

When a transport encounter is completed by air or ground, the clinicians will complete the medical record and associated quality improvement reporting debrief. In the debrief encounter, the system will prompt the clinician to file a PSAP report for any potential or actual patient safety concerns. In accordance with the Air Methods Safety Management System (SMS), teammates are accountable to report hazards and safety concerns observed during the course of their work. If a potential or actual safety concern associated with a transport encounter is identified but not reported, the reporting inquiry will follow the Just Culture framework and clinicians will be provided with an opportunity to report identified safety concerns by any member of the healthcare team, including peer reviewers, Clinical Base Leads (CBLs), Physician Medical Directors, Clinical Directors, or Patient Safety leadership.

PSAP reports are a component of the Air Methods patient safety and healthcare quality improvement process and are part of the Patient Safety Work Product (PSWP) generated by the organization with the intent to improve patient safety and healthcare quality. The reports and all work product generated from them are protected with all applicable privilege and confidentiality protections specific to health information, medical peer review, and healthcare quality improvement.

PSAP reports include patient information that is protected health information (PHI) as defined by the Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule and are collected and maintained within a closed system in order to provide sufficient confidentiality and privacy protection. Any disclosure of PSWP for business operations is managed by Air Methods executive leadership team and general counsel. Air Methods teammates who use PSAP information to participate in patient safety and healthcare improvement activities are subject to all applicable privacy and confidentiality rules. Any questions or concerns about privacy or confidentiality related to the reporting or disclosure of PSWP will be directed to the Air Methods Compliance Office.

PSAP reports contain both a narrative and objective data reporting section. Reports are specific to one transport encounter and may include multiple safety concerns and investigation components. Clinicians must report only objective information related to direct care of the patient in the patient care record; the PSAP report may include additional details specific to the clinical event (e.g., what happened, contributing factors, details of the event disclosure to the receiving facility and Air Methods leadership, equipment taken out of service, etc.).

PSAP reports are used for reporting hazards and risks specific to clinical care delivery. Any other safety concerns (aviation, operations, etc.) can be reported through the AIDMOR system or other safety reporting channel.

68.4 PSAP Evaluation and Investigation

Following submission, the PSAP report will be evaluated and investigated by both the regional clinical leadership team and the patient safety team. Leaders may request more information about a safety concern or may schedule a case review to further evaluate and mitigate risks to patient or clinicians safety. Case reviews are a confidential activity designed to evaluate a clinical case through the lens of our Just Culture and intended to identify opportunities to improve patient safety and healthcare quality; all applicable privacy and confidentiality protections are extended to case review activity. Case reviews will follow the policy designated in the Medical Operations Manual.

When a PSAP is investigated, there are several possible next steps:

- Track and Trend. A safety report may be evaluated and closed to track and trend, with no immediate action item identified for mitigation. For example, a PSAP report may be submitted for an uncomplicated device dislodgement event which was mitigated for the patient at the point of care: here, there are no immediate systems mitigation actions required and this event report may be closed and aggregate data on this type of event will be trended for quality, safety, or risk concerns.
- Investigation and Mitigation. A safety report may be evaluated where a specific event or issue identified will require further investigation, cause analysis, or risk mitigation prior to report closure. For example, a PSAP report may be submitted for medical equipment failure when a cardiac monitor cable fails during the course of patient care; the cable will be taken out of service and repaired or replaced, and the PSAP report can be closed with the aggregate data on this type of event evaluated for trends.

- Cause Analysis. A safety report may be reported and the leader may determine that a significant adverse/safety event or a Serious Safety Event may have occurred. The Section 69 Patient Safety and Clinical Investigation process as outlined in the Medical Operations Manual will commence to determine the cause impact, and appropriate mitigation actions. The PSAP report will be closed when that process is complete.

68.5 Patient Safety Event Classification

A patient safety event is an event that potentially or actually results in unintended harm to the patient by an act of commission or omission rather than by the underlying disease or condition of the patient. Events which reach the patient are designated as patient safety events and events which have the potential to reach the patient but do not are near-miss events.

Patient safety events are defined by the Air Methods Patient Safety Event Classification through a standardized process and standard operational definitions:

- **Serious Safety Events:** events which result in severe harm or death.
- **Safety Events:** events which result in no detectable harm through moderate harm.
- **Near-Miss Events:** events which had the potential to cause harm except for a break in the chain of causality.

Presence or absence of a safety event is determined by first evaluating the case for any deviation in performance standards. This includes human performance standards and also includes such standards as mechanical performance of patient care equipment or deviation in internal or external professional guidelines or policies, etc.

Even if a patient safety event did not occur, the clinical course may involve an adverse outcome which was not a result of any failure in technology, device, process, or practice decision. Such outcomes may still require a clinical case debrief, clinical investigation, or evaluation of risk in relation to clinical or operational decision-making and actions.

All events will be categorized with this system for individual and aggregate data analysis. All potential Serious Safety Events (SSEs) will be communicated with Patient Safety leadership and will undergo root cause analysis for risk mitigation as soon as possible. Any other event may prompt cause analysis at any level, particularly if a trend is identified in clinical care or operations.

FIGURE 68.1: Patient Safety Event Classification Process

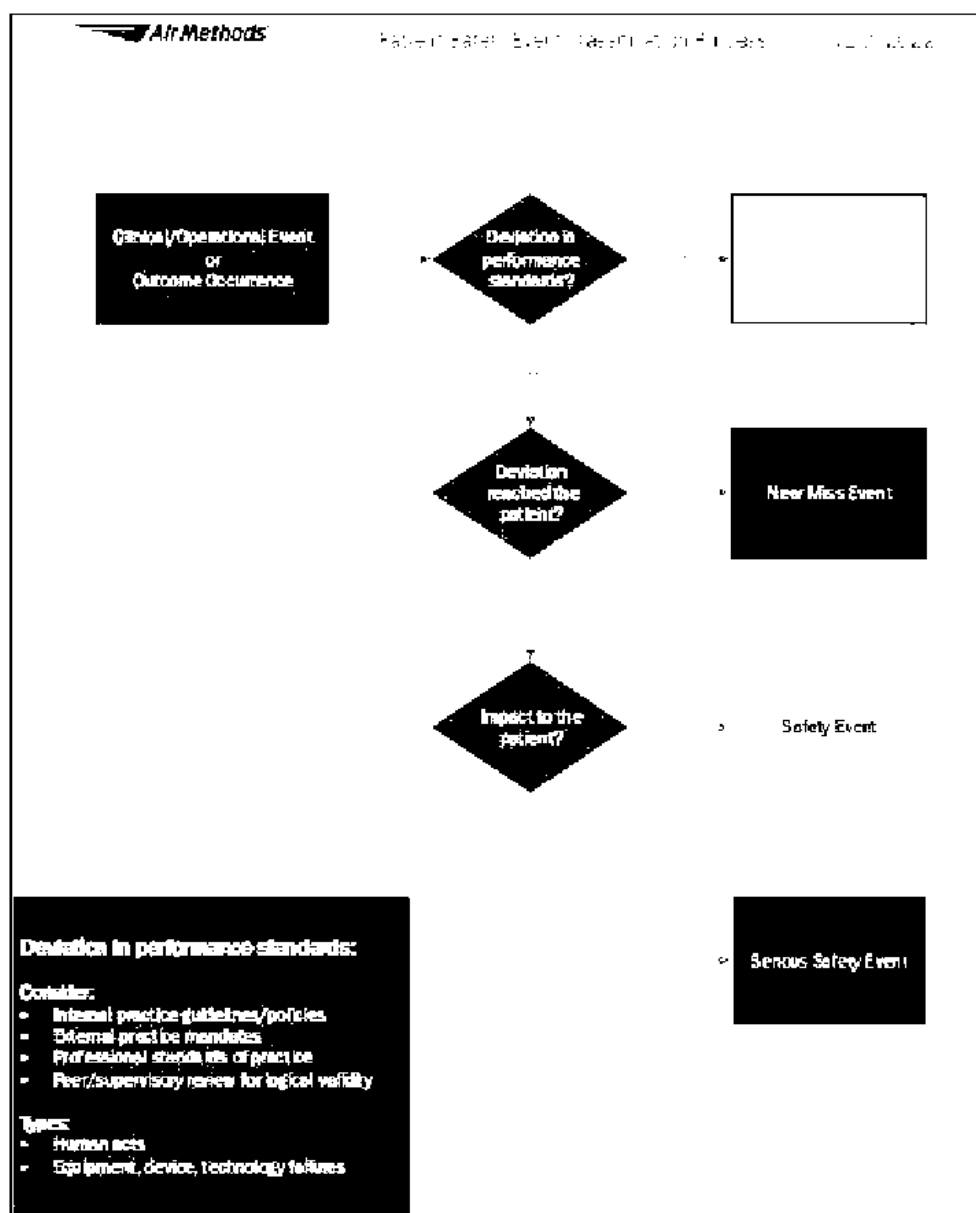


TABLE 68.1 Patient Safety Event Classification

Patient Safety Event Classification			v1.0, 2022
Safety Event Classification	Level of Harm	Code	Operational Definition
Serious Safety Event	Death	SSE1	Patient Safety Event: Incident which reached the patient and caused death through failure to recognize and treat a condition or incorrect/insufficient treatment provided for a condition. For example: • Lethal arrhythmia unrecognized and untreated which progresses to arrest/death. • Inadvertent needle thoracostomy placement into the aorta which results in exsanguination/death. Patient Outcome: Unexpected death not related to the natural or expected course of the patient's illness or underlying condition. On balance of probabilities, death was brought forward in the short term by the incident.
Reaches the patient	Severe Permanent or Temporary Harm	SSE2	Patient Safety Event: Incident which reached the patient and caused severe harm, defined as bodily or psychological injury (including pain or disfigurement) that interferes substantially with functional ability or quality of life. For example: • Chest tube placement into a solid organ which requires surgical evaluation and treatment. • Needle thoracostomy outside of the pleural space requiring prolonged hospitalization for evaluation and treatment of the patient's condition. • Delayed recognition of esophageal intubation resulting in prolonged hypoxia. Patient Outcome: Symptomatic, requiring life-saving intervention or major medical-surgical intervention; shortened life expectancy or major, permanent or temporary harm or loss of function.

Patient Safety Event Classification			v1.0, 2022
Safety Event	Moderate Permanent or Temporary Harm	SE 3	Patient Safety Event: Incident which reached the patient and caused moderate harm, defined as bodily or psychological injury adversely affecting functional ability or quality of life, but not at the level of severe harm. For example: - Any iatrogenic condition which requires an extended ED/hospital encounter to diagnose and/or treat, such as a patient's arm becoming caught in the stretcher system and sustaining a laceration which required sutures, or a peripheral IV infiltration requiring administration of antidote therapy. - Medication error which requires additional monitoring or treatment. Patient Outcome: Symptomatic, requiring intervention (e.g., additional minor/moderate operative procedure, additional therapeutic treatment) and/or an increased length of stay; or, causing minor/moderate permanent or temporary harm, or loss of function.
Reaches the patient.	Mild Temporary Harm	SE 4	Patient Safety Event: Incident which reached the patient and caused mild harm, defined as bodily or psychological injury resulting in minimal symptoms or loss of function, or injury limited to additional treatment, monitoring, and/or increased length of stay. For example: - Patient bumped their head on the side of the aircraft and sustained a minor abrasion which did not require additional evaluation or treatment. - Peripheral IV infiltration not requiring treatment above the level of general observation. - Preventable delay in care not caused by safety operations with known impact or strong potential for clinically significant impact later in clinical course. Patient Outcome: Symptomatic; symptoms are mild, loss of function or harm is minimal or intermediate, but short-term, and minimal (e.g., extra observation, investigation, review, or minor treatment) or no intervention is required.

Patient Safety Event Classification			v1.0, 2022
	No Detectable Harm	SE 5	Patient Safety Event: Precursor incident which reached the patient but did not cause apparent harm. The safety event occurred and impacted patient care, but the patient suffered no apparent harm as a result of the event; care was able to be delivered in a manner consistent with internal and external standards of care. For example: • Oxygen miscalculation requiring change of aircraft to access sufficient oxygen supply for the patient. • Equipment malfunction which may or may not have required change in patient care operations, but which did not produce apparent harm. • Stretcher dropped but did not injure the patient. • Preventable delay in transport not ascribed to safety/risk operation that did not impact clinical course. Patient Outcome: Asymptomatic. No symptoms are detected, and no treatment is required, Not able to discover or ascertain the existence, presence, or fact of harm, but harm may exist; insufficient information is available, or unable to determine any harm. Harm may appear later.

Patient Safety Event Classification			v1.0, 2022
Near Miss Event Does not reach the patient	Almost Happened	NME 6	Patient Safety Event: Near miss/close call event that did not reach the patient OR any unsafe condition or circumstance which increased the probability of a patient safety event occurring. This event could have had adverse consequences but did not and was indistinguishable from fully fledged adverse events in all but outcome. - Medication labeled incorrectly with error detected prior to administration. - Flight aborted for weather after risk assessment completed. - Unable to obtain vascular access on patient prior to transport. - Equipment malfunction with equipment not taken on a flight. Patient Outcome: No safety event reached the patient. The error or capacity to cause harm was caught by an error detection barrier prior to reaching the patient. The system worked, and the patient was unharmed.
Not a Safety Event	No Safety Event Happened	NS 7	Patient Safety Event: None. An event was reported but no actual or potential safety event occurred in this patient encounter.

68.6 PSAP Action

Following event classification and any necessary cause analysis, action items may be identified which will improve teammate or clinical systems safety. The action item, plan for completion, accountable leader, and identified time for completion should all be documented in the safety reporting system, with any applicable documents or correspondence saved to the system for future reference and evidence of loop closure.

Action items able to be managed at the local or regional level will be documented by the responsible leader; any action items which would benefit from enterprise-wide application (e.g., interdisciplinary Safety Risk Assessment, Safety Bulletin, policy change, etc.) should be communicated with Patient Safety Leadership for enterprise patient safety awareness and leadership loop closure.

The PSAP report may be closed prior to completion of all action items specific to safety mitigation but can be reopened by Patient Safety leadership for any additional information to be added as it becomes available. Written notes should be entered in the file to account for additional information noted.

When patient safety-related action items are requested outside of the PSAP reporting system (e.g., referred from a CAPA or hazard register), these action plans and projects will be accounted for in that registry system and results made available to clinical and safety leadership through the usual standard safety reporting forums.

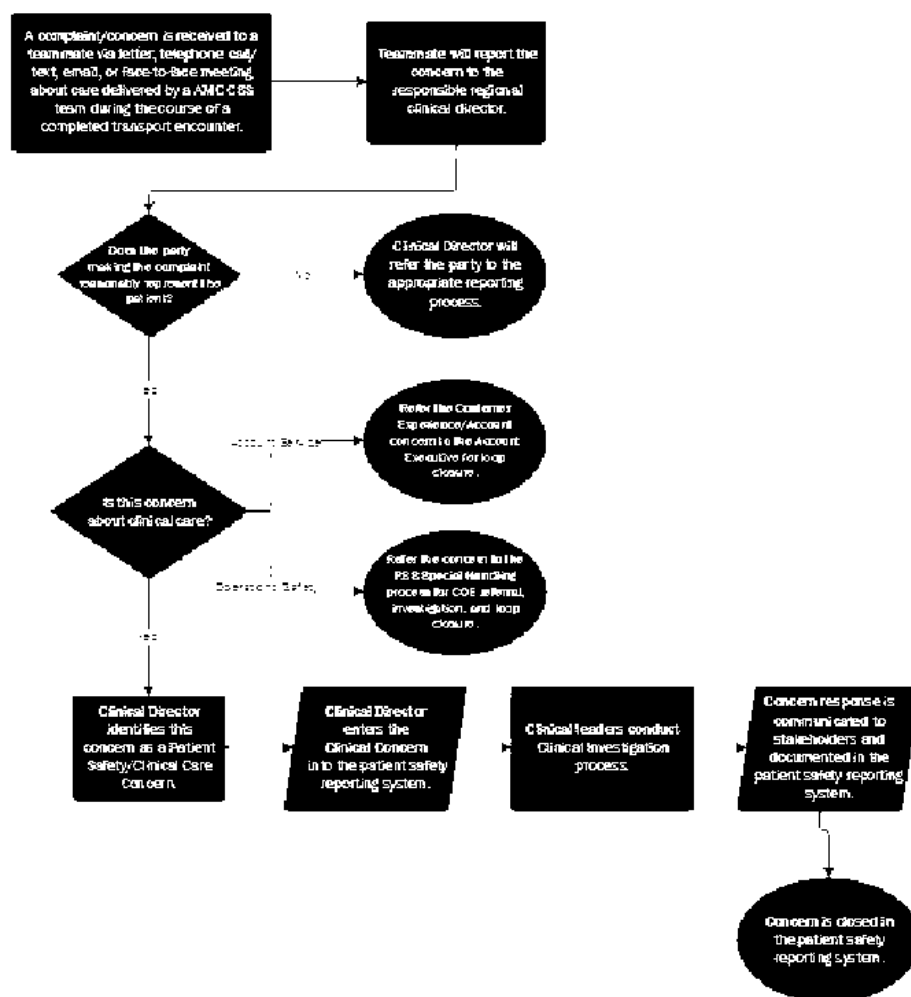
68.7 Patient Safety and Clinical Care Concerns

Patient safety and clinical care concerns ('clinical concerns') may be reported by patients, families, or healthcare system representatives after a patient flight/transport is completed.

These events will be individually evaluated to determine who will handle the concern and how it will be mitigated and reported. Any concern for clinical care or patient safety will be thoroughly evaluated to ensure that it is routed to the proper channel and follows a compliant and uniform process. If a concern is reported to Air Method about a flight which does not concern patient safety or clinical care, it will follow the usual PBS Special Handling Process for billing/account complaints or the Customer Experience/Sales process for general customer service complaints.

The Patient Safety team will coordinate and report the clinical complaints/concerns related to care delivered during completed transports in the patients safety reporting system and ensure that communication, action tracking, and loop closure are completed for any concerns about patient safety or clinical care within a timely manner. Screening and management of clinical concerns follow this process:

FIGURE 68.2: Patient Safety Clinical Care Concerns



This process is specific to external stakeholder or patient concerns. Internal concerns about clinical care will be relayed to the appropriate clinical leader for evaluation and mitigation through the usual quality improvement process. Any concerns with clinical or enterprise risk management implications will be escalated by the Patient Safety Team to Air Methods legal and clinical leadership.

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69 Patient Safety and Clinical Investigation

69.1 Scope

Investigation of patient safety or clinical care concerns related to Air Methods CBS Programs.

69.2 Background

Clinical leaders will investigate concerns with a standardized approach to identify hazards and risks specific to any clinical event which has impacted the patient and has the potential to be a serious safety event (SSE) or could potentially impact clinical or business operations. This process is outlined in such a way to promote identification of hazards and risks, isolation of root and proximal causes, and facilitation of timely mitigation action to resolve any issues.

69.3 Procedure

69.3.1 Administrative: Communication

- a. When a patient safety or clinical care concern is identified that has potential to be a serious safety event (SSE) or a significant clinical event occurs which could potentially impact clinical or business operations, the Clinical Director I (CD1) will communicate with the Clinical Director II (CD2) and Director of Patient Safety (DPS) to determine next steps. The CD2 will communicate the status to the regional VP of Clinical Services (CVP) and the CVP will communicate the status to the Chief Nursing Officer (CNO).
- b. In this instance, the Clinical Directors will notify the Area Manager (AM) of the investigation process and flight status determination. The AM will notify the HR Business Partner (HRBP) and the Regional VP of Customer Experience (RVP), as necessary.
- c. If the patient safety or clinical care concern potentially represents a serious safety event (SSE) as designated in the Patient Safety Action Program (PSAP) policy, the DPS will communicate with PBS to hold the patient's bill pending the investigation and determination.

69.3.2 Administrative: Information Collection and Storage

- a. The clinicians will complete the Patient Care Record (PCR) and PSAP report in the patient safety reporting system as soon as possible. This requirement must be completed before leaving the base unless approval is provided by the Clinical Director.
- b. General record storage (documentation, note-taking, narrative summaries) for the clinical investigation shall be in the patient safety reporting system and uploaded to the Leadership section. Any Patient Safety/Risk Management documentation will be uploaded to the Patient Safety/Risk Management tab.
- c. The CDs will ensure that the PCR is uploaded to the patient safety reporting system and will facilitate upload of any applicable data record from the cardiac monitor and/or ventilator electronic record, and any other supporting documentation pertinent to the patient care event.
- d. The CDs may request written statements from each of the clinicians and any involved internal or external participants as needed, should additional information apart from the medical record be required. Statements and email will be uploaded and stored in the Leadership tab.
- e. The CDs will coordinate with the Medical Director as necessary to obtain patient status and follow-up information. This information will be stored in the Leadership tab of the patient safety reporting system.
- f. The CDs will coordinate with any referring or receiving healthcare teams for any additional information necessary to evaluate the clinical concern. This information will be stored in the Leadership tab of the patient safety reporting system.

69.3.3 Investigation: Case Review

- a. The CD will schedule the review of the case. Meeting attendees will include the Medical Director, CD2/CD1, Clinical Educator/Regional Clinical Education Manager, and PSD. Optional attendees include the CBL, CVP, CNO, AM, REP, and HRBP, as determined by the nature of the clinical event.
- b. The case review will begin with the purpose statement designed in the Case Review policy, reminding participants that our clinical case review process is confidential and follow the Just Culture process.
- c. The case review will proceed as outlined in the Case Review policy. Case review notes will be stored in the Protean Leadership tab.

69.3.4 Investigation: Evaluation

- a. Following the case review meeting, the CD2 will coordinate a meeting with the PSD and the involved leaders to determine the level of safety event classification, any necessary cause analysis process, and all necessary safety and risk mitigation action items specific to clinical quality and safety protections. All SSE events will undergo root cause analysis, as detailed below.
- b. If there are any extenuating circumstances, clinical leadership may decide to convene the Multidisciplinary Incident Review Board (IRB) to review the event and all facts, conditions, and circumstances relevant to the case. The IRB process shall follow the guidance provided in the Air Methods SMS and the Multidisciplinary Incident Review Board Policies and Procedures Manual. The IRB process will not commence until the RCA process is complete, as the root cause conclusions and risk controls must be presented to the board as part of the event evaluation. Substitution testing may be implemented during the IRB process and can be coordinated with Clinical Education leadership. Notes and records of these proceedings shall be stored in the Leadership tab of the patient safety reporting system.

69.3.5 Investigation: Cause Analysis

- a. All SSE occurrences will undergo root cause analysis (RCA) to determine what caused the incident. Clinical event RCA follows the general guidance provided by the Air Methods Safety Management System (SMS) and focuses on clinical evaluation and clinical risk mitigation as documented in the Clinical Root Cause Analysis Record.
- b. The RCA will be facilitated by the PSD and credible, actionable risk controls will be agreed upon by the participants with accountable owners, implementation plans, and expected dates of completion noted.
- c. The RCA record is a part of the patient safety event reporting system and will be stored in the Leadership tab as patient safety work product. The RCA will be summarized by the PDS and presented to the CSC as required by the SMS process.

69.3.6 Investigation Outcomes: Action Plans and Next Steps

- a. **No issues identified: clinical investigation of the clinical concern did not find a patient safety event or any systems/teammate issues.** Process is complete and clinicians return to flight status, if not already active.

- b. **Issues identified: unintentional error or process deviation by teammate.**
Clinical investigation of the concern found an unintentional error or knowledge gap which represents risk to the organization but which can be addressed safely with existing improvement/learning processes, such as a Performance Improvement Plan (PIP) or Education Learning Plan (ELP). Clinical leadership coordinates the appropriate risk mitigation action plan. When the plan is implemented, the process is complete and the clinicians return to flight status, if not already active.
- c. **Intentional deviation from professional practice standards by a teammate: Clinical investigation of the concern found that the teammate(s) displayed an intentional violation of professional practice standards.** Teammates displayed a lack of integrity, dishonesty, falsification of documentation, intentional misrepresentation, harm to patient or others, disrespect to customers or others, noncompliance with company policy, and/or a violation of state or federal law. Clinical leadership shall determine the need for further investigation via IRB process and/or employment status decisions, as necessary. Any report required of this by a state board or local EMS authority shall be coordinated by clinical leadership and documented in the Leadership tab of the patient safety reporting system.

69.3.7 Investigation: Reporting/Communication

- a. Communication around clinical investigations and root cause analysis outcomes will be communicated to: regional clinical leadership shall communicate the findings to the regional VP, who will communicate them to the EVP and any other applicable ELT stakeholder.
- b. Serious safety events and the associated cause analysis outcomes will be reported to the ELT/SLT as described in the Air Methods SMS and recorded in the enterprise risk registry as required. All other record keeping will be maintained in the patient safety reporting system. The PSD will notify the Legal/PBS teams to process the patient account according to the procedures detailed in their Special Handling process.

69.3.8 Administrative: Process Completion

- a. When the investigation, outcome determination, and appropriate leadership communication are complete, the PSAP report and debrief session may be closed.

70 Patient Transports Utilizing a Ground (Surface) Ambulance

70.1 Purpose

To provide guidelines for Air Methods clinicians to safely perform patient transports by ground when unable to be completed by air due to operational reasons or patient condition.

70.2 Policy

- In cases where an interfacility transport request is unable to be completed due to weather, maintenance, or patient weight, the clinical team may be requested to perform the entire transport by ground. In these cases, the clinicians should respond to the bedside to assess the patient and discuss the transport request and logistics with the referring agency.
- Occasionally, due to remote landing zones and unavailability of helipads near the referring or receiving hospital, it may be necessary to utilize ground ambulance transport for partial phases of a patient transport.
- On scene responses may also require ground ambulance assistance, i.e. patient condition dictates that safe air transport cannot be completed and patient is to be transported to closest facility (i.e. cardiac arrest with active CPR).
- Every attempt shall be made to utilize a ground EMS agency that is either accredited by the Commission on Accreditation of Medical Transport Systems or which upholds the same standards as the Commission.
- The patient transport may be declined in instances where there are safety limitations which prevent the transport from being completed by ground in a safe manner, including:
 - The staff has timed out for fatigue/rest (and has notified the administrator on-call).
 - The clinical situation is beyond the clinical team's normal scope of care or training and specialty resources are unavailable at the time of transport; clinical leadership should be consulted prior to declining the transport.

- Ambulance availability: There is not a ground ambulance in acceptable working condition, with enough oxygen and/or capabilities to support the mission available.
- Inclement weather: The safety of the ambulance crew, flight team and the patient are of the utmost priority. All ground transports shall be placed on hold when severe weather or road conditions exist. These conditions may include ice, snow, heavy rains, heavy winds, tornadoes, or other severe inclement weather conditions.

In all cases where weather, personnel, or equipment is in question, clinicians will complete the Ground Ambulance Risk Assessment - 5723 and consult with clinical leadership for guidance.

- Patients transported in ground ambulances will be transported only on stretchers which are approved for and lock into those ground ambulances.
- Clinicians must also transport critical care equipment/medications off the aircraft to ensure all necessary equipment is on board the ambulance while in transit to or from the aircraft (to include functioning suction, sufficient oxygen, and sufficient power supply for all equipment) for the duration of the transport plus a 20 minute reserve to account for any unscheduled delays. The clinicians will ensure all essential on-board equipment is in working order (e.g., on-board wall O2, on-board suction, electrical outlets, and inverter) prior to loading the patient.
- Clinicians must complete the Air Methods Ground Transport Checklist - 5711 and the PACO Tool calculator prior to each ground transport; the checklist should be uploaded into Protean along with the patient care record and pertinent documents for review.
- Clinicians will notify AirCom at the following times and/or every 45 minutes throughout the ground transport.
 - Arrival on scene
 - Leaving referring (provide an estimated time to the receiving)
 - Arrival at the receiving
 - Arrival back at base
- Clinicians must stay with the patient throughout the entire transport to definitive care. In certain circumstances, care may be transferred to an equal level of care, e.g., another program flight team. Relinquishment of patient care must follow Air Methods PCG and Medical Operations Manual Policy Section 20 Continuum of Care.

- Clinicians are required to use seatbelts and shoulder harnesses provided in the ground ambulances. Clinicians must the patient and all equipment are appropriately secured prior to movement of the ambulance and during all phases of transport to ensure patient and crew safety.
- AirCom will generate a “flight number” for all flight requests, including those completed by ground. This number shall be utilized while completing the usual transport related forms including:
 - Authorization and Consent
 - Physician Certification Statement – PCS-01
 - Patient Care Record - 5201.
 - Protean Post-Flight Debrief, CQI & UR upon the completion of the transport
 - Air Methods Ground Transport Checklist - 5711 (should be included in the PCR information in Protean, but not included in emsCharts as this is for internal quality reference only)
- The ground ambulance must be staffed by the transporting agency in such a manner that state licensing regulations are met (i.e. If the Air Methods crew is a Nurse-Nurse crew and the state requires a Paramedic on board).
- The Air Methods crew may provide the transport agency with a copy of the PCR and applicable face-sheets for the purposes of billing.
- This policy applies to ground transports of patients that occur on an occasional basis. To the extent that such transports occur on a more frequent basis, a formal business agreement should be established. Leadership should contact the Legal Department for further evaluation of the arrangement. The transport should be completed in the meanwhile, and leadership should be advised following the transport. All such ground transports are subject to applicable anti-kickback requirements, and in no case shall any remunerations be offered, given, or accepted to or from a ground ambulance provider in exchange for referrals or the promise of referrals. The purpose of such ground transports are solely for patient care and continuity of care for Air Methods’ patients.

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71 Patients with Potential for Violent Behavior

71.1 Purpose

To address the procedure for transporting the psychiatric patient or those who exhibit anxiety, agitation, confusion, or delirium, define transport considerations, and the procedure for refusing to transport this type of patient when there is an issue of safety.

71.2 Policy

- Obtain appropriate data regarding the patient's history of presenting illness/injury, past medical history, medications, and allergies.
- Identify the risk factors for behavior that may escalate to violence:
 - History of Violence.
 - Behavior indicating potential violence through posture, speech, or motor activity.
 - Alcohol or drug intoxication or withdrawal.
 - Alteration of consciousness, to include post-ictal patients.
 - Psychiatric disorder with acute processes or behaviors occurring.
 - Central nervous system trauma with altered level of consciousness with actual combative behavior.
 - Patients with cognitive disabilities such as Autism Spectrum Disorder or dementia have the potential to become violent when placed in a foreign, high-stimulation environment.
 - Any patient in law enforcement custody.
 - Any patient that the pilot or clinical flight team considers as representing a danger to the safe transport of the patient.

Identify previous incidences of aggressive behavior exhibited by the patient while in the care of the sending agency. Aggression is likely to reoccur, and actions should be taken to prevent a violent event in flight.

- For patients who pose a threat to the safety of air transport, ground transport accompanied by the flight crew should be considered. If it is necessary to transport by air, every attempt should be made to ensure that the patient is safely controlled prior to loading the patient into the helicopter. Safe loading may include mechanical restraint and treatment of acute agitation, potentially with airway management, when necessary to ensure the safety of the patient and the crew. Clinical decision-making to justify the level of restraint must be documented.
- All patients who exhibit anxiety or meet the criteria for potential escalation should have two intravenous lines (IV) inserted for the transport. Peripheral IV lines should have additional security with tape added. Any tubing should be placed out of reach of the patient in preparation for escalation of anxiety to agitation.
- If the referring or receiving facility requests patients deemed at risk to be transported without an IV or sedation, the clinicians shall contact Medical Control and the Clinical Director (or AOC) to determine the plan of care and transport.
- Maintain enough straps to safely secure the patient. Apply extremity restraints to any patient observed to be agitated or with the potential to become agitated.
- Should a patient become violent while in flight, it may be necessary to treat acute agitation of the patient by administering sedative/anxiolytic medication. This should be done in accordance with the current [Patient Care Guidelines](#). Should administration of these medications be necessary for restraint purposes, the clinician must document the reasons for administration and the patient's response to the medications.
- If unable to secure and control the patient, ask the pilot to land the aircraft as soon as it is safely possible in order to reassess the situation and gain control.
- In any instance where there is a patient who is considered a threat to safety for transport by air, then the flight crew may cancel transport, and the patient may be transported by ground.
- In the event the patient is transported by ground after the crew has initiated patient contact, the flight crew shall accompany the patient and ground EMS to the closest appropriate facility.
- Any refusal of transport or treatment by a patient should be addressed directly with Medical Control and the Clinical Director (or AOC) for guidance.

72 Performing Care Separate from an Accepted Flight Request

72.1 Purpose

To provide guidance for clinicians to deliver emergency care not associated with an accepted flight request. These events include individuals who present on site at an Air Methods base in need of emergency medical assistance. Requests for the flight clinicians to respond and provide care when the aircraft is unavailable will also fall under this policy.

72.2 Policy/Procedure

- If an individual arrives at an Air Methods Base seeking medical assistance the clinician should:
 - Ensure local EMS is activated (Call 911).
 - Pilot notifies AirCom of a potential request and shows the base as committed to a patient.
 - Clinicians respond to the emergent needs of the presenting patient following Air Methods patient care guidelines while awaiting EMS to arrive.
 - Once EMS arrives, decide jointly the most appropriate transport mode for the patient.
 - If the patient will be transported by ground, it is preferred that the flight clinicians accompany the patient with all their equipment per standard ground transport policies.
 - If local, ground EMS response to the base is delayed and the flight clinicians believe air transport is most appropriate and feasible, AirCom will make an official request via recorded line for air transport approval from local EMS command and we will act accordingly.
 - Documentation:
 - If the patient ends up being transported with the flight team in attendance, an emsCharts ePCR will be completed in the same manner as a normal transport request.
 - If the patient is not transported with the flight team in attendance, an emsCharts ePCR shell chart will be completed. A disposition must be selected that best reflects the patient encounter. Some examples are: "Treated, Transported by Other EMS" or "Treated, Transported by Private Vehicle."

- If one of the Air Methods flight crew on-site needs emergency care the clinicians should:
 - Ensure local EMS is activated (Call 911).
 - Notify AirCom and have them place another air ambulance on standby if appropriate.
 - Clinician(s) responds to the emergent needs of the teammate following Air Methods patient care guidelines while awaiting EMS to arrive.
 - Once EMS arrives, decide jointly the most appropriate transport mode for the patient.
 - If the patient will be transported by air, it should not be by the initial team on-site to accompany the patient, it should be a second responding air ambulance performing the transport.
 - Documentation:
 - In this scenario, if the patient is flown by Air Methods, the transport team will complete an emsCharts ePCR in the same manner as a normal transport requests.
 - For a flight team member who provides care, but does not transport, an emsCharts ePCR shell chart will be completed. A disposition must be selected that best reflects the patient encounter. Some examples are: "Treated, Transported by Other EMS" or "Treated, Transported by Private Vehicle."
- Non-emergent care of a fellow clinician is not permitted beyond first aid, such as assisting with a small bandage, etc. No non-emergent medications will be administered to a teammate under any circumstances, i.e. antiemetics, IV fluids, etc.
- If a request for the flight clinicians is made for a local response to the scene of an accident when the aircraft is unavailable the following applies:
 - Air Methods on duty clinicians will not respond to the scene of an accident or medical emergency in their private vehicle;
 - On duty clinicians may only respond to a scene request with local EMS in a licensed response vehicle, (i.e. ambulance) and must bring all Air Methods emergency equipment and medications per the ground transport policy.
 - Clinicians should consult the Clinical Director for further guidance and approval to respond in an emergency vehicle.

- Clinicians should ensure AirCom is aware of the request and has asked local EMS to provide an ambulance to pick them up if one is available.
 - In true disasters and mass casualty incidents, exceptions may apply, consult the Clinical Director on call.
- Off duty clinicians who encounter emergencies in the community and choose to provide medical care are not acting as Air Methods employees. These activities will fall under local Good Samaritan laws which should be reviewed by individuals wishing to perform them.

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73 *Photography Use by Clinical Staff*

73.1 Purpose

To address Air Methods policy regarding the use of cameras when responding to or completing a patient transport and during all flight operations

73.2 Policy

- Unless specifically permitted as a requirement to comply with policies and procedures included elsewhere in the Medical Operations Manual or at the direction of the Clinical Director or Area Manager, photography of any kind is strictly prohibited for the following situations:
 - At all times and in all phases of flight operations, whether or not a patient is on board.
 - When responding to the aircraft after a flight request has been received.
 - When the aircraft is on the ground at the scene of a flight request or on a hospital helipad for the referring or receiving facility.
 - Photographs of the scene of an accident or medical emergency are prohibited.
 - For instances where photography is authorized or has been approved, care must be taken to comply with the below requirements.
 - Photographs must never capture an image of any part of a patient or any part of equipment or documentation that is displaying information about the patient.

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74 Pre-employment Screening and Interviewing

74.1 Purpose

Air Methods, LLC is committed to employing the highest qualified clinical personnel. To accomplish this, a commitment to pre-employment screening and interview process will be followed.

74.2 Policy

Each potential clinical crewmember candidate will undergo the following process:

- The application will be screened in accordance with the minimum requirements set forth by the Air Methods as defined by the appropriate job description.
- All candidates must complete a post-offer background screen, physical, and drug test.
- The candidate will be interviewed in a panel format which will be led by the Area Manager.
- There will be a standardized clinical candidate exam delivered in a proctored on-line environment with questions derived from nationally recognized curriculums (e.g., ACLS, PALS, National Registry, CCRN, CEN) for use as an evaluation tool. The candidate will be notified of pass or fail status by recruiting. A passing score must be obtained prior to proceeding to the scenario testing and panel interview.
 - If a candidate does not have an appropriate score on the written exam they may re-take the exam after 30 days.
 - The candidate may re-test up to two times during a 12 month period.
 - A passing score on the clinical candidate exam is good for 6 months. If the candidate has not received or accepted an offer of employment within 6 months they will need to re-take the exam.
 - Managers cannot provide feedback on specific test questions, but may provide general areas of improvement for future study.
 - If candidates fail the candidate exam they should not proceed through scenario testing or the panel interview; the exam needs to be provided as the first phase of candidate testing.
- There will be standardized scenario-based evaluations derived from nationally recognized curriculums.

- The Regional Clinical Director will participate in and oversee pre-employment screening and interviewing for clinical base leads, and clinical director positions.
- The Medical Director should be consulted regarding the selection of clinical employees as appropriate.
- The Credentialing Team needs to review and verify credentials prior to making an offer for any clinical position. The clinical recruiter will complete the form and attach the credentials at Clinical Applicant Credentialing Review. The Credentialing Team will then review and notify the clinical director and clinical recruiter if the candidate is cleared for hire or if there are problems with their credentialing.
- Internal transfer applicants will need to be evaluated by the RCD to ensure proper licensure and credentialing is complete prior to transfer. Additional evaluation of clinical competency is not needed, provided the employee is in good standing. Clinical leadership should use the Re-orientation to Flight Status/Internal Transfer Assessment – ME-5645 form, which should be filed with HR for placement in the employee's file following completion of the form.
- Employees should refer to the Teammate Handbook for further information on internal transfers.
- Programs being assimilated into the Air Methods CBS through contractual arrangement or acquisition must provide training records for clinical employees.
 - These records must clearly outline the employee's didactic and clinical education which must be in alignment with CAMTS initial education standard and the Air Methods initial education standard.
 - If proper documentation is present, the new employee will be tested through written and oral/simulation examination to validate competency. If clinical employees are not able to successfully pass this testing, they will be required to attend remedial training prior to being utilized to meet program staffing needs.
 - If documentation is not present and accounted for prior to hire, all new employees will be mandated to complete an approved initial education and training process per the Air Methods and CAMTS standards.
 - Regardless of previous flight experience, all new employees will be required to:
 - Attend new hire orientation in Greenwood Village, Colorado or equivalent determined by corporate education team.
 - Demonstrate skill and equipment competencies.
 - Successfully complete a patient care guideline exam.
 - Receive a safety orientation by a member of the aviation staff.

75 Purchase of Clinical Supplies

75.1 Purpose

Every effort should be made to consolidate orders for clinical soft goods and drugs so that bases are ordering once or twice monthly. McKesson is the preferred vendor for most clinical items; other select vendors have been identified by the purchasing department and may also be utilized. Any pricing request or other inquiry to a new vendor should be made through the purchasing department.

75.2 Policy

Controlled substances should always be ordered separately from other supplies, goods and drugs, even if they are sourced from the same vendor at the same time as other items are requested. Complete a separate McKesson cart and separate purchase order for controlled substances.

- Contact the Air Methods purchasing department with questions about how to order items; do not place orders using a corporate credit card or a personal card. Complete guidelines can be found on the FlightDeck at <https://flightdeck.airmethods.com/resources/PurchasingPolicies>.

75.3 Procedures

Upon receipt of any package from a vendor, the following processes must be followed to reconcile items ordered with items received for corporate accounting purposes:

75.3.1 Receipt Inspection

- SEARCH all boxes to ensure you have all packing slips (A bill of lading is not considered a packing slip).
- INSPECT each item carefully.
- VERIFY condition of item and it is undamaged.
- ENSURE that each line on the packing slip is reviewed and the shipped/delivered quantity is correct and clearly marked.
 - Ensure that you received everything indicated on the packing slip and it doesn't indicate something you did not receive.

75.3.2 Discrepancies

- If there are ANY discrepancies add comments to @opspurchasing on your Coupa purchase order.



- If you did not receive a packing slip. From the McKesson site hover over reports and select view packing slip.



- Add purchase order number and select Apply Changes

SET VIEW PACKING SLIP CRITERIA

This report has information for the last 6 months only.

Enter Invoice, Order #, PO or Date Range and then click Apply Changes.

The Account field is optional for Invoice, Order # and PO searches.

Account

Invoice Order # PO

From To

- Print packing slip.

PACKING SLIP

PRINT DATE: 03-01-2021

PAGE 1 of 1



MCKESSON

SHIP TO: 55217864 AIRLIFE: 1815 AIRPORT LOOP KERRVILLE, TX 78028-9596		BILL TO: 4323873 AIR METHODS CORP INC 5502 S QUEBEC ST GREENWOOD VILLAGE, CO 80111		SHIPPED FROM: MCKESSON MEDICAL-SURGICAL INC 4210 RATING DRIVE, STE 100 GRAPENHUE, TX 76012		
CUSTOMER NUMBER: 85736 ORDERED BY: E520223		INVOICE NUMBER: 21648620 ORDER NUMBER: 97709118 50 ORDER DATE: 3-1-2021		SHIPPED FROM LICENSE: 0035834		
FOR: Destination						
SPECIAL INSTRUCTIONS: CHRISTOPHER A WALKER						
REGULATORY LICENSE: 88557						
Item / Mfg	Qty	UOM	Shipped	To Follow	Description	Cust
LN# Number Ordered Bin Loc					Vendor	Item #
1 191997 2 EA 1					REXLIGHT W PUPIL GAUGE DISP (6 PK)	
2 * 467951 1 CT 0 1					DOPAMINE HCL VL 100MG 5ML (25 CT)	
THE ABOVE LINE WILL BE SENT TO YOU FROM OUR HOUSTON DISTRIBUTION CENTER						

For inquiries related to the Drug Supply Chain Security Act (DSCSA), please email info@mcusercontent.com.

For the DSCSA Traceability Report, please log into McKesson Supply Manager™, then select "DSCSA Traceability Reporting" under the "Reports" tab.

Returns of any pharmaceutical (Rx) product will require a return authorization number (RA) from McKesson Medical-Surgical's Customer Service Center.

McKesson's Terms of Sale shall apply to all purchases. Any disagreements between any order placed under this Agreement and McKesson's reporting shipment must be reported to McKesson for resolution within ten (10) calendar days of McKesson's invoice date except for payment disputes which shall be reported to McKesson for resolution within thirty (30) days of McKesson's invoice date. Please include invoice and serials fees. Failure to report within the designated time shall constitute a waiver of any claims due to such discrepancies.

Contact your Customer Service Center if Safety Data Sheets are needed.

All Package Insert information can be found at the website <http://www.mckesson.com> or <http://www.mckesson.com>. If you have trouble accessing the website and need package insert information, please contact Customer Service and request a copy.

NOTE - ** Next to Line Number Indicates Shipping from Another Location

NOTE - ** Next to Line Number Indicates Line Under Review

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75.3.3 Complete Receiving Action in Coupa

For assistance with Coupa, please refer to the Resource Documents and Videos that can be found on FlightDeck and at this link >>> [Coupa Reference Guides and Videos](#).

76 *Regional Patient Care Guidelines* (PCG) Differences

76.1 Purpose

To define the process for approval and posting of regional PCG differences.

76.2 Policy

76.2.1 Section I: State or County-Specific PCG Differences

- Certain state or county Emergency Medical Services (EMS) governing agencies require specific equipment, medications, or other items to be carried on the aircraft.
- Such differences are discussed with all Physician Medical Directors in the region and require approval from the Clinical Director and Physician Advisory Board.
- The specific state or county rule or policy must be submitted.
- The difference is listed in the regional or program PCG publications and the regional differences are collated and included in the PCG Program Differences section of the FlightDeck.

76.2.2 Section II: Physician Medical Director or Hospital/Healthcare System PCG Differences

- Any Physician Medical Director may propose a PCG difference. The proposed difference must include the rationale and any medical literature to support the proposed difference.
- A hospital or healthcare system may have specific treatments that they expect us to provide (e.g., Tranexamic acid (TXA) in the current PCGs is indicated for adult patients only but a receiving facility may expect the administration to pediatric patients). This proposed difference must include a policy or other statement from the hospital or healthcare system regarding the practice variation.
- These proposed differences will be forwarded to the Clinical Director and Medical Director. The CD and MD will consider the differences and forward to the Physician Advisory Board, along with rationale and evidence supporting the difference.

- Proposed differences will be outlined on Form(s) New/Revised Medication Request Form 5170 and New/Revised Patient Care Guidelines Request Form 5171, see the Appendix for example forms. Please send completed forms to pcgcommittee@airmethods.com for review. Form(s) New/Revised Medication Request Form 5170 and New/Revised Patient Care Guidelines (PCG) Request Form 5171 are available on FlightDeck under Corporate Publications.
- The proposed difference then require approval by the PCG Committee with three possible decisions:
 - Include the proposed difference in the national PCGs.
 - Approve the proposed difference as a regional PCG difference (preferred) or program PCG difference.
 - Deny the proposed difference.
- If approved as a regional or program PCG difference, the difference is listed in the regional or program PCG publications and the regional differences are collated and included in the PCG Differences section of the FlightDeck: Recurrent submission of approved regional or program PCG differences is not required.

76.2.3 Section III: PCG Committee

- Composition: The PCG Committee is comprised of the following:
 - Physician Advisory Board.
 - Director of Education and/or Division of Education Manager.
 - At least one Regional Clinical Director.
 - At least one Flight Line Clinician.
- Duties:
 - Reviews and updates the national PCGs annually.
 - Reviews all proposed differences.
 - Provides follow-up to the region and the original individual(s) that proposed the difference(s).
 - Reviews all approved regional and program PCG differences annually.
 - Coordinates with Education and Training to create any education required regarding the PCGs and PCG differences.

77 *Re-orientation Plan to Return to Flight Status*

77.1 Purpose

To ensure proper training and competency for the clinical flight crew member returning to flight status after being absent or off flight status due to resignation or extended clinical leave.

77.2 Policy

NOTE Re-orientation to Flight Status/Internal Transfer Assessment – ME-5645 Form should be completed for every clinician and become part of their training records.

77.2.1 Up-To-Date Status

Ensure all of the following are up-to-date:

- Occupational Health Physical and associated requirements.
- Required OSHA and Compliance Modules must be completed.
- Required Certifications (ACLS, PALS, BCLS, NRP, TPATC, Primary State License, etc.). Needs to be uploaded to the appropriate online tracking site. If anything is outstanding, arrangements will be made to assist you in completing the appropriate challenge or recertification course.
- Credentialing team must approve and validate all certification in Protean.
- Ensure Up-to-Date Intubation Requirements.
- Ensure Invasive Skills/Vent Lab is current.
- Ensure FAA crew member training-otherwise schedule in next available class.
- Other state, regional, and local certifications.
- Equipment Competencies.

77.2.2 Re-Orientation Plan

This plan is for reorienting employees that have been out for one year or less.

- Complete the following items:
 - Assigned online training modules for all quarters the teammate was on leave.
 - Patient Care Guideline Exam.
 - RSI Exam and Scenario.
 - Competency Day/Invasive Skills Lab within 1-year.
 - Simulation Lab.
 - Clinical requirements as required by state and regional requirements. See regional requirements.
 - Electronic Documentation/Charting Review.
 - Protean Review.
 - Base Updates with Clinical Lead.
 - Review all Clinical Safety Bulletins.
 - Review of all updated policies and procedures.
 - Additional items as assigned by Clinical Education Manager.
- Aviation Specific:
 - FAA/AMRM Update as needed.
 - Aircraft Configuration.
 - Equipment Configuration.
 - NVG Training (Initial or Recurrent as required by base assignment).
 - Radio Operations.
 - Any specific changes that have taken place during leave.
- Observed Flight Time (3-6 Months from Leave of Active Flight Status).
 - Minimum of one (1) 12-hour flight shift riding as a third crew member.
 - Minimum of one (1) patient flight.
 - Minimum of one (1) completed patient chart.

- Observed Flight Time (6 – 12 Months from Leave of Active Flight Status):
 - Minimum of three (3) 12-hour flight shifts riding as a third crew member.
 - Minimum of five (5) patient flights.
 - Minimum of five (5) completed patient charts.
- Regional Clinical Education Manager will review all flights for quality before the completion or re-orientation.
 - RCEM will notify the Area Manager and Clinical Director of completion of re-orientation.

NOTE If an employee has been gone from Air Methods for greater than one year, they will be required to attend new hire orientation held in Greenwood Village, Colorado in addition to the above specified requirements.

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78 Research Committee

78.1 Purpose

Foster research within Air Methods by mentoring, facilitating, and supporting research projects while encouraging responsible research practices.

78.2 Membership

- Standing Members:
 - (1) Research Chair
 - (2) Physician Medical Directors
 - (1) Regional Clinical Director
 - (1) Clinical risk and compliance
 - (3) Education leadership team
 - (1) Clinical Manager
 - (2) At-large clinicians
 - Additional members encouraged to join.
- Term:
 - Research committee membership may be terminated by the Chair due to failure to attend, and/or actively participate in research committee functions.

78.3 Application for Membership

- Submission of current Curriculum Vitae (CV) and cover letter to research committee at clinicalresearch@airmethods.com.

78.4 Selection

- Will be reviewed by Research Chair and discussed with Research Committee.

78.5 Roles and Responsibilities

- Research Chair:
 - Schedule meetings, communicate with committee, represent committee to Air Methods executive management, designate alternate when unavailable, point of contact for primary investigators, and facilitate information as IRB Administrator to IRB.
- Active Members:
 - Compliance with all assigned research education.
 - Review all submitted research proposals to enhance support and mentorship.
 - Serve as liaison to principal investigators for assigned research proposals.
 - Communicate and report progress for assigned research proposals.
 - Encourage and mentor future researchers.

78.6 Meetings

- Meetings will be scheduled monthly at a minimum or more frequently as required.
- Meeting minutes will be recorded and posted in designated online data storage application for research committee's review.

79 Research/IRB Proposal Submission

79.1 Purpose

To provide guidelines for submitting research proposals to Air Methods, LLC.

79.2 Procedure

- Complete the Air Methods Institutional Review Board Application Form ME-5647 Revision 1 on FlightDeck.
 - Submit the completed application to Clinicalresearch@airmethods.com for review by the Research Committee. A sample of this form is located in the Appendix A - Sample Forms of this manual.
- Requests should be submitted at least three months before projected start date.
- Requests will be reviewed by the Research committee based on the following criteria:
 - Relevance to industry.
 - Ethics.
 - Ensuring Federal Wide Assurance (FWA) guideline compliance.
 - Data security.
 - Resources required for participation (Personnel, medication and equipment).
- Guidance will be given within three months.
- The Principal Investigator may be asked to present project to the research committee via webinar or in person.

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80 Retainment of Records

80.1 Purpose

To establish a standard for the length of time and location for clinical records that must be retained at the base or corporate level.

80.2 Procedure

It is the responsibility of the Clinical Base Lead to maintain the records in a secure fashion at the base. Records should be kept in an organized fashion so that records may be easily located for production to inspecting agencies or personnel.

Record Name	Length Must be Retained	Location
Blood Cooler Tracking	10 Years	Base/Online
Daily Blood Check Log	10 Years	Base/Online
Blood Tracking Comments Log	10 Years	Base/Online
Blood Fridge Dickson Wheel	10 Years	Base/Online
Patient Records	All patient records are housed and maintained at PBS.	No patient records are to be retained at the base*
Fetal Heart Monitoring Records	10 Years	Patient Business Services
Aircraft Checklist	3 Years	Protean
Monthly CQI Meeting Minutes	3 Years	Determined by base
Monthly Safety Meeting Minutes	3 Years	Determined by base
All records regarding Narcotics, including, but not limited to, Narcotic Perpetual Log, 222 Forms, EOM Narcotic Audits, etc.	3 Years	Narcotic Management File
*With the exception of Fetal Heart Monitoring Records.		

Fetal Heart Monitoring (FHM) Records will be retained, in their entirety, from select bases that utilize FHMs, if the FHM has the capability of printing and storing the records. FHM Records will be kept in a locked storage cabinet until shipped to PBS. FHM records will be shipped to PBS at a minimum of every 45 days. FHM records will be shipped via a trackable mail service.

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81 Ride-Along and Media Policy

81.1 Purpose

To educate qualified personnel riding along with Air Methods programs regarding program history, patient care, operations, and helicopter safety.

81.2 Policy

Personnel will be informed as to safe helicopter operations; will gain greater understanding of patient care in the air medical environment and will be able to anticipate the need for helicopter and fixed wing service in their field of practice by participating in the Ride-Along program.

Flight programs that offer a ride-along observation program will adhere to the following criteria:

- Qualified participants can include fire, law enforcement, EMS, healthcare related personnel, etc.
- Ride along participant's affiliation with services and schools must be verified.
- Foreign citizens must show a copy of their U.S. Visa or Green Card.
- The minimum age for participants will be 18 years old. On rare occasions there may be a special situation that necessitates a ride-along for < 18 years old. If this is approved at the program level, parental consent must be obtained prior to the ride-along.
- A Ride-Along Application - 5199 will be filled out on each participant that will include a signed Ride-Along Patient Confidentiality - 5197 and the patient confidentiality information (sample forms are located in the Appendix of this manual – the actual forms are located on FlightDeck > Resources > Company-Wide Forms (Air Methods)).

- Ride-along candidates will be directed on proper attire as prescribed by the program.
 - At a minimum, the ride-along participant will have a uniform that covers their arms and legs and a pair of shoes that protects their feet. No offensive, political, or unprofessional attire is allowed.
 - All ride-along participants will provide a copy of their drivers license or ID card to the program. The program should make a copy of the ID (and Vise/Green Card if required) and keep in a secured location at the base. Ride-along participants must carry their ID(s) on them throughout the entire ride-along experience.
 - Air Methods clinicians facilitating the ride-along must ensure that AirCom is notified of the ride-along and their emergency contact information.
 - Air Methods clinicians will ensure that the ride-along participant is educated on the use of cellular phones and any safety precautions during transport. A safety briefing must be given in accordance with GOM with each transport.

81.3 Media Ride-Along Policy

- The focus of a "ride-along program" is to provide education. This does not include the media. Since the patient does not have the ability to authorize this non-essential person, they have a reasonable expectation of privacy.
- On the rare occasion, Air Methods will contract with a photographer to take pictures for the purpose of promotion of our business. Approval must be obtained following the non-revenue aircraft movement policy in the GOM.
- Photographers are treated as a business associate and restrain the release of content only for those purposes allowed or where the patient has consented to their release.
- Air Methods does not allow media ride-along when there is any contact with a patient. When possible we stage photos showing a mock patient for advertising or media purposes or encourage media to use imagery obtained that was within the public eye and not facilitated by our Company. Photos used for documentation must be protected like any other patient health information (PHI).
- It is the responsibility of the clinical flight crew to ensure that all HIPAA laws are complied with and patient privacy is always protected.
- All media ride-alongs must be facilitated through Corporate Communications.

82 Scheduling & Travel Guidelines for HPS & Cadaver Labs

82.1 Purpose

To outline the requirements related to the scheduling, travel, and attendance of annual simulation and cadaver labs.

82.2 Policy

82.2.1 Scheduling of Annual Training Labs

- The central scheduling team will schedule all clinicians for their two annual training labs.
- Clinicians will attend two mandatory training labs in accordance with the current version of the Education Blueprint.
 - Simulation Lab.
 - Cadaver (invasive skills).
- Clinicians who have attended new hire orientation during the year will not attend simulation and cadaver labs during the same calendar year, as these training requirements were met while in new hire orientation.
- The scheduling team will post all scheduled training for each clinician in the scheduling software system.

82.2.2 Notification of Scheduled Labs

- Clinicians will be notified of their scheduled lab dates via email from the scheduling team.
- Notice will include training center location and date of training.

82.2.3 Attendance for Scheduled Labs

- A scheduled lab is treated the same as a scheduled shift.
- It is the company's expectation that clinicians report to lab when scheduled and on time.
- Clinicians unable to attend their scheduled lab due to illness or other unexpected reason, are required to call their Area Manager a minimum of 8 hours prior to their scheduled lab start time.
- Reporting for lab late, leaving early, no show, or unexcused absence may result in disciplinary action.
- Reporting late to lab may result in the clinician having to be rescheduled.
- Clinicians who miss their scheduled lab must work with your Area Manager, central scheduling team, and the travel coordinator to reschedule.

82.2.4 Dress Code for Lab Attendance

- Flight suits and closed toed shoes are required.
- Reference materials normally carried on the flight suit are allowed.
- Miscellaneous items normally carried for patient care such as a stethoscope should be brought to the lab.
- You do not need to bring your helmet.

82.2.5 UKG Dimensions Schedules and Shift Coverage

- Training dates will be posted in the scheduling software by the scheduling team.
- Clinicians will work with their Area Manager to cover shift(s) before and/or after scheduled lab dates to accommodate travel if needed.
- Rescheduling the lab date should be considered an exception. If a lab date needs to be rescheduled, the clinician should work with the scheduling team to select a lab date. If a lab date needs to be rescheduled within three weeks of attending the lab, the clinician will need approval from clinical education manager and Area Manager.

82.2.6 Trading a Lab Date

- Clinicians who would like to attend an alternate scheduled lab should attempt a trade with another clinician or find an alternate lab in the scheduling system with available training slots.
- Any backfilling of shift coverage required as a result of trading a lab is the responsibility of the clinicians trading.
- No trades will be allowed < 21 days from a scheduled lab.
- Process to complete a trade of a scheduled lab:
 - Contact that clinician to arrange the trade.
 - Once both clinicians agree to trade, and shifts have been backfilled as needed, email your Area Manager and scheduling team.
 - The travel coordinator will be responsible for booking travel for clinicians involved in the trade no less than 21 days in advance of the lab date.

82.2.7 Travel for Attending Labs

- Clinicians may not book their own travel for simulation or cadaver lab training.
- Travel for all clinicians will be booked by the designated travel coordinator through the designated travel system platform following the guidelines as outlined in the current Travel and Entertainment Policy.
- All travel is subject to approval of the travel audit system.
 - Each clinician needs to establish a profile in the travel software to include closest airport and home address.
 - The travel coordinator will send the travel itinerary to the clinician through the designated travel software.
- All attempts will be made to book travel no later than 21 days in advance of the scheduled lab date.
- In order to reduce out of pocket expenses, clinicians are encouraged to obtain a corporate credit card so the clinician can pay for meals, rental cars, and other incidentals.
- Be sure to allow sufficient travel time to arrive at the lab on time.
- Notify the designated travel coordinator and your Area Manager immediately if you experience travel delays which will result in missing the lab or arriving late to the lab.

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83 Scope of Care and Specialty Transport

83.1 Purpose

To establish guidelines for acceptance of patient transport requests, considering the appropriate mode of transport, staffing configurations, number of patients transported simultaneously, availability and response times, and any exceptions to normal scope of care requests accepted.

83.2 Policy

- Air Methods, LLC provides rapid response, medically equipped aircraft, and ambulances that transport critically ill and injured patients requiring acute care from the scene of an injury/illness or a referring agency/hospital to a receiving hospital 24-hours a day, 7-days per week.
- All patient transport requests will originate through AirCom/Communication center. Crewmembers will not engage in “freelancing” or self-dispatch to an incident or disaster.
- Air Methods programs respond to interfacility and pre-hospital transport requests for all patient age groups, according to the local and regional differences specific to that area.
- When an interfacility transport request is made for a neonatal or pediatric patient admitted to an intensive care unit (i.e., NICU or PICU), clinicians will respond to the bedside and complete a patient assessment. Clinicians will consult with the receiving physician to determine the plan of care for transport and will coordinate care with the referring physician and/or the Air Methods physician medical director as necessary.
- When a request is made for a neonatal or pediatric patient to be transported to an emergency department or medical/surgical unit, clinicians will respond to the scene or bedside, complete a patient assessment, and are encouraged to consult with any sending/receiving physician or Air Methods physician medical director to discuss or determine the plan of care.
- The Hamilton Ventilator intended patient population ranges from pediatric 3kg ideal body weight to adults 139kg ideal body weight. If equipped with neonatal mode(s) and utilizing the neonate setting the range is 0.2kg-30kg.

- Air Methods flight clinicians can provide emergent care for patients suffering from the following conditions: cardiovascular/ventricular assist devices, pulmonary, neurological, endocrine, infectious, surgical, and traumatic injuries.
- Air Methods clinicians will adhere to the Patient Care Guidelines and Medical Operations Manual policies to ensure safe patient care during transport.
- Air Methods aircraft are medically equipped to provide continuous patient monitoring and therapeutic modalities consistent with critical care therapy for traumatically injured and critically ill patients.
- Air Methods' clinical flight or critical care team composition will consist of a critical care nurse and paramedic. In some circumstances, the team configuration may consist of two critical care nurses due to clinical staffing needs while still adhering to local and state regulations.
- It is not the practice of Air Methods aircraft and personnel to routinely transport two patients in a single aircraft. Refer to the Section 95 Two Patient Transport for specific information regarding two patient transports.
- If an Air Methods program/aircraft is not able to accept a patient transport due to any reason the request will be offered to the next closest CAMTS accredited program.
- Throughout Air Methods certain base locations are approved to complete specialty transports depending on customer's needs. These transports include neonate, nitric oxide, IABP, VAD, or ECMO. Flight programs approved to complete specialty transports are required to adhere to the following guidance.
 - Prior to implementation, it is recommended a needs assessment be completed to validate the specialty transport program need.
 - Clinical Director will identify additional medical equipment needs and coordinate with aviation and maintenance to ensure it meets the certificate compliance to include a means to safely secure the device in the aircraft.
 - Clinical Director will investigate ground ambulance logistics if a medical device is utilized for fixed-wing transports.
 - All local, state, and federal regulatory requirements must be met when implementing a specialty transport program.
 - The specialty transport program must have approval from the following prior to accepting specialty missions.
 - Medical Director
 - Clinical Director
 - Clinical Educator

- Regional Aviation Manager
- Area Manager
- Regional Safety Director
- Regional Maintenance Director
- Clinical Director will coordinate with the Medical Director to develop patient care guidelines specific to the specialty transport mission profile if not accounted for within the Air Methods national Patient Care Guidelines.
- Clinical Directors will ensure the development of a specialty mission playbook to be updated annually and available for all teammates while on shift outlining:
 - Configuration Pictures to include aircraft seating chart.
 - Loading/Unloading Procedures.
 - Securing Procedures.
 - EMI testing.
 - Additional Equipment needed with weight.
 - Non-Air Methods team weights if applicable.
- NICU
 - All isolettes in use must be capable of being opened from their secured position to provide full access to the infant and manage the airway if necessary.
- Intra-Aortic Balloon Pump
 - Clinical Director will coordinate with the Regional Clinical Education Manager to develop and implement initial and recurrent IABP device-specific training.
 - At a minimum, one of the transport clinicians must have completed the Air Methods IABP training requirements to include loading and unloading of the patient and pump device. Refer to the Air Methods Community Based Services Division Training Blueprint 2024 for training requirements.
 - Clinicians are encouraged to refer to the Carry-On Baggage and Equipment Storage and Security policy for guidance when securing specialized medical equipment.
- ECMO
 - Refer to Section 29 ECMO (extracorporeal membrane oxygenation) Specialty Team, Transport Team, Composition, and Competency policy for ECMO transport requirements.

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84 Security of Aircraft

84.1 Purpose

To ensure the security and integrity of the Air Methods aircraft, medical equipment, and supplies.

84.2 Policy

- Air Methods aircraft and medical equipment will be located in a secure area of the base when not flying a mission.
- At the beginning of each shift, and prior to each mission the vehicle and equipment will be surveyed for signs of tampering internally and externally.
- The Pilot-in-Command (PIC) or designee will maintain the security of keys used to gain access to the base and the aircraft.
- Pilots and clinical crew will review training to recognize signs of aircraft tampering with the lead pilot or designee on an annual basis.
- Should the aircraft be left unattended, the flight crew will perform an inspection assessing for signs of possible tampering in and around the entire aircraft and its contents. Any signs of tampering will be reported immediately to Admin-on-call person who will notify the appropriate law enforcement agency and corporate officials.
- In the event the aircraft is left at a remote location, all medications to include controlled substances and durable medical equipment (cardiac monitors, ventilators, IV pumps, IV fluid warmers, pulse oximeters, ETCO2 monitors, and portable suction) and the clinical iPad will be taken with the clinical flight team.
- Crewmembers are responsible for locking the doors to the office prior to leaving the base to respond to a flight request.
- If any signs of tampering, suspicious, or unusual activity has occurred, the aircraft shall immediately be removed from service until it can be deemed safe for operations by appropriate authority.

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85 Social Media: Internet

See the Teammate Handbook – Internet and Email Usage Policy and Social Media Policy. This manual is located on FlightDeck > Resources > Corp Pubs > Human Resources Publications.

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86 Specialty Transports

86.1 Purpose

Air Methods Community Based Services (CBS) is committed to providing the scope of care necessary for all patients ultimately transported. To ensure that all patients are provided high quality patient care, specialty transports must meet the following standards prior to approval and implementation.

86.2 Policy

- For any flight program wishing to incorporate specialty transports into their scope of care, a core curriculum will be developed based on industry standards for critical care transport specific to the specialty area. All core curriculums will be approved by the RCD prior to implementation.
- There must be written operational and clinical standards specifying the specialty transport and the scope of care to be provided.
- When an Air Methods CBS team partners with a local specialty team to complete a patient transport, clinicians must collaborate with our specialty partners to ensure that all patient care standards and documentation requirements are met. The plan for patient care must be communicated and mutually agreed upon, maintaining the highest level of safety with clear medication administration crosschecks. Documentation of patient assessment and clinical intervention should be completed in accordance with existing contractual agreements in order to avoid double documentation and ensure all documentation standards are maintained.
- Use of a Doppler to monitor fetal heart tones by the Air Methods CBS team is required for all high-risk OB transports; the addition of continuous fetal heart monitoring is program-specific and requires pre-approval by the Regional Clinical Director. Additional education is required for teams using continuous fetal heart monitoring.
- Education and training of the clinical personnel must correspond to the scope of specialty service provided. Evidence of on-going clinical competency must be documented.

- Per Diem Specialty Teams/Providers must comply with air medical industry general/education standards, as referenced by CAMTS (all training must be documented and verified on an annual basis):
 - Patient transport consideration
 - In-flight treatment modalities
 - Altitude physiology
 - General aircraft safety & ground operations
 - Aircraft emergency procedures & survival training
 - EMS communications/radio
- All equipment and supplies must be secured according to the CFRs. There must be access to the equipment while secured in-flight. An isolette must be capable of being opened from its secured position in order to provide full access to the infant in the event of complicated airway problems or extrication from the isolette becomes necessary.
- All state/local regulatory requirements related to the type of specialty transport must be met.
- The Regional Clinical Director should be used as a resource for planning and implementation of proposed specialty transport services.
- Prior to implementation of a specialty transport program, a needs assessment must be submitted by the Regional Vice President to the RCD to ensure all appropriate training and equipment is in place.

87 Strategic Clinician Pairing

87.1 Scope

All Air Methods CBS Programs.

87.2 Objective

To ensure the safe and effective provision of emergency medical services by forming strategic crew schedules composed of clinicians that complement each other's experience levels and expertise at Air Methods.

**Each Monday, please review the email received containing information related to clinician pairings for the upcoming 90 days. You can access the [Smartsheet link](#) here.*

87.3 Policy

87.3.1 Team Composition

Whenever possible, teams should be composed of a mix of clinicians that complement each other's experience levels and expertise. This mix allows for mentorship opportunities and knowledge transfer while maintaining safe and effective patient care.

87.3.2 Clinician Characteristics

A "green" clinician is defined as an individual with less than one year of experience in their role at Air Methods, or as determined by leadership based on other relevant factors, such as previous flight experience with Air Methods or another air medical flight company.

87.3.3 Clinician Pairing

87.3.3.1 Green on Green

The composition of a team with all "green" clinicians or clinicians with a recent Performance Improvement Plan (PIP) or Education Learning Plan (ELP) should be an exception and not the standard practice. There will be instances where a "green" clinician is scheduled with another "green" clinician due to the limited availability of experienced clinicians in that area. When this happens, leadership is notified and will be available to provide any extra support or resources that might be needed.

87.3.3.2 Clinician Improvement Opportunities

Any clinician issues a recent ELP or PIP will also be taken into consideration when scheduled with another “green” clinician or another clinician with the same extra educational requirements. For example, it is not recommended that a clinician on an ELP fly with another clinician also on an ELP or PIP.

87.3.3.3 Leadership Review

Leadership will regularly review clinician schedules and pairing. Leaders should consider contacting any pairings that do not meet the recommendations above, prior to or at the beginning of their shift if clinicians are working together for the first time. Topics to discuss could include:

- Crew roles at the beginning of the shift and prior to each flight/
- Completion of daily checklists.
- Reviewing PCGs in route to transport request.
- Resource utilization (contact info for CD, AM, MD, AirCom, AOC).

The composition of teams will be continually assessed by leadership and adjusted based on the progress of clinicians and the evolving needs of the organization.

88 Substance Abuse Testing

See the Employee Handbook Drug and Alcohol Abuse section. This manual is located on FlightDeck > Resources > Corp Pubs > [Human Resources Publications](#).

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89 Survival Kits on the Aircraft

89.1 Purpose

The purpose of this policy and these procedures is to establish what survival kits will be carried on-board the aircraft, and how and when these kits will be inventoried and checked for expiration dates.

89.2 Policy

- Survival kit content and management will follow direction from the General Operations Manual policy entitled Emergency Equipment. Survival kits, appropriate for the season and area of operation, shall be carried. The contents of these kits shall be determined by the senior lead pilot, program aviation manager, or regional aviation manager, unless otherwise specified.
- Clinicians will assist with inventory and check the survival kit contents for expiration dates and operational readiness on the first of each month. Clinicians will log this action within the Protean Daily Checklist. Items requiring replacement will be reported to the Lead Pilot and Area Manager.

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90 Transition to Backup Aircraft

90.1 Purpose

To provide a comprehensive plan to ensure appropriate transition from primary aircraft to backup aircraft in the event of extended maintenance.

90.2 Policy

When a spare Air Methods aircraft arrives at a CBS base it is the expectation that the aircraft will be transitioned with medical equipment within one hour of arrival.

- Notify AirCom that the aircraft will be out of service until transition is complete.
- The Aircraft Transfer Checklist - 5182 (a sample of this form is located in the Appendix of this manual – the actual form is located on FlightDeck > Resources > Company-Wide Forms (Air Methods)) shall be used for all transfers.
- Ensure communication/radio programming is operational and meets state and local requirements prior to in-service status.
- Ensure all on-board medical equipment is operational (Refer to the aircraft checklist for a detailed list of items).
 - Complete a flow check for all oxygen and medical air outlets.
 - Collaborate with the pilot to identify and test the aircraft "ON"/"OFF" oxygen supply switch.
 - Verify the aircraft suction unit is operational and there is adequate suction.
 - Ensure on-board outlets and inverters are operational.
 - Ensure the patient stretcher system is operational.
- Ensure all medical equipment can be secured in accordance with the General Operations Manual (located on FlightDeck > Resources > Corp Pubs > Flight Operations Publications).
- Notify the Clinical Director and Area Manager if there is a problem placing the aircraft into service within one hour of starting the transition.

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91 *Transport of Family Member (Non Crew) Passengers*

91.1 Purpose

To provide standards for air transport of family member or non-crew passengers.

91.2 Policy

The consensus of all crewmembers is required when transporting a family member passenger and final approval is to be made by the pilot. All minors require parental consent. Factors to be considered include:

- Patient acuity
- Aircraft type
- Family member demeanor
- Family member language spoken (must be able to communicate with flight crew)
- Availability of PFDs (if applicable)

Clinicians will adhere to the General Operations Manual policy 3.8 Loading and Unloading Patients or Passengers when operating on a rotor-wing aircraft.

Clinicians will adhere to the General Operations Manual policy 4.6 Loading and Unloading Patients or Passengers when operating on a fixed-wing aircraft.

Clinicians will adhere to the General Operations Manual policy 2.11 Deplaning Aircraft After Landing or Enplaning Before Takeoff for both fixed-wing and rotor-wing aircraft.

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92 *Transport of Patients with Advanced Directives/Do Not Resuscitate*

92.1 Purpose

It is the policy of the medical transport team to honor the medical treatment preferences of patients. Advance Directives, which are valid under state law and consistent with this medical transport team's policy, will be honored. The presence or lack of an Advance Directive will not impact the patient's access to or provision of care. Patients will be supported in their decisions about their treatment. Care necessary to meet emotional, spiritual, and comfort needs, including adequate pain relief, will be provided without regard to treatment decisions in an Advance Directive.

92.2 Definitions

In the absence of state-specific definitions, the following may be utilized:

Advance Directive - An Advance Directive is a legal document allowing an adult person 18 years of age or older to give direction about future medical care or to designate another person(s) to make medical decisions. An Advance Directive becomes applicable only if the individual loses decision-making capacity. The medical transport team may consider that Advance Directives policies vary by state.

Cardiopulmonary Resuscitation (Resuscitation) - Cardiopulmonary Resuscitation means the use of any or all of the following in order to maintain life in the event of cardiac or respiratory arrest: artificial ventilation including tracheal intubation, manual or mechanical assisted ventilation, external cardiac massage, electrical defibrillation, and standard Advanced Cardiac Life Support (ACLS) drug therapy.

Patients with Decision-Making Capacity - Patients with decision-making capacity demonstrate the ability to reason and to consider relevant information. They display an emotional, physical, and intellectual capacity to deal with issues, including consequences and alternatives.

Power of Attorney for Health Care - Power of Attorney for Health Care is a document that identifies another individual(s) as an agent to make healthcare decisions when the patient is unable to do so.

92.3 Policy

1. Medical transport team will provide resuscitation unless there is a valid Do Not Resuscitate (DNR) order or state-accepted equivalent. Patients with a DNR order will continue to receive appropriate treatment therapies including palliative measures.
2. Communication of Treatment Limitations/DNR Order - The referring physician or nursing staff will communicate to the medical transport team that an order to limit treatment/DNR has been written. The medical transport team caring for the patient is responsible for communication to the receiving hospital staff that a DNR or other designated resuscitation status has been written to limit treatment.
3. The medical transport team will respect any written DNR or treatment limitation orders during the transfer from sending facility to receiving facility unless the patient or decision-maker rescinds the order.
4. Patients with decision-making capacity have the right to make treatment decisions for themselves, including refusal of life-sustaining treatment. Advance Directives do not apply as long as the patient has decision-making capacity.
5. Advanced Directives obtained in the pre-hospital environment will be communicated to the receiving hospital. If the patient has lost decision-making capacity and cannot communicate their wishes, the team will resuscitate the patient and report the Advance Directives to Medical Control. Resuscitative efforts will be discontinued only after receiving order from Medical Control.
6. Patients who previously had decision-making capacity but no longer do, and have made a clear and explicit statement (orally or in writing) to limit treatment, shall have that decision honored.
7. Review, Revision, and Revocation - Treatment Limitations/DNR orders can be revoked or revised at any time by the patient with decision-making capacity.
8. Where this policy conflicts with state EMS rules/regulations, the state EMS rules/regulations will take precedence.
9. If a patient with a valid written DNR order expires during transport, the vehicle should proceed to the closest facility or note the GPS coordinates of where the patient expired.
10. Air Methods clinicians should remain with the deceased until cleared by the coroner's office where the patient expired.
11. Air Methods clinicians must contact Medical Control to inform of the patient's expiration.
12. All treatment equipment should be left in place until cleared by the coroner's office.

93 *Transport of Patients in Cardiac Arrest*

93.1 Purpose

This policy is intended to give some guidance to providers being asked to treat and/or transport a cardiac arrest patient as part of a scene or interfacility call. The content is intended to provide guidance in delivering the best possible patient care and assisting the providers/healthcare facilities to which we respond and thus should not be considered absolute. The physician medical director should be accessed as a resource to provide support and guidance to our providers regarding these transports.

93.2 Intra-arrest transport

- All parties must understand the extremely poor prognosis.
 - Sending MD
 - Receiving MD
 - Family
 - Air medical crew
- Good CPR en route must be ensured.
 - Mechanical CPR device.
 - Manual CPR with feedback (CPR quality, EtCO₂).
 - Puts crews at some risk (no safety restraints).
 - Short (<15 min) flights only.
 - Controlled take-off & landing sites.
 - Low risk of hazards.
- Definitive therapy must be offered at receiving facility.
 - Intra-arrest PCI
 - ECMO
 - Operating room with trauma surgeon.
 - Penetrating trauma without TBI.
 - Interventional neurology services.
 - Salvageable patient with suspected neurological cause of arrest.

- Special consideration may be given for pediatric patients due to psychosocial aspects and potential to perform CPR while restrained.

93.3 Post-arrest transport

- May transport immediately post-arrest if above criteria met or after consultation with medical control or clinical direction.
- Require some stability, particularly if all above criteria not met.
 - Vital signs compatible with life for >15 min:
 - SBP >80 mmHg
 - SpO₂ >80%
 - EtCO₂ >25 mmHg
- All parties must understand the extremely poor prognosis.
- Special consideration may be given for pediatric patients due to psychosocial aspects and potential to perform CPR while restrained.

93.4 Futility considerations

- Do Not Attempt Resuscitation status or family request to cease resuscitative efforts.
- Lack of physiological response after “reasonable” resuscitation duration.
 - Persistent “bradysystole” (ECG <30-40/min).
 - EtCO₂ decreasing or persistently low (<15 mmHg).
 - Absent neurological signs.
 - Blinking, breathing, biting tube, spontaneous movement.
 - Poor baseline health and/or quality-of-life.
 - Acute (e.g. TBI, CVA) or chronic (e.g., dementia) brain injury.
 - Terminal illness (e.g., cancer, advanced CHF, O₂-dependent COPD).
 - Institutionalized (e.g., skilled nursing facility).
 - Reasonable resuscitation duration.
 - At least 30 minutes for all patients.

- At least 45 minutes for more salvageable patients.
 - Witnessed arrest (including EMS-witnessed) with immediate CPR.
 - Initial shockable rhythm.
 - Hypothermic arrest.
 - Periods with return of spontaneous circulation (ROSC).
 - Age <70 years

NOTE These represent guidelines rather than strict protocols, and online medical control should be consulted for any questions or concerns.

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94 *Transport of Prisoners*

94.1 Purpose

To specify the procedure personnel shall follow when a request to transport a prisoner patient by helicopter is received.

Crew safety is the number one priority when transporting prisoners. Team members who believe their safety is in jeopardy at any time shall have the authority to cancel the transport. Under no circumstances shall an uncontrolled combative patient be placed in the aircraft.

94.2 Policy

Air Methods Communications must advise requesting agencies of transfer of this Transport of Prisoners policy and procedure during the call intake, at this time police/correction officer and patient weight will be obtained.

- Only one patient in police/correction officer custody may be transported at a time. The aircraft may not be reconfigured for a second patient if the patient is in custody.
- The police/correction officer should be asked to search the patient for weapons prior to loading the patient into the aircraft (both scene and interfacility flights).
- Patients in custody must be placed in four-point physical restraints attached to the stretcher. The type of physical restraint (shackles or soft restraints) is a decision to be made by the police/corrections officer, if one will accompany the patient. If the patient is not accompanied by an officer, handcuffs/shackles will not be allowed, rather utilize soft restraints. The transport nurse or paramedic shall be responsible for assessing distal circulation during transport to ensure circulation is not compromised. Refer to Patient Care Guidelines policy Section 7.36 Restraints for documentation guidelines.
- If the patient is to be restrained using handcuffs or shackles locked with a key, the flight crew and accompanying law enforcement officer must brief prior to the flight to ensure the key is present and each member knows its location.
- The transport team shall remain in contact with the pilot via intercom at all times.

- The police/correction officer should accompany the patient during transport. The decision whether or not to accompany the patient during transport will be made by law enforcement, with approval of the PIC, and if aircraft limitations do not prohibit the additional weight of a guard. It is preferred that the police/corrections officer sit in the rear cabin if the airframe allows them to do so. If this is not possible due to aircraft configuration, it is permissible to allow the law enforcement officer to sit up front.
- Air Methods General Operations Manual states that law enforcement officers are permitted to carry their weapon on board the aircraft. The transport team shall brief the patient on aircraft safety and air medical operations.
- The transport team shall ensure the patient has patent intravenous access prior to flight. Should the patient become agitated, difficult to control, or become freed from handcuffs, please refer to Combative Patient Policy. (There must be clinical indications beyond agitated behavior to warrant prophylactic intubation).
- Observers shall not participate in incarcerated patient transports.
- In the event the aircraft receives a request to respond to a scene flight at a correctional facility, the transport team will ensure the aircraft lands in a secure area outside of the prison. At no time shall the transport team enter the correctional facility to retrieve a patient.
- The Air Methods Communications Center shall be immediately notified if any in-flight emergency occurs and if an emergency landing is necessary. The transport team shall request that local police be dispatched to the scene if this occurs.
- Final authority for mission acceptance lies with the pilot-in-command.

95 Two Patient Transport

95.1 Purpose

To provide the standard regarding two-patient transportation by Air Methods, LLC aircraft.

It is not the practice of Air Methods Community Based Services aircraft and personnel to routinely transport two patients in a single aircraft/single mission.

If a base/location is pre-approved by regional COE Leadership for two-patient transport, the below guidelines must be adhered to.

95.2 Policy

Two-patient transportation may be performed under the following situations:

- A second aircraft is unavailable, or it will be significantly delayed.
- The number of patients needing rapid transport exceeds the capabilities of local EMS and available air ambulances.
- Triage is completed and it is determined by the flight crew that returning for the second patient may be detrimental to the patient's clinical condition.
- An infant born in the field and transported with the mother due to the unavailability of a specialty transport team(s).

Two-patient transport may **ONLY** be performed if, one of the above criteria is met **AND** all of the following is true:

- The responding aircraft is configured for two-patient transportation (e.g., two stretchers, sleds, or approved patient transportation devices that can be safely secured according to guidelines set forth by the FAA and provide safe patient transportation).
- The aircraft is safely capable of transporting two patients according to aircraft weight and balance.

- Postpartum mother/infant dyad transports may be conducted when all of the following conditions are met:
 - Only one mother and one infant may be transported on the same aircraft. Multiple birth patients (twins, triplets) must be transported separately.
 - No spouse is permitted on the aircraft when mother/baby are transported together.
 - Both patients must be hemodynamically stable and maintain their own natural airway, with a stable clinical course predicted for the duration of the flight. If either patient requires ventilatory or cardiopulmonary support, the patients must be transported separately. At no time will two critically ill patients be transported simultaneously.
 - A complete history and physical assessment of each patient must be completed and communicated with the sending and receiving physicians prior to transport in order to establish a collaborative plan for patient safety throughout the transport. If additional resources are needed for safe patient care or long transport time, the patients must be transported separately. The team can work with the sending facility to plan to request a second aircraft or to complete both transports separately, depending on the needs of the patients.
 - Physician involvement is essential to the plan for safe postpartum and neonatal care. Communicate prior to departure with both the referring and receiving physicians about the patient status, duration of transport, and proposed plan of care for transport. Contact your medical director for any questions or concerns.
 - Safe securement of both patients is required. Each patient requires their own stretcher. The infant must be secured to the stretcher in an approved infant securement device (e.g., NeoMate, MedKids Board, BabyPod, etc.). Use of a lap, carseat, or placement in an immobilization device without securement to a stretcher are all expressly prohibited practices.
 - Handoff communication between referring and receiving facilities must be confirmed and documented. Ensure that both patients are properly identified, and that clinical documentation accompanies both patients to the receiving facility (e.g., newborn screening test completion, newborn medication administration, newborn laboratory studies, any maternal treatment administration, etc.). Document the names and titles of all involved clinicians in the medical record with the information provided.

- The Pilot-In-Command (PIC) is aware of the request and approves the transportation of two-patients in regards to safe transport and weight and balance.
- Equipment must be available to adequately provide to both patients simultaneously during transport (e.g., oxygen supply, cardiac monitor/defibrillator, NIBP, SaO₂, ETCO₂, and other specialty equipment as needed).

All medical equipment and stretcher systems utilized for a second patient will be checked daily.

- The transportation of two-patients is deemed appropriate by the clinical flight crew and will not endanger the crew or either patient, patients requiring advanced airway maintenance or those in shock with potential for cardiac arrest shall not be transported together.
- If needed medical control should be contacted for advisement on the completion of the two-patient transport.

NOTE It is understood that two-patient transportation is not routinely practiced and may not be possible depending on certain situations (e.g., airframe, weight and balance, patient acuity, required equipment). It is at the discretion of the flight crew, and if necessary, clinical management/clinical director consult at the time of the request if Two-Patient Transport can be safely performed.

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96 Video Laryngoscope (CMAC Video Recording, File Transfer, File Deletion)

96.1 Purpose

All CMAC Video Laryngoscope intubations will be recorded for quality improvement review and continuing education opportunities. Video files from the device will be uploaded into Protean as an attachment to the Utilizations Review and under the Clinical tab. Files are not to be uploaded into emsCharts. Once confirmed that the file has been uploaded into Protean, files will be manually deleted from the CMAC Pocket Monitor.

****IMPORTANT**** Accessing and transferring files is permitted solely through an Air Methods-issued computer. Personal computers are strictly prohibited from accessing CMAC Pocket Monitor files. Under no circumstances should files be uploaded to any cloud-based service outside of Protean for personal storage. Saving or storing files on any local Air Methods computer is prohibited; files must be promptly deleted from the device, and the trash emptied on the computer upon completion of the designated tasks. Failure to adhere to this policy may constitute a breach of PHI and a violation of HIPAA regulations. Frontline teammates found to be non-compliant with this policy may face disciplinary action.

96.2 Intubation Recording

The following steps will outline the process.

1. Prepare the standard equipment for the procedure as normal utilizing the RSI Checklist.
2. Once the CMAC Pocket Monitor is turned on, press and hold the blue button until the light starts flashing green. This identifies that the recording has started. Complete the CMAC warming steps and RSI checklist procedures. This ensures that the intubation process is captured in its entirety.

NOTE Pushing the blue button will only take a snapshot image. You must hold the button down until it starts flashing green to start video recording.

3. Insert the laryngoscope and perform the intubation by recording and capturing the ET tube placement into the trachea.
4. While the CMAC Video Laryngoscope is still being inserted, remove the Bougie or Stylette and inflate the cuff as appropriate.

5. Remove the CMAC Video Laryngoscope.
6. Press and hold the record button to stop the video recording. It will change from flashing green to a solid blue to indicate recording has stopped.
7. Turn the device off and package appropriately.

NOTE In the event of a failed first attempt, **do not stop** the video recording in between attempts. Remove the device to provide appropriate oxygenation and ventilation. Once you are ready for the next attempt, repeat the procedure with the CMAC Video Laryngoscope, which is already in record mode from the initial attempt.

96.3 Accessing the File/Uploading within Protean

While the following is a step-by-step guide, you are welcome to review a video of the process [HERE](#).

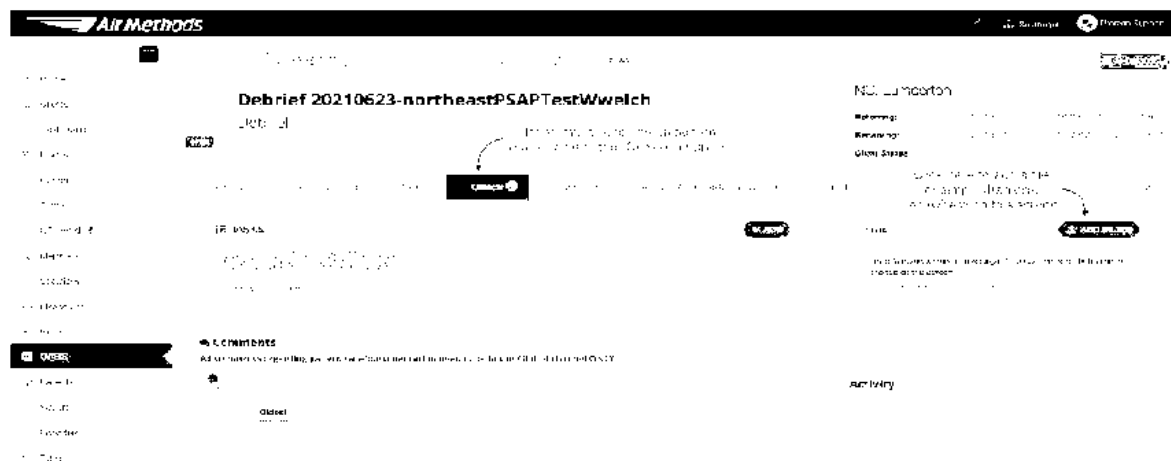
The following steps outline the process to access, upload the file to Protean, and Delete the file from the CMAC Pocket Monitor. Once the Flight Debrief and Utilization review have been entered into Protean > Oasis, this step can be completed.

1. Obtain the CMAC Pocket Monitor Data Transfer Cable from the Medbank. Both teammates are needed to issue the cable from Medbank.
2. Plug the CMAC Pocket Monitor data transfer cable into the FCMAC Pocket Monitor and then plug the cable into the Air Methods computer at the base.
3. The CMAC Pocket Monitor will show up under File Explorer as an attached External USB Storage titled C-MAC PM.

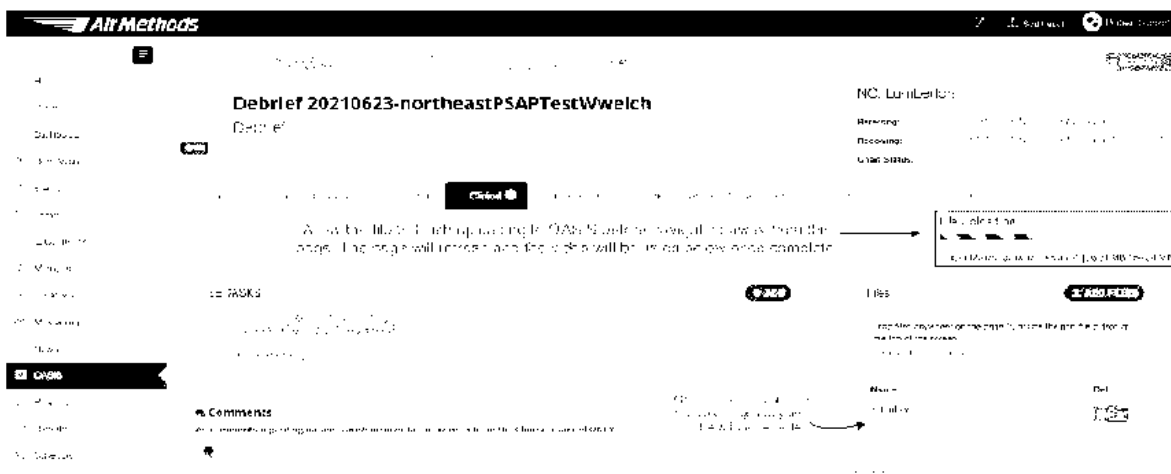
External USB on Computer	Folders on CMAC Pocket Monitor	Images saved within Device Folder
- CDisk (C:) - C-MAC PM (D:) - C-MAC PM (D:) - Network	1. 201 202	1 201_001 201_002 202_001 202_002

4. Identify the correct folder from the intubation. Folders are created automatically for events.
5. Open the folder and select the correct file. There should only be one video file unless the video was started and stopped more than once.

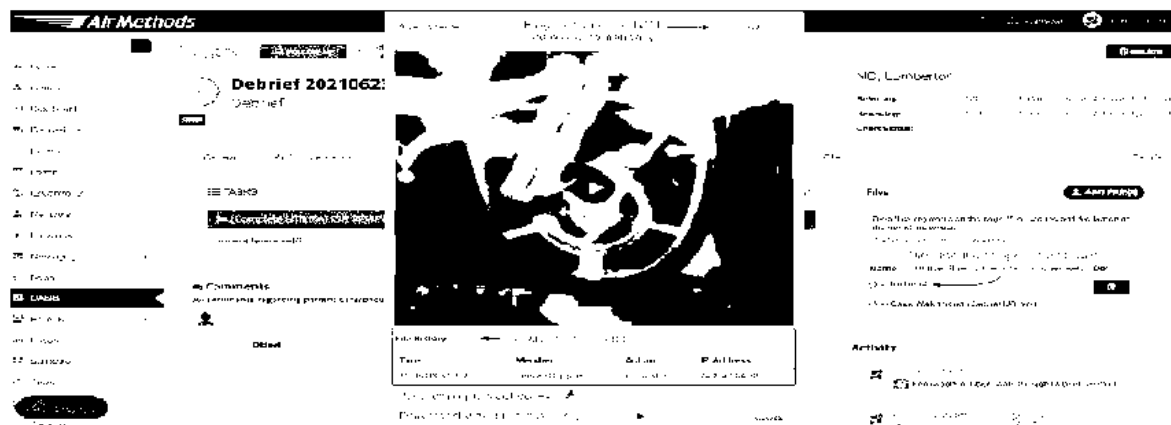
6. In Protean > Oasis and under the Debrief for the flight, select the Clinical tab.
7. Click the ADD FILE(s) button on the right.
8. Select the files from the device to upload.



9. Wait for the file to be uploaded. This is visible by a status bar showing progress.



10. Once complete, verify that the recording will play and has been uploaded correctly.



11. Delete the entire folder from the CMA Pocket Monitor.
12. Empty the Trash on the computer.
13. Disconnect the CMAC Pocket Monitor and cable.
14. Return the CMAC Data Transfer Cable to the Medbank.

REMINDER Files should not be uploaded to any cloud-based service outside of Protean for personal storage under any circumstances. Failure to adhere to this policy may constitute a breach of PHI and a violation of HIPAA regulations and subject individuals to the disciplinary process.

96.4 Policy for Identifying Recorded Intubation Videos for Education/Research Presentations

At Air Methods, our commitment to effective education and research drives the need for a clear policy regarding the identification, management, and usage of recorded intubation videos. Our objective is to ensure these videos are utilized responsibly, adhering to ethical and legal standards while promoting learning and advancement in airway management.

96.4.1 Flagging Process

While the following is a step-by-step guide, you are welcome to review a video of the process [HERE](#).

Recorded intubation videos undergo a flagging process within the Protean system.



This flagging signifies their potential use in crew debriefing sessions, educational activities, or research endeavors only. The Protean flagging system marks videos eligible for such purposes. At the conclusion of crew debriefing sessions, videos flagged for this purpose are unflagged at the discretion of the relevant Clinical Director.

96.4.2 Approval Process for uses outside Protean

Videos flagged for educational or research purposes outside of Protean must undergo thorough review and approval by designated personnel within Air Methods. These individuals (Who does this? - Race, Murguia, Norman, or others) ensure that the video aligns with educational and/or research objectives while safeguarding patient privacy. Any videos containing HIPAA-related information are strictly prohibited from being used outside of Protean.

96.4.3 Documentation

Upon approval, each video will be cataloged within the centralized repository known as the Air Methods Intubation Video Recording Library. This documentation will contain the purpose of usage, authorized personnel accessing the video, and relevant metadata. Details such as the date of recording, specific intubation errors, and the video's significance from educational or research perspectives are documented. Additionally, assurances are provided that no HIPAA information is contained within the video.

96.4.4 Storage and Access

Approved videos are securely stored in accordance with Air Methods IT policies, with access restricted to authorized personnel only, including only those in Education, Research, PAB, and Clinical Directors (Any others?). IT storage and access controls will be utilized to safeguard the privacy and integrity of these videos. All videos are transferred to the Air Methods Intubation Video Recording Library using the designated form: Air Methods Video Recording Library Downloaded Form.

96.4.5 Smartsheet Integration

To streamline management and tracking, a Smartsheet Worksheet serves as the reference for all videos utilized in education and research. This Smartsheet library tracks the last user accessing the videos, metadata changes, and documentation of individuals utilizing the videos for education or research purposes.

96.4.6 Periodic Review

Regular audits are conducted to ensure ongoing compliance with the policy and update the repository as necessary. The Director of Research (or other designated Clinical Member) is responsible for reviewing videos quarterly. This review ensures that all videos remain relevant and accurate for educational and research purposes. Any videos failing to meet storage policies are promptly removed from the storage library.

96.4.7 Disposal

Videos that no longer serve a purpose in education or research are securely deleted in strict accordance with Air Methods data retention policies. This ensures that outdated or unnecessary videos are responsibly managed, and privacy is maintained.

Appendix A - Sample Forms

Sample forms are shown in this section. The most current version of the actual forms are located on the FlightDeck > Resources > [Company-Wide Forms \(Air Methods\)](#).

Aircraft Transfer Checklist - 5182-CS

 Aircraft Transfer Checklist		Form #: 5182-CS Revision #: 1 Date: ED	
From Tail Number:			
To Tail Number:			
Cabin:	Initials	Miscellaneous	Initials
Airway supplies/bag		Flow Meters for O ₂	
RSI kit		Wet Suction unit (in working order)	
Trauma supplies/bag		Radiol Frequency cards	
Pulse Oximeter/Capnography		Portable Radiol (if applicable)	
Pulse Oximeter/Capnography supplies		ICS checkal positions	
Medication bag		Narcotics keys	
Survival Bags (Qty 3)		Base Keys	
O3 kit		Cip board	
Environmental bag		Toughbook	
Monitor/Defibrillator		Radiol Frequencies Verified	
Monitor/Defibrillator supplies		Sharps Container	
Doppler/Feta Monitor		OSHA PPE Supplies	
Doppler/Feta Monitor Supplies			
Medical Protocol Book			
Tadown belt/straps (if applicable)		IOX O ₂ amount (in working order)	
O rings		Cascade O ₂ amount (in working order)	
Ventilator			
Ventilator supplies		Lights In aircraft: In working order	
IV warmer		OPERATIONAL SUPPLIES	
Infusion Pump		Log book	
Infusion Pump supplies		Nav Maps	
O ₂ tank		Flashlight	
Drawer/wal supplies		Flight Manual	
Portable Suction		Aircraft keys	
Life Vests (if applicable)		Flare Gun kit	
Gurney Cushion (aircraft specific)			
Backboard			
KFD		Thomas Brothers Maps	
Traction Device		List all applicable	
Cardboard splints			
C-collars			
Cleaning supplies		DEEP CLEANING	
Blankets		Surfaces	
Headsets		Gurney	
Latex Allergy kit		Beneath seats/base	
		Contaminated supplies removed	
Crew member sign-off		Cabinets/drawers	
Nurse:			
Medic:			
File:			

Ground Transport Checklist - 5711-CS

 Air Methods		Ground Transport Checklist		Form #: 5711-05 Revis on #: 1 Date: T30	
☐	Ensure safe patient and crew compartment	☐	Confirm all Air Methods Equipment mounted to ambulance properly		
☐	Securing and seatbelts for all crewmembers / Stretcher and ability to secure a 1st eq. patient	☐	Vent / AC power cord / Charger / Spare battery		
☐	Environmental controls (heat / AC)	☐	Medication bag		
☐	Speed and vehicle conditions (due to access to these items consult leadership)	☐	☐ IV fluids, blood, and blood products		
☐	Obtain on board oxygen tank size and PSI	☐	Suction bag / Pump bag		
☐	Ensure oxygen connections are compatible with Air Methods Equipment	☐	Monitor / AC power cord / Charger / Spare battery		
☐	Complete PACO ₂ ensure appropriate reserves	☐	Suction / Power		
☐	☐ If required, obtain additional O ₂ tanks	☐	Confirm function and operations of the following ambulance systems:		
☐	Confirm functional onboard equipment	☐	Breathes		
☐	☐ Remote Operation	☐	Oxygen		
☐	☐ Functional AC outlets for equipment charging	☐	Suction		
☐	Confirm working suction and redundant system	☐	Interior lighting		
☐	☐ Single portable (Operation requires note)	☐	Restraint system (both patient and crew)		
☐	Proper restraint system in patient (3 restraints: hips and shoulder straps and ability to secure 4 restraints in ambulance)	☐	Environmental controls as appropriate (heat / air conditioning)		
☐	Use Extensuator in ambulance (Serviceable)	☐	Stretcher operations and limitations		

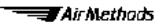
Clinical Division Root Cause Analysis and Action Plan - 5555-CS (Page 1 of 5)

		Clinical Services Division Root Cause Analysis (RCA) and Action Plan		Form #: 5555-CS Revision: 1:1 Date: 1/1/2024
Incident Date:		Investigator:		
Section One: RCA Tool				
The RCA tool is designed to assist in finding causes of problems and developing actions for eliminating, preventing, and minimizing future incidents.				
The following steps should be completed:				
1.	Define the problem.			
2.	Map the process (if required)			
3.	Gather data			
4.	Cause/Effect Analysis			
5.	Solutions and prevention steps development (The 5C's: Cause, Control, Corrective, Consequence, and Cost)			
6.	Implementation			
7.	Monitoring			
8.	Communication of lessons learned			
Section Two: Analysis				
Level of Analysis		Questions	Findings	
What happened? Define the problem.	Utilize a timeline form.	List pertinent events in the order they occurred.		

Clinical Division Root Cause Analysis and Action Plan - 5555-CS (Page 2 of 5)

		Clinical Services Division Root Cause Analysis (RCA) and Action Plan		Form #: 5555-CS Revision #: 1 Date: 1/30/
Level of Analysis	Questions	Findings		
Why did it happen?	The process or activity in which the event occurred.	What normally happens?		
		What steps were involved in it contributed to the event?		
	Human Factors	What human factors were relevant to the outcome?		
	Equipment Factors	Did the equipment performance affect the outcome?		
	Controlable Environmental Factors	What controlable factors directly affected the outcome?		
	Uncontrolable External Factors	Are there any uncontrollable external factors truly beyond the organization's control?		
	Other	Are there any other factors that have directly influenced this outcome?		

Clinical Division Root Cause Analysis and Action Plan - 5555-CS (Page 3 of 5)

		Clinical Services Division	
		Root Cause Analysis (RCA) and Action Plan	
		Form #: 5555-CS	
		Revision #: 1	
		Date: TBD	
Level of Analysis	Questions	Findings	
Factors:	Human Resource Factor	To what degree is staff properly qualified and competent for their responsibilities? What human factors were relevant to the outcome?	
	Training Needs: identified	How can orientation and continuing education be improved?	
	Information Management: status	To what degree is communication among personnel accurate?	
	Leadership Issues: Safety Culture	To what degree is the culture conducive to risk identification and resolution?	

Clinical Division Root Cause Analysis and Action Plan - 5555-CS (Page 4 of 5)

		Clinical Services Division Root Cause Analysis (RCA) and Action Plan		Form #: 5555-CS REVISED #: 1 Date: TBD
Section Three: Action Plan (Executive Summary)				
	Risk Reduction Strategies	Measures of Effectiveness		
For each of the findings identified in the analysis as needing attention, indicate the planned action, expected implementation date, and associated measure of effectiveness. Check to be sure that the selected measure will provide data that will permit assessment of the effectiveness. Consider whether pilot testing of a planned improvement should be conducted. Improvements to reduce risk should ultimately be implemented in all areas where applicable, not just where the event occurred. Identify where the improvements will be implemented.	Action Item #1:	Measure:		
	Action Item #2:	Measure:		
	Action Item #3:	Measure:		
	Action Item #4:	Measure:		
	Action Item #5:	Measure:		
	Action Item #6:	Measure:		

Clinical Division Root Cause Analysis and Action Plan - 5555-CS (Page 5 of 5)

	Clinical Services Division Root Cause Analysis (RCA) and Action Plan	Form #: 5555-CS Revision #: 1 Date: 12/23/2024
Section Four: Components of Executive Summary		
The summary will include the following components:		
1. Situation:		
2. Background:		
3. Assessment:		
4. Recommended Actions:		

New Clinical Product Request - 5716-CS

		New Clinical Product Request				Form 5716/CS	
						Revision #: 1	
						Date: TBD	
Date Requested							
Physician Stakeholder					Email	Phone #	
Name of Person Completing the Form					Title	Phone #	
Device Name							
Part number description (manufacturer) or product number							
Device description (include dimensions)							
Pictures of device (web link)							
Company name							
Company website of device and supplier (Hyperlink to device)							
Supplier Contact Information	Name		Phone		Email		
Projected Cost per Unit							
Potential usage per year							
Is the device certified for use in the aircraft?	☐	YES	☐	NO			
Is the device for one location or companywide/CBS division	☐	One location	☐	Companywide/CBS			
Is this a new product or a replacement (select from the dropdown list)							
If this is a replacement, what is being replaced?							
If this is a replacement, how is it an improvement?							
Why should it be used? (select from the dropdown list)							
What is the current issue that this product will solve?							
Will the trial product be FREE?	☐	YES	☐	NO			
Is there any FREE training for the device trial?	☐	YES	☐	NO			
Is there any supporting documentation for use?	☐	YES	☐	NO	Describe		
Is the Conflict of interest statement of submitter signed?	☐	YES	☐	NO			

[illegible]

Refusal of Transport and/or Treatment - 5196-CS (Page 1 of 2)

		Refusal of Transport and/or Treatment		Form Number: 5196-CS Revision: 2 Effective Date: 06/23/2024 Page 1 of 2
Patient Name:	Date of Birth:	Transport #		
Crew must complete the following:				
Is the patient alert and oriented, appearing to have the mental capacity to refuse transport and/or treatment?		☐ YES ☐ NO		
<i>If NO, patient CANNOT refuse transport or treatment.</i>				
Is the patient of the age of consent for medical care in the applicable state?		☐ YES ☐ NO		
<i>If NO, is patient's parent or legal guardian present and refusing transport and/or treatment for patient?</i>				
		☐ YES ☐ NO		
<i>If NO, patient CANNOT refuse transport or treatment.</i>				
Patient is refusing:		Specific Treatment(s) (list):		
☐ Transport ☐ All Treatment ☐ Specific Treatment				
Crew must perform each of the following tasks (indicate completed by checking each box):				
☐ advised the patient that refusal of Transport and/or Treatment(s) could result in serious complications or even death.				
☐ explained the severity of the patient's injuries and/or condition to the patient.				
☐ gave the patient verbal instructions about how to seek treatment if they change their mind.				
☐ The patient (or parent/legal guardian) verbally confirmed their understanding of the above information.				
☐ advised the patient that they may obtain a copy of this form and/or Patient Care Report by utilizing the QR code or web address.				
☐ contacted medical control via AirCom and notified Dr.				
Signature of Crew Member		Print Name		Date
Patient Release:				
I have been fully advised of the risk of serious complications or even death that may result from my refusal of transport and/or treatment. I understand these risks but nevertheless refuse transport and/or treatment by Air Methods, LLC and its clinicians. I understand that Air Methods clinicians are not physicians and are not authorized or qualified to make a definitive diagnosis of my condition. I understand that if I change my mind and wish to receive transport and/or treatment in the future, I should contact 911. On behalf of myself and my heirs, I hereby voluntarily release and discharge Air Methods and its subsidiaries, affiliates, officers, directors, employees, agents, successors and assigns from any and all liability resulting from my refusal to accept transport and/or treatment.				
Signature of Patient (or Parent/Legal Guardian)		Printed Name of Signatory		
Witness Signature #1		Printed Name of Witness #1		
Witness Signature #2		Printed Name of Witness #2		
☐ Patient refused to sign form				

Refusal of Transport and/or Treatment - 5196-CS (Page 2 of 2)

	Refusal of Transport and/or Treatment	Form Number: 5196-CS Revision: 12 Effective Date: 08/25/2024 Page 2 of 2
Instructions:		
Due to the potential litigation that may result from patient refusal, it is important to fully complete this form. All boxes/lines must be completed on this form. In addition, accurate and thorough documentation must be included in the Patient Care Report (PCR). Documentation in the PCR must follow the Air Methods Patient Care Guidelines and include the following information regardless of the patient's cooperation: • Mental Status • All assessments that can be performed visually • Objective evidence of mental impairment with drugs and/or alcohol • Patient's demeanor during refusal of care and response to presentation of Form 5196 - Refusal of Transport and/or Treatment • If applicable, the circumstances or reasons why the crew was unable to fully advise the patient about the risk and potential consequences of refusing transport and/or treatment • Advice and instructions given to the patient (or parent/legal guardian) • Witness(es) names and duties (including age by name or identifying number, such as badge number) • Crew should always attempt to perform and document (if patient allows) - o A full assessment - o At least one set of vital signs Additionally, the crew must contact Medical Control via AirCom to inform the physician of the circumstances of the refusal regardless of the patient's cooperation. Questions about the patient's capacity to refuse should be addressed with Medical Control during the notification call. At least two witnesses must be present for the refusal. Witnesses may be Air Methods clinicians or other witnesses on scene. All witnesses, regardless of whether they sign the form or not, must be documented in the PCR. Patients that wish to obtain a copy of this form or the Patient Care Report may request the record utilizing the following QR code or by going to www.airmethods.com/contact and sending a Medical Record Request:		
		

Ride-Along Application - 5199-CS

 Ride-Along Application		Form Number: 5199-CS Revision: 17 Effective Date: TBD Page 1 of 1
Directions		
1. Fill out application completely. Application and waivers must be received prior to Ride-Along. It is the applicant's responsibility to follow-up, and confirm, the class the applicant has requested. 2. Applicant must be 16 years of age or older <u>may be eliminated from the program based on weight, aircraft performance, or PIC decision.</u> 3. A copy of current licensure should be submitted with application (if applicable) 4. Applicant must be employed or otherwise volunteer for a public service entity or in a hospital setting.		
Application		
Name	Orientation Date	
Address	Weight	
City, State, Zip	Age	
Agency Affiliation		
Phone (Day)	Evening	
Name of Emergency Contact		
Phone (Day)	Evening	
Reasons for becoming a Ride-Along:		
Please list reasons for wishing to participate in the Ride-Along Program.		
Signature of Ride-Along Applicant		
Date		

Ride-Along Patient Confidentiality - 5197-CS

	Confidentiality of Patient Information Ride-Along Program Participant Verification	Form Number: 5197-CS Revision: 1 Effective Date: TBD Page 1 of 1
Agreement		
I understand that Air Methods, LLC ("Air Methods") provides services to patients that are private and confidential. I understand that in the rendering of emergency medical services, patients provide personal information and that such information may exist in a variety of forms such as electronic, oral, written, or photographic and that all such information is strictly confidential and protected by federal and state laws. I understand that Air Methods has legal, ethical, and moral obligations to protect the confidentiality of its patients' data. Air Methods is required by law to maintain the privacy of certain confidential information, including any and all medical information of a patient's past, present, or future health condition. Such information may only be used and disclosed for treatment, payment, and healthcare operations as defined by HIPAA. By signing this document, (1) I acknowledge that I am prohibited from sharing patient information, in any form, with anyone unless required to do so by law; (2) I agree to abide by all confidentiality requirements as provided herein, and (3) I will act in accordance with HIPAA/privacy requirements. If, at any time, knowingly or inadvertently breach the patient confidentiality practices, I agree to notify the Privacy Officer of Air Methods, LLC immediately. In addition, I understand that any breach of patient confidentiality may result in termination of current and future opportunities to participate in the Ride-Along Program.		
Ride Along Signature:		
Signature: _____		
Date: _____		
Printed Name: _____		

Ride-Along Participant Waiver of Liability - 5198

 Ride-Along Participant		Form Number: 5198 Revision: 4 Effective Date: 04/25/2022
Waiver of Liability Hold Harmless Agreement		
Section One: Participant Information		
Name:	Orientation Date:	
Address:	Weight:	Age:
City:	State:	Zip:
Agency Affiliation:		
Phone (Day):	Phone (Evening):	
Name of Emergency Contact:		
Phone (Day):	Phone (Evening):	
What are your reasons for wishing to participate in the Ride-Along Program?		
Section Two: Waiver of Liability/ Hold Harmless Agreement		
I, _____, request permission to ride along in an Air Methods Corporation helicopter. In consideration for being granted permission to ride in an Air Methods Corporation helicopter, I hereby indemnify, hold harmless, release, and discharge Air Methods Corporation and any of its officers, employees, and agents from any and all claims, losses, or liabilities to me, my employer, my assigns, my heirs, my executors, and personal representatives now and forever, by reason or on account of injury to myself or my property, whether by reason of accident, intent or neglect during such time that I am in a vehicle or aircraft owned or operated by Air Methods Corporation, or in the company of an officer, employee, or agent of Air Methods Corporation who is discharging his/her duties, and regardless of whether such liability arises in tort, contract, strict liability, or otherwise to the fullest extent allowed by law. In addition, I agree to indemnify and hold harmless Air Methods Corporation, its employees, agents, and assigns for any and all claims, losses, or liabilities which arise as a result of my conduct, whether it be intentional, negligent, or accidental while I am a participant in the Ride-Along Program, including, but not limited to such times that I may be in an Air Methods Corporation helicopter or in the company of an officer, employee, or agent of Air Methods Corporation while she/he is acting or discharging his/her duties on behalf of Air Methods Corporation. I further agree to abide by all rules and regulations applicable to the Ride-Along Program. I have been advised and am aware of the risks and dangers associated with emergency medical transport, and I am aware that such activities include the risk of injury and even death and I hereby elect to voluntarily participate in said activities, knowing that the activities may be hazardous to my property and me. I agree to respect the confidential nature of all information regarding patients and transports and to comply with the confidentiality policies and procedures established by Air Methods Corporation which have been provided to me. I understand that Air Methods Corporation provides emergency medical services to patients and will respect the privacy rights of the patients. I assume all risks of death, injury, loss, or damages to my person or property, whether due to accident or neglect, and neither I nor any of my representatives shall have any claim against Air Methods Corporation, its officers, or employees, by reason of my death, injury, loss, or damage.		
Section Three: Signature(s)		
IN SIGNING THIS AGREEMENT, I CERTIFY THAT I HAVE CAREFULLY READ THIS DOCUMENT AND I FULLY UNDERSTAND ITS CONTENT. I AM AWARE THAT THIS IS A RELEASE OF LIABILITY AND A CONTRACT AND I SIGN IT OF MY OWN FREE WILL.		
Signature of Applicant: _____		
Signature of Parent (if applicant is a minor): _____		
		Date: _____

Uncrossed Emergency Blood Release Form - 5646-CS

		Uncrossmatched Emergency Blood Release				Form #: 5646-CS Revision #: 1 Date: 06/20/2023	
Patient's Name:				Date:		Time:	
Flight Number:		Flight Crew:					
Receiving Hospital & Room #:		Attending Physician:					
BLOOD CERTIFIED AS COMPATIBLE FOR THIS PATIENT CANNOT BE SUPPLIED BECAUSE OF INSUFFICIENT TIME FOR GROUPING, TYPING AND/OR CROSSMATCHING total number of Packed Red Blood cells and Liquid Plasma during transport and hospital staff notified							
☐ Patient received that crossmatch needed.							
☐ Female of childbearing age received O Positive Packed Red cells and may need RhoGAM. The units of blood referenced above were infused in the interest of protecting this patient from exsanguination, but we do not take responsibility for complete compatibility of the blood with that of the patient. The unit number(s) is/are:							
Unit Number	Type	Start Time	End Time	Amt. Infused	Pt Temp Start/End		
					/		
					/		
					/		
					/		
Vital Signs at Receiving Hospital:		Time	HR	BP	RESP	TEMP	
				/			
The patient exhibited signs/symptoms of a transfusion reaction during transport.				☐ YES	☐ NO (Mark One)		
If yes, what was the reaction?				Time of reaction:			
					Date:		
Flight Crew 1 Signature		Printed Name					
					Date:		
Flight Crew 2 Signature		Printed Name					
					Date:		
I understand the information above regarding the emergency administration of uncrossmatched blood and am aware of the need to complete crossmatch and possible administration of RhoGAM.							
Signed _____, RM				Date:			
Receiving Nurse							
The clinical condition for the above-named patient is sufficiently urgent to necessitate this emergency transfusion before compatibility testing could be completed. The benefits of the transfusion outweigh the potential risk.							
Signed _____, M.D.				Date:			
Receiving Physician							
** Flight crew will provide a photocopy of this form, in addition to the completed PCR including all vital signs, to the receiving facility. The original will be maintained with the patient's flight records. **							

Medbank (Cubex) Guide

Medbank (Cubex) Otterbox Processes:

- Otterbox forced cycle count
 - Controlled substance items within the Medbank (Cubex) are duplicated and renamed with an ****OTTERBOX**** designation. Using Ativan as an example, there are two options:
 1. Ativan 2mg
 2. ****OTTERBOX**** Ativan 2mg
 - The ****OTTERBOX**** medications are available to account for the medications kept inside the Otterbox itself. These medication items have different attributes from all others and will allow for a "forced cycle count" when an accessed cycle count is initiated by the crew.
 - Medbank (Cubex) will prompt the crew to count Otterbox medications at each shift change regardless of whether or not they were accessed during the previous shift.
 - Each time an accessed cycle count is initiated, Medbank (Cubex) will require (force) the Otterbox medications to be counted.
 - Discrepancies, if any, will be revealed to the crew and reported on MyQLink.
 - Cycle count transactions will also be available daily on the "Transactions by Employee & Witness, C15b" report and all other reports that include cycle count transactions.
 - For visibility of Otterbox medications on hand at a particular base, Clinical Directors can run the "Inventory on hand, C11" report.
 - The workflow for all crew transactions in Medbank (Cubex) remains the same. Application of the serialized PIN tag to the Otterbox is required and input of the PIN tag number in Medbank (Cubex) is required.

Changes in Otterbox configuration - type or concentration of medication:

Because Medbank (Cubex) is tracking the number and type of medications kept in the Otterbox, if one or both of those change, Medbank (Cubex) will need to be updated.

- **Adding medications to the Otterbox:**
 - If a new medication or additional vials of an existing medication are to be added to the Otterbox, the crew will perform a restock transaction to the ****OTTERBOX**** medication bin. For example, if a crew is carrying two Ativan 2mg vials and they need to start carrying three vials, they will perform a restock transaction of one ****OTTERBOX**** Ativan 2mg." Now the Medbank (Cubex) will show three in stock and will expect the crew to find three (3) when the next accessed cycle count is done.
- **Removing medications from the Otterbox:**
 - If medications need to be removed from the Otterbox because a base runs out of something or they won't be carrying as much of a particular medication, they will issue the number of vials they are removing to "Duty Crew." For example, if a base runs out of Fentanyl 100mcg and they use the last two in their Otterbox on a patient, they will issue the two ****OTTERBOX**** Fentanyl 100mcg vials to "Duty Crew." If they are replacing those vials with two Fentanyl 250mcg vials, they will then restock two ****OTTERBOX**** Fentanyl 250mcg vials.

Replacing the Otterbox Tag Lock and Entering the Pin Number in Medbank (Cubex)

- Whenever the Otterbox is opened for any reason, the Tag Lock PIN number from the tag that was removed must be entered in Medbank (Cubex). A new Tag Lock must be placed and the new Tag Lock PIN number is required to be entered in the Medbank (Cubex).
 - Issue Transactions
 - Whenever an Issue Transaction is initiated, a "Validation Code" field will appear on the Medbank (Cubex) screen.
 - The number of your PIN tag should be entered in this field.
 - -The Validation Code field is required and you will not be able to proceed until your PIN number is entered.
 - For transactions where you are issuing something that is not going in the Otterbox, enter NA in the Validation Code field so that you can proceed with your transaction.
 - Return Transactions
 - Whenever a Return Transaction is initiated, a "PIN" field will appear on the Medbank (Cubex) screen.
 - The number of the PIN tag placed on the Otterbox is required to be entered in this field.

Medbank (Cubex) Reports

- The following Medbank (Cubex) automatic Multi-site reports will be set up for distribution.
 - To the RCD:
 - ControlledSubs > Discrepancies, Unresolved, C2 ALL
 - Frequency: Daily
 - Bases: All bases within the respective region(s)
 - Transactions > Transaction by Employee and Witness, C15bALL
 - Frequency: Daily
 - Bases: All bases within the respective region(s)
 - To the CBL:
 - Controlled Subs > Discrepancies, Unresolved, C2
 - Frequency: Daily
 - Bases: All bases within the respective area(s)
 - Transactions > Transactions by Patient AM, C14am
 - Frequency: On the 1st of each month for the preceding month
 - Bases: Assigned base(s) only
 - Inventory > Inventory on Hand, C11
 - Frequency: Daily
 - Bases: Assigned base(s) only
 - Inventory > Potentially Expiring Inventory, C45
 - Frequency: On the 1st of the month for the future 60 days
 - Bases: Assigned base(s) only

Replenishing Administered Meds

- If a clinical teammate has administered meds to a patient, they will need to replace the meds used and log what was administered and wasted in the Medbank (Cubex). If medications need to be restocked to the Otterbox, always ensure this step is complete before transferring the Otterbox to the next shift. (If the clinicians administered Ativan, please follow the separate Ativan replacement procedures found in this Guide)

1. Login to the Medbank (Cubex)
2. Return the Otterbox to the Medbank (Cubex) first and document the Tag Lock PIN tag number in the PIN field when accessing the Otterbox
3. Select Home

NOTE If a medication was drawn up and the entire amount was wasted, follow the steps below to restock that medication to the Otterbox. Otherwise, proceed to step “4” below.

- Select Exit.
 - Login to the Medbank (Cubex).
 - Select WASTED ENTIRE VIAL ON A FLIGHT from patient list and select Issue/Return.
 - Select Issue Items.
 - Type and search for the medication to restock and select Next.
 - Enter the amount of the medication that was wasted which is being issued to the patient “WASTED ENTIRE VIAL ON A FLIGHT” (the entire amount) e.g., if 10mg of Versed was drawn up and none was used, issue 10mg to the patient “WASTED ENTIRE VIAL ON A FLIGHT.”
 - Enter the Security Tag PIN number in the Validation Code field and next to that number, enter the flight number and select Next.
 - Select Issue Items Now.
 - Have the crew partner scan their fingerprint to be a witness.
 - Input the quantity of meds that are in the compartment once it opened and select Next.
 - Verify that the total being issued is accurate and select Next.
 - Select Exit.
 - Apply the security tag to the Otterbox or replenish any additional medications if needed.
4. Select Add New Patient.
 5. Enter the flight number as the Patient ID, then enter the Patient Name and select Issue/Return.
 6. Select Issue Items.
 7. Type to search for the item that was administered and select Next.

8. Enter the amount that was administered in drug dose, e.g., MGs, MCGs, to the patient, which in turn will record the remainder as wasted automatically.
9. Enter the Security Tag PIN number into the Validation Code field and select Next.
10. Select Issue Items Now.
11. Have crew partner scan their finger print to be a witness.
12. Input the quantity of meds that are in the compartment once it opened and select Next.
13. Verify the total being issued is accurate and select Next.
14. Choose one of the two following steps to perform next.
 - If the clinical teammate wishes to leave the Otterbox in the Medbank (Cubex) at this point, Select Exit first, and then Issue the Otterbox to DUTY CREW, stock with the new medication and seal the Otterbox with the new PIN tag. Perform a return of the Otterbox to the Medbank (Cubex) and select Exit.
 - If the clinical teammate wishes to retain the Otterbox on their person at this point, Select Exit first, and then perform an ISSUE transaction for the Otterbox, restock the medications as needed and seal the Otterbox with the new PIN tag.
15. The Clinical Director should be notified via email with an explanation for any "Full Dose Waste."

Cycle Count Procedures

NOTE If Ativan needs cycle counting, see Ativan Appendix below.

Shift Change Cycle Count

Performed every time a clinician arrives to or leaves shift:

1. Login to the Medbank (Cubex).
2. Select Cycle Count.
3. Select Cycle Count Options.
4. Check the box under "Select items accessed since last Cycle Count" on the "Schedule 2" row.
5. Select Next.
6. Have your crew partner scan their fingerprint to be a witness.

- The drawer will open for the first item and you will input the quantity found into the quantity field.
- Repeat steps "6" & "7" for each additional item remembering to check the listed expiration date and change if necessary.
- When you are done, you will be taken back to the Cycle Count List screen.
- Select Exit.

Cycle Count by Item

Process for counting singular items in Medbank (Cubex) by either item name or bin number.

Circumstances may exist where it is desirable to count only one item or group of items on demand without initiating a full cabinet cycle count. This may be used to confirm the count on a bin where a discrepancy exists or to confirm the count of recently stocked medications.

- Begin by logging in and selecting "Cycle Count."

FIGURE 1. Cycle Count

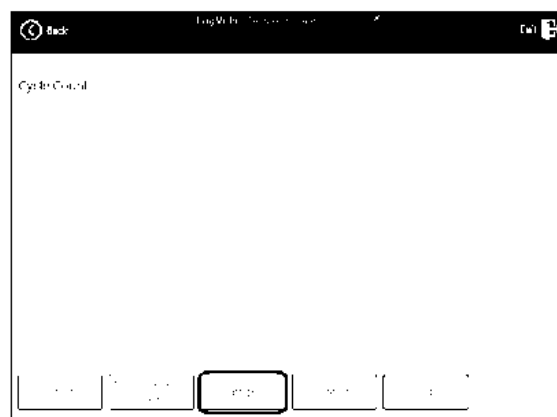
The screenshot shows the 'Cabinet #01' screen. At the top, there is a 'Back' button and an 'Exit' button. Below the title, it says 'Welcome, All Items, AirMethods's Patient.' There is a search bar with 'All' selected and 'Search' and 'Clear' buttons. A table lists items and their counts:

Item Name	Count
Item 001	10
Item 002	20
Item 003	30
Item 004	40
Item 005	50
Item 006	60
Item 007	70
Item 008	80
Item 009	90
Item 010	100

At the bottom, there are four buttons: 'Back', 'Cycle Count', 'Exit', and 'Done/Reset/Wait'. The 'Cycle Count' button is highlighted with a red box.

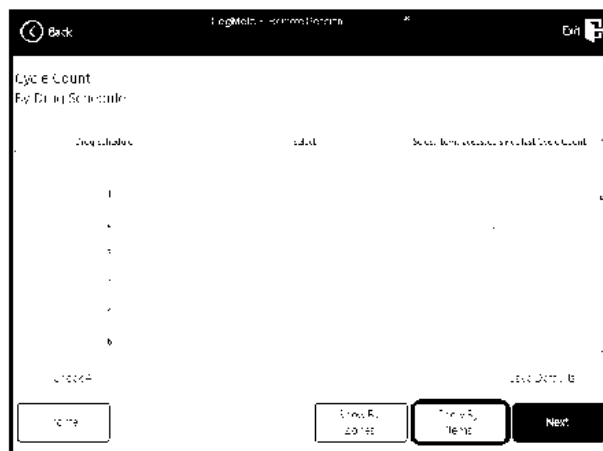
2. Click on “Options”

FIGURE 2. Options



3. Click on “Show By Items”

FIGURE 3. Show by Items



4. You can now search for the item you want to count. In the below example, we're searching for a MiniMed pump, but the process works the same way looking for medications.
5. Type in the search field the first few letters of the item you're looking for and the list will populate. Put a check-mark next to the item you want to count and click "Next."
6. You can select as many items or bins to count as you need on this screen.
7. You can also search for and select different items by entering new search keywords and placing check marks next to the needed items.
8. Once you have selected all the items you want to count, click "Next" and Medbank (Cubex) will ask you to count them one by one.
9. Controlled substances require a witness and external items do not. Witnesses must remain present through the entire transaction.

FIGURE 4. End

Cycle Count

Search

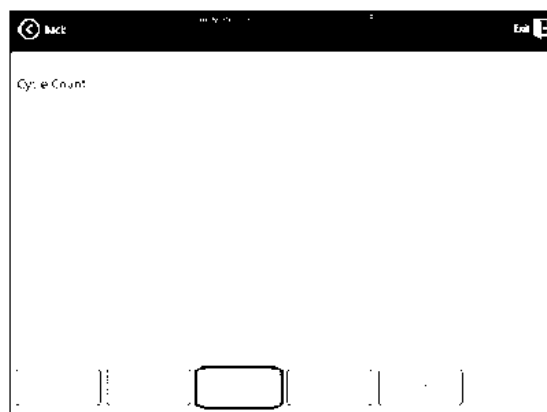
Select	Bin	Qty	Description	Location	Date	Action
☒	101	1	MiniMed 630G	101	12/23/2024	Count
☐	102	1	MiniMed 630G	102	12/23/2024	Count
☐	103	1	MiniMed 630G	103	12/23/2024	Count

Home Print Scan Next

10. If you need help with the above procedure, you can send an email to [Medbank \(Cubex\)help@airmethods.com](mailto:Medbank (Cubex)help@airmethods.com).

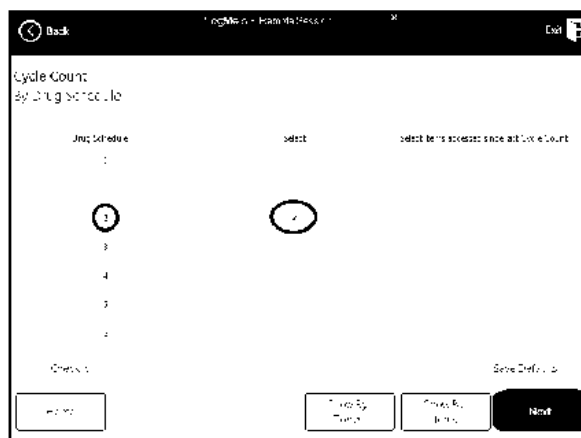
2. Click on “Options”

FIGURE 6. Options



3. Place a check-mark next to the Schedule 2 in the first column and click “Next.”

FIGURE 7. Next



4. Medbank (Cubex) will now cycle through all internal medications requiring a witness for each one. Enter the number of medications found in each bin when it opens and inspect the vials/ampules for damage, tampering and fluid clarity.
5. Check for expiration dates of the medications in the bin. Expired meds need to be moved to the corresponding “expired” bins (see the Expired Med process). Update the expiration date for the bin to the “first to expire” date as needed.

Process to Restock Received Meds

NOTE If Ativan needs restocking, see Ativan Appendix below.

1. Login to the Medbank (Cubex)
2. Select Restock
3. Select Other Stocking Options
4. Select Restock Item
5. Search for the item being restocked (ensure you choose an item description that has a zero in the “On Hand” column next to it. This means that it is an empty cubie and can accept the meds you are going to add. If there are no “zero” cubies, you can select one that already has stock, but ensure you don't add to it more than the cubie can hold) and tap on the description to add a check mark to the item.
6. If you have more than one item to restock, repeat step “5” until your list is complete.
7. Select Show Restock List
8. From the list of items to restock, tap on the QTY field for each item and input the quantity you need to restock (Only choose a quantity that will fit in the cubie that you have selected. Each cubie has a Max and Min number assigned to it, but these are for inventory control only. If the cubie can physically fit the medications in it, you can restock to that cubie, even if it stocks it with more than the Max value.)
9. Select Restock Items Now
10. Have a Witness scan their finger and stay present to audit the restock

11. Verify the quantity found and the quantity being restocked is correct

NOTE If the number you selected to restock will not fit, you can change the restock amount on this screen. This will result in extra medications at the end of your restock. After you are finished, you will need to perform another Restock to stock these extra medications.

NOTE At the end of this current restock, return to line "2" to perform another Restock.

12. Repeat steps until all items have been restocked
13. Select Exit

Process to Remove Expired Meds

NOTE If refrigerated stock of Ativan needs removing, see Ativan Appendix below.

Once meds have expired, perform the following steps to remove the expired meds and restock them into the Medbank (Cubex) under their Expired versions and secure into the Medbank (Cubex) expired bins.

NOTE If you are moving expired medications from the Otterbox into the Medbank (Cubex) expired bins for storage, only perform Step 2 and then follow the "Supplement to Removing Expired Meds" found below Step 2

Step 1:

- For expired meds found in the Medbank (Cubex). Perform Destock Waste Transaction for the expired meds:
 - Login to the Medbank (Cubex).
 - Select Cycle Count.
 - Select Destock Expiring Meds.

- Enter the number of days into the future you wish for the Medbank (Cubex) to search for expired/expiring meds.
- Place a check-mark next to each med you wish to "Destock" to expiration.
- Select Next.
- Have your crew partner scan their finger to act as a witness.
- Input the quantity of expired meds you are removing that are in the compartment once it opened and select Next.
- Verify that the total being issued is accurate and select Next.
- Repeat until all expired items have been issued and then select Exit.

Step 2:

- For expired meds found in the Otterbox or once expired meds have been removed from the Medbank (Cubex). Take the expired items and perform a supplemental restock:
 - Login to the Medbank (Cubex).
 - Select Restock.
 - Select Other Stocking Options.
 - Select Restock Items.
 - Search for the name of the item prefaced with an "EXPIRED" - e.g., EXPIRED-FENTANYL 250MG, and tap on the description to add a check mark to the item.
 - If you have more than one item to restock, repeat step "e" until your list is complete.
 - Select Show Restock List.
 - From the list of items to restock, tap on the QTY field for each item and input the quantity you need to restock.
 - Select Restock Items Now.
 - Have a Witness scan their finger and stay present to audit the restock.
 - Verify that the quantity found and the quantity being restocked is correct.
 - Repeat steps until all items have been restocked.

NOTE If you must now restock your Otterbox to replace expired medications, follow the steps under "Supplement to Step 2 - Removing Expired Meds" below.

- Supplement to Step 2 - Removing Expired Meds

NOTE If you now need to restock your Otterbox because you removed an expired medication from the Otterbox, follow the below steps.

NOTE If Ativan is one of the medications you need to restock, see Ativan appendix below.

1. Login to the Medbank (Cubex).
2. Select EXPIRED INVENTORY from patient list.
3. Select Issue/Return.
4. Select Issue Items.
5. Type to search for the item that you expired from the Otterbox and select Next.
6. Enter the amount that you wish to issue in drug dose, e.g. MGs, MCGs, to the EXPIRED INVENTORY patient. (For example, if you are restocking Fentanyl 250mcg to the Otterbox, you will enter "250."
7. Enter the new Security Tag PIN number in the Validation Code field and write "Replaced Expired Meds" in the PIN field and select Next.
8. Select Issue Items Now.
9. Have your crew partner scan their finger print to be a witness.
10. Input in the quantity of the meds that are in the compartment once it opened and select Next.
11. Verify that the total being issued is accurate and select Next.
12. Select Exit.
13. Place new Security Tag on the Otterbox.

Damaged or Tampered Container Process

Damage Found in Otterbox:

1. Call Clinical Director to report.
2. Complete the Controlled Substance Damage Report in Protean (upload pictures of the damage to the form in the designated area).

3. Waste medication with a witness, return the Otterbox to the Medbank (Cubex) and record the PIN of the tag you just removed from the box into the PIN field.
4. Complete a PSAP if container damaged during patient encounter.
5. Replace damaged container following the below procedure.
6. Login to Medbank (Cubex).
7. Select DAMAGED MEDICATION from patient list.
8. Select Issue/Return.
9. Select Issue Items.
10. Type to search for the item that was damaged.
11. Enter the entire amount of medication that you wasted, including medication that spilled out of its container on the floor or inside the drug bag.
12. Enter the Security PIN Tag number in the Validation Code field and write "Replaced Damaged Vial/Ampule" in the PIN field and select Next.
13. Select Issue Items Now.
14. Input the quantity of meds that are in the compartment once it opened and select Next.
15. Verify that the total being issued is accurate and select Next.
16. Select Exit.
17. Place new security tag on the Otterbox.

Damage Found in Medbank (Cubex):

1. Call Clinical Director to report.
2. Complete the Controlled Substance Damage Report in Protean (upload pictures of the damage to the form in the designated area).
3. Follow the below procedure to remove the damaged vial/ampule from the Medbank (Cubex) stock.
4. Login to the Medbank (Cubex).
5. Perform cycle count on the drawer that houses the damaged medication.
6. Select Cycle Count.
7. Select Cycle Count Options.
8. Select Show By Items.
9. Check the Select button next to the item bin that houses the damaged vial/ampule.
10. Select Next.

11. Have your crew partner scan their fingerprint to be a witness.
12. Enter the quantity of only the **UNDAMAGED** vials/ampules. **NOW REMOVE THE DAMAGED VIAL/AMPULE FROM THE CUBIE AND WASTE THE MEDICATION WITH A WITNESS.**
13. When you are done, you will be taken back to the Cycle Count List screen.
14. Select Exit.

Medbank (Cubex) Power Loss - Steps To Access Meds

If the Medbank (Cubex) should lose power or otherwise become non-functional, follow the below steps to access and continue using any needed medications.

1. Notify the Clinical Director to determine if documenting on a paper Controlled Substances Perpetual Inventory Log - 5424 is required.
2. Obtain the key box code from the Clinical Director and access the Medbank (Cubex) to remove the Otterbox if needed.
3. If a medication from the Medbank (Cubex) is needed to resupply the Otterbox, use the physical key to gain access to the Medbank (Cubex) drawer. Individual Cubies will not open without power - a screwdriver or similar device may be used to pop open the Cubie housing the needed medication. This will damage the affected Cubie and require a replacement. Replacement Cubies can be ordered by contacting the Clinical Director. This step must be approved by the Clinical Director.
4. In the perpetual log, document the removal of the medication from the Medbank (Cubex) and placement into the Otterbox. In addition, document the number of medication ampules/vials that remain in the Cubie that was opened.
5. These specific medications must be counted at each shift change and every time a medication is used until the Medbank (Cubex) can be repaired.
6. Until the Medbank (Cubex) is repaired, the duty crew will use the physical Medbank (Cubex) keys to access medication.
7. The on duty RN will maintain the Medbank (Cubex) key on his/her person at all times. In the event the base is out of service for no RN, the Medbank (Cubex) key will be secured in a lockbox and the code sent to the Clinical Director.
8. Once the Medbank (Cubex) is back online, the perpetual log should reflect "Medbank (Cubex) back online" in the documentation on the form. The logs should be stored in the narcotic management file and maintained for three years.
9. In the Medbank (Cubex), add the names of all patients (if any) that received medication during the Medbank (Cubex) malfunction.

10. Perform an ISSUE transaction for all meds used to each appropriate patient (this should match the medications and patients that were documented on the perpetual log during the Medbank (Cubex) malfunction).
11. Perform a full CYCLE COUNT.
12. Contact the Clinical Director to replace or re-assign damaged Cubies as needed and report on the status of the Medbank (Cubex).
13. The Clinical Director will re-secure the Medbank (Cubex) key in the lockbox and change the code immediately.

Medication Not Available in Medbank (Cubex) to Replenish Otterbox

In the event that needed medications are not available in Medbank (Cubex) to replenish the Otterbox, the following steps must be completed to ensure that all meds are restocked and assigned appropriately:

1. When attempting to issue medications to a patient (replenishing the Otterbox) you may get a message that reads, "You may not issue more than is available." This means the med you are trying to issue is not available in Medbank (Cubex).
2. If you are attempting to issue more than one vial/ampule of a medication, e.g. you're trying to issue 4 mg of Ativan; you should attempt to issue only 2mg first. There could be vials available but fewer than you are trying to replenish.
3. Notify your Clinical Director immediately. The Clinical Director will determine if your base needs to be out of service because of the shortage.
4. Notify your Clinical Base Lead to report that medications need to be ordered ASAP.
5. Perform an accessed cycle count which will update the on-hand quantities of medications in the Otterbox.
6. Lock the Otterbox with a new serialized tag and return the Otterbox to the Medbank (Cubex).
7. Depending on whether you'll be in or out of service, you should leave the Otterbox in the Medbank (Cubex) (if going OOS) or issue it back out (if you are remaining in service). The Clinical Base Lead will be responsible for ensuring that when the medications are delivered to replenish your stock, the following steps take place.
8. When the medications arrive, those meds will be "Restocked" into the Medbank (Cubex) per the normal procedure. Do not stock the Otterbox directly from the shipment package. All medications must be "Restocked" into the Medbank (Cubex) first.

9. The patient that originally received the medication should then be recalled in Medbank (Cubex) and the medication "Issued" to that patient, thus replenishing the Otterbox to its complete format.
10. Perform an accessed cycle count which will update the on-hand quantities of medication in the Otterbox.
11. Notify the Clinical Base Lead and Clinical Director that this transition is complete.
12. The Clinical Base Lead will report any discrepancies during the monthly narcotic audit.

Storing Controlled Substances Outside the Medbank (Cubex)

- Storing Benadryl and Zofran in the Air Methods approved safe:
 - If necessary, due to space limitations in the Medbank (Cubex), Benadryl and/or Zofran may be stored in a key lockable lock box which is to be placed in a key lockable safe. Confirm with your Clinical Director to assure your Medbank (Cubex) has been configured to allow for this alternate storage scenario.
 - Access to meds stored in an external safe will require two keys, one key to the safe and another key to the lockbox with the meds stored inside.
 - Issuing these medications to replenish your aircraft supply works the same as issuing meds from within the Medbank (Cubex).
 - Cubex will automatically issue any needed keys to the user when access to a key-locked medication is needed.
 - This applies to all transactions that result in access to these medications.
 - When following the process to issue medications, Medbank (Cubex) will automatically include the needed key in the issue list.
 - Medbank (Cubex) will also prompt the user to return the key and will post an alert on the login screen if you forget to return the key. No alteration to the established method of issuing medications is required.
- Storing Ativan in the Air Methods approved refrigerator with Q-lock:
 - No alteration to the established method of issuing medications is required for Ativan.
 - Follow the same process for issuing medications - Medbank (Cubex) will automatically include the key in the issue list.
 - Medbank (Cubex) will prompt the user to return the key and will provide an alert on the login screen if you forget to return the key.

- Storing Controlled Substances Received but Unable to Stock Immediately:
 - If circumstances exist where a box containing controlled substances is received but cannot be immediately stocked into the Medbank (Cubex) (e.g. there is a pending flight request), the box must be secured in a key lockable external safe. Follow the below steps to put the box in the safe:
 - Assure box is sealed well with tamper resistant tape.
 - Sign in to Medbank (Cubex) and click on “Restock.”
 - Select “Other Stocking Options.”
 - Select “Restock Item.”
 - Type in “Pending Restock Box”, click on the item and select “Show Restock List.”
 - Type in the number of boxes that need to be secured in the safe.
 - Select “Restock Items Now.”
 - Since a key is also required, the Medbank (Cubex) will prompt you automatically to issue that key. No additional actions are required aside from following the prompts.
 - Use the key to access the safe and secure the boxes inside.
 - Return to the Medbank (Cubex) and follow the on screen prompts to return the key.
 - Once you are ready to stock the meds into the Medbank (Cubex), perform an Issue transaction to Duty Crew for the item called “Pending Restock Box.”
 - Now that you have the box in your possession, you can open the box and restock the meds into the Medbank (Cubex) following the restock procedure outlined in this guide.
- Storing Controlled Substances Going to MedFlats While Pending FedEx Pick-up:
 - Place medications being returned in a box, sealed with tape and FedEx label applied.
 - This box will need to be secured in an external safe until FedEx arrives to accept the package.
 - Follow the below steps to secure your MedFlats package while waiting for FedEx to pick it up.
 - Assure you have followed the MedFlats policy and the contents of the box are accurate, including the packing list.
 - Assure meds are securely and safely packed, the box is sealed with tape and the FedEx label is applied.

- Sign in to Medbank (Cubex) and click on “Restock.”
- Select “Other Stocking Options.”
- Select “Restock Item.”
- Type in “Pending FedEx Pick-Up Box”, click on the item and select “Show Restock List.”
- Type in the number of boxes that need to be secured in the safe.
- Select “Restock Items Now.”
- Since a key is also required, the Medbank (Cubex) will prompt you automatically to issue that key. No additional actions are required aside from following the prompts.
- Use the key to access the safe and secure the boxes inside.
- Return to the Medbank (Cubex) and follow the on screen prompts to return the key.
- When the FedEx employee arrives, perform an Issue transaction to Duty Crew for the item called “Pending FedEx Pick-Up Box.”
- This process requires two clinicians at the base when FedEx arrives to pick-up - if a second clinician is not present, contact the Clinical Director to serve as a witness to perform the Issue transaction.

Air Methods Lockbox Addresses

If a check is received at the base from a payer, please send the check to the appropriate lockbox below.

Follow these steps:

1. Secure check in an appropriate envelope (as determined by Pitney Bowes)
2. Find what company the check is addressed to
3. Locate the company name below
4. Ship to the company name and address listed below

TABLE 1. Lockbox Addresses

Rocky Mountain Holdings	PO Box 713362 Cincinnati, OH 45271-3362
LifeNet DBS ARCH Air Medical Service, Inc.	PO Box 713383 Cincinnati, OH 45271-3383
Mercy Air Service	PO Box 84621 Seattle, WA 98124-5921
Omni Flight Helicopters, Inc.	PO Box 731116 Dallas, TX 75373-1116
Native American Air Ambulance, Inc.	PO Box 29675, Dept. 2057 Phoenix, AZ 85038-9675

TABLE 2. Altitude Associated Changes in Physiological and Environment Parameters

Altitude (feet)	Barometric Pressure (mmHg)	PO₂	Degrees F	Gas Expansion	SpO₂ (%)
0 (sea level)	760	103	59	1	98
1,000	733	98.2	55.4		
2,000	706	93.8	51.8		
3,000	681	89.5	48.4		
4,000	656	85.1	44.8		
5,000	632	81	41.2		
6,000	609	76.8	37.6		
7,000	586	72.8	34		
8,000	565	68.9	30.4	1.3	93
9,000	542	65	27		
10,000	523	61.2	23.4	1.5	87
11,000	503	57.8	19.8		
12,000	483	54.3	16.2		
13,000	465	51	12.6		
14,000	447	47.9	9.1		
15,000	429	45	5.5		84
16,000	412	42	1.9		
17,000	396	40	-1.7		
18,000	380	37.8	-5.2	2	72
19,000	364	35.9	-8.7		
20,000	349	34.3	-12.3	24	66
22,000	321	32.8	-19.5		60
25,000	282	30.4	-30.1	3	
30,000	228		-47.9	4	
40,000	141		-56.5	7.6	

TABLE 3. Expected Oxygen Delivery

Nasal Cannula		Simple Mask		Non-Rebreather		Venti-Mask	
LPM	%FIO ₂	LPM	%FIO ₂	LPM	%FIO ₂	LPM	%FIO ₂ (color)
1	25	4	35	6	60	2	24 (Blue)
2	29	6	45	7	70	4	28 (White)
3	33	8	50	8	80	6	31 (Orange)
4	37	10	55	9	90	8	35 (Yellow)
5	41	12	60	10	Near 100	10	40 (Red)
6	45	14	65	>10	100		

TABLE 4. Time of Useful Consciousness

Altitude	Normal	Rapid Decompression
40,000+	15 sec. or less	15 sec. or less
40,000	15-20 sec.	7-10 sec.
35,000	30-60 sec.	15-30 sec.
30,000	1-1.5 min.	30-45 sec.
25,000	3-5 min.	1.5-2.5 min.
22,000	10 min.	5 min.
15,000	20-30 min.	10-15 min.

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